MSc Marriage and Family Therapy

The Lecture Library

Twenty-eight course books in one file

This single file contains every volume of the library, in programme order: eight courses in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. Each course keeps its own cover, imprint, contents, running heads and folio, so the file can be printed whole or the individual volumes printed from the separate PDFs in `pdf/`.

Use the bookmark panel of your PDF reader to jump to any course or to any chapter within a course. Every volume is also available as a standalone file, and the same text is published as a browsable site in `site/`.

Courses in this library

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents of each volume: how to use the book; course description and rationale; learning outcomes; assessment; the fourteen-week plan; fourteen teaching weeks; an integration chapter; a glossary; a self-assessment paper with model answers; and further reading.

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 701

Fundamentals of Marriage and Family Therapy

14 teaching weeks · 10 practicals · 8 worked cases · 7 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This is the course that turns a helping professional into a systemic one.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 701 — Fundamentals of Marriage and Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

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MFT 701	**Fundamentals of Marriage and Family Therapy**	One	1.71
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Course leader's summary. This is the course that turns a helping professional into a systemic one. It covers the intellectual history of family therapy, the epistemology that separates it from individual work, the profession's identity and scope, and the basic architecture of a first session. Everything later in the programme — theory, technique, assessment, ethics — assumes the vocabulary built here.

How to use this book

Each week contains four things: **the idea** (what you must understand), **the practice** (what you must be able to do), **the case** (what it looks like in a room), and **a reflective task** (what it asks of you personally). Read the week before the lecture; do the practical in the lab; write the reflection within 48 hours, while it is still uncomfortable.

The book is written to be read straight through, but it is also designed for revision: the integration chapter at the end compresses the whole course into two pages, and the glossary defines every term the examination may use.

1. Course description and rationale

Most mental health training teaches us to look inside a person: their thoughts, their symptoms, their attachment history. Family therapy teaches us to look between people: the loop, the triangle, the rule that nobody wrote down. This is not a rejection of individual psychology. It is an additional discipline of attention, and it changes what counts as the problem, the cause, and the cure.

Consider three common presentations:

Presentation	Individual formulation	Systemic formulation
A 15-year-old refuses school	Anxiety disorder, school phobia	A child whose symptoms hold a parental conflict at bay; the school refusal is the family's most effective way of preventing an argument about the marriage
A wife is "depressed"	Major depressive episode	A woman in a triad with her husband and his mother, carrying the family's unspoken grief about a lost pregnancy
A man "works too much"	Workaholism, avoidance	The person who regulates the family's emotional temperature by being absent, in a system that rewards his absence

None of these systemic formulations is automatically more true. Each is a hypothesis with different implications for where you intervene. The mature clinician holds both — the individual and the relational — and chooses deliberately.

Why this course first. Because students who begin with technique learn to do something they cannot explain, and students who begin with theory learn to explain something they cannot do. MFT 701 begins with framing: what kind of problem is this, and therefore what kind of help is required?

2. Learning outcomes

By the end of the course, students will be able to:

#	Outcome	Assessed in
LO1	Explain the epistemological shift from linear to circular causality, and its clinical consequences	Exam, Week 2 log
LO2	Trace the historical development of family therapy and locate contemporary models within it	Exam
LO3	Describe the profession's identity, scope of practice, and relationship to allied professions	Assignment
LO4	Conduct a structured first session, including joining, problem definition, and contracting	Skills assessment
LO5	Draw a three-generation genogram and generate hypotheses from it	Assignment
LO6	Critically discuss the common-factors evidence and its implications for model choice	Exam essay
LO7	Identify their own family-of-origin patterns and describe their effect on clinical work	Reflective log

3. Assessment

Component	Weight	Due
Reflective practice log (weekly, 300–400 words)	20%	Fortnightly
Genogram and hypothesis assignment (2,000 words + diagram)	30%	Week 9
Two-hour written examination (seen case + essay + short answers)	50%	Week 15

Skills gate. Students must pass a 15-minute observed first session (Week 6) before proceeding to MFT 706. The gate is pass/fail with one resit.

4. The fourteen-week teaching plan

Week	Topic	Practical	Core reading
1	What is family therapy? The relational turn	Triad: "the problem as described"	Nichols ch. 1; Handbook ch. 1
2	Linear and circular causality; first- and second-order cybernetics	Observation of a recorded argument	Watzlawick, Pragmatics ch. 1
3	A short history: from schizophrenia research to evidence-based practice	Timeline construction	Nichols ch. 2–3
4	Systems thinking: boundaries, feedback, homeostasis, rules	Feedback-loop mapping exercise	Becvar & Becvar ch. 2

Week	Topic	Practical	Core reading
5	The profession: identity, scope, settings, teamwork	Role boundaries case discussion	AAMFT code, scope sections
6	The first session I: joining, and the referral question	**Observed skills gate**	Minuchin & Fishman ch. 1
7	The first session II: defining the problem in the family's language	Triad: problem definition	Haley, Leaving Home ch. 1
8	The genogram as a hypothesis machine	Genogram lab (own family)	McGoldrick, Genograms ch. 1–2
9	Formulating relationally: the 5-P grid applied to a family	Formulation workshop	Dallos & Draper ch. 4
10	Culture, class and power in the consulting room	Reflexivity exercise	McGoldrick, Ethnicity & Family Therapy
11	Evidence: what works, for whom, and the common factors	Appraisal of one RCT and one qualitative study	Wampold ch. 2
12	Ethics, risk and the limits of the systemic frame	Risk-screen rehearsal	MFT 704 pre-reading
13	Integration I: three models applied to one family	Live demonstration + discussion	Nichols ch. 10
14	Integration II: your emerging clinical stance	Position statement presentation	Course review

Week 1 — What is family therapy? The relational turn

The idea. Family therapy is a frame, not a set of techniques. It is the decision to treat the relationship as the client. That decision has four consequences:

1. **The unit of assessment changes.** You must hear from more than one person, and notice what happens between them.
2. **The unit of intervention changes.** You intervene in sequences, rules, meanings and structures, not only in individual cognition.
3. **The definition of success changes.** Success may be a family that argues well, not a family that never argues.
4. **The therapist's position changes.** You are not a technician outside the system; you are a participant who affects it (Figure 1).

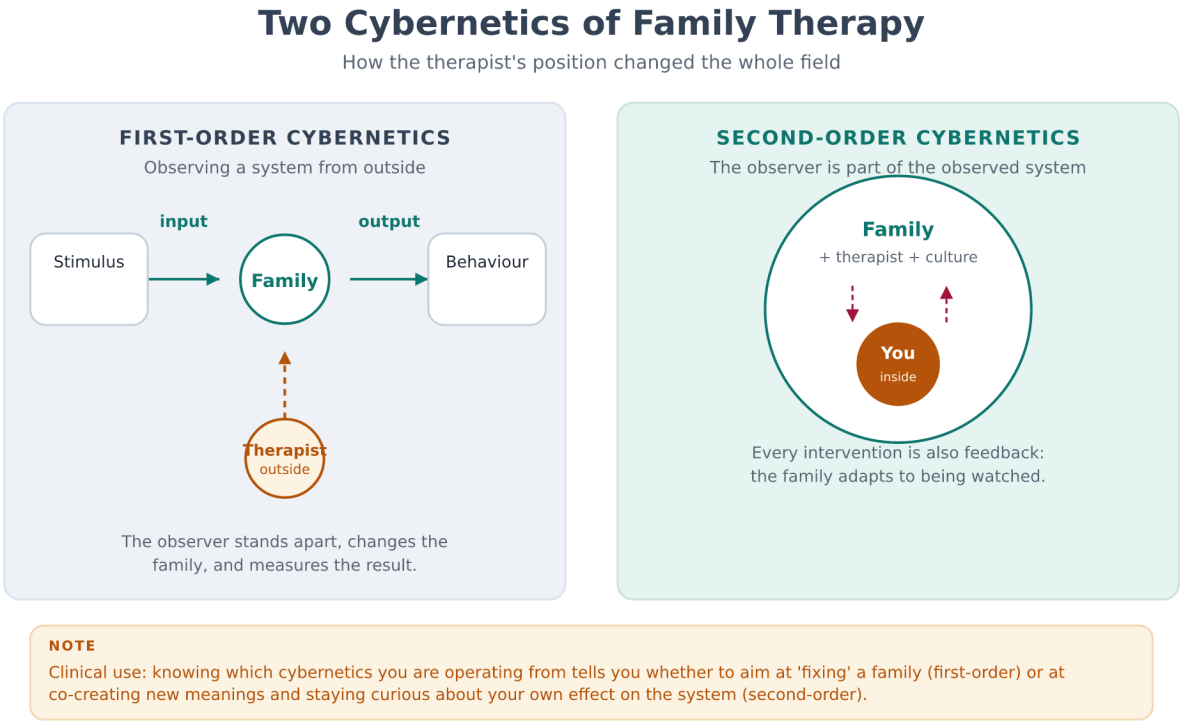


Figure 1.1 — First- and second-order cybernetics: where the therapist stands

A working definition. Marriage and family therapy is the assessment and treatment of relational systems — couples, families, and the wider networks in which they live — using interventions that target interaction patterns, meaning systems, and the emotional bonds between people.

What it is not. It is not "seeing everyone together" as a matter of policy. Sometimes the right systemic move is an individual session within family work. It is not a belief that individuals do not exist, or that suffering is imaginary. It is a disciplined focus on context.

> **CASE**
> Mrs Okonkwo brings her 9-year-old son, Ebuka, for "stubbornness". In the first ten minutes she mentions, twice, that his father "is never home". The linear frame says: assess and treat Ebuka's behaviour. The systemic frame says: ask who else is affected by the behaviour, what happens after it, and what would change in the household if it stopped. The second question is not a rejection of the first; it is a widening of the field in which the first can be treated.

Practice (lab). In triads, one person describes a current difficulty for three minutes. The "therapist" may only ask questions that include another person ("who notices when…", "what does your mother do when…"). Debrief: what did the widening do to the description?

> **REFLECT**
> Write down the last significant conflict in your own family. Now write it again from the point of view of the person you understood least. Note what changed in your body as you wrote.

Week 2 — Linear and circular causality

The idea. Linear causality asks who started it and what caused it. Circular causality asks what maintains it and how each response feeds the next. This is the single most important conceptual shift in the course.

The shift is not merely philosophical. It changes what you do:

Linear question	Circular question
"When did the depression begin?"	"What happens in the house when she stays in bed?"
"Why does he lose his temper?"	"What does she do just before he raises his voice?"
"Who is to blame?"	"How does the blaming itself work?"

Punctuation. Each family member punctuates a sequence differently — she says he withdraws, he says she nags — and each believes the other started it. The therapist's job is not to decide who is right but to make the sequence visible so that both can see the same film (Figure 2).

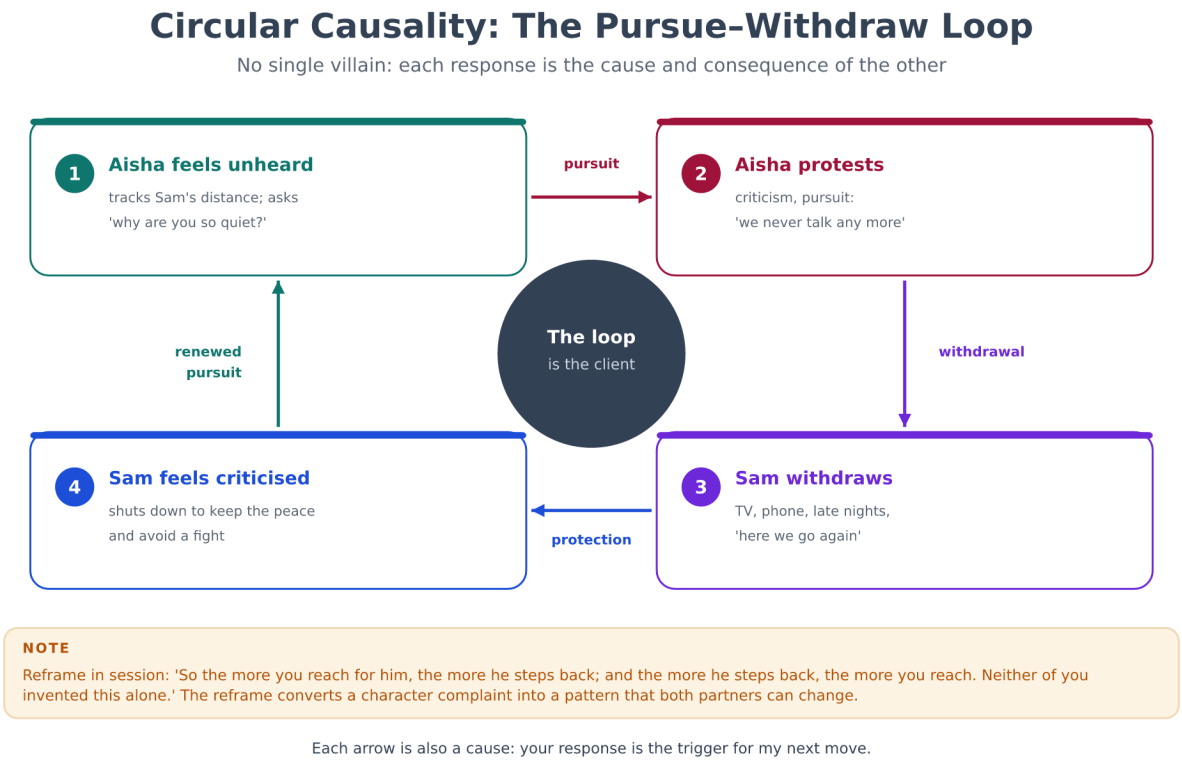


Figure 2.1 — The pursue–withdraw loop: a circular formulation

First-order and second-order cybernetics. First-order cybernetics treats the observer as outside the system: you measure, you intervene, you evaluate the result. Second-order cybernetics accepts that the observer is part of the system being observed: the family responds to being watched, and your presence is data. Contemporary practice is mostly second-order, which is why reflexivity (Week 10) is a clinical skill rather than a personality trait.

> **CASE**
> In a couple session, Folake says Dele "starts every fight". Dele says Folake "is never satisfied". Rather than adjudicate, the therapist asks each to describe, minute by minute, what happened last Tuesday. The sequence that emerges is: Folake raises a worry → Dele answers with a joke → Folake escalates → Dele leaves the room → Folake follows him → Dele goes silent for two days. Nobody started it. The loop started it.

Practice (lab). Watch five minutes of a recorded (consented) or scripted argument with the sound off. Write the sequence as a loop of four to six steps, with an arrow for each move. Then watch with sound and compare your loop with what the speakers said caused it.

> **KEY**
> If you can draw the loop, you can intervene in it. If you can only name a villain, you are still thinking linearly.

Week 3 — A short history of the field

You cannot critique a tradition you cannot narrate. This week gives you the story in five movements.

1. The pre-history (1900s–1950s). Child guidance clinics, marriage counselling, and social casework treated families, but individually. Psychoanalysis dominated, and the family was understood as the source of individual pathology rather than as a system in its own right.

2. The research rupture (1950s–1960s). Several independent groups — Bateson's Palo Alto project, Lidz and Wynne studying families of people diagnosed with schizophrenia, Bowen at Menninger, Ackerman in New York — began observing whole families and describing patterns rather than causes. Bateson's team gave us the double bind hypothesis and, more durably, the vocabulary of communication and paradox.

3. The heroic era (1960s–1970s). Charismatic founders built schools: Minuchin (structural), Haley and Madanes (strategic), Satir (experiential/human validation), Whitaker (symbolic-experiential), Bowen (multigenerational), Boszormenyi-Nagy (contextual), the Milan group (systemic/milan). Techniques were bold, sessions were long, and therapists were often filmed behind one-way mirrors.

4. The critique and the turn to meaning (1980s–1990s). Feminists (Hare-Mustin, Goldner, Walters) demonstrated that "neutral" systemic descriptions hid gendered power. Social constructionism and postmodernism (Anderson & Goolishian, White & Epston) shifted attention from structure to story. The Milan team's later work introduced curiosity and the reflecting team.

5. Evidence and integration (1990s–present). Manualised models were tested in randomised trials (behavioural couples therapy, EFT, multisystemic therapy, functional family therapy, multidimensional family therapy). Attachment, neuroscience and trauma research entered the field. The current mainstream is integrative: model-informed, evidence-aware, culturally situated.

Why the history matters clinically. Every school was a reaction to a limitation of its predecessor. Structural therapy reacted to an over-focus on insight; narrative therapy reacted to an over-focus on expertise; trauma-informed care reacted to an over-focus on technique. When you find yourself irritated by a model, ask which limitation it is answering — you will usually find your own blind spot there.

PRACTICAL

Build a one-page timeline in small groups. For each decade, write (a) the dominant idea, (b) the model it produced, (c) the critique that ended its dominance. Present one decade each.

Week 4 — Systems concepts you will actually use

Six concepts do most of the clinical work. Learn them precisely; vagueness here produces vague therapy.

1. System and subsystem. A system is a set of elements in interaction such that a change in one produces a change in the others. Subsystems (the couple, the siblings, the parental dyad) are organised by boundaries.

2. Boundary. The invisible rules about who participates and how. Boundaries range from diffuse (enmeshed: little privacy, feelings shared before they are owned) to rigid (disengaged: autonomy without belonging). Healthy is not the middle of a line; healthy is appropriate to the task — the parental subsystem needs a clearer boundary than the sibling subsystem (Figure 5 in MFT 702).

3. Feedback. Negative feedback corrects deviation (a thermostat, a bedtime routine); positive feedback amplifies it (an escalation, a panic spiral). Families contain both, always. Symptoms often persist because a negative-feedback loop protects a positive-feedback loop underneath.

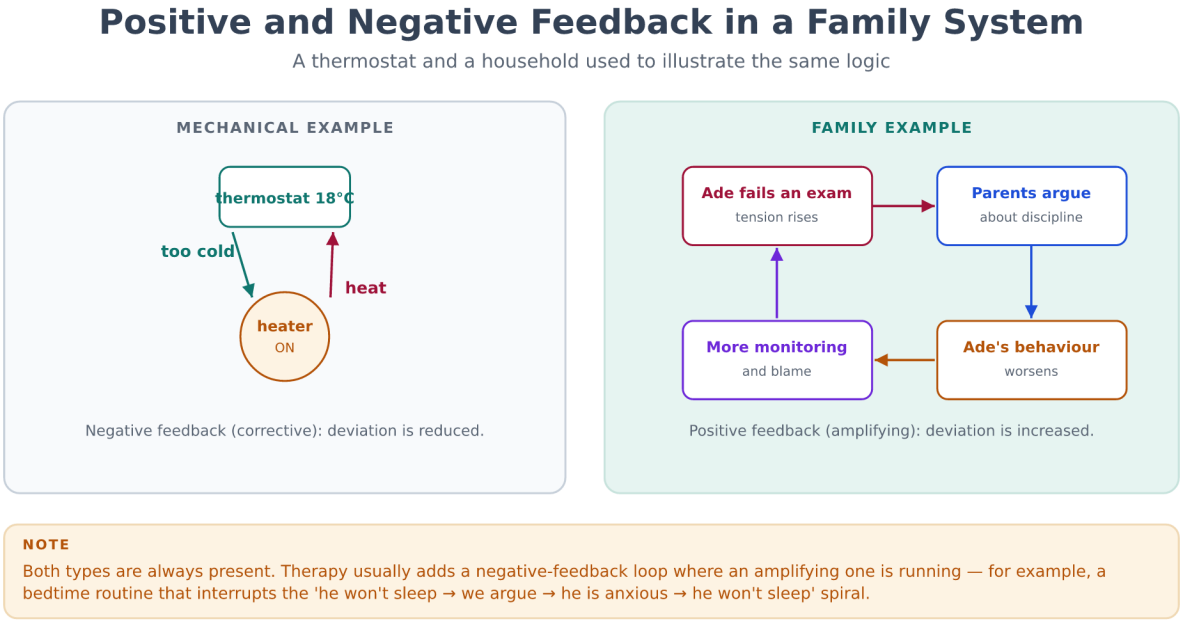


Figure 4.1 — The same feedback logic in a thermostat and in a family

4. Homeostasis. The system's tendency to maintain its familiar organisation, including familiar pathology. This is why a family may resist improvement: the drinking, the fighting, or the symptom has become load-bearing.

5. Rules. Recurrent patterns that prescribe and proscribe behaviour ("we do not discuss money"; "no one is allowed to be angry at Father"). Most family rules are never spoken; they are inferred from what produces anxiety.

6. Triangles. When tension between two people exceeds what the dyad can hold, a third is recruited — a child, an affair, a job, an in-law, a symptom. Triangles distribute anxiety; they also freeze the dyad's capacity to resolve it (covered fully in MFT 715).

> **CASE**
> Since the father's redundancy, the household has run a nightly cycle: he criticises the children's homework, the mother defends them, he accuses her of undermining him, she goes to bed early, and the eldest daughter stays up to calm him. Mapping it: the father's shame (unspoken) → criticism → mother's protection → his isolation → daughter's role as regulator. Nothing here is "the problem". The system has found a way to hold his shame — at the cost of a 14-year-old's adolescence.

Practice (lab). Using the case above, label each element: boundary, feedback type, homeostatic function, rule, triangle. Then answer: what would have to change elsewhere if the daughter stopped regulating?

Week 5 — The profession: identity, scope and settings

Family therapy is a distinct profession with its own training standards, codes and legal standing, and it is also a transdisciplinary competence. You need both facts.

Scope of practice. Assessing and treating relational distress; couple and marital therapy; family therapy with children and adolescents; parent guidance; separation and divorce work; work with stepfamilies; group work with relational focus; consultation to organisations. In most jurisdictions, family therapists do **not** prescribe medication, conduct forensic custody evaluations of their own clients, or give legal advice.

Settings. Private practice, community mental health, hospitals (including paediatric and psychiatric liaison), schools, child protection services, addiction services, employee assistance programmes, faith-based counselling centres, NGOs in humanitarian settings, and court-connected mediation.

Teamwork. You will work alongside psychiatrists, paediatricians, social workers, teachers, probation officers, imams and pastors, and community health workers. Two skills matter more than you expect: writing a referral that says what you want, and knowing the boundaries of what you may disclose.

Professional identity. Three tensions recur in the literature and in practice:

1. **Expert or collaborator?** The field has moved from therapist-as-director to therapist-as-curious- participant, but families often want expertise. The mature position is "expert in process, not in their lives".
2. **Neutral or advocating?** Systemic neutrality does not require moral neutrality about violence.
3. **Specialist or generalist?** You will be asked to work with issues you were not trained for. The professional answer is competence boundaries plus referral, not improvisation.

> **CASE**
> A school refers a 12-year-old for "aggression". The school wants an assessment letter; the mother wants the school to stop the bullying; the father does not want to attend. Assume nothing. Ask who defined the problem, who benefits from that definition, and what the referral is for. Then write down your scope: what you will assess, what you will not, and to whom you will send what.

> **PRACTICAL**
> Draft a one-page "Scope of practice" statement for yourself as an intern. Include the three things you will not do without supervision.

Week 6 — The first session I: joining

The idea. Joining is the process of becoming part of the system sufficiently to influence it, without becoming so much a part of it that you lose the capacity to observe. Minuchin called it the therapist's "accommodation" to the family's style. It is not friendliness; it is deliberate.

Four joining moves:

1. **Track the family's language.** Use their words, not your diagnostic ones. If they say "he is stubborn", ask about "stubborn" before you ask about "oppositionality".
2. **Validate what is true in each person's account.** Not "you are right", but "that makes sense given what you have been carrying".
3. **Match pace and formality.** Some families want a structured interview; others need ten minutes of narrative before any structure is tolerable. Ask: "How would you like to use this hour?"
4. **Make the structure explicit.** Who is here, why, for how long, who knows, what is confidential. Anxiety drops when the frame is clear.

What to cover in a first session (the "minimum viable first session"):

Element	Sample wording
Contract and limits	"I will keep what is said here private, except where someone is at risk of harm."

Element	Sample wording
Each person's view	"I would like to hear from each of you — what worries you most?"
The attempted solutions	"What have you already tried? What happened?"
The pattern	"So when X happens, what does Y do? And then?"
Risk screen	"Is anyone being hurt at home? Any thoughts of ending your life?"
Feedback	"Here is what I think I heard. What have I got wrong?"
Next step	"I would like to see all of you again next week, including your father."

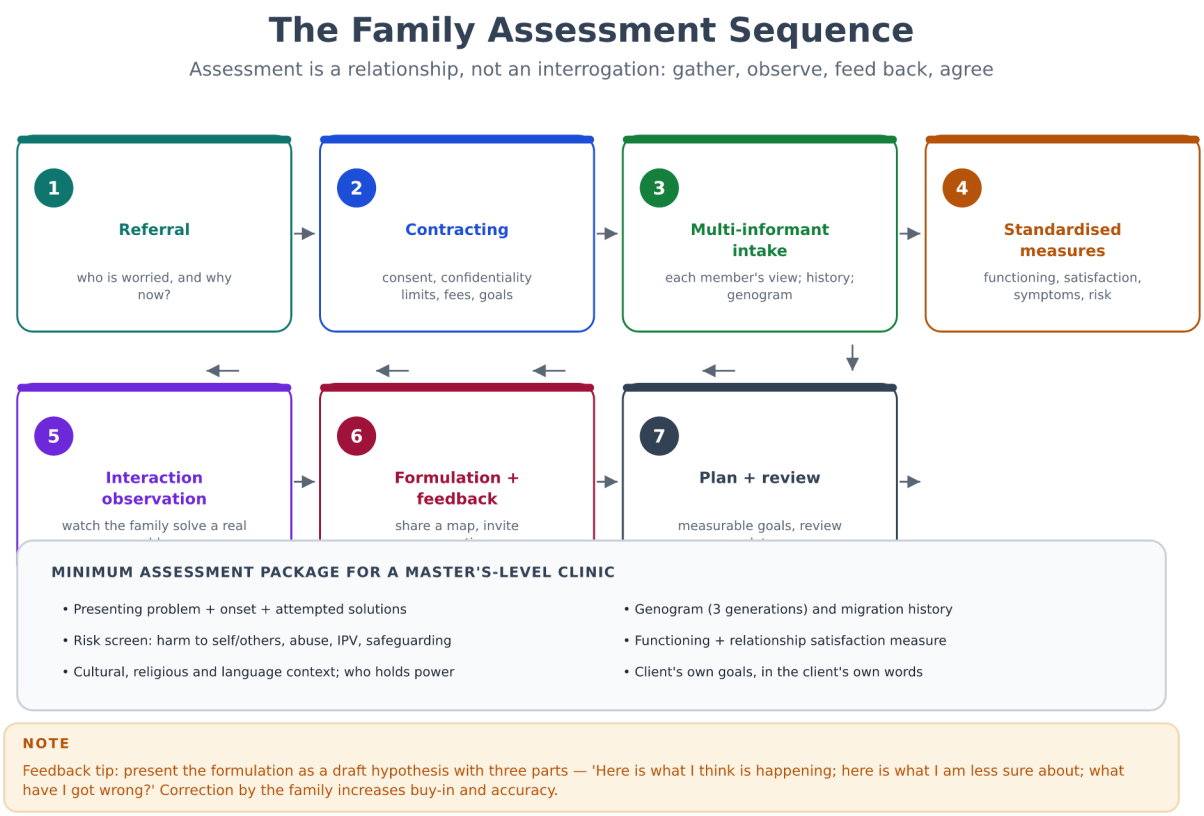


Figure 6.1 — The assessment sequence: assessment is a relationship, not an interrogation

CASE

A family arrives furious with each other and with the referrer. The therapist's first sentence is not a question but an acknowledgment: "You have been sent here by three different people and none of them asked you what you wanted. Let's start there." Joining succeeded before any information was gathered.

PRACTICAL (SKILLS GATE)

Fifteen minutes, observed. You must (1) state confidentiality and its limits, (2) obtain a description of the problem from at least two people, (3) ask one circular question, (4) complete a risk screen, (5) offer a summary and invite correction. You may use one index card of prompts. Feedback uses the four-point structured sheet in the programme handbook.

Week 7 — The first session II: defining the problem

The referral question is rarely the clinical question. Your task is to negotiate a problem definition that is (a) specific, (b) interactional, (c) owned by the family, and (d) changeable.

From vague to specific. "He is disrespectful" → "When I ask him to help, he looks at his phone and says nothing. That happens most evenings, and then I shout."

From individual to interactional. "She is manipulative" → "When she cries, her father gives in and you feel undermined; then you withdraw, and she comes to me." Notice: no blame is removed, but the description now includes three people and a sequence.

From imposed to owned. Ask each person to complete: "I will know this has worked when…". Write the answers down and read them back. If the parents' answers are incompatible, that incompatibility is the work.

From unchangeable to changeable. "I want him to respect me" is a relational outcome that no one can deliver on demand. "I want us to agree on one consequence for phone use, and both apply it for a week" is testable.

CASE

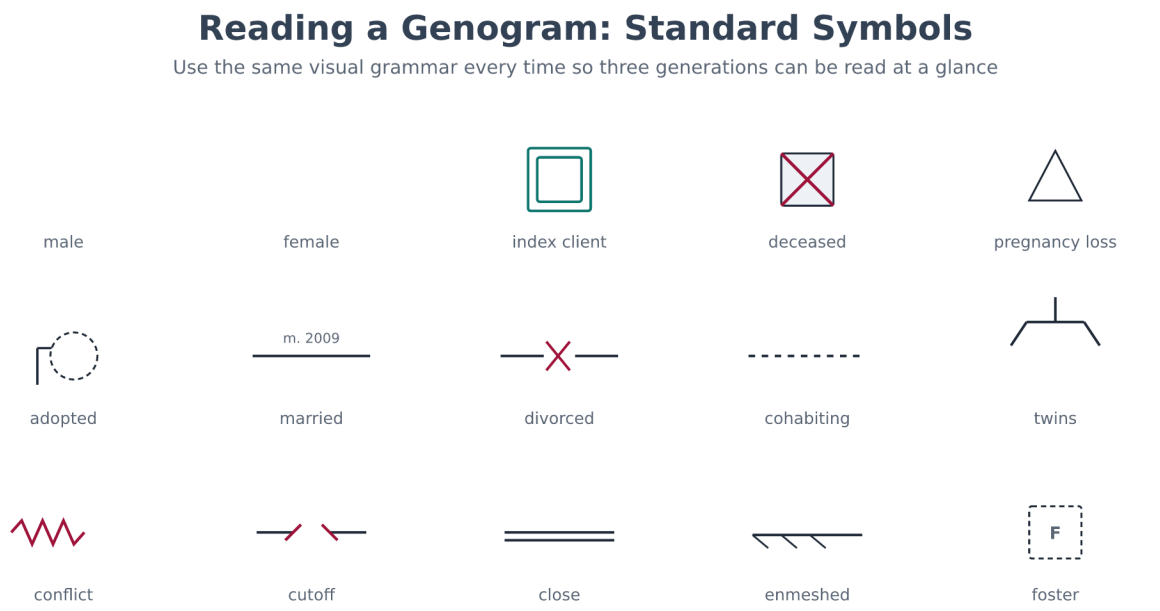
The Adeyemis come because their 16-year-old, Amara, has refused school for six weeks. The referral says "school refusal". The negotiated definition becomes: "The mornings have become a battle that everyone loses; Amara does not leave the house; her mother stays home from work; her father works later; and her brother has started sleeping in her room." The problem is now a system, not a girl.

Practice (lab). Take a complaint a peer brings from placement (disguised) and rewrite it four times: specific, interactional, owned, changeable. Read the final version to the "family" and ask whether it is recognisable.

Week 8 — The genogram as a hypothesis machine

The genogram is a three-generation map of family structure, relationships and events. Drawing it is simple; using it is not — the value lies in the hypotheses it generates and the questions it permits.

Standard symbols (Figure 3) and conventions: squares for males, circles for females, horizontal line for marriage, slash for divorce, cross for death, double outline for the index client. Relationship lines: double for close, zigzag for conflict, broken with slashes for cutoff, triple for enmeshed.



NOTE

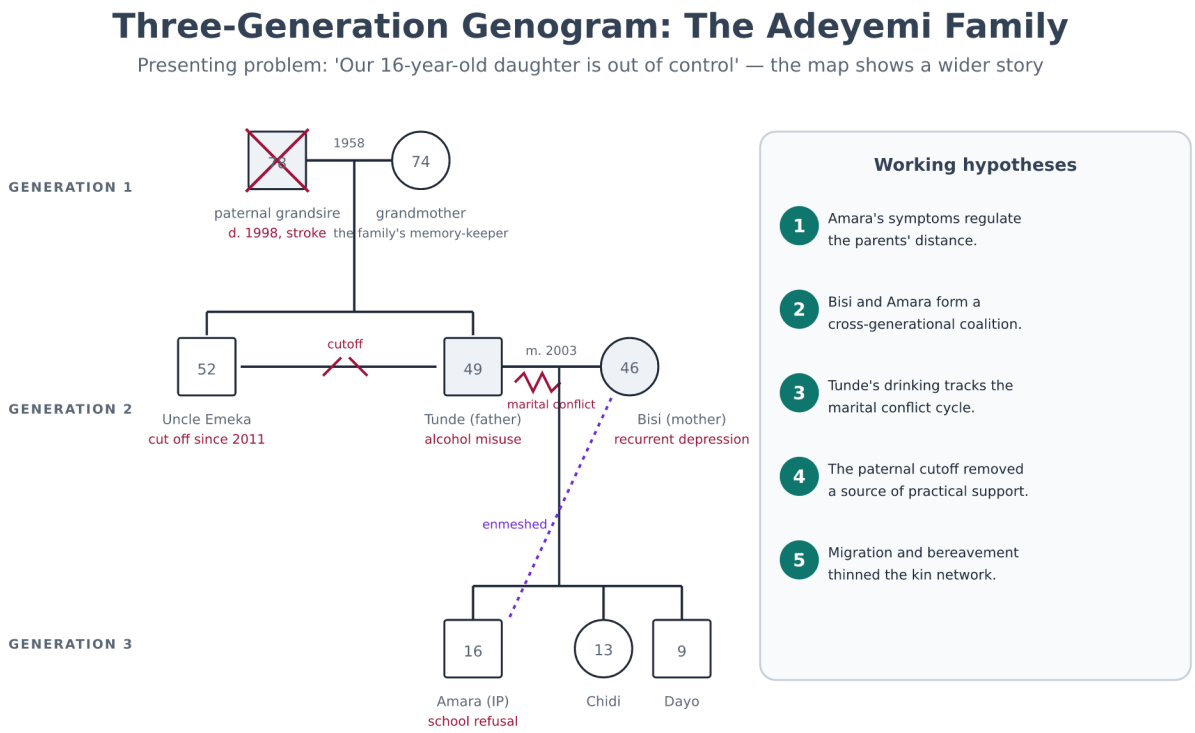
Also record: ages or year of birth inside the symbol, date and cause of death above it, occupation, migration, religion, and significant secrets (affairs, incarceration, abuse) using a key that you explain to the family before you draw.

Figure 3 — Genogram symbols: learn the grammar once and three generations become readable

What to ask while drawing. Age and year of birth; occupation and migration; deaths and their causes; separations, divorces, remarriages; who is close to whom; who does not speak to whom; who is the "responsible one"; who "had a breakdown"; family secrets; repeating names; repeating ages at which things happened.

From map to hypothesis. A good genogram yields three kinds of hypothesis:

1. **Structural** — coalitions, cutoffs, over/under-functioning pairs.
2. **Temporal** — patterns repeating at the same age, or symptom onset clustering around anniversaries.
3. **Legacy** — loyalty, indebtedness, roles inherited ("the eldest daughter always cares for everyone").



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 8.1 — A worked three-generation genogram with hypotheses

Cultural note. In many families the genogram must include people who are not biologically related but are structurally central — a foster child, a co-wife, a pastor, a long-term house help. Ask, "who else belongs to this family?" before you decide the map is complete.

PRACTICAL

Draw your own three-generation genogram from memory first, then correct it with a relative. Note every place where your memory and theirs differ. Those differences are the assignment.

Week 9 — Formulating relationally

A formulation is a provisional, testable explanation that connects the presenting problem to the system that maintains it, and points to where you would intervene.

Use the 5-P grid (presenting, predisposing, precipitating, perpetuating, protective) and add two relational columns: who is involved and what function the symptom serves.

Domain	Adeyemi family
Presenting	Amara, 16, school refusal × 6 weeks; parental conflict over discipline
Predisposing	Maternal depression; paternal alcohol misuse; migration severed the extended-family buffer
Precipitating	Father's redundancy; best friend left the city; transition to senior secondary
Perpetuating	Pursue–withdraw cycle between parents; mother–daughter coalition; avoidance rewarded by anxiety relief; disrupted sleep
Protective	Both parents attend; church community; Amara's engagement with art; an aunt who is trusted

The intervention sentence. End every formulation with one sentence of the form: "Given X, Y and Z, the most useful focus now is W, because it is the most changeable and the most load-bearing." For the Adeyemis: "Given the coalition and the parental distance, the most useful focus now is the couple's conflict cycle, because it is what the symptom is currently regulating."

Share it. A formulation that is never spoken is a private theory. Offer it as a draft with three parts: here is what I think, here is what I am unsure about, here is what I need from you.

> **ASSIGNMENT (30%)**
> 2,000 words plus a genogram. Choose a disguised or composite family from placement or a detailed role-play. Include: genogram, 5-P grid, circular formulation with a diagram, three competing hypotheses, one intervention plan with rationale, and a reflection on your own reactivity to this family.

Week 10 — Culture, class and power

Every family you meet is embedded in a culture, and so are you. Culture is not a variable to be "controlled for"; it is the medium in which the problem is defined.

Four practical disciplines.

1. **Ask rather than assume.** "In your family, who usually makes the big decisions? Who is consulted? Is that how you want it to be?"
2. **Locate yourself.** Complete the same cultural genogram for your own family. Your views on hierarchy, individuation, duty and disclosure are not neutral.
3. **Notice the power in the room** — including yours. The family may be polite because of your status, not because they agree.
4. **Interrogate "normal".** Models that prize individuation can pathologise families who prize interdependence. Ask what the family's own picture of a good life is.

Language. If the family thinks in their first language about emotional matters, the session may need that language or an interpreter. Interpreters are not furniture: brief

them, seat them beside you, ask them to interpret speech not summaries, and never ask them to translate a disclosure you have not prepared for.

Religion and spirituality. In much of the world, faith is not an "aspect" of family life; it is the organising frame. Ask what the family's faith says about the problem, who in the religious community knows, and whether they want that person involved. Do not compete with the pastor or imam; coordinate.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent twice. She does not want the police involved and does not want her family told. The therapist must hold three things at once: respect for her autonomy and faith, a safety plan that does not depend on him changing, and honesty about the limits of confidentiality. (Full treatment in MFT 712 and 724.)

> **REFLECT**
> Where in your own family would a therapist from a different culture misread the structure? Write 300 words on what they would get wrong and why.

Week 11 — What the evidence says

Students often arrive believing either that therapy "just works" or that only manualised treatments do. The evidence is more interesting than either position.

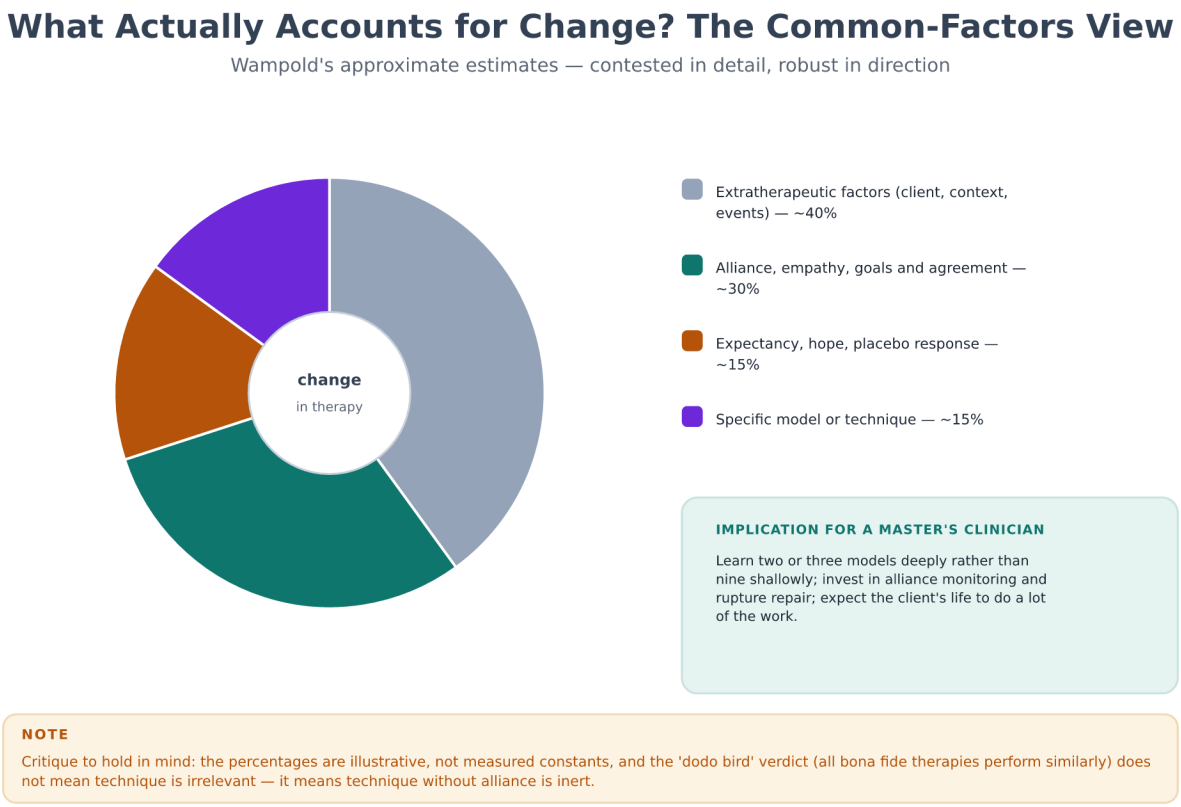


Figure 11.1 — Common factors and model effects, with a critique

Four findings worth carrying into practice.

1. **Family and couple therapies work.** Meta-analyses consistently find moderate-to-large effects for couple therapy in relationship distress, and for family-based treatments in adolescent behaviour problems, eating disorders (family-based treatment for anorexia is a first-line intervention), substance misuse and psychosis relapse.
2. **Alliance matters, and it is relational.** In couple and family work, the alliance is not one relationship but several, and shared sense of purpose predicts outcome better than any single dyad's alliance with the therapist.
3. **Model differences are smaller than model loyalties suggest.** Common-factors research attributes much of the variance to client factors, alliance and expectancy. Technique is not irrelevant: it organises what the therapist notices and does, and it matters more for specific problems.
4. **Drop-out is the silent failure.** In couple and family work, 20–40% of families end therapy unilaterally. Attendance, engagement and early change are the best predictors of retention. Track them.

How to read a paper in ten minutes. Read the abstract; then the design (who, how many, compared to what); then the outcome measure (does it measure what you care about?); then the effect size, not the p-value; then the limitations; then ask whether the sample resembles your clinic.

> **CRITICAL READING**
> Choose one randomised trial of a family therapy model and one qualitative study of family therapy experience. Write 500 words comparing what each can and cannot tell you.

Week 12 — Ethics, risk and the limits of the systemic frame

Ethics is not a chapter at the end of the course; it is the frame that makes the work possible. Here we establish the four issues every family therapist meets in the first month.

1. Confidentiality in a multiperson system. There is no single confidentiality. Agree the rules at the start: what is said in the joint session is shared; what is told to you individually may need to be brought into the open; secrets that affect safety cannot be kept. Write the policy down and give it to the family.

2. Risk. Screen for suicide, self-harm, violence, abuse and neglect in every new case, and document it. You cannot assume a colleague has done it.

3. Competence. Taking a case you are not trained for is not courage. It is a risk to the client and to you.

4. Multiple relationships and power. Small communities, religious networks and family ties make dual relationships likely. The test is not "is there a second relationship?" but "does the second relationship impair the first, or exploit the first?"

> **CASE**
> During an individual session, a husband tells you he has resumed an affair and asks you not to tell his wife. He is not at risk and no one is being harmed in the sense that requires disclosure. What do you do? (Work this case in groups; then compare with the decision model in Figure 33, MFT 704.)

Documentation. Write notes as if a colleague will take over tomorrow and a court may read them in a year: date, attendees, presenting issues, risk screen result, interventions, plan, and who was told what.

Week 13 — Integration I: three lenses on one family

This week is a rehearsal for MFT 702. You will apply three models to the same case and notice that each produces a different question, a different focus and a different first move.

Model	Key question	First move with the Adeyemis	Likely risk
Structural	What is the hierarchy, and where are the boundaries?	Strengthen the parental subsystem; block the mother–daughter coalition	Imposing a structure the family experiences as foreign

Model	Key question	First move with the Adeyemis	Likely risk
Bowen/multigenerational	How does anxiety travel, and who absorbs it?	Coach the parents to manage their own reactivity; map the projection onto Amara	Slow; family may want symptom relief now
Solution-focused	What is already working, and what would "one point better" look like?	Find exceptions to school refusal; scale the parents' confidence	May bypass the function of the symptom

The point of the exercise is not to choose a winner. It is to notice that a family presents a plural reality and that your model determines what you can see. Good clinicians can move between at least two frames without losing coherence.

> **PRACTICAL**
> In groups, take the same case and produce three one-page plans. Present all three. Then answer: which would you start with, and what would make you change your mind?

Week 14 — Integration II: your emerging clinical stance

Write a two-page **position statement**: what you believe about how relational problems develop, what you think therapy does, what you will not do, and what you still do not know. This is not an exam answer; it is the beginning of a professional identity that will be revised for the rest of your career.

A strong statement includes: one theoretical commitment, one epistemological commitment (how you know what you know), one ethical commitment, one cultural commitment, and one honest limitation.

> **REFLECT**
> Read your Week 1 reflection again. What has changed? What has not? Write 300 words to yourself, dated, to be reread at the end of the programme.

Integration: the whole course on two pages

1. The frame. Treat the relationship as the client. Widen the field; include context; notice the observer.

2. Causality. Move from who-started-it to what-maintains-it. Draw the loop. Punctuate differently.

3. History. Five movements: pre-history, research rupture, heroic era, critique and meaning, evidence and integration. Each school answers a limitation of its predecessor.

4. Concepts. System, subsystem, boundary, feedback, homeostasis, rule, triangle. Six concepts, most of the clinical work.

5. Profession. Know your scope, your setting, your team, your limits. Refer well.

6. The first session. Join, contract, hear everyone, screen for risk, map the pattern, summarise, invite correction, agree next step.

7. The problem definition. Specific, interactional, owned, changeable.

8. The genogram. Three generations, structural and temporal hypotheses, cultural completeness.

9. Formulation. 5-P plus "who is involved" and "what function the symptom serves", ending with one intervention sentence.

10. Culture and power. Ask, locate yourself, notice power, interrogate normal.

11. Evidence. Family therapy works; alliance matters; models overlap; drop-out is the silent failure.

12. Ethics. Confidentiality is negotiated, risk is screened, competence is bounded, dual roles are examined, notes are written for strangers.

13. Integration. Multiple lenses; move between frames without losing coherence.

14. Stance. A written, revisable position on what you believe and what you do not know.

Glossary

Boundary — the implicit rules governing participation and closeness between subsystems. **Circular causality** — an account in which each element both causes and is caused by the others. **Coalition** — an alliance of two or more people against a third. **Cybernetics** — the study of control and communication in systems; first-order (observer outside), second-order (observer inside). **Detriangulation** — the process of extricating a third party from a dyadic conflict. **Enmeshment** — diffuse boundaries with excessive emotional sharing and low autonomy. **Disengagement** — rigid boundaries with little emotional contact. **Feedback (negative)** — a process that reduces deviation and restores stability. **Feedback (positive)** — a process that amplifies deviation and produces escalation. **Genogram** — a three-generation diagram of family structure, relationships and events. **Homeostasis** — the system's tendency to maintain its existing organisation. **Hypothesis** — a provisional, testable explanation that guides enquiry. **Joining** — the therapist's deliberate accommodation to a family's style in order to work with it. **Linear causality** — an account in which A causes B in one direction. **Punctuation** — the way each participant marks the beginning of a sequence, thereby assigning blame. **Reframe** — relabelling a behaviour so that it is understood in a new, usually relational, frame. **Subsystem** — a unit within a system organised by boundaries (couple, siblings, parental dyad). **System** — a set of interacting elements such that a change in one changes the others. **Triangle** — a three-person configuration that absorbs and stabilises dyadic tension.

Self-assessment

Multiple choice (answers below)

1. A therapist says, "The real question is not who started the argument but what keeps it going." This statement reflects: (a) linear causality (b) circular causality (c) psychodynamic causality (d) moral causality

2. A father's drinking has the effect of preventing his wife and son from discussing the son's expulsion. In systemic terms the drinking is: (a) random (b) homeostatic (c) an individual symptom only (d) irrelevant to the family

3. "We never discuss money" is best described as: (a) a boundary (b) a family rule (c) a triangle (d) a reframe

4. In second-order cybernetics the therapist is: (a) outside the system (b) part of the observed system (c) a neutral observer (d) irrelevant

5. The best first move with a family that arrives furious at the referrer is: (a) correct the misunderstanding immediately (b) acknowledge their experience of being sent (c) begin the assessment (d) ask who is the identified patient

6. A negotiated problem definition should be specific, interactional, owned and: (a) clinically imposed (b) changeable (c) blame-allocating (d) historical

7. Which is not an element of a minimum first session? (a) confidentiality and limits (b) a risk screen (c) a full genogram (d) a summary and invitation to correct

8. The strongest predictor of drop-out in couple and family work is: (a) diagnosis (b) therapist's model (c) early engagement and early change (d) family size

9. "Multigenerational transmission" belongs to which tradition? (a) structural (b) Bowen (c) Milan (d) solution-focused

10. The statement "expert in process, not in their lives" describes: (a) neutrality about violence (b) the therapist's stance (c) a legal requirement (d) a diagnostic rule

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 c · 9 b · 10 b

Short answer (150 words each)

1. Explain the difference between negative and positive feedback using a family example.
2. Why is "punctuation" clinically useful? Give a sample question.
3. Name three limits of confidentiality in family work and how you would state each to a family.
4. What is the difference between a genogram and a formulation?
5. Summarise the common-factors critique of model loyalty in four sentences.

Essay questions (choose one, 1,500 words)

1. "Family therapy is a frame, not a technique." Discuss with reference to at least two models and one case from your placement.
2. Critically evaluate the claim that systemic neutrality is impossible — and undesirable — in cases of domestic violence.
3. Using a disguised case, demonstrate how a linear formulation and a circular formulation would lead to different treatment plans. Which would you choose and why?

Practical and clinical log requirements

For each session you observe or conduct, record:

Field	Requirement
Date and duration	Exact
Attendees	Relationship to index client; ages; who was absent
Referral source and question	In the referrer's words
Presenting problem	In each attendee's words
Risk screen	Completed Y/N; result; action
Pattern observed	One loop or sequence, described behaviourally
Intervention	What you did, and why
Response	What happened next
Supervision	Who, when, what was discussed
Plan	Next step, with review date

The log is assessed for **specificity and consistency**, not for how impressive the case was. A log of ordinary sessions written precisely will score higher than a dramatic case written vaguely.

Further reading

Core texts

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Dallos, R., & Draper, R. (2015). An Introduction to Family Therapy (4th ed.). Open University Press.

Foundations and history

- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Bateson, G. (1972). Steps to an Ecology of Mind. Chandler.
- Hoffman, L. (1981). Foundations of Family Therapy. Basic Books.
- Gurman, A. S., Lebow, J. L., & Snyder, D. K. (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Practice and technique

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Haley, J. (1980). Leaving Home. McGraw-Hill.
- McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). Norton.

Culture and critique

- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford.
- Hare-Mustin, R. (1978). A feminist approach to family therapy. Family Process, 17(2), 181–194.

Evidence

- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Shadish, W. R., & Baldwin, S. A. (2003). Effects of family and marital psychotherapies: A meta-analysis. Journal of Consulting and Clinical Psychology, 71(6), 1080–1097.

Codes and standards

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- International Association of Marriage and Family Counselors. Ethical Code.

Next: MFT 702 — Theories of Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 702

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MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	**Theories of Family Therapy**	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Nine major schools, each treated as an answer to a specific question. You will learn each model well enough to do a first session in it, critique it, and integrate it — and you will practise moving between models on the same family until the choice becomes deliberate rather than habitual.

How to use this book

Theory is learned in three passes. **Pass one:** understand the model's core question and its intervention. **Pass two:** apply it to a case you know. **Pass three:** critique it and ask what it cannot see. Each week in this book gives you all three, plus the practical that makes the pass real. Do not memorise lists of techniques; memorise what problem each model believes it is solving.

1. Course description

There is no single "correct" family therapy. There are traditions, each with a theory of how problems develop, a theory of change, a preferred focus of attention, and a characteristic set of techniques. A clinician who knows only one model does not have a theory; they have a habit.

This course therefore pursues three aims: **comparative competence** (you can describe and demonstrate each major model), **critical appraisal** (you can name each model's evidence base and its blind spots), and **integration** (you can combine two models coherently in one case rather than dithering between them).

Organising question. For each model, ask: What does this model believe is the problem, and what does it therefore do first?

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the theoretical foundations, core concepts and change mechanisms of nine major models	Exam
LO2	Demonstrate a model-consistent first session for at least three models	Skills assessment
LO3	Compare models on focus, technique, role of the therapist and evidence	Comparative matrix
LO4	Identify the blind spots and cultural assumptions of each model	Essay
LO5	Construct a coherent integrative plan for a single case using two models	Case assignment
LO6	Justify model choice with reference to the client, the evidence and your own competence	Viva

3. Assessment

Component	Weight	Due
Comparative matrix (nine models × six dimensions)	20%	Week 7
Observed model-consistent session (20 min)	20%	Weeks 8–10
Integrative case study (2,500 words)	30%	Week 12
Written examination	30%	Week 15

4. The fourteen-week plan

Week	Model / topic	Practical	Key text
1	How to compare theories: six dimensions	Matrix construction	Nichols ch. 1
2	Psychoanalytic and object-relations family therapy	Genogram + interpretation exercise	Scharff & Scharff
3	Bowen's multigenerational theory	Detriangling role-play	Bowen (1978)
4	Structural family therapy	Enactment and boundary-making	Minuchin (1974)
5	Strategic and MRI brief therapy	Directive design	Haley & Madanes
6	Milan systemic therapy	Circular questioning	Palazzoli et al. (1980)
7	Experiential and human validation (Satir, Whitaker)	Sculpture exercise	Satir (1988)
8	Cognitive-behavioural and behavioural couple therapy	Behavioural exchange design	Epstein & Baucom
9	Emotionally focused therapy	EFT enactment	Johnson (2019)
10	Solution-focused brief therapy	Miracle question and scaling	de Shazer (1985)
11	Narrative therapy	Externalising conversation	White (2007)
12	Integration: combine two models without incoherence	Case integration workshop	Lebow (2014)
13	Evidence, culture and critique of each model	Debate	Sprenkle et al.
14	Your theoretical position	Position paper presentation	—

5. The six dimensions of comparison

Every model can be described on the same six axes. Learn to fill this grid from memory; it is the spine of the exam.

Dimension	The question it answers
Theory of problem	How does this model believe problems develop and persist?

Dimension	The question it answers
Unit of focus	Individual, dyad, family structure, meaning system, or context?
Theory of change	What must happen for things to improve?
Therapist position	Director, coach, collaborator, witness, expert?
Signature techniques	What does this model characteristically do?
Evidence base	What is the strength and quality of supporting research?

> **PRACTICAL**
> Build the matrix in a spreadsheet on day one. Add a row each week. By Week 7 you will have a revision document worth more than any summary anyone could give you.

Week 2 — Psychoanalytic and object-relations family therapy

The question it asks. What unconscious processes, carried from earlier relationships, are being re-enacted in this family now?

Core concepts. Object relations (internal working models of self and other formed in early relationships); projection and projective identification (disowned parts of the self are located in another person); splitting (people experienced as all-good or all-bad); repetition compulsion; the family as a shared unconscious system.

Therapy. Longer-term; the therapist attends to transference and countertransference, interprets patterns, and helps the family "hold" difficult feelings rather than evacuate them into one member. Frameworks include group-analytic family therapy and the Tavistock tradition.

Signature technique — interpretation, used sparingly. "I notice that whenever your mother speaks about her own mother, you begin to speak for her. I wonder whether that is how the family has always managed her sadness."

Contribution. It gave the field its respect for depth, for what is not said, and for the therapist's own emotional response as data.

Blind spots. Long, expensive, thin evidence base in its classical form; risks pathologising parents; the language can feel blaming ("the intrusive mother") and can ignore material context.

> **CASE**
> A young man cannot keep a job; his mother calls his employers before he does. Object-relations reading: the mother's own abandonment anxiety is projected into her son's incompetence, and his failure keeps her needed. Interpretation may be less useful than an experiment: agree that she will not call, and predict that both will feel anxious for two weeks. The prediction turns insight into something testable.

Week 3 — Bowen's multigenerational theory

The question. How is anxiety transmitted across generations, and who in the system absorbs it?

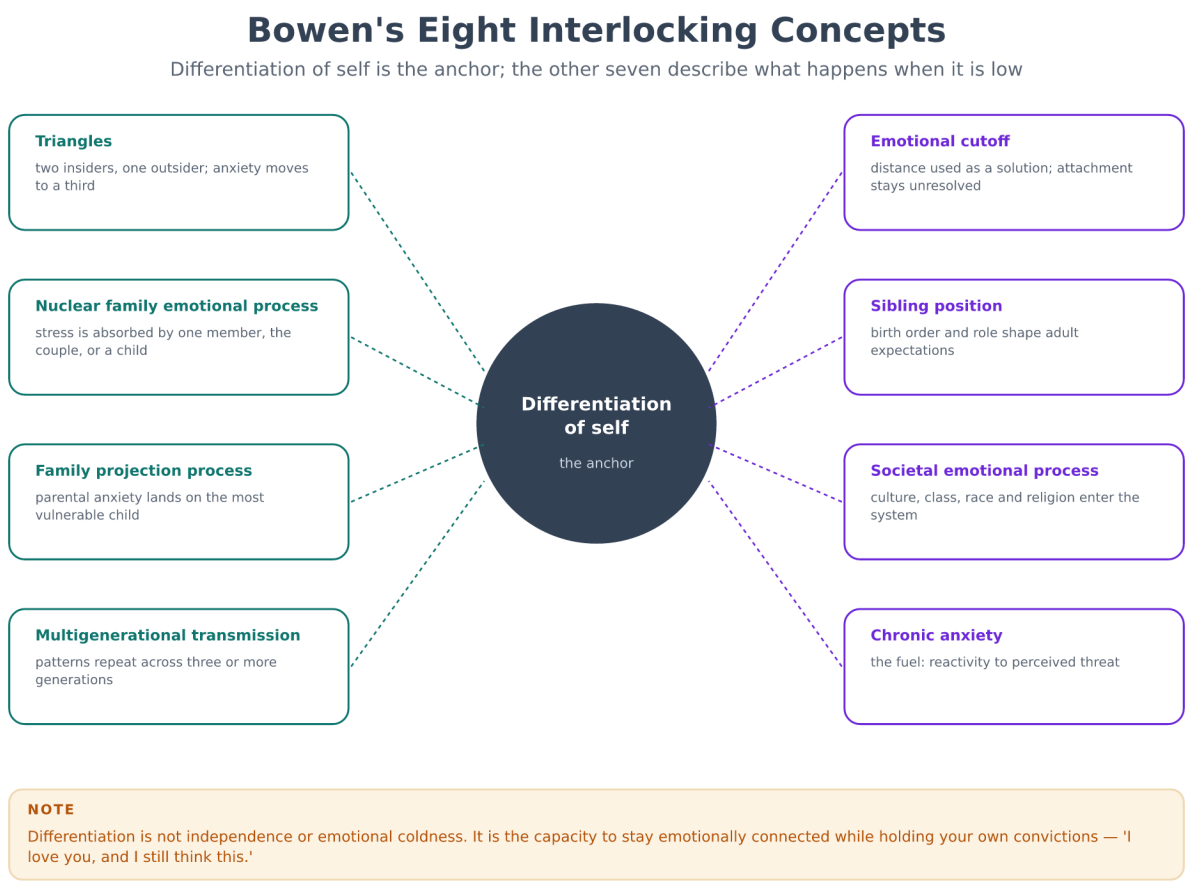


Figure 3.1 — Bowen's eight interlocking concepts

Core concepts. Differentiation of self (the capacity to stay connected while thinking for oneself); triangles; the nuclear family emotional process (marital conflict, dysfunction in a spouse, projection to a child, emotional distance); family projection process; emotional cutoff; multigenerational transmission; sibling position; societal emotional process; chronic anxiety.

Therapy. The therapist is a **coach**, not a fixer. The work is largely with the most motivated person (often "the over-functioner"), who learns to stay in contact with difficult relatives while managing their own reactivity. Genograms are the primary tool; the goal is differentiation, not symptom removal.

Signature technique — detriangling. "When your mother criticises your sister, what happens inside you? What would it look like to stay interested without taking a side and without leaving?"

Contribution. Genuinely systemic over time; explains why patterns repeat; insists the therapist do their own family-of-origin work.

Blind spots. Slow; assumes a level of choice and mobility not everyone has; can under-weight structure, gender and economics; evidence is largely clinical rather than trial-based.

> **PRACTICAL**
> Triads. One person describes a triangle in their family. The therapist practises three moves: name the triangle, ask about the client's own reactivity, and refuse to take a side. Rotate; the "client" rates how neutral (not cold) the therapist was.

Week 4 — Structural family therapy

The question. What is the organisation of this family, and how does its structure maintain the problem?

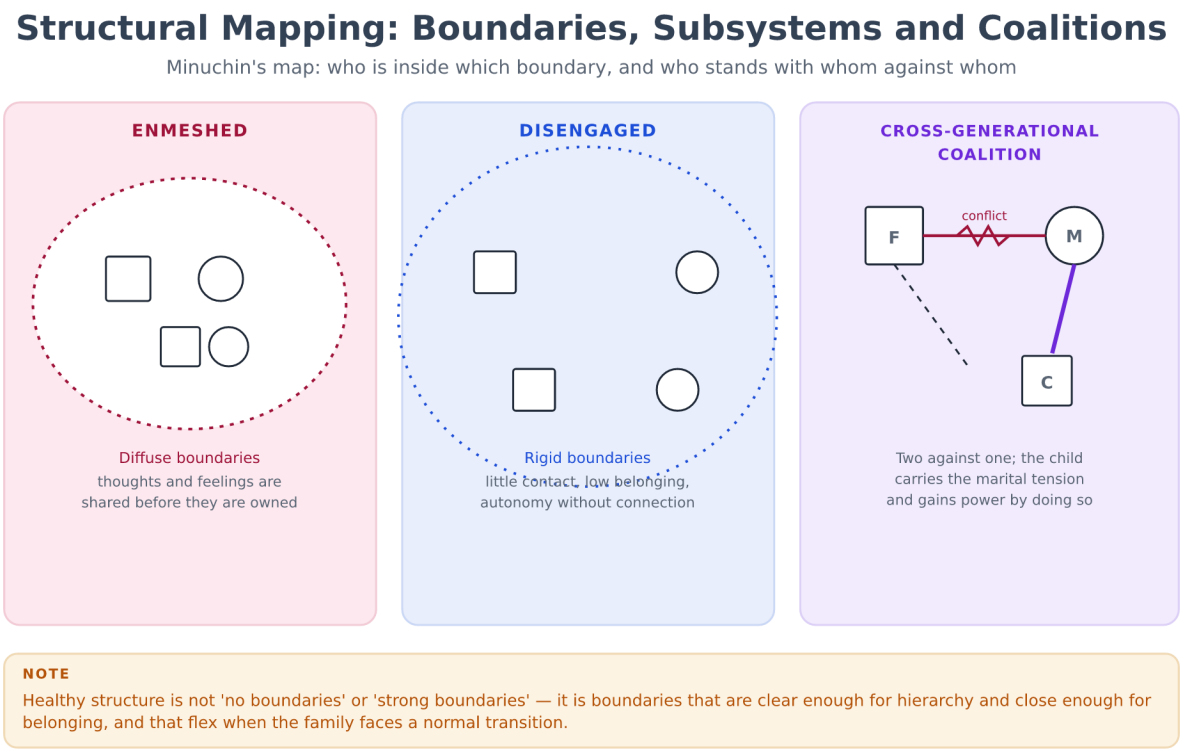


Figure 4.1 — Structural mapping: boundaries, subsystems, coalitions

Core concepts. Structure (the repetitive patterns of interaction that organise the family); subsystems; boundaries (clear, diffuse, rigid); hierarchy; alignment and coalition; enmeshment and disengagement; complementarity; the identified patient as the symptom-bearer of a structural problem.

Therapy. The therapist **joins** and then **challenges** the structure in the room. Change happens through doing, not talking: enactments, boundary-making, unbalancing, raising intensity, and marking hierarchy. Sessions are active and often directive.

Signature techniques.

- **Enactment:** "Say that to her, not to me. I'll listen."
- **Unbalancing:** deliberately supporting one member to shift the system's equilibrium.
- **Boundary-making:** "This is a conversation for the two of you; the children will wait outside."
- **Intensity:** "This is the fourth time you have laughed when your wife described the miscarriage. What is that laugh?"

Contribution. Fast, observable, highly teachable; strong evidence for structural family therapy in child and adolescent problems and in family-based treatment for substance misuse.

Blind spots. Risks imposing a middle-class norm of two-parent hierarchy; can be experienced as controlling; under-attends to inner life and to history.

> **CASE**
> The Adeyemi family. The structural reading: the parental subsystem is weak and the mother–daughter coalition crosses the generational line; the father is structurally peripheral and reacts with criticism when he is pulled in. First move: a session with the parents alone to plan one consistent rule and one shared evening, before any work with Amara. Second move: a joint session in which the parents tell Amara the plan together while the therapist blocks her attempts to negotiate with her mother alone.

Week 5 — Strategic and MRI brief therapy

The question. What is the family doing that keeps the problem going — especially their attempted solutions?

Core concepts. The problem is maintained by current interaction, not by the past; attempted solutions often persist because they "almost" work; the symptom may serve a function in the system; resistance is a response to the therapist's approach, not a property of the family; brief, focused, pragmatic.

Therapy. The therapist is an **active strategist** who designs interventions — tasks, directives, reframes, and sometimes paradoxical instructions. Sessions are few; between-session tasks do the work; termination is early by design.

Signature techniques.

- **Reframing:** "Your daughter's tantrums may be her way of keeping you two in the same room."

- **Ordeals:** a task that must be performed whenever the symptom occurs, making the symptom more costly than the change.
- **Paradoxical directive (with caution):** prescribing the symptom, under close supervision, in carefully selected cases.
- **Pretend technique:** "For one week, pretend to have the symptom on purpose for ten minutes a night."

Contribution. Broke the assumption that therapy must be long; emphasised the therapeutic power of therapist pragmatism and of clear, behavioural tasks.

Blind spots. Manipulation risks; weak evidence for paradoxical techniques; may be superficial in cases with trauma or severe mental illness; the "function of the symptom" can be speculative.

> **PRACTICAL**
> Design a one-week task for a family you know (disguised) that makes the current attempted solution impossible to continue, without instructing them to do the opposite in so many words. Present it; the group judges whether it is clear, doable, and safe.

Week 6 — Milan systemic therapy

The question. How do the family's beliefs and premises organise their behaviour — and how can curiosity disturb them?

Core concepts. The family as a meaning-making system; the **hypothesising, circularity and neutrality** stance; positive connotation (all behaviour is understood as serving the system); rituals; **circular questioning** (questions about differences and relationships rather than facts); the reflecting team and the "irreverent" later Milan style.

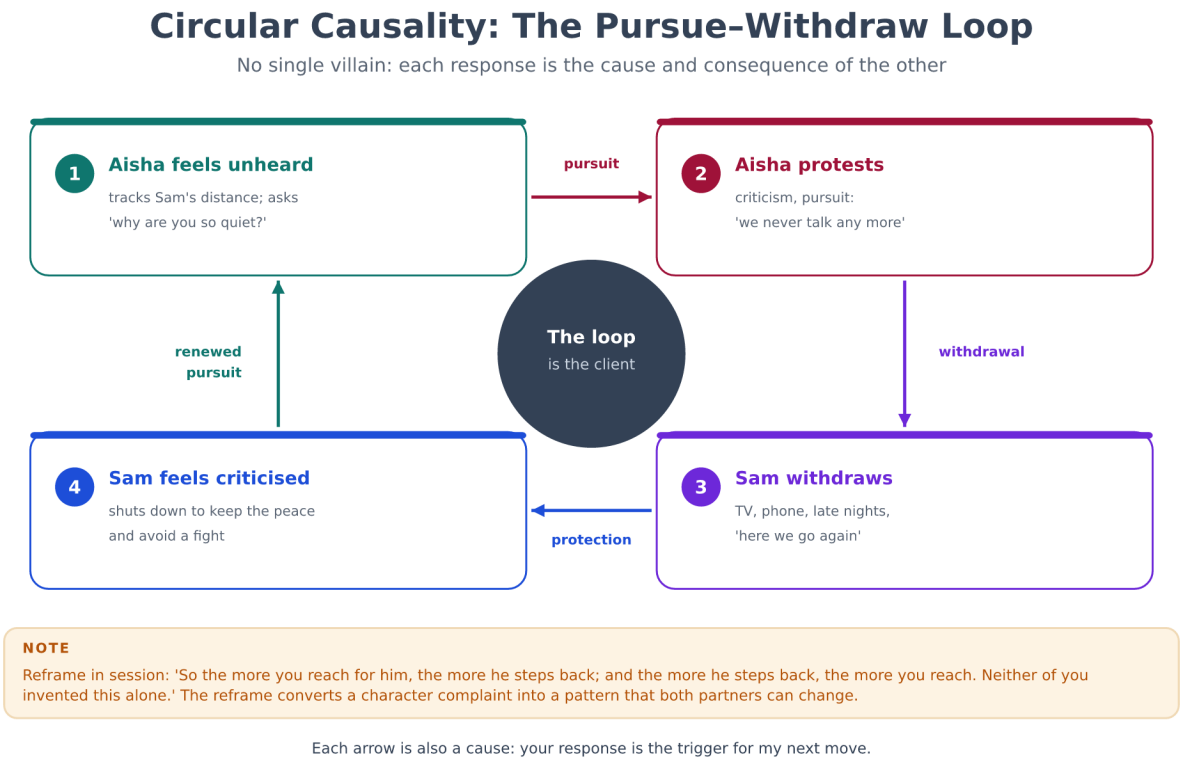


Figure 6.1 — Circular causality in practice

Therapy. The team observes behind a screen; the therapist asks circular questions; a consultation break produces an intervention that positively connotes the family's behaviour, often prescribing a ritual. The modern version is non-directive: the team's reflections are offered as multiple perspectives, not as verdicts.

Signature technique — circular questioning.

- "Who in the family is most worried about your mother's sadness?"
- "If your brother were here, what would he say your father does when you cry?"
- "On a scale from 1 to 10, how close are your parents? And how close would they say they are?"
- "What is the difference between the way your mother treats you and the way she treats your sister?"

Contribution. Gave the field its most transferable skill — the circular question — and its most useful epistemic stance, curiosity.

Blind spots. Can be experienced as evasive in crisis; neutrality is ethically dangerous in violence; team-based format is expensive.

PRACTICAL

Ten-minute interview with a volunteer "family" (two peers). You may ask only circular questions. An observer counts how many were genuinely circular (about difference/relationship) and how many were disguised facts. Aim for 80%.

Week 7 — Experiential and human validation models

The question. What would happen if this family could feel and say the truth to each other, right now?

Key figures. Virginia Satir (family sculpture, communication stances, self-worth, "the five freedoms"); Carl Whitaker (symbolic-experiential therapy, absurdity, the therapist's use of self); Kempler and the Gestalt-influenced tradition.

Core concepts. Growth rather than pathology; the present moment as the site of change; expression of authentic feeling; the therapist's own aliveness as a therapeutic instrument; family sculpture as non-verbal enactment of structure.

Signature techniques.

- **Family sculpture:** family physically arrange themselves to show the emotional map; then rearrange.
- **Communication stances** (Satir): placating, blaming, super-reasonable, irrelevant, and congruent.
- **Here-and-now enactment:** "Tell her now. I want to see what happens to your face when you say it."

Contribution. Humanised the field; insisted on warmth, humour and the person of the therapist. Many of its insights survive inside EFT and other emotion-focused models.

Blind spots. Dependent on the charisma and health of the therapist; thin manualisation; risk of emotional flooding without containment.

CASE

In a family session, the father has not cried in 20 years. Whitaker-influenced response: the therapist does not push the tears, but stays with the father's jaw and breathing, wondering aloud about what it costs to hold it. When the father finally speaks, the therapist keeps the room still long enough for others to respond. The intervention is pace, not technique.

Week 8 — Cognitive-behavioural and behavioural couple therapy

The question. What behaviours and beliefs maintain the distress, and how can they be changed directly?

Core concepts. Behavioural exchange and communication training (TBCT); cognitive restructuring of relationship standards and attributions; the role of selective attention (noticing negatives and missing positives); homework as the engine of change; skills deficit versus skills performance.

Therapy. Structured, time-limited, with agenda and homework. Skills are taught and rehearsed in the room, then practised between sessions. Integrative behavioural couples therapy addresses both behaviour and acceptance, with attention to culture and values.

Signature techniques.

- **Behavioural exchange:** each partner lists small, achievable caring behaviours; each does them daily.
- **Communication training:** speaker–listener with a talking object (see Figure 35, MFT 716).
- **Attribution retraining:** "When he is late, what do you assume? What else could it mean?"
- **Problem-solving:** define one problem, generate options, choose, trial, review.

Evidence. Among the strongest in the field; effective for relationship distress, with good evidence for depression treated in a couple format.

Blind spots. Can feel mechanistic; may under-weight emotion and attachment; assumes a degree of motivation and literacy; "skills" can be performed in session and abandoned at home.

> **PRACTICAL**
> With a peer, design a behavioural exchange contract: three behaviours each, defined observably, cheap, daily, and mutually agreeable. Then critique it: what could make this fail in a real family within two weeks?

Week 9 — Emotionally focused therapy

The question. What attachment need is underneath the anger, and how can the cycle be de-escalated so the bond can be repaired?

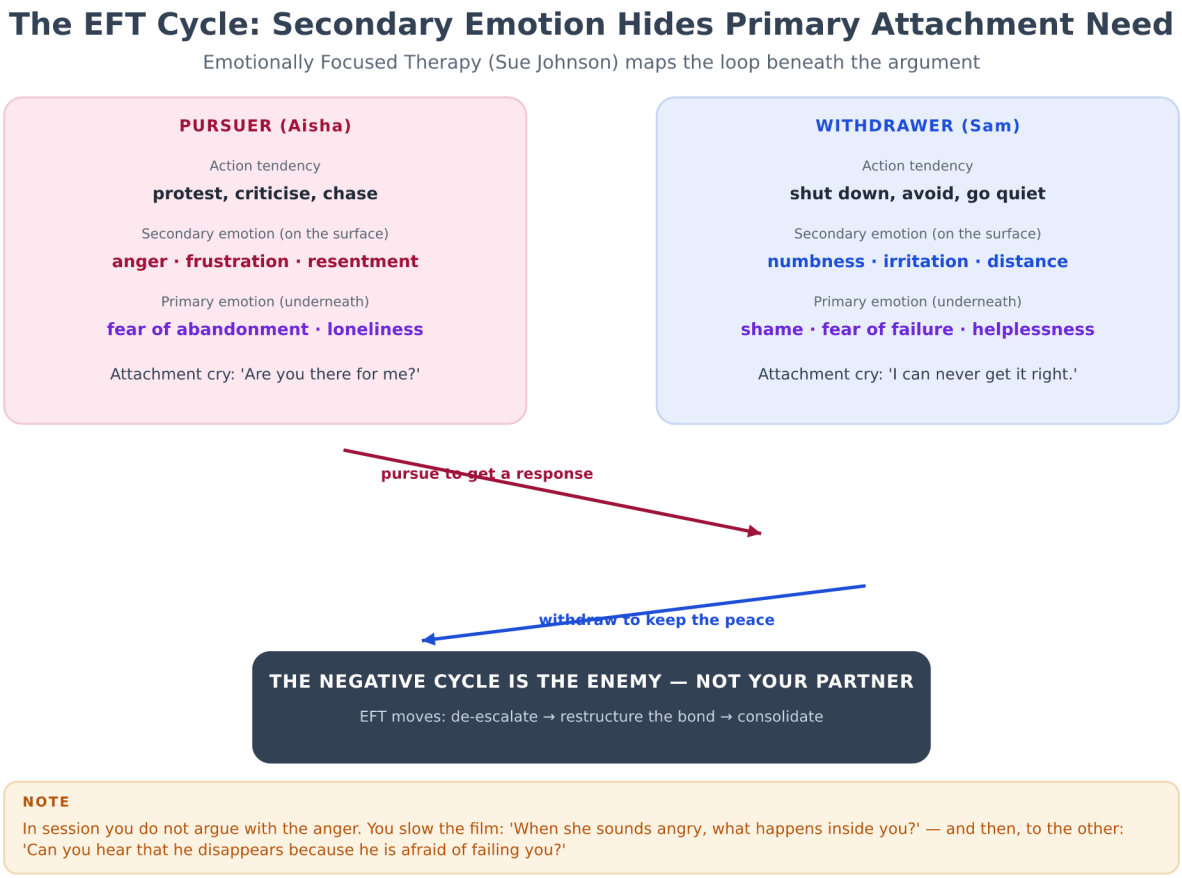


Figure 9.1 — The EFT cycle: secondary emotion and primary attachment need

Core concepts. Attachment theory as the map (see Figure 17); the negative cycle (pursue–withdraw, attack–defend, both-withdraw); primary versus secondary emotion; the "attachment cry"; softening events; the therapist as a process consultant and secure base.

Therapy in three stages. 1) **De-escalation** — identify the cycle, access primary emotion, reframe the problem as the cycle. 2) **Restructuring** — withdrawer re-engagement, pursuer softening, enactment of new responses. 3) **Consolidation** — integrate, problem-solve, prepare for recurrence.

Signature techniques.

- **Tracking the cycle:** "What happens inside you when she says that?"
- **Deepening primary emotion:** "Under the anger — is there something like fear of being alone?"
- **Enactment:** "Can you tell him that now, looking at him?"
- **Softening:** the pursuer's protest turns into an attachment expression, and the withdrawer responds.

Evidence. Among the most robust in couple therapy (over 30 trials, large effect sizes, durability in follow-up). Considered a first-line couple therapy for relational distress.

Blind spots. Requires a specific in-session emotional intensity and skills; less suited to ongoing coercive control; cultural calibration needed (the language of "needs" may feel self-indulgent to some families).

> **CASE**
> Aisha and Sam. Third session: the therapist slows a familiar argument to 20% speed. Aisha's anger is reframed as protest; Sam's silence as fear of failing. The first enactment — Aisha, quietly: "I get loud because I think you are leaving" — is the turning point. The next eight sessions are the work of making that statement safe to repeat at home.

Week 10 — Solution-focused brief therapy

The question. What is already working, and what would one small step look like?



Figure 10.1 — Solution-focused brief therapy in five moves

Core concepts. Solution talk versus problem talk; the miracle question; scaling; exceptions; coping questions; the client as expert; small, observable change; "if it works, do more of it".

Therapy. Conversational, non-normative, often 3–6 sessions. The therapist listens for competence and deliberately amplifies it; homework is usually "notice what works".

Signature techniques.

- **Miracle question:** "Suppose tonight a miracle happens and the problem is solved. What will you notice first tomorrow morning? Who else will notice?"
- **Scaling:** "On 0–10, where are you? What would one point higher look like this week?"
- **Exception questions:** "Tell me about a time the arguing stayed small. What were you doing differently?"
- **Coping question:** "With everything you have faced, how have you managed to keep going?"

Evidence. Good evidence for depression, with brief formats showing effects comparable to longer treatments in some trials; widely used in child protection and school settings.

Blind spots. Can bypass necessary grief or trauma work; risks ignoring risk; may feel invalidating when a family needs to tell the story of what happened.

> **PRACTICAL**
> Full miracle-question sequence in pairs (8 minutes), then a one-point scaling and a concrete task. Observer records: did the therapist ask for concrete behavioural detail, and did the client's posture change?

Week 11 — Narrative therapy

The question. Which story is dominating this family's life, and who wrote it?

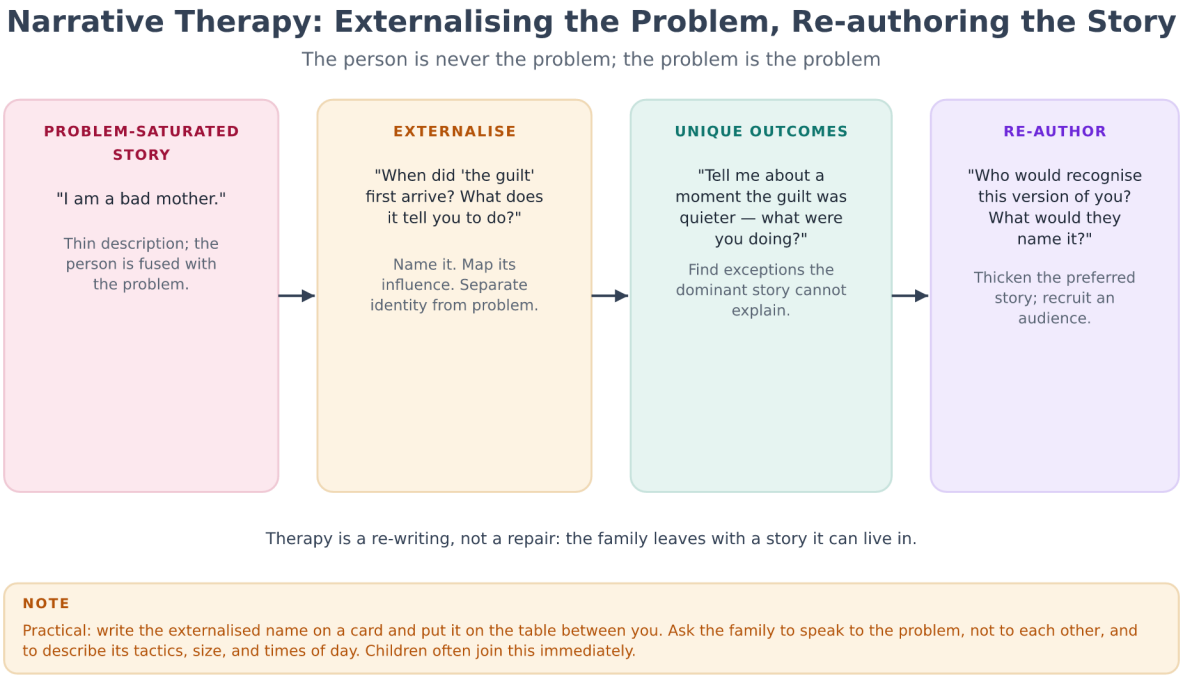


Figure 11.1 — Externalising and re-authoring the preferred story

Core concepts. The person is not the problem; the problem is the problem; dominant and preferred stories; thin versus thick descriptions; unique outcomes; externalising conversations; re-membering; definitional ceremonies; the therapist as decentred but influential.

Signature techniques.

- **Externalising:** "When did 'the guilt' first arrive in your life? What does it whisper to you?"
- **Mapping the influence:** "When does it have the most power? When does it lose power?"
- **Unique outcomes:** "Tell me about a moment when 'the shouting' was quieter."
- **Re-authoring:** "If you were to name this new version of yourself, what would you call it? Who would recognise it?"
- **Letter-writing** and **outsider-witness groups** to thicken the preferred story.

Contribution. The most culturally adaptable of the schools; central in work with children, with communities, and with people who have been defined by a diagnosis.

Blind spots. Can under-attend to material reality and risk; the linguistic sophistication can be inaccessible; evidence base is growing but smaller than EFT or CBT.

> **CASE**
> A mother referred by the school as "negligent". The therapist does not argue with the label but asks when "the exhaustion" took over, what it makes possible, what it prevents. Within thirty minutes the mother has described three occasions when she "fought the exhaustion and won", including a week when she arranged a rota with her sister. The referral, still true, is no longer the whole story.

Week 12 — Integration without incoherence

The question. How do you combine models so the client is not paying for your confusion?

Three legitimate forms of integration:

1. **Assimilative integration.** A home model, enriched by techniques from elsewhere. Most clinicians work this way. The home model keeps the formulation coherent.
2. **Sequential integration.** Different models at different phases — for example structural work to establish safety and hierarchy, then EFT for the couple's bond, then solution-focused work to consolidate.
3. **Theoretically integrated models.** Purpose-built blends (integrative behavioural couples therapy; attachment-based family therapy), coherent because they were designed together.

Rules for staying coherent.

Rule	Why
One formulation, one primary target	Prevents scattergun technique
Name the model out loud in supervision	Makes your reasoning auditable
Change model for a reason you can state	"Because the conflict escalates when we go near feelings"
Do not mix incompatible theories of change in one move	Reframing and behavioural contracting pull in different directions
Review at fixed intervals	Integration fails by drifting, not by decision

Trap to avoid: technique salad. Symptom-tracking from CBT, a miracle question, a family sculpture and an externalising conversation in one session does not make you integrative; it makes you unpredictable, and the family cannot tell what is expected of them.

> **ASSIGNMENT (30%)**
> One disguised case; two models; one formulation each; a reasoned choice of sequence; a predicted risk of the combination; a plan for how you would know it is not working.

Week 13 — Evidence, culture and critique

Model	Strongest evidence	Weakest evidence	Main cultural critique
Structural	Child/adolescent behaviour; substance misuse	Adult mood disorders	Imposes a particular family hierarchy
Bowen	Clinical tradition; modest outcome data	Few RCTs	Assumes mobility and choice; slow
Strategic/MRI	Some RCTs in child problems	Paradoxical techniques	Manipulative; may ignore trauma
Milan	Weak in classical form	Rituals, team format	Neutrality unethical in violence
Experiential	Moderate, largely process research	Manualisation	Charisma-dependent; middle-class
CB couple therapy	Strong	Severe relational trauma	Mechanistic; literacy-dependent
EFT	Strong, replicated	Coercive control	Individualist language of needs
Solution-focused	Moderate (depression, brief formats)	Severe mental illness	Can bypass grief and risk
Narrative	Growing; strong qualitative base	Large trials	Requires verbal fluency

The reflexive question. For each row, ask: whose family is this model describing? Models were developed largely with white, Western, middle-class, two-parent families in clinic settings. When you apply them to a polygynous household, a three-generation

compound, a family separated by migration, or a family whose authority structure is religious rather than developmental, you must translate — and say that you are translating.

Week 14 — Your theoretical position

Write 1,500 words: your home model, why it fits you, what it cannot do, the models you borrow from and under what conditions, and how you will know when to change your mind. Defend it in a ten-minute viva.

Integration: nine models on one page

Model	Problem is…	Change happens by…	Therapist is…	First move
Object relations	Unconscious re-enactment	Making the unconscious usable in the room	Container/interpreter	Notice the transference; interpret gently
Bowen	Undifferentiated anxiety across generations	Differentiation of self	Coach	Genogram + detriangling
Structural	Dysfunctional organisation	Restructuring in the session	Director/joiner	Enactment; strengthen hierarchy
Strategic	Attempted solutions	Interrupting the sequence	Strategist	Task, reframe, directive
Milan	Meaning premises	Curiosity disturbing belief	Curious team member	Circular questions
Experiential	Blocked feeling and authenticity	Present-moment expression	Real person	Sculpture; here-and-now
CB couple	Behaviour and attribution	Skills and rehearsal	Teacher/coach	Behavioural exchange
EFT	Attachment injury and cycle	Accessing primary emotion; enacting new responses	Process consultant	Track and reframe the cycle
Solution-focused	Problem-saturated attention	Amplifying existing solutions	Collaborator	Miracle/scale question
Narrative	Dominant story	Externalising and re-authoring	Decentred, influential	Externalising question

Glossary

Circular questioning — questions about differences and relationships that reveal pattern rather than fact. **Differentiation of self** — the capacity to stay emotionally connected while holding one's own position. **Enactment** — directing family members to speak to each other in the session rather than through the therapist. **Externalising** — locating a problem outside a person, as a thing with tactics and a history. **Negative connotation / positive connotation** — interpreting behaviour as serving the system, rather than as defective. **Paradoxical directive** — prescribing the symptom in order to change its function (used rarely, with supervision). **Reframe** — relabelling behaviour within a new frame that makes change possible. **Restructuring** — changing the family's organisation in the session through enactment and boundary work. **Scaling** — asking a family to rate a state on a numerical scale to make change visible and specific. **Unique outcome** — a moment when the dominant story did not hold, used as a foothold for a preferred story.

Self-assessment

Multiple choice

1. "The problem is the problem; the person is not the problem" is a principle of: (a) structural (b) narrative (c) Bowen (d) strategic

2. A therapist says, "For the next week, I want you to argue for exactly fifteen minutes at 8 p.m., then stop." This is closest to: (a) narrative (b) EFT (c) strategic (d) solution-focused

3. Positive connotation is associated with: (a) Milan (b) CB couple therapy (c) object relations (d) solution-focused

4. Which model places differentiation of self at its centre? (a) structural (b) Bowen (c) EFT (d) Milan

5. "Who would notice first if the problem were solved?" is a technique from: (a) structural (b) solution-focused (c) narrative (d) experiential

6. In EFT, the behaviour that conceals a primary attachment emotion is called: (a) enactment (b) secondary emotion (c) reframe (d) restructuring

7. Which model is most associated with family sculpture? (a) Satir (b) Haley (c) Minuchin (d) White

8. "Unbalancing" is a technique of: (a) structural therapy (b) solution-focused therapy (c) object relations (d) narrative therapy

9. Which model has the strongest replicated randomised evidence in couple therapy? (a) Bowen (b) Milan (c) EFT (d) experiential

10. Assimilative integration means: (a) blending theories equally (b) using a home model enriched by borrowed techniques (c) alternating models by session (d) refusing to commit to a model

Answer key

1 b · 2 c · 3 a · 4 b · 5 b · 6 b · 7 a · 8 a · 9 c · 10 b

Short answer

1. State the theory of change of structural therapy in two sentences, then name one case type where it is a poor fit and say why.
2. Write three circular questions you would ask a family in which a father has just lost his job.
3. What is the difference between positive connotation and reframing?
4. Give two reasons why narrative therapy suits work with children.
5. Name two incompatible assumptions that would make a structural-plus-narrative formulation incoherent.

Essay (choose one, 2,000 words)

1. Compare structural and EFT approaches to the same couple case. Which would you choose first and why?
2. "Neutrality is a luxury the powerful can afford." Discuss with reference to Milan therapy and domestic violence.
3. Critically evaluate the claim that theoretical integration is a sign of maturity rather than confusion.

Further reading

Comparative

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
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- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
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- Haley, J. (1987). Problem-Solving Therapy (2nd ed.). Jossey-Bass.
- Palazzoli, M. S., Cecchin, G., Boscolo, L., & Prata, G. (1980). Paradox and Counterparadox. Aronson.
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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	**Human Development Across the Lifespan**	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You cannot assess a family without knowing what is developmentally ordinary. This course covers physical, cognitive, language, social-emotional and moral development from conception to later life, always in family and cultural context — and it trains you to distinguish normal variation, delay, regression and symptom.

How to use this book

Development is not a list of ages; it is a set of **tasks** and a set of **negotiations** between a person and their family. Each week here gives you the developmental task, the family task that accompanies it, what goes wrong, and what it looks like in a consulting room. Use the milestone grid (Figure 23) as a reference you return to, not as a checklist to memorise.

1. Course description

Human development is the backbone of family assessment. A two-year-old's tantrum, a nine-year-old's lying, and a fifteen-year-old's secrecy are not three versions of "defiance": they are three different developmental moments that require three different responses. Clinicians who do not know this either pathologise normal development or miss genuine delay.

The course integrates **neurodevelopmental, cognitive, attachment, systemic and cultural** perspectives. It treats development as bidirectional: children shape parents as much as parents shape children, and families develop too (a theme developed in MFT 719).

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe physical, cognitive, language and social-emotional milestones from 0–18 and in adulthood	Exam
LO2	Distinguish normal variation, delay, regression and symptom in a presenting concern	Case assignment
LO3	Apply attachment theory to family assessment	Case assignment, exam
LO4	Explain developmental tasks of adulthood and later life and their family implications	Exam
LO5	Evaluate developmental theory critically, including its cultural assumptions	Essay
LO6	Communicate developmental information to parents without jargon or alarm	Skills assessment

3. Assessment

Component	Weight	Due
Developmental case analysis (2,000 words)	30%	Week 8
Parent-feedback role-play (10 min, observed)	20%	Week 11
Written examination	50%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why developmental knowledge is clinical knowledge	Milestone self-audit
2	Prenatal development, temperament and the transition to parenthood	Temperament interview
3	Infancy: attachment, synchrony, regulation	Still-face observation
4	Toddlerhood: autonomy, language, limit-setting	Tantrum consultation role-play
5	Early childhood: play, theory of mind, initiative	Play observation
6	Middle childhood: industry, peers, school, self-concept	School-refusal case
7	Adolescence I: puberty, identity, autonomy	Adolescent interview
8	Adolescence II: risk, peers, digital life, mental health	Risk/development differential
9	Adult development: intimacy, work, generativity	Life-structure interview
10	Couple development across the lifespan	Couple timeline exercise
11	Parenting across stages; communicating with parents	**Observed parent feedback**
12	Later life: cognition, loss, caregiving, legacy	Caregiver family meeting
13	Culture, poverty and development	Critical reading seminar
14	When development goes wrong: delay, disorder, trauma	Differential diagnosis workshop
15	Integration: the developmental lens in assessment	Case presentations

Week 1 — Why developmental knowledge is clinical knowledge

Three clinical errors are prevented by developmental literacy:

1. **Pathologising the ordinary.** A two-year-old who says "no" to everything is doing the developmental task of autonomy, not manipulating the family.
2. **Missing the abnormal.** A five-year-old who has lost previously acquired language is not "being difficult"; regression is a red flag requiring urgent referral.

3. **Mis-timing the intervention.** A parent is told to "reason with" a three-year-old whose prefrontal circuitry cannot yet support the reasoning demanded. The intervention fails and the parent concludes they are failing.

Four categories you must distinguish — this distinction recurs in every case assignment:

Category	Definition	Example
Normal variation	Within the expected range, differing in style and pace	Walking at 11 months vs 15 months
Delay	Functioning below the expected range but progressing	No words at 24 months, but progressing
Regression	Loss of previously acquired skill	A four-year-old stops speaking
Symptom	Behaviour that signals distress rather than development	New soiling at seven after a bereavement



Figure 1.1 — Developmental milestones from 0 to 18: a screening map, not a diagnosis

PRACTICAL

Write your own developmental timeline from memory: first word, first step, a significant illness, a move, a loss. Then ask a parent or sibling to correct it. Note which events you had forgotten and who remembered them.

Week 2 — Prenatal development, temperament and becoming a parent

Prenatal and perinatal. Brain development, maternal stress and nutrition, birth experience, and the "fourth trimester". Clinically relevant: prematurity, birth trauma, postnatal depression (which affects up to 1 in 5 mothers in some settings and is strongly associated with relational and child outcomes), and paternal postnatal depression.

Temperament. Nine dimensions (Thomas & Chess) usually simplified to three profiles: easy, slow-to- warm, and difficult/feisty. Temperament is not destiny but it is **goodness of fit** that matters: a highly sensitive child in a loud, unpredictable household will struggle more than the same child in a predictable one.

The transition to parenthood. The couple must reorganise: time, sleep, money, division of labour, sexuality, relationship with grandparents, and identity. Research consistently finds relationship satisfaction dips after the birth of a child — and that the quality of the co-parenting alliance, more than the amount of conflict, predicts child outcomes.

> **CASE**
> A couple presents with "no time for each other" four months after their first baby. The developmental frame normalises the strain (this is the most common point of relational collapse), then asks the systemic question: which grandparent has been recruited into the parenting triangle, and what did the couple stop negotiating when the grandmother moved in?

Practice. In pairs, conduct a 10-minute temperament interview with a parent (or peer playing one): sleep, feeding, reaction to change, intensity, mood. Then identify the goodness of fit with the household.

Week 3 — Infancy: attachment, synchrony and regulation

The idea. Attachment is not "bonding" and not "love"; it is a behavioural system that organises a child's strategies for seeking safety from a caregiver under stress.

The classic patterns (Ainsworth's Strange Situation; see Figure 17 in MFT 710): secure, insecure- avoidant, insecure-resistant/ambivalent, and disorganised (added by Main & Solomon, associated with frightening or frightened caregiving). In adulthood the same dimensions appear as attachment styles.

Synchrony and rupture–repair. Healthy development does not require perfect attunement — it requires frequent enough attunement and effective repair. Tronick's still-face experiments show how quickly infants respond to a caregiver's unresponsiveness, and how quickly they recover when the caregiver returns.

Co-regulation is the precursor of self-regulation. A baby does not learn to calm alone; they learn to calm with someone, repeatedly, until the pattern is internalised. This has direct clinical implications: telling an overwhelmed parent to "let him cry it out" without attention to the family's capacity and beliefs is a technique imposed on a relationship.

> **PRACTICAL**
> Watch a consented 3-minute video of a caregiver and infant (or a still-face clip). Note: gaze, vocalisation, touch, pace. Then describe one moment of rupture and one of repair in behavioural language. Avoid interpretation for the first five minutes.

Week 4 — Toddlerhood: autonomy, language and limits

Developmental tasks. Autonomy versus shame and doubt (Erikson); language explosion (from ~50 words at 18 months to 300+ and two-word combinations by 24 months); symbolic play; beginning of self-regulation.

The clinical translation of "no". A toddler's "no" is the emergence of a separate self. The parental task is to hold the limit without humiliating the child — "You may not hit. You may be angry." This is the earliest training ground for the emotional vocabulary the family will use for the next fifteen years.

Language and family. Delay in expressive language is the most common developmental concern presented to clinics. Distinguish: late talker (normal variation with good comprehension), bilingual language confusion (normal), hearing loss, global delay, and autism spectrum presentation. Always ask about **hearing** and about **which language is spoken at home and by whom**.

> **CASE**
> A grandmother raises a 2-year-old while both parents work. She reports he "does not listen". Assessment finds: he is spoken to mostly in commands, in two languages, by three adults with different rules. The intervention is not a behaviour chart; it is a family agreement about one set of five rules, one language of instruction, and ten minutes of child-led play a day with each caregiver.

Week 5 — Early childhood: play, theory of mind, initiative

Tasks. Initiative versus guilt; fantasy play; theory of mind (understanding that others have different beliefs — typically emerging around 4); moral development begins as consequence-based reasoning; school readiness.

Play is the work. Play develops language, self-regulation, perspective-taking, and the capacity to tolerate frustration. Children who have experienced adversity often play less

symbolically and more repetitively — a clinical observation that underpins play therapy (MFT 727).

The clinical skill: observing play without hijacking it. Describe what the child does; do not direct it, and do not interpret it prematurely ("that must be your father"). Interpretation before trust produces a performance, not a communication.

> **PRACTICAL**
> Ten minutes of child-led play with a volunteer child or peer. Your only tools: describe, reflect, imitate, and set one limit. Debrief: how many times did you ask a question? (Aim for fewer than three.)

Week 6 — Middle childhood: industry, peers, school

Tasks. Industry versus inferiority; mastery and competence; peer group and friendship rules; self-concept becomes comparative ("I am good at maths, bad at football"); moral reasoning becomes more reciprocal.

What changes in the family. The child's world widens; parents move from direct control to monitoring and scaffolding. School becomes a second system with its own rules, and the family–school interface becomes a clinical site.

School refusal — a developmental and systemic differential. Ask: is this anxiety, separation difficulty, bullying, a learning difficulty, a family crisis, or a bid to protect a parent? The distinction matters: the same behaviour has opposite treatment implications.

> **CASE**
> A nine-year-old refuses school on Mondays only. Developmental enquiry: what happens on Sunday nights? The answer: the parents' weekly argument about money. The child's presence at home on Monday keeps the mother from leaving for work, and the argument stops. The symptom is a Monday-shaped solution to a marital problem — and it is also genuinely distressing to the child.

Week 7 — Adolescence I: puberty, identity, autonomy

Tasks. Identity versus role confusion; pubertal change; separation-individuation without severance; peer belonging; emerging sexuality; moral and ideological commitment.

The neuroscience, briefly. The adolescent brain is not "broken": it is specialised. The reward system matures before the regulatory system, which produces sensation-seeking and heightened sensitivity to peer approval alongside gradually improving impulse control. This is an argument for scaffolding, not for lecturing.

The family task. To renegotiate the relationship: less control, more influence; more privacy, more accountability. Families that cannot renegotiate produce two common

patterns: escalation (control increases as autonomy increases) and abdication (parents withdraw entirely).

> **CASE**
> Amara, 16. The parents describe her as "secretive"; she describes them as "spies". The developmental reframe: privacy is a developmental need, and secrecy is what privacy becomes when there is no negotiated trust. The intervention is a negotiation: what will she share, what will they ask, and what happens when the agreement is broken?

Week 8 — Adolescence II: risk, peers, digital life and mental health

Risk behaviour is normative in moderation and dangerous in combination: substance use, risky driving, unsafe sex, self-harm, disordered eating. Risk is not a moral category; it is a developmental phenomenon shaped by peers, family, school and access.

Digital life. Assess function, not just quantity: what is the phone doing in this adolescent's life — connection, escape, status, self-soothing, harm? Ask about sleep, cyberbullying, sexual content, and the family's rules about devices. Banning without substituting an alternative rarely works.

Mental health. Half of all lifetime mental disorders begin by mid-adolescence. Screen, do not guess. Distinguish transient distress from disorder by **duration, impairment and severity** — and use the same developmental question: is this a normal task being struggled with, or a disorder?

> **PRACTICAL**
> Conduct a 12-minute adolescent interview with a peer who has been briefed to be reticent. You may not ask more than two questions in a row without reflecting something back. Observer records: ratio of questions to reflections; did the adolescent elaborate?

Week 9 — Adult development

Tasks. Intimacy versus isolation; career and work identity; generativity versus stagnation; the "social clock" (the culturally shared timetable for marriage, children, promotion, retirement).

Why this matters clinically. Most couple problems presented in clinics are adult developmental problems wearing a relational mask: a 32-year-old's struggle with intimacy, a 45-year-old's reckoning with a career, a 58-year-old's grief for the life they did not have.

Life structure and the "seasons" of adulthood (Levinson; vaillant's longitudinal work). Treat these as metaphors with empirical support, not as laws. The strongest empirical

finding is **continuity with change**: people change, but in patterned ways that reflect early relationships and current context.

> **CASE**
> A 45-year-old man presents with "irritability". Developmental enquiry finds two things: his father died at 45, and his eldest child is about to leave for university. He is not depressed in the clinical sense; he is renegotiating mortality and role. The intervention is narrative and relational — mourning the father he barely knew, and planning what his own next decade is for.

Week 10 — Couple development

Couples develop too. A workable map (used again in MFT 710 and 718):

Phase	Central task	Common failure
Formation	Commitment and identity as a couple	One partner never really chooses
Early marriage	Building rituals and boundaries with families of origin	In-law triangles
Young children	Dividing labour; protecting the couple bond	Co-parenting replaces partnering
Adolescent children	Re-negotiating autonomy, revisiting one's own adolescence	Escalation or abdication
Launching	Re-intimacy after children leave	The "empty nest" that was already empty
Later life	Facing illness, retirement, loss together	Caretaking replaces relating

> **PRACTICAL**
> With a peer, draw a couple timeline: five high points, five low points, one turning point each. Then ask: which developmental transition explains the lowest point? Present in 5 minutes.

Week 11 — Parenting across stages and talking to parents

Parents do not need lectures; they need **accurate, specific, hopeful information** in plain language. The skill has four parts:

1. **Normalise first.** "What you are describing is very common at this age."
2. **Give one developmental fact.** "At four, the part of the brain that manages impulse is still being built, so he cannot yet do what you are asking consistently."
3. **Give one concrete action.** "Try naming the feeling and offering two acceptable choices."

4. **Set a review.** "If this is still happening in three weeks, or if X or Y appears, come back."

What to avoid: technical vocabulary; predictions ("he will grow out of it"); blame; over-reassurance when the concern is real; and overwhelming a parent with five recommendations.

> **PRACTICAL (OBSERVED)**
> Ten-minute parent-feedback role-play using the four-part structure. Assessor rates: plain language, one fact, one action, review point, and warmth.

Week 12 — Later life

Tasks. Integrity versus despair; adaptation to retirement; bereavement; caregiving; grandparenthood; legacy and meaning.

Clinical realities. Cognitive change must be distinguished from delirium, depression and dementia; hearing loss is a major and under-treated contributor to isolation and apparent confusion. Caregiving burden falls disproportionately on women and on the "responsible" child, and it frequently reopens old sibling hierarchies.

Grief. Use the dual-process model (Figure 22, MFT 712): healthy grieving oscillates between loss-oriented and restoration-oriented activity. In many cultures, mourning is collective and prescribed; the clinician's task is to support the rites rather than to impose a private model of "working through".

> **CASE**
> Three siblings disagree about their mother's care. The eldest, a daughter, has done everything; the youngest, a son abroad, sends money and criticises. The clinical task is not to decide who is right but to make the loyalty ledger explicit: who owes what, to whom, and by what accounting? (This is contextual therapy's territory; see MFT 702 and 715.)

Week 13 — Culture, poverty and development

Development is not culture-free. Three critiques you must be able to state:

1. **The norms are Western.** Attachment classifications, individuation timetables and "school readiness" were normed on particular populations. A child who sleeps in the parental bed until five is not necessarily enmeshed; in many cultures this is ordinary.
2. **Poverty changes development.** Chronic stress, food insecurity, crowding and insecure housing affect attention, sleep, and the capacity for consistent parenting. Diagnosing "inconsistent parenting" without naming material conditions is a category error.

3. **Extended and distributed caregiving is normal.** In much of the world, children are raised by networks — grandparents, aunts, older siblings, co-wives, foster kin. Attachment theory travels better when it is asked "who does this child seek when frightened?" rather than "is the mother attuned?"

> **CRITICAL READING**
> Read one cross-cultural critique of attachment theory and one study of caregiving in a non-Western setting. Write 500 words on what you would change in your own assessment template as a result.

Week 14 — When development goes wrong

Build a differential habit. For any concern, run the four-category check (variation, delay, regression, symptom) and then ask the five systemic questions:

1. **When did it start, and what else changed then?**
2. **Who noticed first, and who disagrees?**
3. **What does the family do when it happens, and what happens next?**
4. **What would improve if it stopped — and what would get worse?**
5. **What has already been tried, and what did that teach us?**

Presentation	Developmental differential	Systemic differential
Aggression at 5	Language delay; impulsivity; trauma	Parental conflict; inconsistent limits; a new baby
Withdrawal at 12	Depression; bullying; neurodivergence	Family secrecy; a parent's illness; migration
Decline at 70	Delirium; dementia; depression; hearing loss	Bereavement; relocation; loss of role
"Not listening" at 3	Hearing; language; attention	Multiple caregivers, conflicting rules

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Week 15 — Integration

The developmental lens does three things for your clinical work: it **calibrates** your expectations, it **dates** the problem (onset relative to transitions), and it **predicts** what the family must renegotiate next. Carry three questions into every first session: What is this person's developmental task right now? What is the family's corresponding task? And what would "on track" look like in six months?

Glossary

Attachment — a behavioural system organising proximity-seeking to a caregiver under threat. **Co-regulation** — the process by which a caregiver's calm and responsiveness regulate the child's arousal. **Developmental task** — the psychosocial challenge characteristic of a life stage. **Disequilibrium** — a period of developmental reorganisation marked by increased conflict or regression. **Goodness of fit** — the match between a child's temperament and the demands of the environment. **Regression** — loss of previously acquired function; a red flag. **Scaffolding** — temporary support that allows a child to do what they cannot yet do alone. **Separation-individuation** — the process of developing autonomy while remaining connected. **Social clock** — culturally shared expectations about the timing of life events. **Theory of mind** — the capacity to attribute mental states to others. **Zone of proximal development** — the gap between what a learner can do alone and with support.

Self-assessment

Multiple choice

1. A four-year-old who stops speaking after a family bereavement is best described as: (a) normal variation (b) delay (c) regression (d) attention-seeking

2. "Goodness of fit" refers to the match between: (a) child and school (b) temperament and environment (c) parents' goals (d) IQ and achievement

3. The most common developmental concern presented to clinics is: (a) motor delay (b) expressive language delay (c) sleep (d) feeding

4. Disorganised attachment is most associated with: (a) daycare (b) frightening or frightened caregiving (c) prematurity (d) bilingualism

5. A toddler's frequent "no" primarily reflects: (a) oppositional disorder (b) emerging autonomy (c) poor parenting (d) language delay

6. Which is not a feature of adolescent brain development? (a) heightened reward sensitivity (b) peer sensitivity (c) mature impulse control (d) later prefrontal maturation

7. A "social clock" refers to: (a) sleep rhythms (b) culturally shared timing of life events (c) school timetable (d) biological ageing

8. In the dual-process model, healthy grieving involves: (a) staying on the loss track (b) avoiding reminders (c) oscillating between loss and restoration (d) suppressing emotion

9. Apparent confusion in an older adult should first prompt assessment of: (a) personality (b) delirium, depression, hearing, dementia (c) family conflict (d) medication refusal only

10. Extended-family caregiving should be assessed by asking: (a) whether the mother is attuned (b) who the child seeks when frightened (c) the parents' marital status (d) the family's income

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish delay from regression, with one clinical example of each.
2. Name three developmental tasks of adolescence and the corresponding family tasks.
3. Why is "let him cry it out" a relational intervention, not only a behavioural one?
4. Give two cultural critiques of attachment theory and one way to adapt your assessment.
5. What are the four parts of effective parent feedback?

Essay (choose one, 1,800 words)

1. "Normal development is a cultural achievement, not a biological fact." Discuss with reference to attachment and independence.
2. Using a disguised case, show how a developmental formulation changed your understanding of a presenting problem that had been framed as behavioural.
3. Critically evaluate the evidence that couple satisfaction declines after the birth of a child, and discuss the clinical implications.

Further reading

- Berk, L. E. (2022). Development Through the Lifespan (8th ed.). Pearson.
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- Siegel, D. J. (2015). Brainstorm: The Teenage Brain. TarcherPerigee.
- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- Keller, H. (2018). Cultures of Infancy (2nd ed.). Routledge.
- LeVine, R. A. (2003). Childhood Socialization. AltaMira.

Next: MFT 704 — Ethical and Legal Issues in Therapy

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	**Ethical and Legal Issues in Therapy**	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This course trains judgement, not just compliance. You will learn the codes, the law, and — more importantly — how to reason when the codes and the law point in different directions, when the family's culture conflicts with the institution's policy, and when your own values are in the room.

How to use this book

Every week presents a **standard**, a **case**, and a **decision**. Ethics is not memorised; it is practised. The weekly incidents in this book are drawn from the ordinary texture of clinical life — the secret, the subpoena, the gift, the neighbour, the unpaid fee — because those are the situations that actually end careers.

1. Course description

Family therapy multiplies ethical complexity. In individual work there are two parties (you and the client). In family work there may be six, with conflicting interests, unequal power, and secrets that each person believes belongs to them. Add a school, a child protection agency, a court, an insurance company, and a faith community, and the question "who is my client?" becomes the central professional question of the course.

The course combines **normative ethics** (codes, duties, principles), **law** (the parts a clinician must know and the parts a clinician must refer), and **professional formation** (the kind of clinician you are becoming).

2. Learning outcomes

#	Outcome	Assessed
LO1	Apply the major ethical codes and principles to relational cases	Exam, case analysis
LO2	Explain confidentiality and its limits in multiperson therapy	Exam
LO3	Conduct and document a risk assessment for suicide, self-harm, violence and abuse	Skills assessment
LO4	Manage secrets, dual relationships, gifts, fees and boundary dilemmas defensibly	Case analyses
LO5	Describe the legal framework for marriage, divorce, custody and child protection in Nigeria	Exam
LO6	Use a structured decision-making model and document the reasoning	Assignment
LO7	Recognise when to refer for legal advice, and how to do so	Viva

3. Assessment

Component	Weight	Due
Ethics incident analyses (4 × 600 words)	30%	Weeks 4, 6, 9, 11
Risk assessment observed skills (15 min)	20%	Week 10
Examination (case-based)	50%	Week 15

4. The fourteen-week plan

Week	Topic	Practical
1	Moral philosophy for clinicians: duties, consequences, virtues, care	Trolley variants in therapy
2	The codes: AAMFT, IAMFC, APA, national associations; informed consent	Consent form drafting
3	Confidentiality in couple and family therapy	Secrecy protocol design
4	Secrets I: the individual disclosure	Scripted incident
5	Secrets II: infidelity, paternity, HIV status	Complex case debate
6	Boundaries and multiple relationships	Boundary audit
7	Competence, supervision, and the duty to refer	Competence self-audit
8	Risk I: suicide and self-harm	**Observed risk assessment**
9	Risk II: intimate partner violence and child protection	Safety planning
10	Legal framework: marriage, divorce, custody, maintenance	Court-report exercise
11	Records, documentation, subpoenas, court testimony	Note-writing lab
12	Money, gifts, barter, and the business of therapy	Fee policy workshop
13	Ethics of research, assessment and diagnosis	Consent and deception cases
14	Integration: your ethical stance and decision model	Viva

Week 1 — Moral philosophy for clinicians

You do not need to become a philosopher, but you need to know which argument you are making when you say "that would be wrong".

Tradition	Central question	Therapy example
Deontology (duty)	What are my duties regardless of outcome?	Keeping confidentiality even when disclosure would be convenient
Consequentialism	What produces the best overall outcome?	Breaking confidentiality to prevent a serious harm
Virtue ethics	What would a person of good character do?	Refusing a gift that would create obligation

Tradition	Central question	Therapy example
Ethics of care	What does this relationship require of me?	Attending to the family's dependence on you
Justice / rights	Are power and resources fairly distributed?	Whose voice is missing from this room?

The four principles (Beauchamp & Childress) remain the most portable framework: **autonomy, beneficence, non-maleficence, justice**. In family work they collide constantly — respect for the father's autonomy may conflict with the mother's safety.

> **PRACTICAL**
> Take three ethical dilemmas from your own life (not clinical). For each, identify which tradition you used, and which you used to justify it afterwards.

Week 2 — The codes and informed consent

What codes do and do not do. Codes set a floor, not a ceiling. They are written to be enforceable, which means they are often silent on the hardest questions. They also vary by jurisdiction and by professional body; know which one governs you.

The core obligations across the major codes:

1. Competence and its limits
2. Informed consent (including consent for recording and for supervision)
3. Confidentiality and its limits
4. Multiple relationships
5. Sexual intimacy with clients and former clients (prohibited; time limits vary)
6. Fees and financial arrangements
7. Advertising and representation
8. Research ethics
9. Cultural competence and non-discrimination
10. Reporting obligations and legal compliance

Informed consent in family therapy is a document and a conversation. It should cover: who the client is; who is seen and in what combination; the confidentiality policy including secrets; recording and supervision; fees and cancellation; the limits of confidentiality (risk, abuse, court order); and the complaint route. Review it aloud in the first session, in the family's language, and record that you did.

> **PRACTICAL**
> Draft a one-page informed-consent document for couple therapy. Exchange with a peer and identify the three sentences most likely to be misunderstood by a family with limited literacy. Rewrite them.

Week 3 — Confidentiality in the multiperson system

There is no single confidentiality in family work; there is a **policy** you negotiate.

Three workable models.

Model	Rule	Advantages	Risks
No-secrets	Anything told individually may be shared with the couple	Simple, transparent	Individual may withhold important material
Full secrecy	Individual disclosures are kept private	Protects individual therapy	Creates a therapeutic secret that distorts the couple work
Selective (negotiated)	The therapist and client agree in advance what will be shared	Flexible and realistic	Requires skill and documentation

Most experienced couple therapists operate a **modified no-secrets policy**: they will not keep a secret that materially affects the couple's work, and they tell each partner this before the first individual session. This protects both the relationship and the therapist.

Documentation. Record the policy in the file, in the family's words if possible, and note the date it was explained. If a secret arises, record what you did and why.

> **CASE**
> During an individual session, a wife discloses that she has been having an affair for two years and has no intention of ending it or telling her husband. No one is at physical risk. Under a no-secrets policy, you tell her at the outset of the individual session that this cannot stay private if the couple work is to continue; you then help her decide how and when to disclose, with a deadline. Under a selective policy, you would negotiate what can be shared. What you must not do is carry the secret silently for months while conducting "couple therapy".

Week 4 — Secrets I: the individual disclosure

Classify before you respond. Three questions: Is anyone at risk? Does this information change the treatment I am providing? Is the client able to disclose it themselves, with support?

Type of secret	Typical response
Harmless (a surprise party)	Keep it; it is not clinically material
Clinically material, no risk (an affair; a debt; a diagnosis)	Negotiate disclosure with a deadline; do not collude
Risk to a third party (HIV status undisclosed; threats)	Assess; follow legal and public-health obligations; document

Type of secret	Typical response
Risk to a child or vulnerable adult	Report; inform; protect
Risk to self	Risk assessment; safety planning; break confidentiality as required

The clinical skill: refusing to be recruited. "I understand you are not ready to tell him. I cannot keep this and also work on your marriage with you, because I would be working with half the truth. Let us plan how you will tell him, and when."

The systemic reality. Secrets are not only information; they are structure. A secret held by one partner and the therapist creates a coalition — and the excluded partner often senses it, which damages the therapy's credibility.

Week 5 — Secrets II: infidelity, paternity, HIV status

Three cases that appear in every couple therapist's career, each with a different legal overlay.

Infidelity. Not a reportable act. The clinical tasks: stop the bleeding, contain the disclosure, assess for sexually transmitted infection risk, address the meaning of the affair (not only its fact), and decide with the couple whether disclosure to a third party (the affair partner's spouse) is required. That last question is a values question with safety implications; consult.

Paternity. A man may discover that a child he is raising is not biologically his. The therapist must not conceal information that affects a child's welfare, must not give legal advice, and must help the family decide who needs to know and in what sequence. Refer for legal advice on paternity, maintenance and custody; the emotional work is yours.

HIV and other communicable conditions. Confidentiality is strong, but public-health law may create duties where a partner is at identifiable risk and the client refuses disclosure. Know your jurisdiction's rule, consult, and document. Never disclose without advice.

> **DEBATE**
> "A therapist who keeps an affair secret for six months is colluding in the harm the affair causes." Argue for and against, then write your own policy in 200 words.

Week 6 — Boundaries and multiple relationships

Definition. A multiple relationship exists when you have another significant relationship with a client or someone closely associated with them. Not all are unethical; all require examination.

The test. (1) Could the second relationship impair your objectivity? (2) Could it exploit the power difference? (3) Would the client experience it as exploitative later? (4) Is it avoidable?

Common in small communities.

Situation	Risk	Defensible approach
Seeing a neighbour's family	Overhearing information; social contact	Discuss openly; agree how to behave at community events
Client is a fellow church member	Role confusion; spiritual authority	Clarify roles; consider referral if you hold a leadership role
A relative of your child's classmate	Dual loyalty	Disclose in supervision; transfer if the risk is unmanageable
Client asks you to be a character witness	Role change; advocacy	Decline; explain; refer to a professional who can assess
Gift-giving after a bereavement	Cultural obligation vs boundary	Accept small cultural gifts; document; discuss meaning
Trading therapy for services	Exploitation risk	Usually decline; if unavoidable, document value and rationale

Touch, self-disclosure, and home visits. Each is sometimes therapeutic and always requires a rationale you can articulate in supervision. The test is never "did it feel right?" but "whose need did it serve?"

> **PRACTICAL**
> Complete a boundary audit of your present life: community roles, family ties, business interests, social media. Identify the three situations most likely to produce a dual relationship in the next two years, and pre-decide your response.

Week 7 — Competence and referral

Competence has four dimensions: knowledge, skills, experience, and appropriateness (personal and cultural fit). You can be knowledgeable and inappropriate: a therapist with unprocessed trauma working intensively with trauma, or a young male therapist seeing a couple in a culture where his authority is contested.

The duty to refer arises when: the problem is outside your training; you lack supervision for it; your personal reactions would compromise care; or the client needs a different modality or medical input.

How to refer well. Name the reason without shaming ("this needs someone with specific training in X"); make a warm handover (a call, not just a name); offer a bridge session if needed; follow up once; and document. A referral is not abandonment.

> **SELF-AUDIT**
> List the ten presenting problems you are most likely to see. For each, rate your competence 1–5 and name the supervisor or service you would refer to. The two lowest ratings are your professional development plan for this year.

Week 8 — Risk I: suicide and self-harm

The standard. Ask directly, ask specifically, ask about means, and document. There is no evidence that asking about suicide increases risk, and substantial evidence that clinicians fail to ask.

A defensible risk assessment includes:

1. **Ideation** — frequency, intensity, duration, controllability.
2. **Plan** — specific method, place, time.
3. **Intent** — "Do you intend to act on this?"
4. **Means** — access to firearms, medication, pesticides, heights.
5. **Previous attempts** — the strongest single predictor.
6. **Protective factors** — reasons for living, responsibility to children, faith, support.
7. **Substance use and impulsivity.**
8. **Family history.**
9. **Current stressors and recent losses.**
10. **Disclosure and support network** — who can be told, who can watch.

Documentation. Record the questions you asked and the answers, your formulation of risk (low/moderate/ high), the plan, who was informed, and the review date. If you decide not to break confidentiality, record why.

In family work. A parent's suicidality, an adolescent's self-harm and a child's disclosure must be handled without turning the family session into an interrogation. Structure: see the person individually first, agree what will be shared, then facilitate the sharing.

> **PRACTICAL (OBSERVED)**
> Fifteen-minute suicide risk assessment role-play with a trained actor. You must ask all ten elements, formulate risk, and agree a safety plan. Pass/fail.

Week 9 — Risk II: intimate partner violence and child protection

Distinguish two patterns.

Situational couple violence	Coercive control
Mutual, low-level, escalates under stress	One partner systematically dominates
Fear is situational	Fear is chronic; surveillance, isolation, threats
Couple therapy may be appropriate after assessment	Couple therapy is contraindicated; risk increases
Safety planning individual	Specialist advocacy and protection

Screening. Screen each partner **separately**, at least once, using direct questions: "Has your partner ever hit, kicked, or pushed you? Has he/she ever made you afraid? Do you feel safe going home today?" Never screen in front of the partner.

Child protection. Know your jurisdiction's reporting duty. In Nigeria, the Child Rights Act (2003, adopted in many but not all states) and state child protection laws create duties and structures; the practical route is usually the state ministry responsible for social welfare, the police family unit, or a designated safeguarding lead. Document the concern, the advice received, and the action taken.

> **CAUTION**
> Conjoint sessions can escalate violence after the session. If there is any indication of coercive control, separate sessions and a safety plan precede any conjoint work. Write it down.

Week 10 — The legal framework a clinician must know

This section is **orientation, not legal advice**. Statutes and state practice vary; verify with a lawyer.

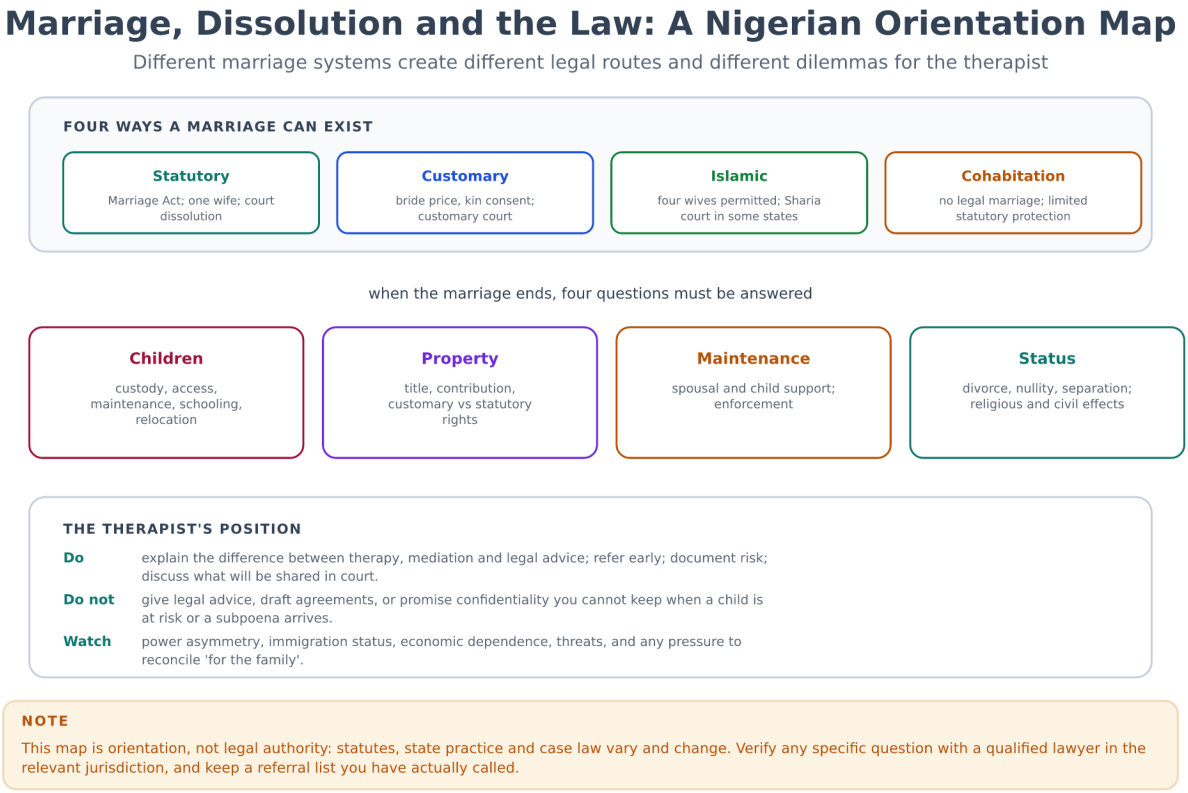


Figure 10.1 — Marriage systems and dissolution questions in Nigeria

Marriage in Nigeria exists in four overlapping forms: statutory (Marriage Act), customary, Islamic, and cohabitation without legal marriage. A family may be married under more than one system.

Dissolution. Statutory divorce requires a court petition (Matrimonial Causes Act, with the two-year rule and recognised grounds); customary and Islamic dissolutions follow their own processes and courts. The therapist's role is not to advise on grounds but to help the family clarify what they are deciding and to refer.

Children. Custody and access decisions are governed by the welfare principle — the child's best interests. Maintenance obligations exist under both statutory and customary law. The therapist may be asked for a report; a treating therapist should generally not conduct a forensic custody evaluation of their own client (role conflict).

Other relevant areas: the Child Rights Act; the Violence Against Persons (Prohibition) Act 2015 (applicable in the FCT and in states that have adopted it); mental health legislation in the states that have adopted the Mental Health Act 2021 framework; data protection obligations regarding client records.

Week 11 — Records, subpoenas and court

Write notes for three readers: your future self, a colleague taking over, and a court. Use factual, behavioural language; avoid speculation and avoid character judgements. Record: date, attendees, risk screen, interventions, plan, and who was told what.

If a subpoena or court order arrives: do not ignore it; do not respond immediately without advice; verify its validity; contact your supervisor, indemnity provider or lawyer; inform the client (unless prohibited); release only what is required, with a privilege log if advised.

Testifying. Your role is to report what you observed, not to advocate for a preferred outcome. Answer the question asked; do not volunteer; do not guess; say "I don't know" when you do not. Distinguish fact, clinical opinion and hypothesis.

> **PRACTICAL**
> Note-writing lab. Write a progress note for a session in which a risk was disclosed and a report to a school was agreed. Exchange; grade each other against the "three readers" standard.

Week 12 — Money, gifts and the business of therapy

Fees are clinical material. They carry meaning about worth, dependency, entitlement and shame. Have a written policy; state it at the start; apply it consistently; discuss non-payment rather than avoiding it.

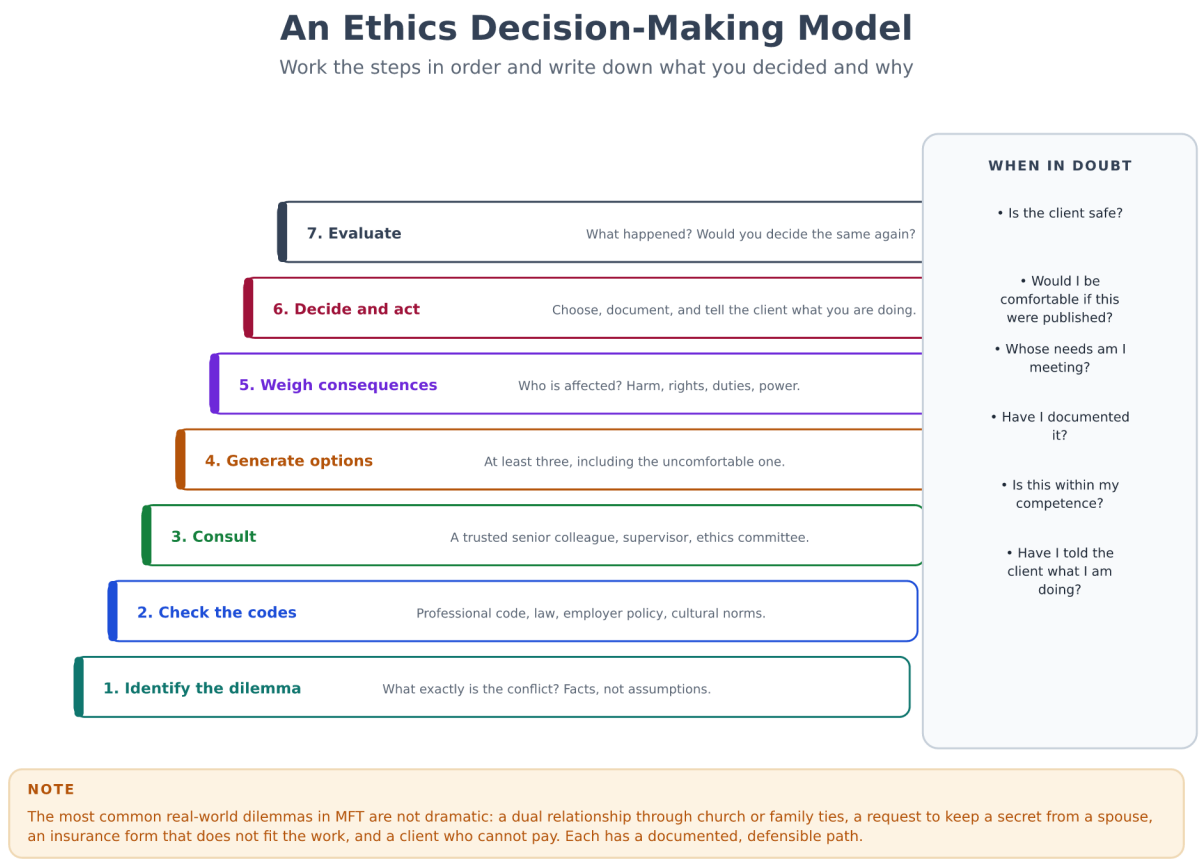
Refusing to reduce a fee sometimes ends therapy; never reducing a fee sometimes ends therapy. Both are true. Decide your sliding scale in advance and document the reason for each exception.

Gifts. Small cultural gifts may be accepted with documentation; expensive gifts should be discussed; gifts with strings should be declined. Ask, "What would it mean to you if I accepted / declined?"

Week 13 — Ethics of research, assessment and diagnosis

- **Informed consent** for research must be voluntary, capacity-appropriate and revocable; families, children and people in crisis require special care.
- **Deception** requires justification, debriefing, and ethics approval.
- **Diagnosis** carries consequences: access to care, stigma, insurance, custody. Diagnose what you can defend and describe what you cannot.
- **Assessment instruments** must be validated for the population and language; using an unvalidated instrument and acting on it is an ethical problem, not just a methodological one.

Week 14 — Integration: the decision model and your stance



NOTE

The most common real-world dilemmas in MFT are not dramatic: a dual relationship through church or family ties, a request to keep a secret from a spouse, an insurance form that does not fit the work, and a client who cannot pay. Each has a documented, defensible path.

Figure 14.1 — A seven-step decision-making model for ethical dilemmas

The seven steps. Identify the dilemma → check codes and law → consult → generate at least three options including the uncomfortable one → weigh consequences for each affected party → decide, act and document → evaluate afterwards.

The documentation habit. A defensible decision is not the same as a correct decision; it is one you can explain. Write the reasoning at the time, not after a complaint.

Your stance. Write 500 words: what you will never do; what you will always consult about; and where your personal values are most likely to distort your clinical judgement.

Glossary

Autonomy — the client's right to self-determination. **Beneficence / non-maleficence** — the duty to benefit and the duty not to harm. **Coercive control** — a pattern of domination, isolation and threat, distinct from situational violence. **Confidentiality** — the duty not to disclose information learned in a professional relationship. **Dual relationship** — a second significant relationship with a client or someone close to them. **Duty to warn / protect** — a legal obligation, in some jurisdictions, to act on a serious threat. **Informed consent** — voluntary agreement based on adequate information and capacity. **Mandated reporting** — a legal duty to report suspected abuse or neglect. **Privilege** — the client's legal right to prevent disclosure in court proceedings. **Subpoena** — a legal order requiring documents or testimony. **Welfare principle** — the rule that a child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. In couple therapy, the most defensible approach to individual secrets is: (a) keep everything private (b) declare a policy in advance and apply it (c) tell the partner immediately (d) refuse individual sessions entirely

2. Asking a client directly about suicide: (a) increases risk (b) does not increase risk and is required practice (c) is only for psychiatrists (d) should be avoided in family sessions

3. Couple therapy with a couple in which there is coercive control is: (a) recommended (b) contraindicated until safety is established (c) harmless (d) the only legal option

4. The four principles of biomedical ethics are: (a) autonomy, beneficence, non-maleficence, justice (b) consent, capacity, confidentiality, competence (c) duty, utility, virtue, care (d) respect, honesty, integrity, courage

5. A treating therapist asked to conduct a custody evaluation for their own client should: (a) accept (b) accept if paid (c) decline because of role conflict (d) accept with supervision

6. The welfare principle means decisions about children are governed by: (a) parental preference (b) the child's best interests (c) the court's convenience (d) financial capacity

7. The strongest single predictor of a suicide attempt is: (a) diagnosis (b) previous attempt (c) gender (d) family income

8. A small cultural gift after a bereavement should usually be: (a) refused (b) accepted, discussed and documented (c) accepted silently (d) reported to the police

9. Screening for intimate partner violence should be done: (a) with the couple present (b) with each partner separately (c) by questionnaire only (d) only if asked

10. A defensible ethical decision is one that: (a) everyone agrees with (b) can be explained and documented (c) produces the best outcome (d) follows the client's preference

Answer key

1 b · 2 b · 3 b · 4 a · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State a confidentiality policy for couple therapy in six sentences a family could understand.
2. List the ten elements of a defensible suicide risk assessment.
3. Distinguish situational couple violence from coercive control, and state the treatment implication.
4. What should you do when you receive a subpoena? Five steps.
5. Why is "I know it is unethical but it was the right thing to do" an inadequate defence?

Essay (choose one, 2,000 words)

1. "The most difficult ethical problems in family therapy are not conflicts between right and wrong but between two rights." Discuss with three cases.
2. A father discloses that his 14-year-old daughter is self-harming and asks you not to tell her mother. Analyse the ethical, legal and clinical dimensions and set out your plan.
3. Critically examine the claim that the professional codes are culturally biased in favour of Western individualism.

Further reading

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). OUP.
- Corey, G., Corey, M. S., & Corey, C. (2019). Issues and Ethics in the Helping Professions (10th ed.).
- Doherty, W. J. (1995). Soul Searching: Why Psychotherapy Must Promote Moral Responsibility. Basic Books.
- Wilcoxon, S. A., Remley, T. P., & Gladding, S. T. (2013). Ethical, Legal and Professional Issues in Counseling (4th ed.). Pearson.
- Child Rights Act (Nigeria), 2003; Violence Against Persons (Prohibition) Act, 2015.

Next: MFT 705 — Research Methods in Marriage and Family Therapy

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	**Research Methods in Marriage and Family Therapy**	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You will become a consumer who can read a paper in ten minutes and know what it is worth, and a producer who can design a study you can actually finish. The course is deliberately practical: every week you do something small with real or simulated data.

How to use this book

Research methods are learned by doing badly, then better. Each week has a **concept**, a **worked example**, and a **small task** you can complete in an hour. The tasks build towards your thesis proposal in MFT 770, so keep every one of them.

1. Course description

Family therapy research is harder than individual therapy research for a structural reason: **people in a family are not independent observations**. If you treat a couple as two independent people, you violate the central assumption of most statistical tests. This course teaches you to see that problem and to handle it in ways that survive examiners and journal reviewers.

The course covers: framing a good question; quantitative, qualitative and mixed designs; sampling and measurement; basic analysis; appraisal of evidence; ethics; and writing a proposal.

2. Learning outcomes

#	Outcome	Assessed
LO1	Convert a clinical hunch into a focused, researchable question	Assignment 1
LO2	Select and justify a design appropriate to the question	Exam, proposal
LO3	Evaluate the reliability and validity of commonly used family measures	Exam
LO4	Handle non-independence and unit-of-analysis problems	Exam, proposal
LO5	Interpret basic descriptive and inferential statistics and effect sizes	Exam, data exercise
LO6	Appraise quantitative, qualitative and mixed-methods studies	Appraisal assignment
LO7	Write an ethical, feasible research proposal	Proposal

3. Assessment

Component	Weight	Due
Question and design exercise (1,000 words)	15%	Week 4
Systematic appraisal of two studies (1,500 words)	25%	Week 9

Component	Weight	Due
Data analysis exercise (set problems)	20%	Week 12
Research proposal (2,500 words)	40%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	What research is for; evidence-based practice and its critics	Find one paper you will use in the thesis
2	From hunch to question: PICO/SPIDER; feasibility	Draft three questions
3	Designs overview: quantitative, qualitative, mixed	Map your question to a design
4	Sampling: who is in, who is out, who never comes	Critique a convenience sample
5	Measurement: reliability, validity, responsiveness	Appraise one couple measure
6	The independence problem: unit of analysis in family data	Worked analysis of couple data
7	Quantitative analysis I: descriptive and comparison	Run descriptives in software
8	Quantitative analysis II: association, prediction, effect size	Interpret a regression table
9	Qualitative methods and rigour	Code a transcript extract
10	Mixed methods; participatory approaches	Design a mixed study
11	Systematic review and critical appraisal	Appraise one meta-analysis
12	Data management, security, documentation	Write a data plan
13	Research ethics and with vulnerable families	Complete an ethics application
14	Writing it up; publication and impact	Proposal presentations

Week 1 — What research is for

Three legitimate purposes. Description (what is happening), explanation (why, and under what conditions), and improvement (does this intervention help, for whom). A study that serves none of these is a filing cabinet exercise.

Evidence-based practice is the integration of best available research, clinical expertise, and client values and context. It is not "follow the manual". Two critiques worth knowing:

1. **Efficacy versus effectiveness.** Trials conducted with selected volunteers under ideal conditions may not generalize to your clinic, where families present with debts, migration problems, and court dates.
2. **The evidence hierarchy is question-dependent.** A meta-analysis of trials answers "does it work on average?"; a qualitative study answers "how does it work, and for whom does it fail?". Family therapy needs both.

> **TASK**
> Find one paper relevant to your intended thesis topic. Write its citation and one sentence stating what it establishes and one sentence stating what it leaves open.

Week 2 — From hunch to question

A weak question: "Does family therapy work?"

A workable question: "Among adolescents referred to a Lagos community clinic for school refusal, does a six-session family-based intervention reduce days absent from school at 12 weeks, compared with usual care?"

Frameworks. For quantitative work use **PICO** — Population, Intervention, Comparison, Outcome. For qualitative work use **SPIDER** — Sample, Phenomenon of interest, Design, Evaluation, Research type.

Feasibility screen. Before you fall in love with a design, ask: Do I have access? How many participants can I realistically recruit in six months? Who will approve this? What is the cost? Is the timeline compatible with the semester? Most failed theses are failed at this step, not at analysis.

Common traps.

Trap	Example	Fix
Too broad	"Family functioning in Nigeria"	Specify population, setting, outcome
Unanswerable with your data	"Does family therapy cause lasting change?"	Narrow to what a 12-week design can show
Two questions in one	"Does it work and why?"	One study, one primary question
Question-shaped advocacy	"Why is family therapy better?"	Make it genuinely open ("how does it compare?")

> **TASK**
> Draft three candidate questions. For each, write the PICO or SPIDER elements, the sample you can access, and the main threat to validity.

Week 3 — Designs overview

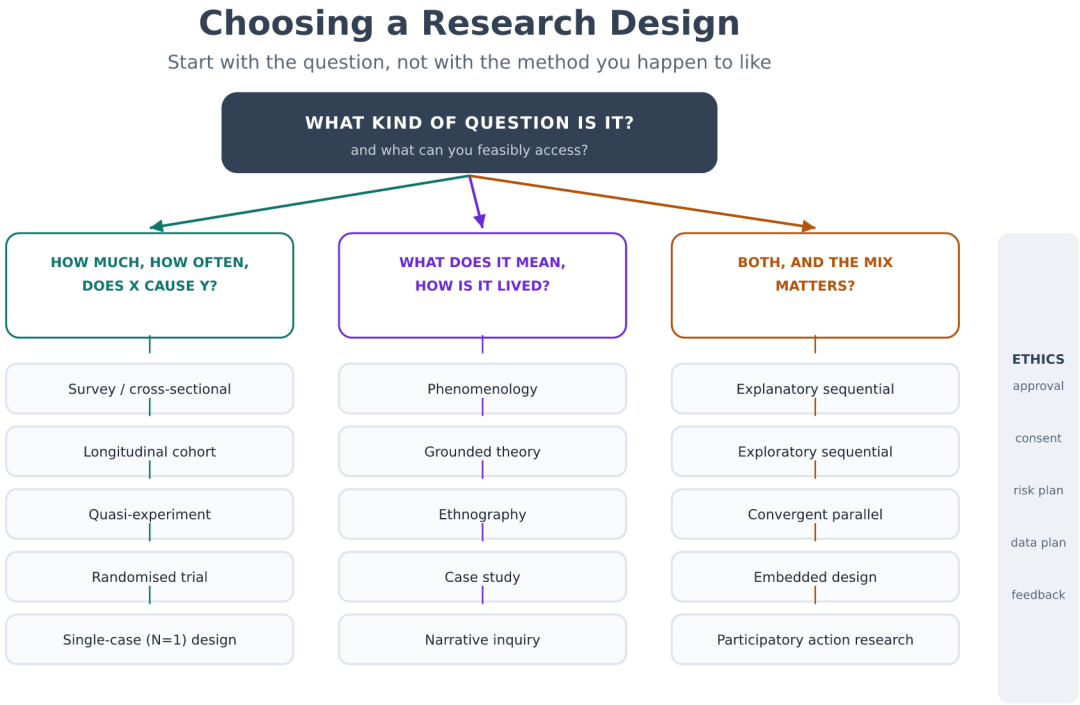


Figure 3.1 — Choosing a research design: start with the question

Quantitative.

Design	Answers	Strengths	Threats
Cross-sectional survey	How common? What is associated?	Cheap, fast	Cannot establish temporality
Longitudinal cohort	What predicts change?	Temporal ordering	Attrition
Quasi-experiment	Does the intervention work without randomisation?	Feasible in clinics	Selection bias
Randomised controlled trial	Does it cause the effect?	Strongest causal inference	Cost, ethics, generalisability
Single-case (N=1) design	Does it work for this family?	Clinically realistic	Limited generalisation

Qualitative.

Approach	Focus	Sample size
Phenomenology	Lived experience of a phenomenon	6–12
Grounded theory	Building a theory from data	15–30, to saturation
Ethnography	Culture and practice in a setting	Prolonged fieldwork
Case study	Depth in one bounded system	1–5 cases

Approach	Focus	Sample size
Narrative inquiry	How people story their lives	3–10
IPA	How individuals make sense of experience	3–6

Mixed methods — see Week 10.

> **TASK**
> Take your strongest question from Week 2 and map it onto two designs. State what each would let you claim, and what it would not.

Week 4 — Sampling

The uncomfortable fact. Clinical samples are almost never representative. Families who attend are different from families who are referred and do not attend, and those are different again from families who are never referred. Your findings describe attenders, not families.

Probability sampling (random, stratified, cluster) permits generalisation. **Non-probability sampling** (convenience, purposive, snowball, quota) does not — and this is fine, if you say so.

In family research specifically:

- **Who is the participant?** Mother, father, child, whole family, or the therapist's rating of the family?
- **Who refuses?** Fathers are the most common absence in family research and in family therapy. A study that reports "family functioning" from mothers' reports is a study of mothers.
- **Sample size.** For quantitative work, do a power calculation before you collect (software: G*Power, or the pwr package in R). For qualitative work, plan for saturation and state how you will judge it.

> **TASK**
> Critique the sample in any published Nigerian family study: who was included, who was excluded, and what does the exclusion do to the conclusions?

Week 5 — Measurement

A measure must be **reliable** (consistent), **valid** (measuring what it claims), and **responsive** (sensitive to change).



Figure 5.1 — Four validity targets and three reliability methods

The four validity targets.

Validity	The question
Internal	Did the intervention, and not something else, produce the change?
External	Can these findings travel to other families, settings and cultures?
Construct	Are you measuring "alliance", or "wanting to please the therapist"?
Statistical	Is the effect large enough, and the test powerful enough, to trust?

Instruments you should know (and check for local validation):

Instrument	Measures	Notes
Dyadic Adjustment Scale (DAS) / DAS-4	Relationship satisfaction	Widely used; short forms available
Revised Dyadic Adjustment Scale (RDAS)	Satisfaction	14 items
FACES-IV	Cohesion and flexibility	Psychometrics debated; use cautiously
Family Assessment Device (FAD)	Family functioning (McMaster)	60 items; subscales
Working Alliance Inventory (WAI-SR)	Therapeutic alliance	Adapt for family work carefully
PHQ-9 / GAD-7	Depression / anxiety	Free, brief, widely validated

Instrument	Measures	Notes
SDQ	Child emotional and behavioural functioning	Parent, teacher, self versions

The cultural validity question. Was this instrument developed and validated with families like the ones you see? If not, report that limitation and consider a local adaptation study (a perfectly good thesis).

> **TASK**
> Take one couple measure. Find two published validations, one in an African sample if it exists. Report: internal consistency, factor structure, and any evidence of measurement invariance.

Week 6 — The independence problem

This is the technical heart of family research.

The problem. In a study of 100 couples, you do not have 100 independent observations; you have 50 couples, and partners' scores are correlated. Standard tests (t-test, ANOVA, OLS regression) assume independence. Violating it inflates Type I error: you will find effects that are not there.

Three solutions.

1. **Analyse at the dyad level.** Compute a couple score (mean, difference, or both) and analyse that. Simple, loses information about within-couple differences.
2. **Treat the dyad as the unit and model non-independence explicitly.** Multilevel models (HLM) with individuals nested in couples nested in therapists. This is the modern standard.
3. **Dyadic models.** The **Actor–Partner Interdependence Model (APIM)** separates actor effects (my predictor on my outcome) from partner effects (my predictor on my partner's outcome). This is often the clinically interesting question.

Worked example. Suppose you measure criticism (predictor) and satisfaction (outcome) in 60 couples. An OLS regression treating all 120 people as independent finds criticism predicts satisfaction, $p = .001$. An APIM re-analysis finds: actor effect significant; partner effect smaller but present; and the standard errors widen. The clinical interpretation changes: criticism damages the person who gives it as well as the person who receives it.

> **TASK**
> Simulate it: in a spreadsheet, create 40 couples with correlated scores. Run a t-test treating all 80 as independent, then compute the couple means and run the test again. Note the difference in p.

Week 7 — Quantitative analysis I

Descriptive statistics. Mean and standard deviation for symmetric data; median and interquartile range for skewed data (income, session counts). Always report n, missingness, and the range.

Comparing groups. t-test (two groups), ANOVA (three or more), chi-square (categorical). Check assumptions: independence, normality (roughly), equal variance.

Clinical significance versus statistical significance. A drop of 2 points on a satisfaction scale can be statistically significant with n = 400 and clinically meaningless. Report **effect sizes** and, where available, the **minimal clinically important difference**.

Common effect sizes.

Statistic	Meaning	Rough benchmark
Cohen's d	Standardised mean difference	0.2 small, 0.5 moderate, 0.8 large
Pearson's r	Association	0.1 small, 0.3 moderate, 0.5 large
Odds ratio	Relative odds	1.5 modest, 2+ substantial
Number needed to treat	How many treated for one extra success	Lower is better

> **TASK**
> Interpret this sentence for a non-researcher in two sentences: "The intervention group showed a greater reduction in distress than controls (d = 0.55, 95% CI [0.21, 0.89], p = .002)."

Week 8 — Quantitative analysis II

Correlation is not causation, and neither is regression. Regression tells you how much of the variance in an outcome is associated with predictors, adjusted for other predictors. It does not tell you the direction, unless the design does.

Reading a regression table. Look at: which predictors are significant; the sign and size of the coefficient (in meaningful units); the confidence intervals; R^2 (variance explained); and whether the model diagnostics were reported.

Moderation and mediation. Moderation: for whom, or under what conditions, does the effect hold? (e.g. does the intervention work better for younger adolescents?) Mediation: through what mechanism? (e.g. does improved communication explain the reduction in distress?). Mediation requires strong assumptions and, ideally, longitudinal data.

> **TASK**
> Take a published regression table from a family study. Write four sentences a clinician could use, including one caveat.

Week 9 — Qualitative methods and rigour

When qualitative methods are the right choice: when you need to know how something is experienced, why a service is not used, what a concept means locally, or how a therapy unfolds. They are not a fallback for people who dislike numbers.

Rigour criteria (Lincoln & Guba; Braun & Clarke for reflexive TA):

Criterion	Practical meaning
Credibility	Do the findings ring true to participants? (member checking, triangulation)
Transferability	Enough context for a reader to judge relevance elsewhere (thick description)
Dependability	An audit trail of decisions
Confirmability	Findings grounded in data, not in the researcher's preferences
Reflexivity	Your own position examined, not hidden

Coding, briefly. Familiarise → code → generate themes → review → define → write. Coding can be inductive (from the data) or deductive (from a framework). Reflexive thematic analysis explicitly treats themes as constructed by the analyst, not excavated from the data.

Sample extracts. Suppose a father says: "They asked me to come, but nobody asked me what I thought was wrong. I sat there like a visitor in my own house." Codes: excluded, not consulted, home as foreign. Candidate theme: "A visitor in my own house": the disengagement of fathers in family therapy. Note how the theme uses the participant's own language.

> **TASK**
> Code the extract above independently, then compare with a peer. Where you disagree, ask whether you are coding the same unit.

Week 10 — Mixed methods and participatory approaches

Why mix. Numbers answer "how much"; words answer "how" and "why". Three designs:

1. **Explanatory sequential** — quantitative first, then qualitative to explain the result.
2. **Exploratory sequential** — qualitative first (to build a culturally valid measure or model), then quantitative.
3. **Convergent parallel** — both at once, integrated in interpretation.

Integration is the hard part. A study that reports a survey and some interviews side by side, without integrating the findings, is two studies in one paper. Use a **joint display** table: finding from strand A

| finding from strand B | convergence / divergence | interpretation.

Participatory action research is particularly well suited to community and family work: families help define the question, collect data, and interpret findings. It strengthens validity and ethics at once.

> **TASK**
> Design a mixed-methods study to understand why families drop out after the first session. Specify the sequence, the integration point, and one joint display row.

Week 11 — Systematic review and critical appraisal

Levels of evidence (roughly, for treatment questions): systematic reviews and meta-analyses > RCTs > quasi-experiments > cohort > case-control > cross-sectional > case series > expert opinion. For questions of meaning, the hierarchy inverts.

Appraising a meta-analysis. Ask: Was the search systematic and pre-registered? Were studies comparable enough to pool? Was heterogeneity (I^2) reported and explained? Was publication bias assessed (funnel plot, Egger's test)? Do the authors' conclusions follow from the data?

A ten-minute paper reading routine.

1. Abstract: design, n, outcome, conclusion.
2. Participants: who, where, how recruited.
3. Comparison: what was the control?
4. Measure: does it measure something you care about?
5. Effect: size and confidence interval, not just p.
6. Limitations: what the authors admit and what they do not.
7. Relevance: your population, your setting, your resources.

> **TASK**
> Appraise one meta-analysis of couple or family therapy. Write 500 words: what it establishes, what it cannot establish, and one clinical implication.

Week 12 — Data management

Principles. Collect the minimum you need; store it encrypted; separate identifiers from data; define retention and destruction; document every decision.

Practical protocol for a master's thesis.

Element	Rule
Identifiers	Separate key file, encrypted, access limited to you and supervisor
Audio/video	Encrypted drive, deletion date agreed with participants

Element	Rule
Transcription	Pseudonymised at the point of transcription, not after
Version control	Date every dataset; never overwrite raw data
Analysis log	One page per week: what you did, what you decided, why
Backup	Two locations, one offline

> **TASK**
> Write a one-page data management plan for your proposed study.

Week 13 — Research ethics

Core requirements. Institutional ethics approval before recruitment; informed consent appropriate to age and capacity; voluntariness (especially for your own clients); protection of confidentiality; a plan for distress and incidental findings; fair selection of participants; and a dissemination plan that returns something to the community.

Specific issues in family research.

- **Consent within families:** if a parent consents, does the adolescent have a genuine choice? Use separate assent and consent processes.
- **Coercion by proxy:** recruiting your own clients creates pressure. Use a third party for recruitment.
- **Group confidentiality:** you cannot guarantee that family members will not repeat what is said.
- **Power and payment:** paying participants can be appropriate compensation or an undue inducement; be transparent about the reasoning.

> **TASK**
> Complete a mock ethics application for your proposal, including consent forms for an adult and for a 14-year-old.

Week 14 — Writing it up

Structure (IMRaD). Introduction (why this question matters; what is known; the gap) → Method (what you did, in enough detail to replicate) → Results (what you found, without interpretation) → Discussion (what it means, limitations, implications) → Conclusion.

Writing rules that examiners reward. One idea per paragraph; active voice where possible; report numbers with units and confidence intervals; state limitations honestly and specifically; do not overclaim in the abstract.

Publication. Target a journal that publishes practitioner research; follow the author guidelines exactly; expect rejection once and revise.

Glossary

Actor–partner interdependence model (APIM) — a dyadic model separating one's own and one's partner's effects. **Attrition** — loss of participants over time. **Confidence interval** — the range within which the true value is likely to fall. **Construct validity** — the degree to which a measure captures the intended concept. **Effect size** — the magnitude of an effect, independent of sample size. **Heterogeneity (I^2)** — the proportion of variation across studies due to real differences rather than chance. **Independence assumption** — the statistical requirement that observations are not correlated. **Measurement invariance** — whether an instrument measures the same construct across groups. **Power** — the probability of detecting an effect that exists. **Saturation** — the point at which new data yield no new themes. **Triangulation** — using multiple methods, sources or analysts to strengthen inference.

Self-assessment

Multiple choice

1. Treating both partners in 50 couples as 100 independent observations primarily threatens: (a) external validity (b) statistical conclusion validity (c) construct validity (d) reliability

2. The APIM distinguishes: (a) pretest and posttest (b) actor and partner effects (c) randomisation and control (d) qualitative and quantitative

3. A qualitative study of lived experience with 8 participants is most likely: (a) grounded theory (b) phenomenology (c) ethnography (d) RCT

4. Cronbach's alpha estimates: (a) test–retest reliability (b) inter-rater reliability (c) internal consistency (d) construct validity

5. The strongest design for causal inference is: (a) cross-sectional survey (b) quasi-experiment (c) randomised controlled trial (d) case series

6. "For whom and under what conditions does the effect hold?" refers to: (a) mediation (b) moderation (c) randomisation (d) sampling

7. Which is not a qualitative rigour criterion? (a) credibility (b) transferability (c) generalisability (d) reflexivity

8. A 95% confidence interval that includes zero suggests: (a) a large effect (b) a non-significant effect at the .05 level (c) a reliable measure (d) a sampling error in the measure

9. The most common absence in family research samples is: (a) mothers (b) fathers (c) adolescents (d) grandparents

10. Reflexivity in qualitative research means: (a) the researcher examines their own position and effect (b) participants review findings (c) the study is replicated (d) data are triangulated

Answer key

1 b · 2 b · 3 b · 4 c · 5 c · 6 b · 7 c · 8 b · 9 b · 10 a

Short answer

1. Convert "does family therapy help adolescents?" into a PICO question.
2. Explain the unit-of-analysis problem in one paragraph a clinician would understand.
3. Name three threats to internal validity in a pre-post clinic study.
4. What is measurement invariance and why does it matter in cross-cultural family research?
5. List five elements of a data management plan.

Essay (choose one, 2,000 words)

1. "The randomised controlled trial is the wrong gold standard for family therapy." Discuss.
2. Design a mixed-methods study of therapy drop-out in a Nigerian family clinic, justifying every methodological choice.
3. Critically appraise the evidence that common factors explain most psychotherapy outcome.

Further reading

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- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288–307.
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Next: MFT 706 — Clinical Practice and Techniques

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	**Clinical Practice and Techniques**	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the skills course. You will leave able to structure a session, ask six kinds of question deliberately, conduct an enactment, reframe without dismissing, assign a task that gets done, and end therapy well. Most of the hours are practice, not lecture.

How to use this book

Technique is a set of **deliberate choices**, not a personality. Each week introduces one family of skills, gives you the language, shows a transcript, and then requires you to perform it. Read the transcript aloud before the lab: your mouth needs to know the sentences before your mind can choose them under pressure.

1. Course description

The gap between knowing about family therapy and doing it is bridged by rehearsal. This course covers the micro-skills (joining, questioning, reflecting, summarising), the meso-skills (structuring a session, managing conflict, conducting an enactment), and the macro-skills (planning treatment, assigning tasks, handling impasse, terminating).

It is assessed almost entirely on **performance**, because a student who can describe an enactment and cannot conduct one is not yet a clinician.

2. Learning outcomes

#	Outcome	Assessed
LO1	Structure a session with a clear opening, middle and close	Observed session
LO2	Use six categories of question deliberately and appropriately	Observed session, exam
LO3	Conduct an enactment and manage its escalation	Observed session
LO4	Reframe, normalise and positively connote without dismissing distress	Skills test
LO5	Design and review between-session tasks	Task portfolio
LO6	Manage conflict, silence and resistance in the room	Observed session
LO7	Plan and conduct a planned ending	Viva, log

3. Assessment

Component	Weight	Due
Skills triad log (weekly)	20%	Fortnightly
Observed session 1 (20 min, mid-semester)	20%	Week 8
Task portfolio (5 designed tasks with rationale)	20%	Week 12

Component	Weight	Due
Observed session 2 (40 min, with feedback)	40%	Week 16

4. The sixteen-week plan

Week	Skill family	Practical
1	The architecture of a session	Structure a 50-minute session
2	Joining and the first five minutes	Opening rounds, three styles
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Questioning I: linear, circular, strategic	Circular questioning drill
5	Questioning II: reflexive, future-oriented, scaling	Solution-focused drill
6	Enactment: making the family talk to each other	Enactment lab
7	Reframing, normalising, positive connotation	Reframe tennis
8	**Observed session 1**	Assessment and feedback
9	Working with conflict and escalation	De-escalation drill
10	Silence, tears, and the therapist's discomfort	Tolerance-of-silence lab
11	Structuring: who sits where, who speaks when, subgroups	Subgroup session design
12	Between-session tasks and homework	Task portfolio workshop
13	Impasses, non-attendance, and drop-out	Difficult-session rehearsal
14	Working with children and adolescents in family sessions	Live supervision
15	Endings: planned, unplanned, and therapeutic	Termination role-play
16	**Observed session 2**	Full assessment

Week 1 — The architecture of a session

A 50-minute family session has five movements. Beginners skip the first and the last, which is why their sessions feel formless.

Movement	Minutes	Purpose
1. Opening / re-joining	3–5	Reconnect, state the frame, ask what has changed
2. Agenda	2–3	Whose concern is most urgent this week?
3. Work	25–30	The core intervention: enactment, questioning, restructuring
4. Consolidation	5–7	Summarise, check, agree the task
5. Close	3–5	Confirm next appointment, name one thing to watch

The "what has changed?" opener. Never start where you left off. Start with the family's own observation: "Before we begin — what has been different this week, even slightly?" This trains the family to notice change and gives you data about what is already working.

> **PRACTICAL**
> Write the five movements on a card. Use it in every triad this semester. By Week 8 you should not need it.

Week 2 — Joining

Three joining styles (choose deliberately):

1. **Tracking** — follow the family's language and pace. "Tell me about 'stubborn' — what does it look like in your house?"
2. **Accommodation** — adopt the family's structure for the moment. With a hierarchical family, address the father first; with a chaotic family, bring structure immediately.
3. **Personal joining** — offer a small, true piece of yourself. "I grew up in a house where nobody raised their voice either. It took me years to learn to argue."

What joining is not. It is not agreement, flattery, or becoming the family's friend. You can join and disagree: "I like the way you two can laugh at this. I am going to say something you may not like."

Three failures of joining.

Failure	Symptom	Repair
Over-joining	You are now part of the family's pattern	Name it: "I think I just took your side. Let me step back."
Under-joining	Family answers in monosyllables	Slow down; ask about something concrete and safe
Premature confrontation	Family defensive from minute five	Return to their account for ten minutes before challenging

> **PRACTICAL**
> Three rounds of five-minute openings, each with a different joining style. The "family" rates: did I feel understood? Did I feel patronised? Did I want to continue?

Week 3 — Listening

Paraphrase — return the content. **Reflect** — return the feeling. **Summarise** — return the pattern across people and time.

Client: "He comes home, eats, and goes to the room. I sit in the parlour with the children and I feel like a widow."

- Paraphrase: "So most evenings he is in the room and you are with the children."
- Reflect: "You feel alone in your own marriage."
- Summarise (later, across people): "So there are two versions of the evening: yours is that he leaves; his is that he arrives already exhausted and afraid of saying the wrong thing. Both of you are alone in the same house."

The 2:1 rule for beginners. Two reflections for every question. It slows the interview, produces richer material, and prevents the interrogation pattern families experience in first sessions.

> **PRACTICAL**
> Ten-minute interview in which you may not ask a single question. Only paraphrase, reflect and summarise. Note how uncomfortable it is, and what the client does when you stop asking.

Week 4 — Questioning I

Six categories, each with a purpose. Learn the label so you can choose under pressure.

Type	Purpose	Example
Linear	Gather facts	"When did the arguing start?"
Circular	Reveal difference and relationship	"Who notices first when your mother goes quiet?"
Strategic	Introduce a direction	"What would happen if you stopped reminding him?"
Reflexive	Invite a new perspective	"If your daughter could hear this conversation, what would she say you both believe?"
Future-oriented	Open possibility	"Suppose this is better in six months — what would be different?"
Scaling	Make change measurable	"On 0–10, how close are you as a couple? What would one point look like?"

The circular question is the field's signature skill. Build it from three elements: a difference (who/what/how much), a relationship (between whom), and a comparison (more than, less than, compared with).

- Weak: "Is your husband supportive?"
- Better: "Who in the family would say your husband is most supportive — and who would disagree?"
- Best: "If your mother were watching the two of you last night, what would she say was different between how he spoke to you and how he speaks to your sister?"

> **PRACTICAL**
> Circular questioning drill: 10 minutes, all questions must be circular. Observer counts and classifies. Target: 80% genuinely circular.

Week 5 — Questioning II

Reflexive questions invite the family to observe itself. They are the safest way to raise something difficult, because the family supplies the content:

- "What do you think your son has learned about marriage from watching the two of you?"
- "If I asked your daughter who is in charge in this house, what would she say?"
- "What is the most useful thing that could happen in this session — and the most uncomfortable?"

Future questions create hope without denying the present:

- "Suppose we meet in a year and this is behind you. What would you want to be able to say you did?"
- "What would your father have wanted for this family?"

The question to avoid: "Why?" in relation to behaviour. "Why did you shout at her?" invites defence or invention. "What happened just before you shouted?" invites sequence.



Figure 5.1 — Solution-focused moves: strengths, miracle, scaling, exceptions, task

PRACTICAL

Convert ten "why" questions into "what/how/who" questions. Compare the answers you get in role-play.

Week 6 — Enactment

Definition. Enactment is the deliberate direction of family members to speak **to each other** in the session, while the therapist moves to the edge of the action.

Why it matters. Most families come to therapy to talk about each other to a third party. Enactment converts description into live interaction, where you can observe and shape it. It is the single most powerful structural technique.

The four-step protocol.

1. **Block the detour:** "Don't tell me — tell her."
2. **Specify the content:** "Tell her what you needed on Saturday, in one sentence."
3. **Direct the process:** "Look at him, not at me." "Say it again, slower."
4. **Intensify or interrupt:** "You just smiled. Say it again with the smile." Or: "Stop. What just happened?"

Transcript (structural enactment with a couple).

THERAPIST:

You have both told me about Saturday. Now I want you to tell each other. Funmi, tell Femi what you needed on Saturday — one sentence, and look at him. **Funmi:** (to therapist) I needed him to — **Therapist:** To him. **Funmi:** I needed you to call and say you would be late. **Femi:** I did call — **Therapist:** Femi, answer her, not me. **Femi:** (pause) I did call. At four. You didn't pick up. **Funmi:** I was with your mother. **Therapist:** Stop there. Funmi — what happened inside you when he said that?

Hazards. Enactment can escalate. Have a stop signal, keep your voice calm and slow, and interrupt before contempt. With coercive control, do not enact conflict; safety first (MFT 704).

PRACTICAL

Enactment lab. Triads with a scripted couple. The therapist may only use the four steps. Observer records: did the therapist stay out of the content? Did the family speak to each other for more than 90 seconds?

Week 7 — Reframing, normalising, positive connotation

Reframing changes the frame, not the facts. It is the most-used intervention in the field and the most easily abused.

Original frame	Reframe	Risk
"He is controlling"	"He is frightened of losing you and tries to hold on"	Can excuse control
"She is manipulative"	"She has learned that tears are the only way to be heard"	Can remove accountability
"You are lazy"	"You are exhausted and have stopped asking for help"	Can be heard as excusing
"Our son is defiant"	"Your son is trying to protect his mother"	Can burden the child further

Three tests before you reframe.

1. **Is it true enough?** A reframe that the family rejects as false damages alliance.
2. **Does it open or close action?** "He is frightened" opens action; "he is a monster" closes it.
3. **Does it remove accountability?** Reframes should increase the family's sense of agency, not reduce it.

Normalising — placing behaviour in a developmental or statistical context: "Arguing about money is the most common argument in the first five years of marriage. It is not a sign that you are failing."

Positive connotation (Milan) — describing all behaviour as serving the family system: "Your arguments may be the family's way of ensuring that nothing important is decided without both of you being fully present." Use sparingly; it can sound like sarcasm.

> **PRACTICAL**
> "Reframe tennis": one student states a complaint, the partner must return three successive reframes, and the first student says which one she would accept and why.

Week 8 — Observed session 1

Twenty minutes with a role-played family. Assessed against five criteria: structure (does the session have a shape?), joining (does each person feel heard?), questioning (at least three circular), one enactment, and a summary with a task.

Feedback is given live by the observing group using the structured sheet, then by the lecturer.

Week 9 — Working with conflict and escalation

The therapist's job during conflict is regulation, not resolution. Five moves:

1. **Slow the pace.** "Stop. Say that again, more slowly."
2. **Name the process.** "I notice that every time she raises her voice, you leave the room. Can we stay with it for two minutes?"

3. **Structural interruption.** "I am going to ask you both to be quiet for one minute and let me speak to your son."
4. **Separate content from process.** "The topic is money. The pattern is that you both start explaining before the other finishes."
5. **Time-out with a return.** "Let's take two minutes. We are not finished."

Escalation signs to interrupt early: raised volume, contempt (eye-rolling, mockery), one person standing, threats, children becoming still or leaving, your own heart rate rising.

> **DE-ESCALATION DRILL**
> Pairs escalate a scripted argument on cue. The therapist must interrupt before the third escalation. Repeat until the interruption happens at the right moment, not the safe one.

Week 10 — Silence, tears, and the therapist's discomfort

Silence is a technique. There are four kinds:

Silence	Meaning	Response
Processing	Someone is thinking	Wait. Count to fifteen.
Emotional	Feeling has arrived	Stay; soften your face; do not rescue
Resistance	Something is not being said	Name it gently: "Something just shifted. What happened?"
Your own	You are lost	Say so: "Let me think for a moment about what I am hearing."

Tears. Do not hand a tissue immediately; it tells the person to stop. Wait three seconds, then offer. Ask: "Is this a sadness that has been waiting a long time?"

Your discomfort is data. If you want to fill the silence, ask what you are avoiding. If you feel irritated with one family member, ask what they represent to you. This is the beginning of using the self as an instrument (developed in MFT 720 and 722).

Week 11 — Structuring the room and the system

Physical structure. Seat families so that coalition patterns are visible and can be interrupted. Put the parents together and the children together; sit between people who never speak; never sit in a line that recreates the hierarchy you are trying to change.

Who attends. Deliberately choose the combination: whole family, parents only, siblings, one parent and one child, individual. State the rationale: "Next week I would like to see the two of you alone, so you can talk as a couple without the children carrying it."

Subgroup sessions. A powerful and under-used technique: see the parents alone to plan a united message, then bring the children in and have the parents deliver it together. The intervention is the difference between the two sessions.

> **PRACTICAL**
> Design a three-session sequence using different attendance combinations for the Adeyemi family (MFT 701). Justify each combination.

Week 12 — Between-session tasks

A task is a test of the system, not homework. It should be small, specific, behavioural, and designed to produce information.

The four-part task formula.

1. **What exactly** ("For the next seven days, at dinner…")
2. **Who does what** ("…each of you names one thing that went well in your day…")
3. **What to notice** ("…and notice what happens to the atmosphere by Thursday.")
4. **What to bring back** ("…we will look at what you noticed next week.")

Five useful task types.

Type	Purpose	Example
Observation	Gather data	"Notice and write down every time the argument stops before it starts."
Ritual	Interrupt a sequence	"One night this week, the parents go out for one hour. The children are told in advance."
Behavioural exchange	Build positives	"Three small caring acts each per day, unannounced."
Communication	Practise a skill	"Two 15-minute speaker–listener conversations, timer on."
Paradoxical (rare, supervised)	Change the symptom's function	"On Tuesday, have the argument on purpose for ten minutes at 8 p.m."

Why tasks fail. Too big, too vague, agreed under pressure in the last two minutes of the session, or the family does not believe the therapist believes they will do it. Write the task down and give them the paper.

> **TASK PORTFOLIO (20%)**
> Five tasks, each with: the presenting problem, the task, the rationale, the predicted failure mode, and the review question you would ask.

Week 13 — Impasses, non-attendance and drop-out

When a session goes flat. Three likely causes: the family is protecting a secret; the therapist has joined too closely to one side; or the work has reached material the family did not consent to. Name the process: "I notice we have been very polite for twenty minutes. What are we all avoiding?"

When someone does not attend. Never pretend it did not happen. "Your father chose not to come today. Let's think about what that means." Explore whether attendance is a form of communication.

When the family drops out. Follow up once, by phone, briefly and without pressure; record the reason if given; and use it as clinical data. In family therapy, drop-out is the most common negative outcome, and it is usually visible early — the first two sessions predict the rest.

Impasses with the therapist. If you are bored, angry, or over-involved, that is information about the system. Take it to supervision before you act on it.

Week 14 — Working with children and adolescents in family sessions

Practical adjustments.

- Shorter sessions for younger children (30–40 minutes), or split sessions.
- Explain the frame to the child directly, not only to the parents: "In this room we talk, and sometimes we draw."
- Give the child a legitimate role: "You are the expert on what happens at school."
- Protect the child from being made the messenger: "That is a question for your mother to answer, not you."
- Never ask a child to choose sides in front of the parents.

The adolescent rule. Speak to the adolescent as the primary person for at least five minutes. Ask their version first. Parents' accounts of adolescents are systematically more negative than adolescents' own reports and than teachers' reports.

> **LIVE SUPERVISION**
> One student conducts a family session with a trained role-play family while the group observes; the supervisor may call in with one suggestion. Debrief with the family's experience.

Week 15 — Endings

Planned ending. Agree it in advance; review what changed and what did not; name the risk of relapse; write a one-page relapse plan with the family; leave the door open.

The four questions of a good ending.

1. What is different now, in your own words?
2. What did each of you do that made the difference?
3. What is most likely to make it come back?
4. What will you do when you notice the first sign?

Unplanned endings. Client-initiated endings are common; acknowledge and, where possible, send one closing message. Therapist-initiated endings (illness, placement change, moving) require a proper handover session — an abrupt ending can replicate an abandonment for the family.

Do not adopt. A warm ending can slip into a friendship. Say what the relationship was: "I have enjoyed working with you. This has been a professional relationship, and it ends here — that is what makes it safe."

> **PRACTICAL**
> Termination role-play: a family that has improved and is anxious about stopping. The therapist must review, name the risk, write the plan and close, in 15 minutes.

Week 16 — Observed session 2

Forty minutes, full assessment, using the criteria below and a self-assessment submitted in advance.

Criterion	Weight
Structure and pacing	20%
Joining and alliance with each member	20%
Questioning (range and timing)	20%
One deliberate intervention well executed	25%
Summary, task and closing	15%

Glossary

Accommodation — adapting one's style to a family's organisation in order to join it. **Circular question** — a question about difference or relationship that reveals pattern. **Enactment** — directing family members to interact with each other in session. **Intensity** — raising the emotional or interactional stakes to produce change. **Positive connotation** — framing all behaviour as serving the family system. **Reflexive question** — a question inviting the family to observe itself. **Reframe** — relabelling behaviour within a new, usually relational, frame. **Subgroup session** — seeing part of the family deliberately, then using the difference. **Unbalancing** — deliberately supporting one member to shift the system's equilibrium.

Self-assessment

Multiple choice

1. The correct response to "she is manipulative" as a first formulation is: (a) agree (b) challenge the parent (c) gather specific behavioural sequences (d) diagnose

2. "Who notices first when your mother goes quiet?" is: (a) linear (b) circular (c) strategic (d) closed

3. The main purpose of enactment is to: (a) gather history (b) observe and shape live interaction (c) calm the family (d) demonstrate expertise

4. A reframe should: (a) excuse the behaviour (b) open new action without removing accountability (c) always be positive (d) replace diagnosis

5. Two reflections for every question is recommended because it: (a) fills time (b) reduces interrogation and enriches material (c) prevents emotion (d) shortens sessions

6. A task is most likely to fail when it is: (a) written down (b) small and specific (c) agreed in the last two minutes (d) behavioural

7. Therapeutic silence after an emotional statement should usually be: (a) filled quickly (b) held for 10–15 seconds (c) ended by offering a tissue (d) followed by a question

8. In a family with coercive control, enactment of conflict is: (a) recommended (b) contraindicated (c) neutral (d) required

9. Asking "why did you shout?" is discouraged because it invites: (a) facts (b) defence or invention (c) circular information (d) emotion

10. A good ending includes: (a) a new diagnosis (b) a relapse plan (c) a friendship (d) a referral to another therapist

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 b · 9 b · 10 b

Short answer

1. Write one example of each: linear, circular, reflexive and scaling question, on the same topic.
2. Give the four steps of enactment with the exact words you would use for step 1.
3. What are the three tests of a good reframe?
4. Name four reasons between-session tasks fail.
5. How would you structure a session for a family with a 7-year-old and a 15-year-old?

Essay (choose one, 1,800 words)

1. "Technique is the servant of the alliance." Discuss with reference to your own observed sessions.
2. Analyse a transcript of your own session and identify three moments where a different choice was available. Justify each alternative with theory.
3. How should a family therapist adapt technique for a family in which the father is structurally absent?

Further reading

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Tomm, K. (1988). Interventive interviewing: Part III. Family Process, 27(1), 1–15.
- Hepworth, M., & Larsen, J. (1993). Direct Social Work Practice. Brooks/Cole. (Micro-skills chapters)
- Butler, C., & Pilling, S. (2020). Competence Framework for Systemic Family and Couple Therapy. UCL.
- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation. UCL Competence Frameworks.

Next: MFT 707 — Assessment and Diagnosis in Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 707

Assessment and Diagnosis in Family Therapy

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SEMESTER ONE

1.71 units · Prerequisite: MFT 701, 703

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MFT 707 — Assessment and Diagnosis in Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 701, 703

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	**Assessment and Diagnosis in Family Therapy**	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words they can use. The course culminates in a full assessment report of publishable quality.

How to use this book

Assessment is the phase of therapy where the most harm and the most help can be done. Work through the weeks in order: the sequence matters, because each week's skill depends on the previous one. The final assignment is a complete assessment report — the same document type you will write for courts, schools and psychiatrists for the rest of your career.

1. Course description

Family assessment answers four questions: What is happening? How does it work? What maintains it? What would help? Diagnosis answers a fifth: Does this presentation meet criteria for a recognised disorder? Both matter, and they are not the same activity.

The course teaches the **multi-informant, multi-method** standard: self-report, collateral report, observation of interaction, standardised measures, records, and the family's own account — integrated in a formulation that is shared, tested and revised.

2. Learning outcomes

#	Outcome	Assessed
LO1	Plan and conduct a multi-informant family assessment	Observed assessment
LO2	Administer, score and interpret at least four standardised measures	Report
LO3	Use DSM-5/ICD-11 criteria appropriately, including their limits	Exam
LO4	Produce a 5-P formulation with relational hypotheses and a risk screen	Report
LO5	Identify medical, substance-related and cultural rule-outs	Exam
LO6	Give assessment feedback to a family in accessible language	Skills assessment
LO7	Write a report that is accurate, defensible and free of jargon	Report

3. Assessment

Component	Weight	Due
Measure administration and scoring (portfolio)	20%	Week 7
Observed assessment interview (30 min)	25%	Week 11

Component	Weight	Due
Full assessment report (3,000 words + appendices)	40%	Week 15
Examination	15%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Purposes of assessment: clinical, legal, institutional	Map the purposes in a case
2	The multi-informant principle; who to see and in what order	Design an assessment plan
3	History taking: development, family, medical, psychiatric, substance	Structured history interview
4	Interaction observation: what to look for and how to record	Live observation coding
5	Standardised measures I: relationship and family functioning	Score and interpret
6	Standardised measures II: symptoms, risk, children	Score and interpret
7	**Measure portfolio due**; psychometrics in practice	Peer review of portfolios
8	Diagnosis I: DSM-5 and ICD-11 — structure, use, limits	Criteria exercise
9	Diagnosis II: differential diagnosis and comorbidity	Case differentials
10	Medical, neurological and substance rule-outs	Referral-writing lab
11	**Observed assessment interview**	Assessment and feedback
12	Relational and systemic diagnosis: beyond the individual	Formulation workshop
13	Cultural formulation and bias in assessment	Cultural formulation exercise
14	Feedback, report-writing and shared decision-making	Report clinic
15	Integration: assessment that changes treatment	Case presentations

Week 1 — Purposes of assessment

Assessment is never neutral. Ask **who wants it and what they will do with it**.

Purpose	Who commissions	What they need
Clinical	The family	Understanding, direction, relief
Institutional	School, employer, agency	A defensible opinion and a recommendation
Legal	Court, solicitor, child protection	Facts, opinion within expertise, documentation
Research	Investigator	Standardised, comparable data
Self-understanding	The family	Narrative coherence

The trap of multiple purposes. A family may come for help and be sent by a court. The therapist must be explicit about which hat they are wearing, because the duties differ — a clinical assessment prioritises the client's interests; a forensic assessment prioritises accuracy for the court and is not confidential in the same way.

> **PRACTICAL**
> Take any referred case. List every party with an interest in the assessment and what each wants. Identify where the interests conflict.

Week 2 — The multi-informant principle

Rule. Never rely on one informant for a relational question. Mothers, fathers, children and teachers systematically disagree, and the disagreement is clinically informative.

A workable order of contact.

1. **Referrer** (5–10 minutes by phone): what is the concern, what has been tried, is there risk?
2. **Whole family** (60–90 minutes): joining, each person's view, interaction observation, contract.
3. **Subgroups** as indicated: parents alone, each child, each partner separately (always screen IPV separately).
4. **Collateral** with consent: school, GP, previous therapist.
5. **Measures** completed between sessions.
6. **Feedback session**: share the formulation, invite correction.

Who is missing? Fathers, paternal relatives, non-resident children, step-parents, grandparents who co-parent, and the person the family has decided not to discuss. Ask: "Who else is part of this family who is not here? Who would disagree with what has been said today?"

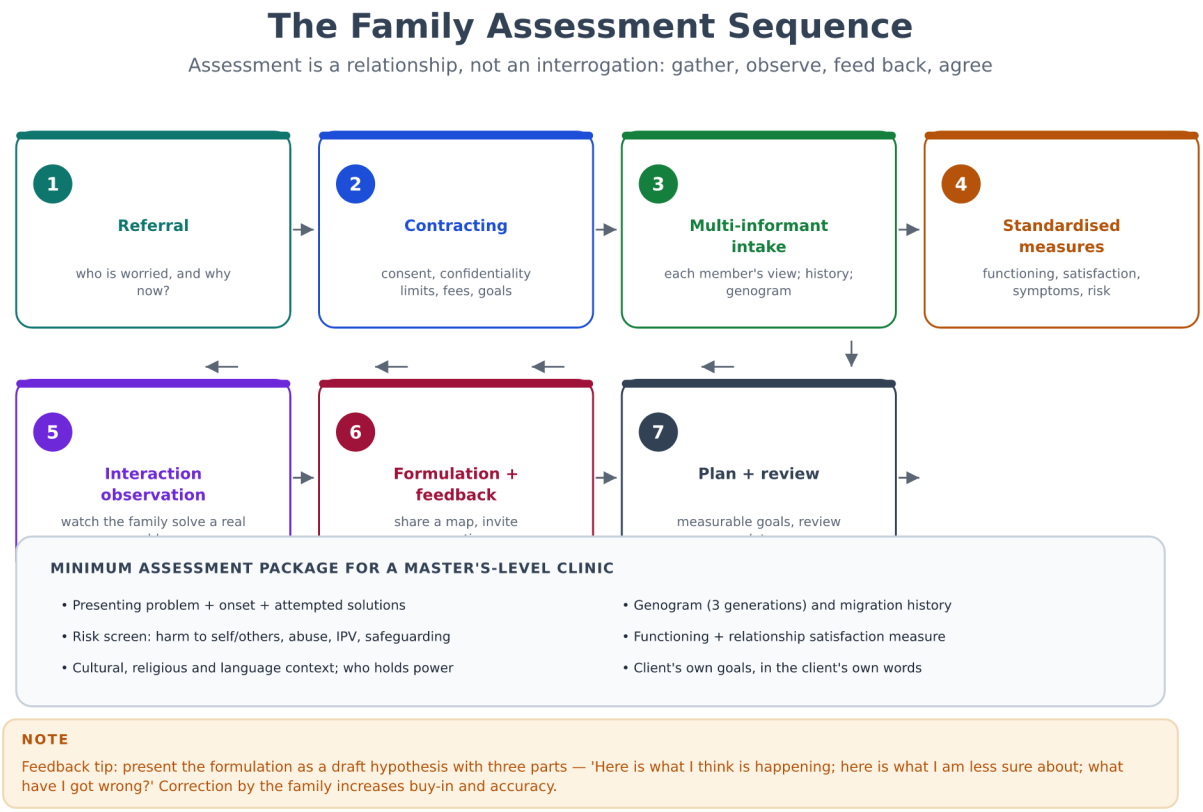


Figure 2.1 — The assessment sequence from referral to review

> **PRACTICAL**
> For a case you know, design the assessment plan: who, in what order, how long, and why. Defend the order.

Week 3 — History taking

A structured history prevents the two classic errors: never asking, and asking everything.

Domains (45 minutes is enough if you are organised).

Domain	Key questions
Presenting problem	Onset, course, severity, triggers, attempted solutions, previous treatment
Developmental	Pregnancy, birth, milestones, illnesses, separations, schooling
Family of origin	Genogram (3 generations), roles, losses, secrets, migration, violence, mental illness
Current family	Household composition, rules, routines, roles, money, division of labour, sexuality
Medical	Chronic illness, medication, sleep, head injury, neurological symptoms
Psychiatric	Previous diagnoses, hospitalisations, self-harm, suicide attempts, treatment response
Substance use	Alcohol, tobacco, cannabis, prescription, other; quantity, frequency, consequences

Domain	Key questions
Social	Work, school, faith community, friends, housing, legal issues, finances
Risk	Harm to self/others, abuse, neglect, IPV, safeguarding
Strengths	What has kept this family going? Who helps? What works?

The strengths column is not decoration. Families referred for assessment have usually been described in deficit terms for years. A careful strengths inventory changes the emotional tone of everything that follows and predicts engagement.

Week 4 — Interaction observation

You do not have to rely on what families say about themselves; you can watch them.

Four standard tasks.

1. **The problem discussion (10 minutes).** "Talk to each other about the problem for ten minutes. I will not interrupt." Observe: who speaks, who is interrupted, who looks at whom, who is silent, how it ends.
2. **The planning task (5 minutes).** "Agree on one change you will make this week." Observe: negotiation, hierarchy, coalition, flexibility.
3. **The positive reminiscence (5 minutes).** "Tell each other about a good day you had together." Observe: affection, repair capacity, humour.
4. **The child-focused task.** "Decide together how to respond to the next tantrum." Observe: who decides, whether the child is consulted, whether the parents support each other.

What to record (behavioural, not judgemental).

Record	Not
"Father interrupted mother 7 times in 10 minutes; she stopped speaking after minute 6."	"Father was dominant."
"Daughter looked at mother before answering each question."	"Daughter is enmeshed."
"Parents agreed on the plan in 90 seconds; the child's objection was not addressed."	"Weak parenting."

The five-minute structural map. Draw the seating, the direction of speech, and the coalitions you observed. It will be more useful than a page of adjectives.

Week 5 — Standardised measures I: relationship and family functioning

Why measure. To obtain a baseline, compare informants, track change, communicate with other professionals, and protect against your own impression management.

Core measures.

Measure	What it measures	Items	Notes
DAS-4 / RDAS	Couple satisfaction	4 / 14	Brief, widely used; cut-offs published
FACES-IV	Cohesion and flexibility	42	Psychometrics debated; use with caution
FAD (McMaster)	Family functioning	60	Six subscales; strong evidence base
Family APGAR	Perceived family support	5	Very brief; useful for screening
WAI-SR	Therapeutic alliance	12	Adapt for family members individually

Scoring discipline. Use the manual's scoring, not a photocopied table. Report raw and standardised scores, the norm group, and the cut-off used. State what a score does not mean: a high DAS score does not mean the couple is safe.

> **PRACTICAL**
> Score an RDAS from a completed (simulated) form: compute the total, compare with the published cut-off, and write three sentences you could say to the couple.

Week 6 — Standardised measures II: symptoms, risk and children

Symptom measures. PHQ-9 (depression; note item 9 is a suicide item and must be followed up), GAD-7 (anxiety), AUDIT (alcohol), DUDIT (drugs), PC-PTSD-5 or PCL-5 (trauma), YSQ or DERS (emotion regulation) as indicated.

Child measures. SDQ (emotional and behavioural, 4–17, parent/teacher/self), RCADS (anxiety and depression), CBCL (broadband; longer), Conners (attention). Always obtain multiple informants and report them separately.

Risk measures. Columbia Suicide Severity Rating Scale (C-SSRS) or a structured clinical interview; do not substitute a questionnaire for a conversation.

Interpretation rules.

1. Measure the same construct with the same instrument at follow-up.
2. Report informant discrepancies rather than averaging them away.
3. Treat screening positives as questions, not diagnoses.
4. Consider literacy, language and cultural fit before interpreting a score.

> **PRACTICAL**
> Administer the SDQ to a volunteer parent and the self-report to a peer playing an adolescent. Compute both. Compare. Write what the discrepancy might mean.

Week 7 — Psychometrics in practice



Figure 7.1 — Validity and reliability: four targets, three methods

Four questions to ask of any instrument.

1. **What is the norm group?** If it was normed on US undergraduates, a Nigerian clinical sample's score needs careful interpretation.
2. **What is the internal consistency?** α ≥ .70 is the usual minimum for research; ≥ .90 is desirable for individual clinical decisions.
3. **Has it been validated locally or in a similar population?** Search before you use.
4. **Is it responsive to change?** A measure that cannot detect improvement is useless for outcome monitoring.

Measurement invariance. The technical question "does this instrument measure the same thing across groups?" is clinically urgent. If a scale behaves differently for men and women, or across languages, then a difference in scores may reflect the instrument, not the people.

Week 8 — Diagnosis I: structure, use and limits

What DSM-5 and ICD-11 are. Classification systems that group patterns of signs and symptoms into categories, using criteria, thresholds and durations. They are reliable vocabularies with imperfect validity: they describe clusters, not known disease entities.

The five DSM-5 axes are gone (they were removed in DSM-5, replaced by a single axial diagnosis plus separate notation of psychosocial stressors, disability and contextual factors). Know the current structure even if you trained on the old one.

Using diagnosis well.

1. Diagnose to communicate, not to explain. "Generalised anxiety disorder" tells a colleague what to expect; it does not tell you why this woman, in this family, at this time.
2. Diagnose only what you can defend with criteria.
3. Record uncertainty explicitly: "provisional", "rule out", "not otherwise specified".
4. Consider the function of a diagnosis in the family: it can unlock care, or become an identity, or a weapon in a custody dispute.

Using diagnosis badly.

- **Premature closure:** the first label constrains all subsequent observation.
- **Diagnostic overshadowing:** the label hides a physical illness, a trauma, or a relational problem.
- **Category fallacy:** assuming that because a category exists, a distinct disease exists.
- **Cultural misreading:** interpreting culturally normative behaviour (spiritual experience, deference, grief expression) as pathology.

> **PRACTICAL**
> Take one case and write two formulations: one purely diagnostic, one purely systemic. Then write the paragraph that integrates them.

Week 9 — Differential diagnosis and comorbidity

The differential habit. For any presenting complaint, generate at least three alternatives before committing. Example: an adolescent's withdrawal.

Possibility	What would support it	What would rule it out
Major depression	Pervasive anhedonia, sleep/appetite change, 2+ weeks	No change in function; mood reactive
Social anxiety	Fear of scrutiny; avoidance specific to social settings	Comfortable with close friends
Trauma response	Intrusion, avoidance, hyperarousal after an event	No identifiable event
Substance use	Changed peer group, money, paraphernalia	Denial plus negative screen
Family crisis	Onset coincides with parental separation	No temporal link
Medical (e.g. anaemia, thyroid, sleep apnoea)	Fatigue, physical signs	Normal investigations

Comorbidity is the rule, not the exception. Depression and anxiety co-occur; substance use and trauma co-occur; child behaviour problems and parental mental illness co-occur. Prioritise: treat what is most dangerous, most disabling, or most treatable first, and treat the family context alongside.

Week 10 — Medical, neurological and substance rule-outs

Refer, do not diagnose. Your job is to notice and to route.

Presentation	Rule out	Route
First episode of psychosis	Organic causes, substance-induced	Urgent psychiatric referral
Sudden confusion in an elder	Delirium, infection, medication	Same-day medical review
Fatigue, weight change, low mood	Anaemia, thyroid, diabetes, sleep apnoea	GP with specific request
Memory decline	Dementia vs depression vs delirium	Cognitive assessment service
Headaches, seizures, personality change	Neurological	Neurology
Erectile dysfunction, low libido	Vascular, hormonal, medication	Medical review before psychologising
Unexplained somatic complaints	Physical illness, somatisation	GP plus psychiatric liaison

Write a referral that works. State what you observed, what you want excluded, why now, and who to contact. "Please assess and advise" produces a delay; "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue and low mood, as I am considering a diagnosis of depression and would like to rule out physical causes before starting therapy" produces a response.

Week 11 — Observed assessment interview

Thirty minutes, assessed: contract and confidentiality, each person's view, interaction observation task, risk screen, strengths, and a closing summary with next steps. The family rates their experience of being assessed on three dimensions: heard, not judged, clear about what happens next.

Week 12 — Relational and systemic diagnosis

Individual diagnosis does not describe a relationship. Three ways of adding the relational layer:

1. Relational descriptors. Not a code, but a description: "a couple in a pursue–withdraw cycle with low repair capacity"; "a family with a cross-generational coalition and a disengaged father"; "a family at a stage transition with no renegotiation".

2. Global functioning of the relationship. Instruments such as the GARF (Global Assessment of Relational Functioning) rate the family or couple's functioning along a continuum, with anchors. Useful for outcome monitoring and for communicating severity.

3. The formulation as the relational diagnosis. The 5-P grid plus relational columns is the most clinically useful "diagnosis" a family therapist produces.

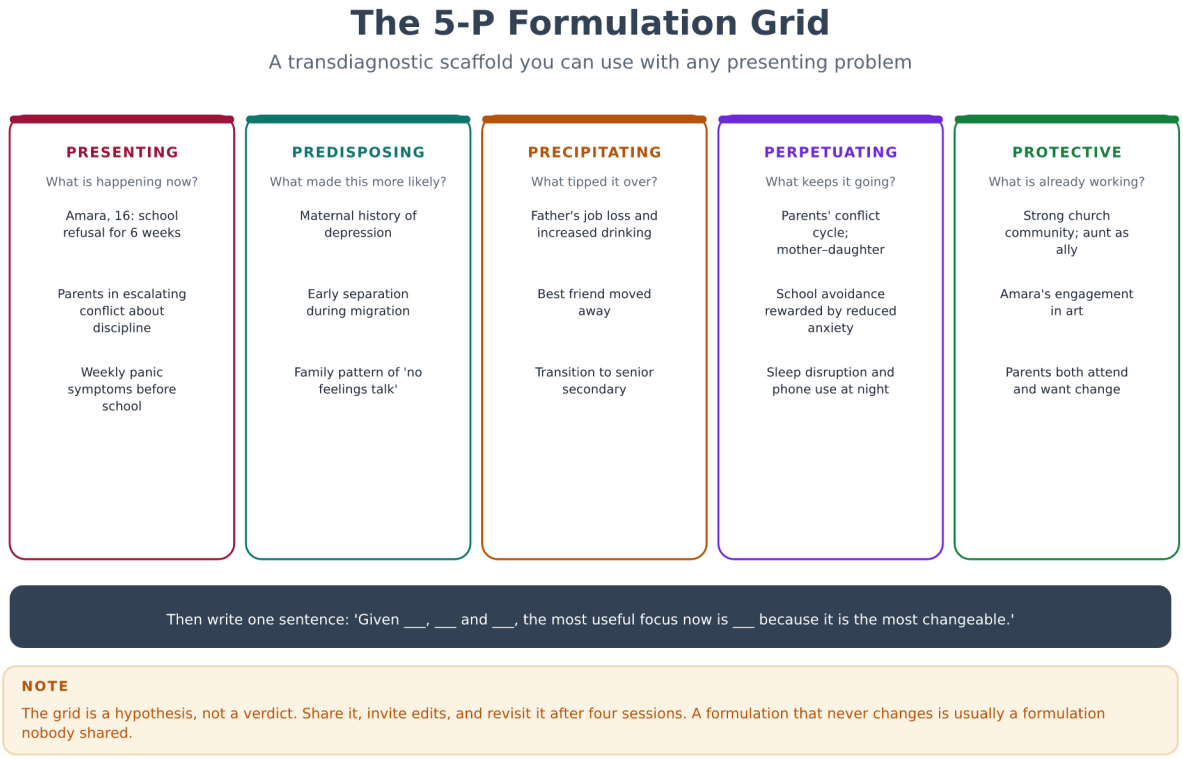


Figure 12.1 — The 5-P formulation grid applied to one family

Worked formulation (Adeyemi family, condensed).

> **PRESENTING.** Amara, 16, has not attended school for six weeks; panic symptoms on school mornings; escalating parental arguments about discipline. **Predisposing.** Maternal history of depression; paternal alcohol misuse; migration removed the extended family's daily support; a family rule that feelings are not discussed. **Precipitating.** Father's redundancy, a best friend's relocation, and the transition to senior secondary school, all within four months. **Perpetuating.** A pursue–withdraw cycle between the parents; a mother–daughter coalition that stabilises the marital distance; the relief Amara experiences when she stays home, which reinforces avoidance; disrupted sleep and night-time phone use. **Protective.** Both parents attend; a trusted aunt; church community; Amara's engagement with drawing; the family's capacity for humour. **Hypothesis (testable).** If the parents can complete one conflict conversation without Amara's involvement, her morning panic will reduce within three weeks. **Focus:** de-escalate the couple cycle while building a graded return-to-school plan that does not depend on Amara's compliance alone.

Week 13 — Cultural formulation and bias

The Cultural Formulation Interview (CFI) — a set of 16 questions from DSM-5 — gives structure to what many clinicians do informally:

1. What brings you here, in your own words?
2. What do you call this problem?
3. What do you think caused it?
4. How severe is it, and what worries you most?
5. What has helped, and what has not?
6. What kind of treatment do you expect?
7. Is there anything about your background or identity I should know?

Sources of bias in assessment.

Bias	Effect	Mitigation
Availability	Over-diagnosing what you saw last week	Use structured criteria
Confirmation	Seeking evidence for your first hypothesis	Actively seek disconfirmation
Cultural	Pathologising difference	Ask about meaning; consult
Gender	Reading assertion in women as pathology; in men as strength	Check your language
Class	Reading material hardship as neglect	Ask about money explicitly
Attribution	Blaming parents for adolescent behaviour	Include context and strengths

---

Week 14 — Feedback and report writing

Feedback is an intervention. Structure it in four parts: what I heard, what I think is happening, what I am unsure about, what I suggest. Then stop talking and invite correction.

The report. Write for three readers: the family, the referring professional, and a future clinician who has never met you. Suggested structure:

1. Identifying information (minimal — no more than necessary)
2. Reason for referral (in the referrer's words)
3. Consent, confidentiality and who was seen
4. Sources of information
5. Presenting concerns, from each informant
6. History (developmental, family, medical, psychiatric, substance, social)
7. Observation of interaction
8. Measures administered, with scores and interpretation
9. Risk assessment and outcome

10. Formulation (5-P plus relational hypothesis)
11. Diagnosis, if any, with criteria met and differentials considered
12. Recommendations, with who will do what
13. Review date and signature

Style rules. No adjectives about people; no speculation without labelling; no jargon the family cannot read; no more than two pages for the family copy.

> **PRACTICAL**
> Write the family-facing summary (one page, no jargon) of your full report, and test it on a non-clinician.

Week 15 — Integration

Assessment is the moment a family learns what kind of problem they have. Get it right and therapy has a spine; get it wrong and every subsequent session is spent working on the wrong thing. The standard is not comprehensiveness; it is **sufficiency** — enough information to make a defensible formulation and a workable plan, gathered in a way that leaves the family feeling respected.

Glossary

Collateral information — data from third parties with consent. **Cultural formulation** — a structured account of the cultural context of a presentation. **Diagnostic overshadowing** — attributing a physical or relational problem to a psychiatric label. **Differential diagnosis** — the systematic consideration of alternative explanations. **GARF** — Global Assessment of Relational Functioning. **Measurement invariance** — whether an instrument behaves equivalently across groups. **Multi-informant assessment** — gathering data from several people who see the person in different contexts. **Rule-out** — a condition that must be excluded before a diagnosis is confirmed. **Screening** — a brief procedure identifying those who need fuller assessment.

Self-assessment

Multiple choice

1. The multi-informant principle exists because: (a) families lie (b) informants see different contexts and disagreement is informative (c) it is legally required (d) it fills the session

2. A high couple-satisfaction score means: (a) the relationship is safe (b) the relationship is satisfactory to the respondent, not that it is safe (c) the couple will stay together (d) no further assessment is needed

3. Diagnostic overshadowing refers to: (a) over-diagnosing (b) a label hiding another problem (c) comorbid diagnoses (d) cultural bias

4. The interaction task "agree on one change this week" is used to observe: (a) memory (b) negotiation, hierarchy and coalition (c) IQ (d) attachment classification

5. Which is the best referral request? (a) "Please assess and advise" (b) "Please see this patient" (c) "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue" (d) "Please confirm my diagnosis"

6. The 5-P formulation stands for presenting, predisposing, precipitating, perpetuating and: (a) psychiatric (b) protective (c) prognostic (d) provisional

7. Screening instruments should be treated as: (a) diagnoses (b) questions requiring follow-up (c) evidence of malingering (d) definitive cut-offs

8. The CFI is: (a) a personality test (b) a cultural formulation interview (c) a child functioning index (d) a court report

9. The strengths inventory matters because it: (a) is required by codes (b) changes tone and predicts engagement (c) replaces diagnosis (d) shortens assessment

10. A treating therapist should generally decline to conduct: (a) an intake (b) a custody evaluation of their own client (c) a genogram (d) an outcome measure

Answer key

1 b · 2 b · 3 b · 4 b · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List the five parties who may have an interest in an assessment and what each wants.
2. Give three behavioural (not judgemental) observation statements.
3. What are the four parts of feedback to a family?
4. Name three biases in assessment and one mitigation for each.
5. Why must IPV screening be conducted separately from the partner?

Essay (choose one, 2,000 words)

1. "Diagnosis in family therapy is both necessary and dangerous." Discuss.
2. Using a disguised case, present a full assessment and defend your formulation against two alternatives.
3. Critically evaluate the claim that standardised family measures are culturally biased.

Further reading

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- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288–307.
- Sperry, L. (2012). Family Assessment: Contemporary and Cutting-Edge Strategies. Routledge.

Next: MFT 708 — Cultural Competence in Therapy

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Cultural Competence in Therapy

14 teaching weeks · 5 practicals · 2 worked cases · 2 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

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MFT 708 — Cultural Competence in Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	**Cultural Competence in Therapy**	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Culture is not a topic added to therapy; it is the medium in which therapy happens. This course moves you from "knowing about cultures" to working skilfully with difference, power and meaning — including your own.

How to use this book

You will be uncomfortable at points in this course. That is not a design flaw; it is the learning. Each week pairs a **concept** with a **practice** that requires you to examine your own position. Do the exercises honestly and keep them private — this is formation, not performance.

1. Course description

The field's language has shifted from cultural competence (a set of knowledge and skills, often taught as a list of group traits) to cultural humility (a lifelong process of self-examination, redress of power imbalance, and institutional accountability) and cultural safety (the outcome judged by the person receiving care).

This course teaches all three, and is honest about their limits. It covers race and racism, ethnicity, religion and spirituality, class and poverty, gender and power, sexuality, disability, migration and displacement, language and interpreting, and the intersection of all of these in a single family.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish cultural competence, humility and safety, and use each appropriately	Exam
LO2	Conduct a cultural assessment using a structured framework	Observed interview
LO3	Analyse power, privilege and intersectionality in a clinical relationship	Reflective portfolio
LO4	Work effectively with interpreters and in a second language	Skills assessment
LO5	Adapt assessment and intervention to extended-family, faith-based and polygynous structures	Case analysis
LO6	Recognise and interrupt racism, ethnocentrism and bias in your own practice	Portfolio, viva
LO7	Critically appraise the cultural assumptions of the major therapy models	Essay

3. Assessment

Component	Weight	Due
Reflexive portfolio (6 entries × 400 words)	30%	Fortnightly
Cultural assessment observed interview (25 min)	25%	Week 9
Case analysis with cultural formulation (2,500 words)	45%	Week 14

4. The fourteen-week plan

Week	Topic	Practice
1	What culture is — and is not	Cultural genogram (own family)
2	Competence, humility, safety: history and critique	Concept comparison
3	The ADDRESSING framework	Structured cultural interview
4	Race, racism and the clinical encounter	Microaggression analysis
5	Ethnicity, tribe, language and migration	Interpreter role-play
6	Religion and spirituality as clinical structure	Faith-history interview
7	Class, poverty and material reality	Budget exercise
8	Gender, power and the politics of the family	Power mapping
9	**Cultural assessment observed interview**	Assessed
10	Sexuality, gender identity and family acceptance	Coming-out family session
11	Disability, chronic illness and care	Accessibility audit
12	Polygyny, extended family and communal child-rearing	Structural adaptation
13	Decolonising therapy: models, manuals and evidence	Critical appraisal
14	Integration: your cultural position statement	Viva

Week 1 — What culture is — and is not

Culture is the shared, learned system of meanings, practices, values and identities through which a group makes sense of the world. It is dynamic, internalised, contested and partially unconscious.

Five things culture is not.

1. **Not nationality.** "Nigerian culture" contains hundreds of languages and dozens of marriage systems.
2. **Not a personality.** Culture sets a range of possible behaviours, not a script every member follows.
3. **Not static.** Families migrate, convert, intermarry and change class; their culture changes with them.

4. **Not only "the other".** You have a culture too, and so does your profession.
5. **Not an excuse.** "That is our culture" can describe a practice or justify coercion; the therapist must be able to hold respect and moral clarity at the same time — particularly about violence.

The clinical implication. Never ask "what do people from X do?" Ask "what does this family do, what does it mean to them, who in the family disagrees, and how does it interact with the problem?"

> **PRACTICAL**
> Draw your own cultural genogram: languages, religions, migrations, class position, ethnic identities, and the rules you absorbed about marriage, money, gender and feelings. Note what you would find difficult to say to a family that did the opposite.

Week 2 — Competence, humility, safety

Concept	Core claim	Strength	Limitation
Cultural competence	Knowledge and skills about groups can be taught	Practical, trainable	Risks stereotyping; assumes culture is a trait
Cultural humility	Lifelong self-reflection, redress of power, institutional accountability	Attends to power and to the self	Can become inward-looking; hard to assess
Cultural safety (Ramsden)	The recipient judges whether care is safe	Centres the client's experience	Requires institutional change, not only individual skill

The synthesis used in this programme. Humility as the stance, competence as the skills, safety as the criterion. You reflect on your position (humility), use structured tools and knowledge (competence), and ask the family whether the encounter was safe (safety).

Why "I treat everyone the same" is not an ethical position. Treating everyone the same preserves the advantages of the majority, because the default is not neutral — it was built for someone.

Week 3 — The ADDRESSING framework

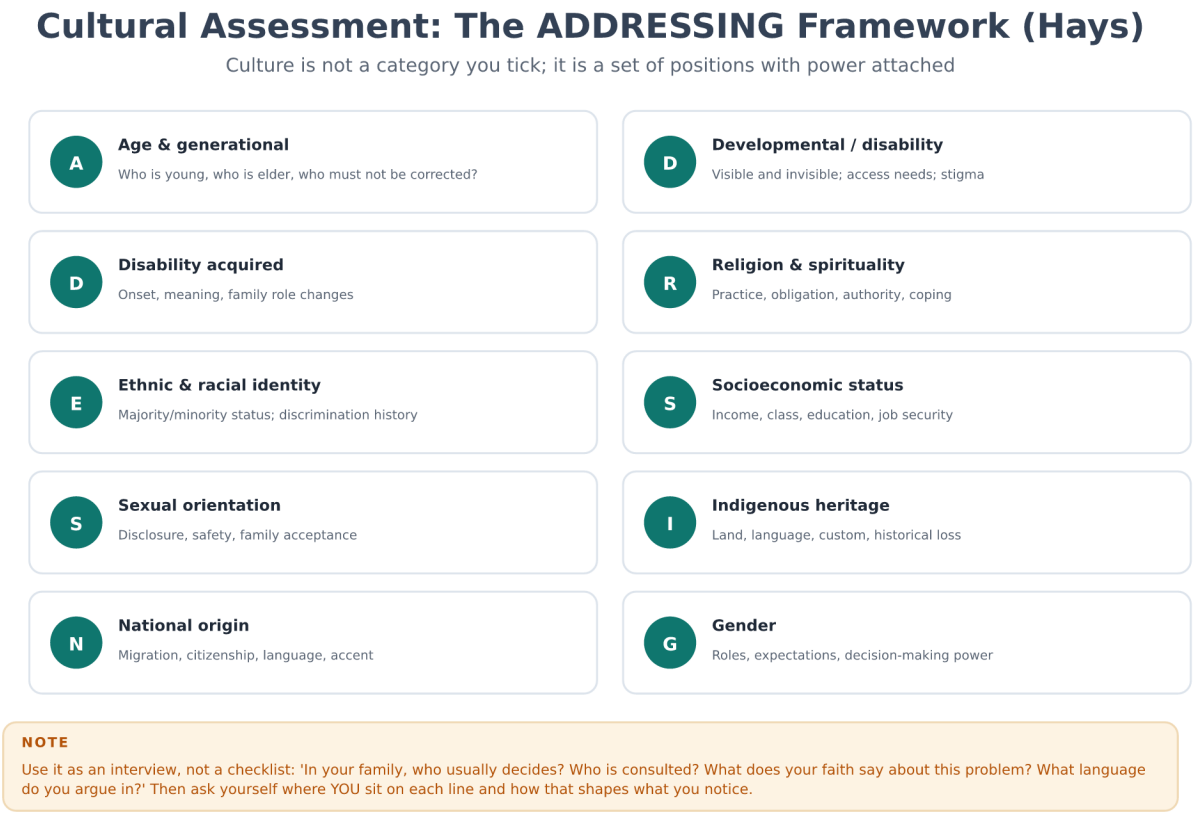


Figure 3.1 — ADDRESSING: ten cultural positions with power attached

Hays's framework asks you to assess ten dimensions, for the family **and for yourself**:

Age and generation · Developmental disability · Disability acquired later · Religion and spirituality · Ethnic and racial identity · Socioeconomic status · Sexual orientation · Indigenous heritage · National origin · Gender.

Interview prompts that work.

- "In your family, who usually decides? Who is consulted? Who is told afterwards?"
- "What does your faith say about this problem? Who in your religious community knows?"
- "Which language do you argue in? Which language do you pray in?"
- "When you were growing up, was money talked about?"
- "What did your family teach you about people from [another group]?"

The power question. For each dimension, ask: is this person in the majority or the minority in this society, in this clinic, and in this room? The same person may be privileged on one axis and marginalised on another — this is intersectionality.

> **PRACTICAL**
> Conduct a 15-minute structured cultural interview with a peer using five ADDRESSING domains. Then reverse roles. Debrief: which question was hardest to ask, and why?

Week 4 — Race, racism and the clinical encounter

Definitions. Race is a social category with no biological basis; its effects are nonetheless real and measurable. Racism is the system of advantage and disadvantage organised around race, operating at interpersonal, institutional and internalised levels.

Three clinical implications.

1. **Racism is a chronic stressor with health consequences** — including hypertension, depression, and adverse birth outcomes. It belongs in the assessment, not in a footnote.
2. **Therapy is not a racism-free space.** Families may test you, protect you, or perform for you. Ask: "Have you had experiences in health settings where you were treated badly because of your background?"
3. **Microaggressions occur in clinical settings** — comments, assumptions, lowered expectations. They are often unintentional and are still harmful.

Four common therapist failures.

Failure	Example	Repair
Colour-blindness	"I don't see race"	Acknowledge difference; ask about its meaning here
Defensiveness	"I'm not racist, but…"	Focus on impact, not intent
Over-attribution	Explaining everything by race	Differentiate race from other factors
Exoticising	Treating the family as a cultural exhibit	Ask about ordinary life too

Talking about race when it has not been mentioned. Wait for a cue, then ask directly: "I wondered whether the fact that I am [X] and you are [Y] matters here. Sometimes it does, sometimes it doesn't." Offering the question is often more important than the answer.

> **PRACTICAL**
> Analyse three microaggression scenarios from a clinical setting: what was said, what was meant, what was heard, what a repair would sound like.

Week 5 — Ethnicity, tribe, language and migration

Ethnicity and tribe. In Nigeria and across Africa, ethnic identity carries histories of majority/minority relations, language, land, and in some cases conflict. Ask about it as a relationship question: how do the two families regard each other's ethnicity? Was the marriage contested? Whose language do the children speak?

Migration. Migration is a family event, not an individual one. Key clinical themes:

- **Split households** — one parent abroad, children with grandparents, "sent" children.

- **Role reversal** — children who acquire the new language become the family's intermediaries.
- **Loss of the extended-family buffer** — the practical support that made parenting possible.
- **Status loss** — a professional who becomes a cleaner; a respected elder who becomes invisible.
- **Legal precarity** — the constant threat of removal, which changes every decision.

Language. A family may think in one language about emotion and another about work. Ask: "In which language do you feel most like yourself? Which language do you dream in?" Offer sessions in the family's preferred language where you can; where you cannot, use an interpreter properly.

Working with interpreters — the standard.

1. Brief before: purpose, confidentiality, first-person interpretation, do not summarise or advise.
2. Seat the interpreter beside you, facing the family.
3. Speak to the family, not to the interpreter ("How do you feel?" not "Ask her how she feels").
4. Allow longer for interpretation; do not fill pauses.
5. Debrief afterwards: what was difficult, what was not said, any cultural information.
6. **Never** use a child as interpreter for anything clinical.

> **PRACTICAL**
> Interpreter role-play: one student interprets, one conducts, one plays a family member who switches languages. Practise briefing, first-person interpretation, and debriefing.

Week 6 — Religion and spirituality as clinical structure

In most of the world, faith is not an "aspect" of family life; it is the organising framework. Asking about it is not optional.

What to ask. What is your faith? How does it shape daily life (prayer, diet, dress, holy days)? What does it say about marriage, divorce, sex, mental illness, healing, and death? Who is your religious authority? Have you consulted them about this problem? What did they say?

Three clinical patterns.

1. **Faith as resource.** Community, meaning, ritual, forgiveness, hope, practical support.
2. **Faith as stressor.** Guilt, prohibition of divorce, deliverance beliefs that attribute mental illness to sin, conflict between religious and medical models.
3. **Faith as site of conflict.** A couple from different traditions; conversion; a teenager leaving the faith; a pastor's authority competing with the therapist's.

Coordination, not competition. With consent, a conversation with the family's pastor, imam or elder can be more powerful than any technique. The aim is a shared plan, not a theological debate. You do not have to share the family's beliefs to take them seriously; you do have to be honest about your own.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent. She will not go to the police; the pastor has counselled her to be patient. The therapist must respect her faith and autonomy, hold the pastor as an ally where possible, and build a safety plan that does not depend on the husband's repentance. (See MFT 712, 724.)

Week 7 — Class, poverty and material reality

The clinical error. Reading the effects of poverty as individual or family pathology: "inconsistent parenting" when the mother works three jobs; "neglect" when the family has no electricity; "disorganised" when the household has eight people in two rooms.

What to ask, without humiliating.

- "Is money tight at the moment? How tight?"
- "What happens at the end of the month?"
- "Who else depends on your income?"
- "Have you ever had to choose between food and a bill?"

Practical action. Have a short list of referral routes: food support, school fee assistance, legal aid, social welfare, health insurance. A therapist who ignores material reality becomes part of the family's helplessness.

The class position of therapy itself. Sessions cost money, require transport and time off, and are conducted in a register many families do not use. Adapt: flexible times, community venues, shorter paperwork, plain language, a written summary the family can keep.

> **PRACTICAL**
> Complete a household budget for a family with two minimum-wage incomes and two children. Then re-read a "poor parenting" case with that budget in mind.

Week 8 — Gender, power and the politics of the family

Power is not the same as influence. Ask: who decides, who is consulted, who is informed, who is financially dependent, who can leave, who would be believed if they spoke?

Gender patterns you will meet.

Pattern	Presentation	Clinical risk if ignored
Traditional division of labour	Mother "does everything" and is exhausted; father "does nothing"	Therapist aligns with mother; father withdraws further
Head-of-household ideology	Father decides; mother adapts	Mother's distress is invisible; children learn silence
Female economic power	Wife out-earns husband	Status threat becomes the real argument
Male emotional restriction	He cannot say what he feels	Therapist misreads flatness as lack of love

The feminist critique that changed the field (Hare-Mustin, Goldner, Walters): early family therapy described "circular" patterns as though both parties had equal power. In violence, in economic dependence, and in cultural constraints, they do not. A circular description that ignores power becomes a sophisticated form of blame-the-victim.

Clinical practice. Name the asymmetry without moralising; ask the family whether they want it to be this way; work with the person who holds more power to use it differently; never insist a woman "communicate better" as a substitute for a man changing his behaviour.

Week 9 — Cultural assessment observed interview

Twenty-five minutes, assessed against six criteria: use of a structured framework, curiosity rather than assumption, power-attentiveness, no stereotyping, integration of culture with the presenting problem, and a respectful close.

Week 10 — Sexuality, gender identity and family acceptance

The clinical reality. In many settings, disclosure is unsafe, and families are torn between religious conviction, social pressure, and love for their child. Your task is not to resolve that for them but to make the conversation possible and to keep everyone safe.

Principles.

1. **Do not assume or ask for labels prematurely.** Follow the client's language.
2. **Assess safety first.** Is it safe at home, at school, at church? Who knows? What is the risk of disclosure?
3. **Work with the family's ambivalence**, not against it. "You have both said you love your son. You have also said you cannot accept this. I want to understand both, without asking you to give either up today."
4. **Never promise a family you will change the child**, and never promise a child you will change the family. Be honest with both.

5. **Know your legal context and your local referral routes**, including safe medical and mental health services.

When the family rejects. Rejection is a process with stages: shock, denial, negotiation, and — in some families — accommodation. Your job is to keep the door open, reduce harm, and protect the most vulnerable member.

Week 11 — Disability, chronic illness and care

Two models. The medical model locates disability in the body; the social model locates it in barriers — physical, attitudinal, institutional. Both matter clinically.

Assessment questions. What is the diagnosis and functional impact? Who provides care, and at what cost? What has the family had to give up? What is the meaning of the condition in this family's belief system (punishment, test, curse, genetics)? What accommodations are needed?

Avoiding two errors. Minimising ("it's not that bad") and catastrophising ("your life is over"). Ask the person directly: "What do you want me to understand about how this affects you?"

Sibling and parent impact. Caregiving burden falls unequally; siblings of disabled children often develop precocious competence and unspoken resentment. Name it as love and loss together.

Week 12 — Polygyny, extended family and communal child-rearing

Do not treat structure as pathology. Household forms that differ from the Western nuclear norm are not automatically dysfunctional. Assess function, safety and fairness within the form.

Polygynous households — assessment prompts.

- Who lives where, and who decides?
- How are resources and time allocated? Who keeps the accounts?
- What are the relationships between wives — allies, rivals, or both?
- How do children understand their relationships to the other wives?
- Is the marriage legal under a statutory, customary or religious system (the answer affects rights)?
- Who in the household holds authority over the children?

Clinical themes that recur: jealousy and comparison, resource competition, co-wife coalitions and sub-coalitions, children as messengers, unequal treatment, and the husband's role as the only permitted channel of communication.

Communal child-rearing. In many families, a child is raised by a grandmother, an aunt, or a foster kin network. This can be protective (multiple attachment figures) or risky (unclear responsibility, unspoken injury). The question is not "who is the mother?" but "who is responsible for what, and does the child know who to go to?"

> **CASE**
> A husband proposes a second wife. The first wife presents with insomnia and anger; he presents with confusion. The therapist neither endorses nor condemns the practice. The work is: what does each understand the marriage to be? What does the first wife have a right to? What will the children be told? Are there religious and legal structures the family wants involved?

Week 13 — Decolonising therapy

The critique. Most of the therapy models in this programme were developed in North America and Europe, tested on samples that were not representative, and then exported as universal. Three specific problems:

1. **Normative assumptions.** Individuation, self-expression and boundary-setting are treated as health. In many families, interdependence, duty and discretion are the markers of maturity.
2. **Evidence hierarchies.** Trials and manualised models carry more status than community knowledge and qualitative research, which subtly determines what can be funded and taught.
3. **Language.** "Enmeshment", "over-involvement", "dependence" are not neutral words; they encode a picture of the good family.

Responses that are more than rhetoric.

- Learn the local literature; cite African scholars; test instruments locally.
- Adapt models explicitly and say what you changed and why.
- Use the family's language and concepts in formulation.
- Include community and faith resources in treatment plans.
- Publish your adaptations so the field's evidence base changes.

And a caution. Decolonisation is not the replacement of one orthodoxy with another. Families are not monoliths, and "that is our culture" can silence a woman or a child as effectively as a Western manual can.

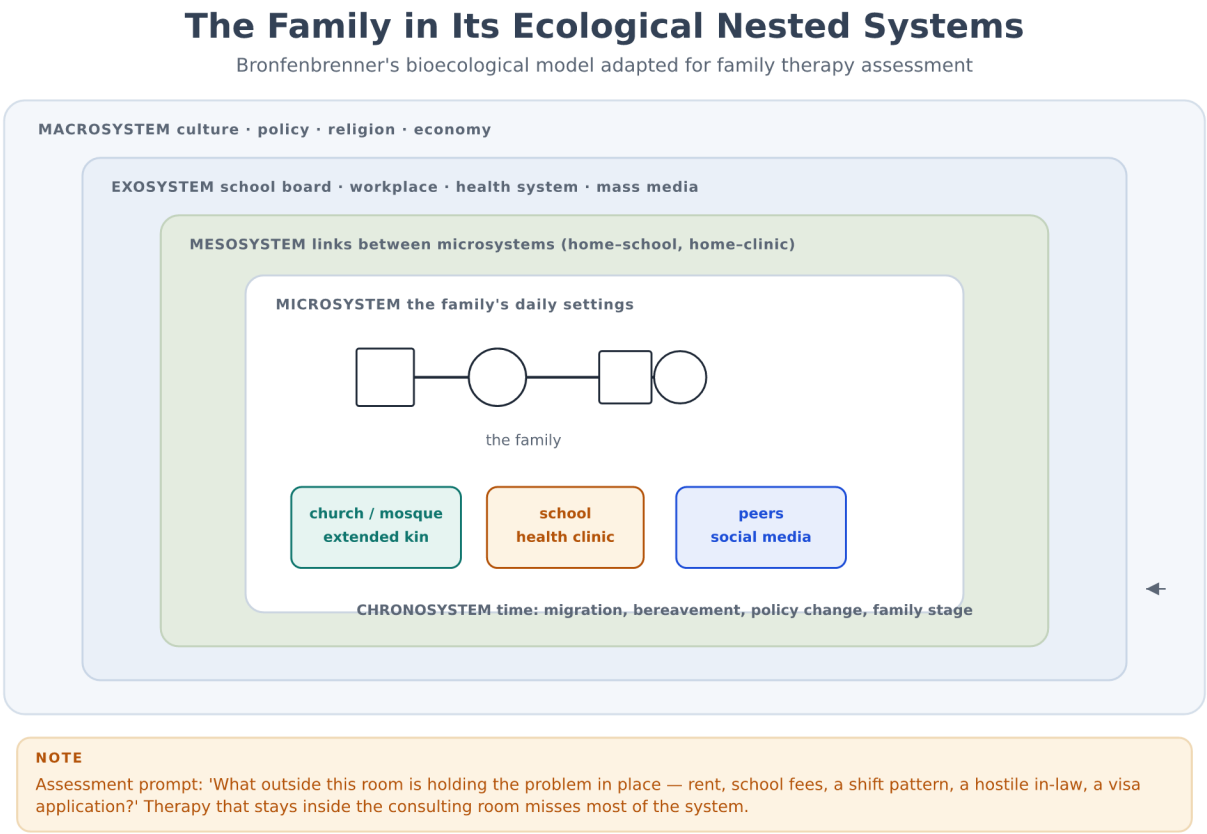


Figure 13.1 — Ecological systems: the family inside institutions, culture and time

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Week 14 — Integration: your cultural position statement

Write 1,000 words: your cultural identities and their advantages and disadvantages; two families with whom you would struggle and why; the adaptations you will make; the assumptions of your preferred model that you will watch; and how you will know when you have got it wrong. Present it in a viva.

Signals that you have got it wrong (learn to recognise them): the family is polite and does not return; you are doing most of the talking; you have explained their culture to them; you feel defensive; you have used the word "they".

Glossary

Cultural humility — a lifelong stance of self-examination and redress of power imbalance. **Cultural safety** — the recipient's judgment that care is safe and free of assault on identity. **Cultural formulation** — a structured account of culture in a presentation (e.g. the DSM-5 CFI). **Ethnocentrism** — judging another culture by the standards of one's own. **Intersectionality** — the interacting effects of multiple social positions. **Microaggression** — a brief, commonplace indignity, often unintentional. **Power** — the capacity to influence decisions, resources and meanings; distinct from influence. **Stereotype threat** — the risk of confirming a negative stereotype about one's group. **Structural competency** — the capacity to recognise and act on upstream social determinants.

Self-assessment

Multiple choice

1. "I treat everyone the same" is inadequate because: (a) it is impossible (b) the default is not neutral (c) clients dislike it (d) it is illegal

2. Cultural safety is judged by: (a) the therapist (b) the institution (c) the recipient of care (d) the code of ethics

3. Asking a family "what does your faith say about this problem?" is: (a) inappropriate (b) essential assessment (c) only for religious therapists (d) a legal requirement

4. An interpreter should: (a) summarise key points (b) interpret in the first person (c) advise the family (d) be the family's child

5. The social model of disability locates disability primarily in: (a) the body (b) barriers in society (c) genetics (d) family dynamics

6. A circular formulation that ignores power risks: (a) complexity (b) blaming the less powerful partner (c) over-diagnosis (d) cultural sensitivity

7. Intersectionality refers to: (a) cultural competence (b) interacting social positions (c) bilingualism (d) family structure

8. A family arrives with a co-wife arrangement. The therapist should first: (a) assess for pathology (b) endorse the arrangement (c) assess function, safety and fairness within the structure (d) refuse to work with them

9. Microaggressions in clinical settings are: (a) always intentional (b) often unintentional and still harmful (c) rare (d) not clinically relevant

10. Decolonising therapy means: (a) rejecting all Western models (b) adapting and testing models, citing local knowledge, changing the evidence base (c) using only local languages (d) avoiding diagnosis

Answer key

1 b · 2 c · 3 b · 4 b · 5 b · 6 b · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish cultural competence, humility and safety in one sentence each.
2. Write four ADDRESSING prompts you would actually use.
3. What are the six steps of working with an interpreter?
4. Give three clinical effects of racism and one assessment question.
5. Name three culturally specific assumptions in EFT or structural therapy and how you would adapt them.

Essay (choose one, 2,000 words)

1. "Cultural humility is a stance, not a technique." Discuss with reference to two clinical encounters.
2. Critically examine the claim that family therapy models pathologise interdependence.
3. Analyse a case in which your own cultural position shaped the assessment, and what you would do differently.

Further reading

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End of Semester One. Next: MFT 709 — Group Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 9 OF 28

MFT 709

Group Therapy Techniques

15 teaching weeks · 5 practicals · 1 figure

SEMESTER TWO

1.71 units · Prerequisite: MFT 706

> “Groups are the most efficient and the most unpredictable clinical format.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 709 — Group Therapy Techniques

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	**Group Therapy Techniques**	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Groups are the most efficient and the most unpredictable clinical format. This course teaches you to design a group, build a culture, work with the process rather than only the content, manage the difficult members, and co-facilitate without performing a marriage in front of the members.

How to use this book

A group cannot be learned from a book alone, but it can be ruined without one. Each week gives you the theory, a facilitation script, and a hazard. The practical sessions in this course run as a real group: you will be members before you are leaders, and you will write about both.

1. Course description

Group therapy is a distinct modality with its own theory of change: members help each other, and that mutual aid is the treatment. Family therapists use groups for couples (multiple couples in one group), parents, adolescents, men who have used violence, people in recovery, the bereaved, and children.

This course covers group design, composition, stages of development, therapeutic factors, process observation, difficult members, co-facilitation, ethics and evaluation. It is grounded in Yalom's interpersonal model and in systemic thinking about the group-as-a-system.

2. Learning outcomes

#	Outcome	Assessed
LO1	Design a group: purpose, membership, size, duration, setting	Group proposal
LO2	Explain and use Yalom's therapeutic factors	Exam
LO3	Identify and work with group process (here-and-now)	Observed facilitation
LO4	Manage difficult members and group crises safely	Viva, observed
LO5	Co-facilitate effectively, including managing disagreement	Peer and tutor rating
LO6	Screen and prepare members ethically	Screening role-play
LO7	Evaluate group outcomes	Evaluation plan

3. Assessment

Component	Weight	Due
Group design proposal (2,000 words)	25%	Week 6
Process reflection log (as member, 6 entries)	20%	Fortnightly

Component	Weight	Due
Observed co-facilitation (30 min)	30%	Weeks 12–14
Examination	25%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why groups work: mutual aid and the social microcosm	First group session (as member)
2	Types of group in family therapy	Observation of a live/recorded group
3	Design: aims, size, duration, open vs closed	Design clinic
4	Screening, consent and preparation	Screening role-play
5	Yalom's therapeutic factors	Factor-spotting exercise
6	**Group proposal due**; group stages I: forming and storming	Process observation
7	Group stages II: norming, performing, adjourning	Leader tasks by stage
8	The here-and-now: working with process	Process commentary drill
9	Difficult members: monopoliser, silent, hostile, help-rejecting	Structured rehearsals
10	Conflict, scapegoating and subgrouping in groups	Live intervention
11	Co-facilitation: division of labour, disagreement, repair	Co-therapy pairs
12	**Observed co-facilitation begins**	Assessed
13	Groups for couples and for parents	Design adaptation
14	Ethics, confidentiality and boundaries in groups	Incident analysis
15	Evaluation and endings	Outcome plan

Week 1 — Why groups work

The core claim. In a group, members are simultaneously helpers and helped. This reciprocal process — Yalom called it mutual aid — produces learning that a one-to-one relationship cannot, because it is horizontal rather than vertical. The leader is not the only source of help; the membership is.

The social microcosm. A group is a small society in which each member eventually behaves as they do outside: the man who must control, the woman who apologises, the adolescent who deflects with humour. The group then becomes a laboratory in which those behaviours are noticed, felt, and changed in vivo.

Systemic framing. A group is a system: it has boundaries (who is in, who is out), roles, norms, a developmental history, and homeostatic mechanisms that resist change.

Everything you learned about families applies. The scapegoat in a group is playing the same structural role as the identified patient in a family.

> **PRACTICAL**
> Begin the experiential group this week. The group runs for the semester with two student co-leaders rotating. Write your first reflection on what it felt like to be a member.

Week 2 — Groups in family therapy practice

Group type	Members	Typical aims
Couples group	4–6 couples	Communication skills, shared normalisation, vicarious learning
Parents group	6–10 parents	Behaviour management, support, reducing isolation
Adolescent group	6–8	Identity, peer feedback, social skills
Men's behaviour change	8–12	Accountability, gender norms, non-violence
Recovery group	8–12	Sobriety support, family reintegration
Bereavement group	6–10	Normalising grief, meaning-making
Psychoeducational	10–20	Information plus discussion
Multi-family group	4–6 families	Family–family learning (strong evidence in psychosis and eating disorders)

Multi-family groups deserve particular attention in this programme: they combine systemic work with mutual aid, are cost-effective, and have good evidence in severe mental illness, eating disorders and adolescent conduct problems.

Week 3 — Design

Eight design decisions. Each should be justified in your proposal.

1. **Aim** — what change, for whom, measured how?
2. **Membership** — who is included, who excluded, and on what criteria?
3. **Size** — 6–10 for interactional groups; larger for psychoeducation.
4. **Duration and frequency** — 8–16 sessions weekly is typical; state the rationale.
5. **Open or closed** — closed builds cohesion; open tolerates attrition.
6. **Setting and materials** — circular seating, privacy, accessibility.
7. **Leadership** — solo or co-facilitated (co-facilitation is strongly preferred for beginners).
8. **Evaluation** — pre/post measures, attendance, satisfaction, and a follow-up.

Composition traps. Mixing victims and perpetrators of violence in one group; mixing adolescents and adults; including two people in a current conflict (e.g. two co-wives, or a

couple in the same parents' group); including a member who knows another member socially without discussing it in advance.

> **PROPOSAL (25%)**
> A 2,000-word design document: population, aims, evidence base, membership criteria, screening protocol, session-by-session outline, facilitation plan, risk protocol, evaluation plan, budget.

Week 4 — Screening, consent and preparation

Screening protects the group as much as the individual. Screen for: fit with the group's aims, current risk, ability to tolerate group feedback, concurrent individual therapy, and practical commitment.

The screening interview (30 minutes).

1. What brings you here, and what do you hope for?
2. What has helped before? What has not?
3. Have you been in a group before? How was it?
4. How do you usually handle conflict and criticism?
5. Is there anyone in the group you know? (Show the member list where appropriate.)
6. Are you currently safe? (Risk screen.)
7. Are you willing to keep confidentiality, knowing it cannot be guaranteed?

Consent must be specific. Explain that confidentiality is a group norm but not a guarantee; that members may be upset by others; that the leaders will break confidentiality only in the circumstances described; and that attendance and punctuality are part of the contract.

Preparation improves outcomes. Tell the member what to expect, what will be asked of them in the first session, and how to introduce themselves. Pre-group preparation reduces drop-out measurably.

> **PRACTICAL**
> Screening role-play with a volunteer who is ambivalent and has been pressured to attend. Practise: exploring ambivalence, explaining the frame, and declining a poor fit without rejecting the person.

Week 5 — Yalom's therapeutic factors

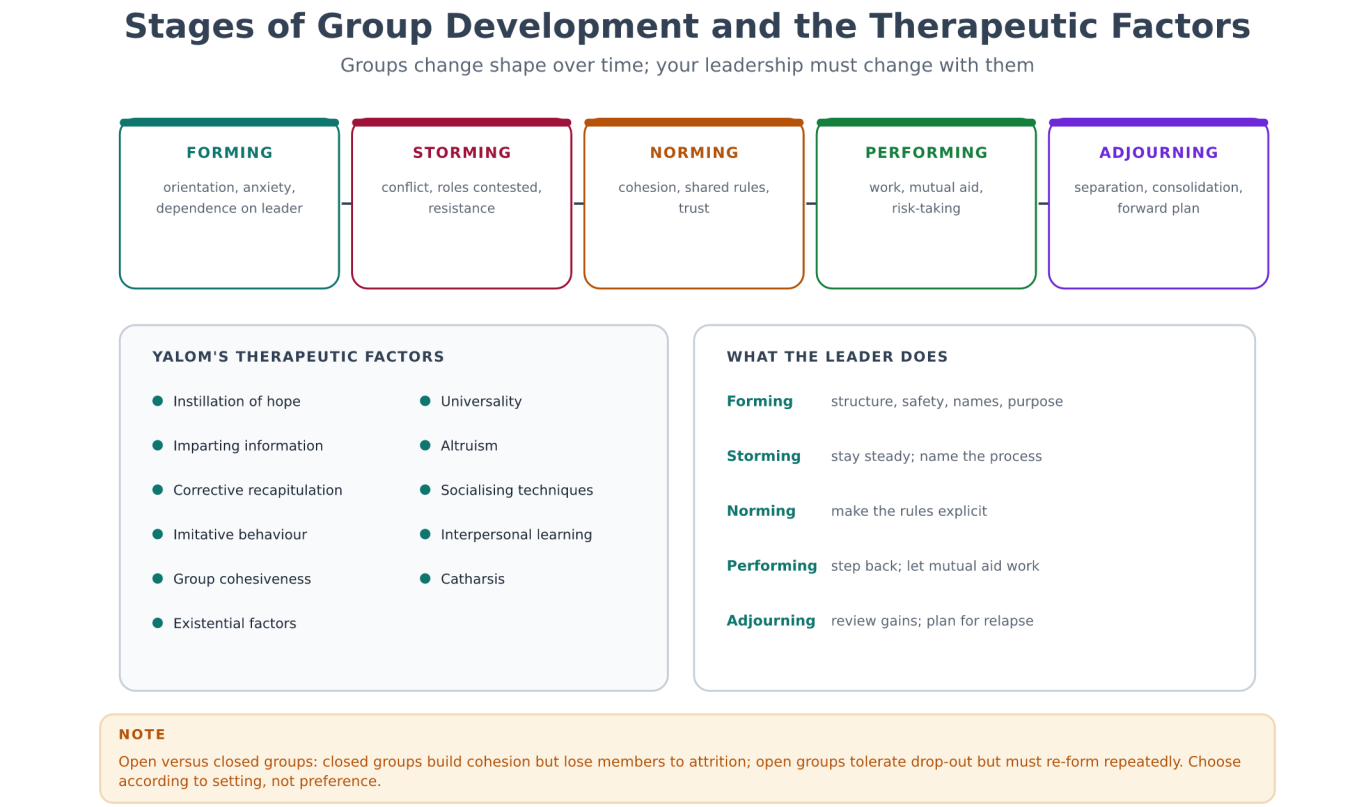


Figure 5.1 — Group stages, therapeutic factors and the leader's changing task

Factor	What it looks like	How the leader strengthens it
Instillation of hope	A member describes recovery	Ask for specifics; keep the member connected to the group
Universality	"I thought I was the only one"	Name the shared experience explicitly
Imparting information	Advice, psychoeducation	Redirect advice-giving into dialogue
Altruism	A member helps another	Point out the help; let the helper be seen
Corrective recapitulation	Old family patterns reappear	Interpret the pattern in the here-and-now
Socialising techniques	Learning social feedback	Model and reinforce it
Imitative behaviour	Modelling the leader or a member	Use yourself deliberately
Interpersonal learning	Feedback changes behaviour	Make the group the site of the experiment
Group cohesiveness	Belonging, attendance, care	Protect the frame; address absences
Catharsis	Emotional release	Contain it; link it to meaning
Existential factors	Responsibility, mortality, isolation	Allow the conversation; do not rush to technique

The beginner's error: chasing catharsis. Tears without integration produce relief for a day and no change. The sequence is: **express → understand → experiment → consolidate**.

Week 6 — Stages I: forming and storming

Forming. Anxiety, politeness, dependence on the leader, search for structure. Members ask: "What are we supposed to do? Is this allowed?"

Leader task: provide structure and safety, model disclosure at an appropriate level, establish norms (punctuality, confidentiality, one person at a time, no advice without asking), and name the anxiety.

Script: "Most groups start quietly. Part of what we will do is notice what we are all avoiding."

Storming. Conflict, role competition, resistance, criticism of the leader, subgrouping. This is not failure; it is the group testing whether it can survive difference.

Leader task: stay steady, do not defend, name the process, and let the group negotiate. Avoid solving it for them.

Script: "There is something happening between the two of you that the rest of us are watching. Can we look at it together rather than settle it?"

> **PRACTICAL**
> Process observation. Two observers code the session for: who speaks, who is interrupted, who is silent, and three moments of process (as opposed to content). Feed back to the group in the last ten minutes.

Week 7 — Stages II: norming, performing, adjourning

Norming. Cohesion, shared rules, mutual trust, humour. Attendance stabilises.

Leader task: consolidate the norms explicitly; use the cohesion to do deeper work.

Performing. The productive stage: mutual aid, confrontation with care, risk-taking, insight.

Leader task: step back. Your best sessions are the ones in which you spoke least and the group worked. Redirect all content to the members: "What does the group think?"

Adjourning. Ending, loss, consolidation, transfer to life outside.

Leader task: review gains, name the loss, have each member say what they gave and received, and make a forward plan. Do not let the group end by attrition.

Termination script: "We have four sessions left. I want us to use them for two things: what each of you has learned, and what each of you will do when the group is not here."

Week 8 — The here-and-now

Two layers. Content is what the group is talking about; process is what is happening between the people while they talk about it. Groups change through process.

The four-step process commentary.

1. **Notice** something concrete: "Every time Ada speaks about her marriage, Femi looks at the clock."
2. **Name it to the group, not about the person:** "I notice something happening here."
3. **Ask the group to look:** "Does anyone else notice it? What do you think it is about?"
4. **Return it to the relationship:** "Femi, what is happening for you when Ada speaks?"

Timing rules. Do not process-comment in the first two sessions (no safety yet). Do not do it to demonstrate skill. Do it when the same pattern has occurred at least twice.

Hazards. Process commentary can feel like exposure. Use the group as the buffer ("does anyone else…"), avoid naming individuals as specimens, and never interpret someone's process before you have checked it with them.

> **DRILL**
> Ten minutes of "process-only": the group may only speak about what is happening here, not about events outside the room. Debrief how uncomfortable it was.

Week 9 — Difficult members

Member type	Behaviour	What it protects	Intervention
Monopoliser	Talks at length; fills silence	Anxiety, fear of invisibility	"I want to bring others in; you may find that hard." Set a turn-taking norm
Silent member	Says nothing; attends	Fear of exposure, cultural deference	Low-demand invitation; do not force; check in privately
Hostile/devaluing	Attacks the leader or a member	Shame, previous betrayal	Name impact; set the boundary; explore what it protects
Help-rejecting complainer	Asks for help then rejects it	Control, fear of change	Move from advice to the function: "What would it cost you to accept help?"
Group mascot	Defuses everything with humour	Avoids grief	"Your jokes are welcome. I also want to know what is underneath this one."
Scapegoat	Becomes the group's problem	Carries disowned material	Protect; interpret the group's function in selecting them
Subgroup	Two or three form a bloc	Safety in numbers	Bring it into the open; do not let it be secret

The principle. Every difficult behaviour is a solution the member has found for a problem. Your task is to understand the problem, not to eliminate the behaviour by force.

Week 10 — Conflict, scapegoating and subgrouping

Escalation in groups is faster than in dyads because of contagion. Interrupt earlier than you would in a family session.

The scapegoat sequence (learn to spot it): a member is different → the group's anxiety attaches to them → the group pressures them → the leader is invited to condemn them → if the leader agrees, the group learns that difference is dangerous.

Leader intervention: name the process, protect the member, and give the group the work: "I notice we have all become very interested in what Femi is doing wrong. What is it about him that makes us comfortable focusing there?"

Subgroups. Two members who meet outside, or a bloc that votes together. Name it without accusation: "I am aware that some of you talk outside. That is allowed, and it affects what happens here. What shall we do about it?"

Week 11 — Co-facilitation

Why co-facilitate. Two sets of eyes, two relational styles, a model of a working relationship, and protection when one of you is triggered.

Division of labour — decide before every session.

- Who opens and closes?
- Who tracks process, who tracks content?
- Who handles risk or a member leaving the room?
- How will you signal a disagreement in the moment?

Managing disagreement in front of the group. Never contradict your co-leader without acknowledging it. Options: (a) defer and discuss after; (b) name the difference openly and model resolution: "I see it differently, and I want the group to hear both." Naming it well is a gift; naming it badly is a rupture.

The classic co-therapy failure: the couple dynamic. Two co-leaders who replicate a parent–child or pursue–withdraw pattern in front of the group. The group will notice and may act it out. Take it to supervision immediately.

> **PRACTICAL**
> Co-therapy pairs plan a session in ten minutes using the division-of-labour checklist, then facilitate with the observed group. The observer records every moment the pair were out of sync.

Week 12 — Observed co-facilitation

Thirty minutes of an experiential group, co-facilitated, assessed against: safety and structure; use of process; inclusion of silent members; containment of difficult behaviour; co-leader coordination; and closing.

Week 13 — Couples and parents groups

Couples group. Four to six couples. Strengths: normalisation ("other couples have this too"), vicarious learning, and reduced shame. Risks: public conflict, competition, and one partner feeling ganged up on.

Structure: 15 minutes check-in (one sentence per person), 20 minutes teaching, 30 minutes exercise (couples work together while leaders circulate), 15 minutes group debrief. Never ask a couple to do a deep vulnerability exercise in front of others before they have done it alone with you.

Parents group. Strengths: reducing isolation, shared problem-solving. Risks: one parent becoming the group's expert, and advice-giving replacing reflection.

Structure: check-in, one skill, one application, one commitment. Keep advice horizontal: "Who has tried something that worked?"

Week 14 — Ethics and boundaries in groups

Issue	Standard
Confidentiality	Explain it is a norm, not a guarantee; revisit when a member leaves
Screening	Never skip it; document it
Dual relationships	Members who know each other; leaders who know members socially
Touch	Discourage between members; use only with explicit norms
Out-of-group contact	Agree a policy; do not become a member's confidant
Recordings	Separate consent from each member; a member may withdraw at any time
Risk disclosure	Leaders follow their duty of care; tell the group if confidentiality must be broken

The most common ethical complaint in groups is not dramatic: it is a member learning that something they said was repeated outside. Prevention: explicit norms, a written agreement, and a leader who models discretion.

Week 15 — Evaluation and endings

Minimum evaluation package. Attendance and drop-out (with reasons), a pre/post measure of the target problem, a session-by-session satisfaction rating, and a follow-up at 8–12 weeks.

Writing up. Report the design, the actual membership, the adaptations you made, the outcome and the limitations. A well-reported small group is a contribution; an unreported one is a private experience.

Glossary

Cohesion — the group's sense of belonging and mutual commitment; a precondition for productive work. **Group-as-a-whole** — the group considered as a single system with its own dynamics. **Here-and-now** — the immediate interpersonal process in the room, as distinct from outside content. **Mutual aid** — the reciprocal helping that occurs between members. **Process** — the relational layer of communication, as distinct from content. **Scapegoating** — the group's displacement of anxiety onto one member. **Social microcosm** — the group as a small society in which members' characteristic patterns appear. **Subgrouping** — the formation of alliances within the group. **Therapeutic factor** — a mechanism of change identified by Yalom (e.g. universality, cohesiveness).

Self-assessment

Multiple choice

1. Mutual aid refers to: (a) the leader helping members (b) members helping each other (c) financial support (d) group cohesion

2. Scapegoating in a group usually functions to: (a) help the scapegoat (b) absorb the group's anxiety (c) improve cohesion (d) test the leader

3. Pre-group preparation: (a) is optional (b) reduces drop-out and improves outcome (c) increases anxiety (d) is only for research

4. The leader's main task in the performing stage is to: (a) direct more (b) step back and let mutual aid work (c) introduce new rules (d) end the group

5. "Here-and-now" work means: (a) discussing childhood (b) working with immediate in-room process (c) using mindfulness (d) homework review

6. Confidentiality in groups is: (a) guaranteed (b) a norm that cannot be guaranteed (c) irrelevant (d) the leader's legal duty only

7. A closed group: (a) admits new members (b) has fixed membership (c) has no rules (d) is always shorter

8. Catharsis without integration typically produces: (a) lasting change (b) temporary relief (c) increased cohesion (d) no effect at all

9. The best composition decision when two members are in an active conflict is to: (a) include both to work it out (b) avoid placing them in the same group (c) seat them together (d) ask them to sign a contract

10. Parallel process in co-facilitation means: (a) the leaders take turns (b) the leaders' relationship replicates a family pattern (c) members copy the leaders (d) two groups run at once

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List five design decisions and the rationale for each.
2. Name five therapeutic factors and one leader behaviour that strengthens each.
3. Give the four steps of process commentary with an example.
4. How would you intervene with a monopoliser without shaming them?
5. What are the four elements of a group evaluation plan?

Essay (choose one, 1,800 words)

1. Design a couples group for eight distressed couples, justifying every design decision.
2. "The group is the treatment, not the leader." Discuss with reference to mutual aid and process.
3. Analyse a difficult moment in your experiential group and set out what you would do differently as leader.

Further reading

- Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books.
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Next: MFT 710 — Couples Therapy

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MFT 710

Couples Therapy

16 teaching weeks · 1 practical · 4 worked cases · 6 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 702, 706

> “Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 710 — Couples Therapy

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 16 teaching weeks · Prerequisite: MFT 702, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	**Couples Therapy**	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them. This course integrates attachment-based (EFT), behavioural (TBCT/IBCT) and systemic approaches, and takes infidelity, violence, separation and cultural context seriously.

How to use this book

Couple work is where the therapist's own relationship history is most easily activated. The reflective tasks in this book are not optional extras; they are the difference between a clinician who can hold two perspectives at once and one who quietly takes sides. Do them.

1. Course description

The course covers: assessment of the couple (including individual sessions and violence screening), the major evidence-based models (EFT and behavioural couple therapy), the Gottman research programme, communication training, infidelity and betrayal trauma, sexual problems, separation and divorce work, and cultural and religious context.

Three frameworks are integrated: **attachment** (why the bond matters), **behaviour** (what maintains the pattern), and **system** (how the pattern organises itself).

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a couple assessment safely, including IPV screening and individual sessions	Observed assessment
LO2	Formulate the couple's negative cycle in attachment terms	Case assignment
LO3	Deliver core EFT and behavioural interventions under supervision	Observed sessions
LO4	Work with infidelity, including disclosure, meaning and rebuilding	Case study
LO5	Address sexual concerns within the couple frame	Exam
LO6	Manage separation, divorce and stepfamily transitions ethically	Case study
LO7	Adapt couple work to cultural, religious and extended-family context	Viva

3. Assessment

Component	Weight	Due
Couple assessment observed (40 min)	25%	Week 7
Formulation and treatment plan (2,000 words)	25%	Week 10
Recorded or live session with analysis (30 min)	30%	Week 15

Component	Weight	Due
Examination	20%	Week 16

4. The sixteen-week plan

Week	Topic	Practical
1	What couples want and what therapy offers; the evidence	Map a couple's goals
2	Assessment I: structure, history, and the couple narrative	Intake role-play
3	Assessment II: individual sessions, IPV screening, secrets	Screening rehearsal
4	The negative cycle: pursue–withdraw and its variants	Cycle mapping
5	Attachment theory applied to couples	Attachment interview
6	EFT I: tracking and de-escalation	Enactment drill
7	**Observed couple assessment**	Assessed
8	EFT II: restructuring and softening events	Softening rehearsal
9	Behavioural couple therapy: exchange and communication	Speaker–listener
10	**Formulation and plan due**; the Gottman programme	Four horsemen practice
11	Infidelity and betrayal trauma	Disclosure planning
12	Sex and intimacy in couple therapy	Sensate focus design
13	Violence, control and when couple therapy is contraindicated	Safety planning
14	Separation, divorce, co-parenting and stepfamilies	Co-parenting mediation
15	Culture, faith and family-of-origin in couple work	Cultural adaptation
16	Consolidation, relapse prevention and ending	**Session analysis due**

Week 1 — What couples want, and what therapy offers

Couples arrive with a presenting complaint (money, sex, in-laws, phone) and an underlying bond question ("are you there for me?"). The complaint is the doorway; the bond is the room.

The evidence, briefly. Couple therapy is effective for relationship distress with moderate-to-large effects. Behavioural couple therapy, cognitive-behavioural couple therapy, and emotionally focused therapy have the strongest evidence base. Emotionally focused therapy has the most consistent evidence for durability, including in high-distress couples.

What predicts a good outcome. Early change, motivation of both partners, shared definition of the problem, low coercive control, and — consistently — the quality of the shared therapeutic alliance.

What predicts failure. Coercive control, undisclosed affairs that surface mid-treatment, untreated addiction, severe individual psychopathology, and one partner who has already decided to leave but has not said so.

> **REFLECT**
> What did your own family teach you about conflict, repair, and what a marriage is for? Write 400 words. Keep it; you will return to it in Week 13.

Week 2 — Assessment I

Structure of a couple assessment (2–3 sessions).

1. **Joint session:** both partners' accounts, the history of the relationship, the current pattern, and the goals.
2. **Individual sessions:** one with each partner — risk screen, secrets policy, individual history, ambivalence, and "is there anything you would not say in front of your partner?"
3. **Feedback session:** formulation, plan, and the agreement.

What to assess.

Domain	Questions
Commitment	"Is there any part of you that has already decided to leave?"
History	How did you meet? What drew you? What was good in the early years?
Current pattern	"Walk me through last Tuesday's argument, minute by minute."
Attempted solutions	What have you tried? What happened?
Family of origin	Genogram of both families; how each family handled conflict, affection, money
Context	Work, money, migration, in-laws, children, faith, health
Strengths	What still works? What do you still enjoy?
Risk	Violence, coercive control, suicide, substance use, child safety

Ask about the good. Couples in distress systematically under-report positives; a careful strengths inventory is both assessment and intervention.

Week 3 — Individual sessions, IPV screening and secrets

Always see each partner alone at least once. This is where safety is disclosed.

Screening questions (ask directly, in private).

- "Have you ever felt afraid of your partner?"
- "Has your partner ever hit, pushed, slapped, or thrown something at you?"

- "Has your partner ever forced or pressured you sexually?"
- "Does your partner control your money, your phone, or who you see?"
- "If you went home tonight and told him you had said all this, what would happen?"

If coercive control is present: stop conjoint work; safety-plan individually; refer to specialist services; document. Conjoint work can increase danger and can silence the victim in the room.

Secrets policy. State it in the first joint session: "If one of you tells me something individually that affects our work together, I will not keep it as a secret. I will help you decide how and when to tell your partner, and if you cannot, I will ask us all to talk about it."

Ambivalence. One partner may already be planning to leave. Ask: "Where are you on a scale from 'I am fully committed to working on this' to 'I have already decided'?" The answer changes the treatment.

Week 4 — The negative cycle

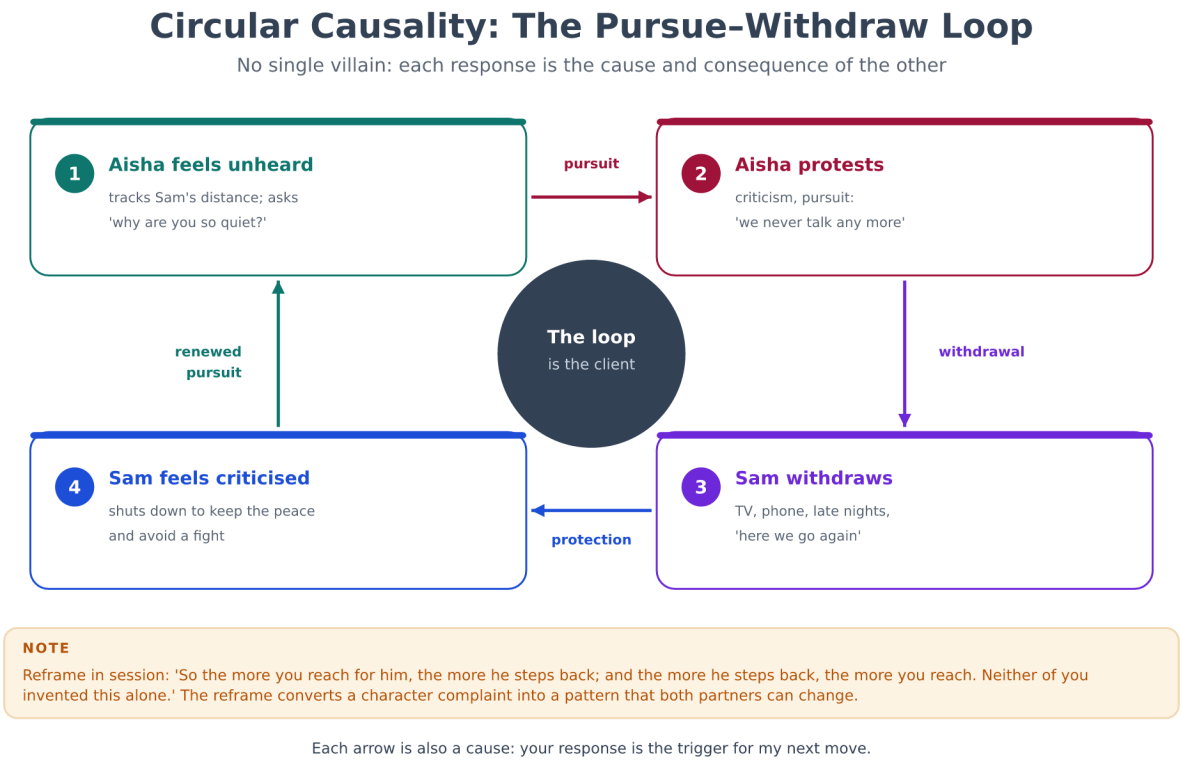


Figure 4.1 — Circular causality: the pursue–withdraw loop

The three common cycles.

1. **Pursue–withdraw.** One escalates to get a response; the other shuts down to keep the peace. The pursuing increases, the withdrawal deepens.

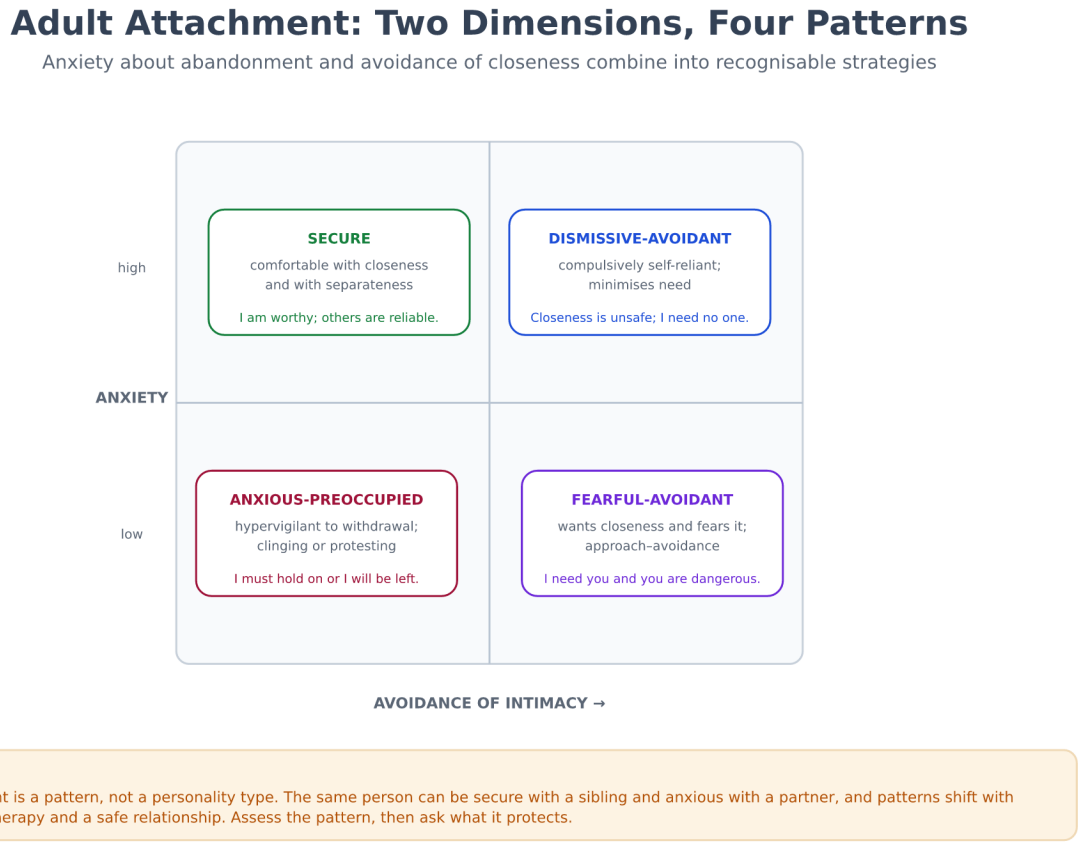
2. **Attack–defend.** Both escalate; complaint is met with counter-complaint; the volume rises and the content becomes irrelevant.
3. **Mutual withdrawal.** Both avoid; the relationship becomes parallel and polite; loneliness without conflict — often the hardest to treat because there is no presenting crisis.

Mapping the cycle in session. Ask each partner for the last three arguments and write the sequence on paper as a loop. Then read it back and ask: "Is this recognisable?" The moment of recognition — often accompanied by a laugh or a sigh — is the therapeutic turn.

The reframe. "The problem is not that you are angry, or that you are cold. The problem is the loop between you, and the loop has a life of its own."

> **PRACTICAL**
> With a role-played couple, map the cycle on a whiteboard in 15 minutes and get both partners to agree with the drawing. The agreement, not the drawing, is the skill.

Week 5 — Attachment applied to couples



> **NOTE**
> Attachment is a pattern, not a personality type. The same person can be secure with a sibling and anxious with a partner, and patterns shift with context, therapy and a safe relationship. Assess the pattern, then ask what it protects.

Figure 5.1 — Adult attachment: two dimensions and four patterns

Core idea. Adult romantic love is an attachment process. Partners monitor accessibility and responsiveness; when the bond feels threatened, protest behaviours appear (criticism, pursuit, withdrawal, numbing), and those protest behaviours are usually the opposite of the need they express.

Two dimensions. Anxiety (about abandonment) and avoidance (of intimacy). Their combination produces four patterns, each with a characteristic strategy and core belief (see figure).

Clinical use.

- Anxious partner: the protest is a request for reassurance. Help them ask for it directly rather than through criticism.
- Avoidant partner: withdrawal is a protection against overwhelm and shame. Help them stay present for longer and return rather than disappear.
- Both: the cycle is an attachment problem, not a character problem.

The attachment question to hold in your head. "What is this person asking for, and what are they doing instead?"

Week 6 — EFT I: tracking and de-escalation

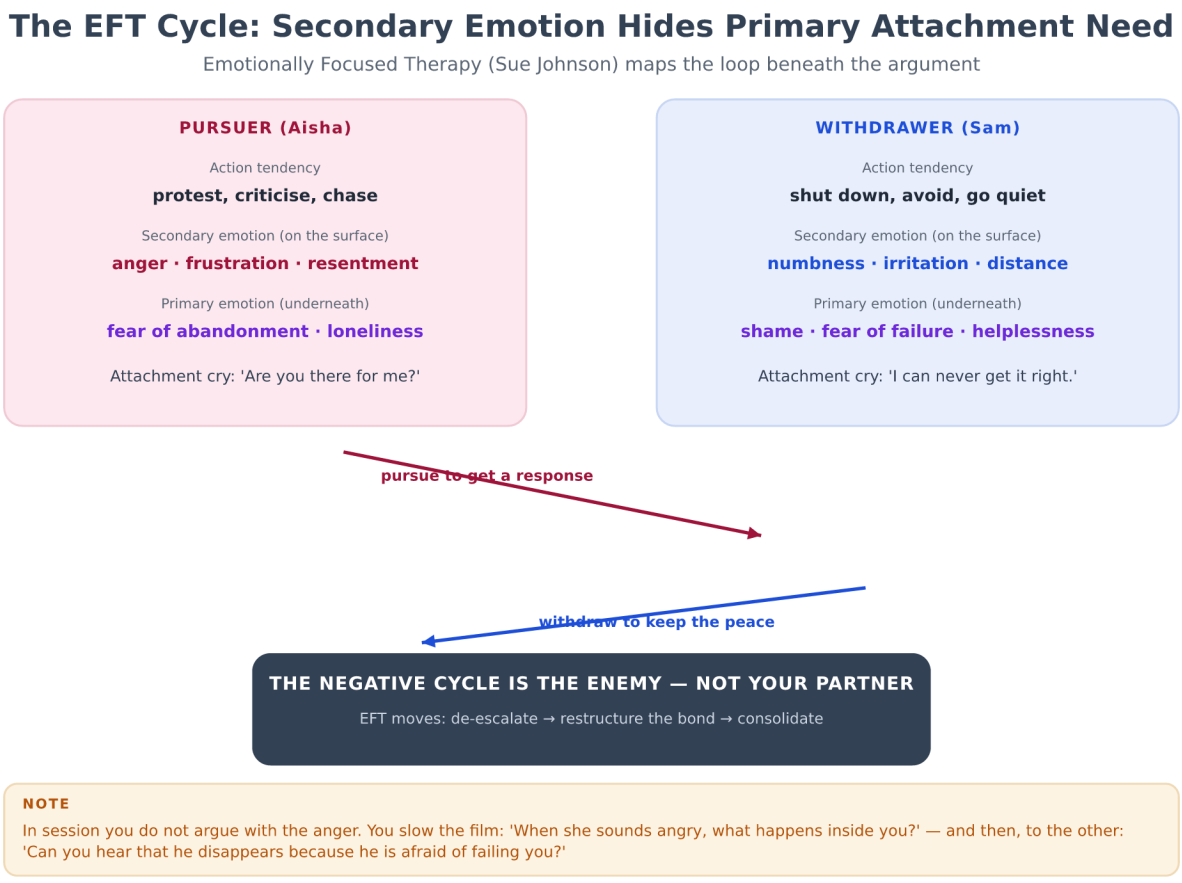


Figure 6.1 — The EFT cycle: secondary emotion and primary attachment need

Stage 1 (de-escalation, sessions 1–8). The goal is that both partners can describe the cycle and see it as the enemy.

Four moves.

1. **Track the cycle** in specific incidents. Slow the film; ask for bodily experience.
2. **Access primary emotion.** "Under the anger — is there something like fear?" Name it tentatively; let them correct you.
3. **Reframe the problem as the cycle.** "Neither of you is the problem. This dance is."
4. **Name the attachment cry.** "So when you say 'we never talk', what you mean is 'I miss you and I am afraid you have gone'."

A worked enactment (Week 6 lab).

> **THERAPIST (TO AISHA):**
> When you get loud, what happens inside your body? **Aisha:** My chest. Like I cannot breathe until he answers. **Therapist:** And if you did not get loud — what would he see? **Aisha:** (pause) That I am frightened. **Therapist:** Can you tell him that? Look at him. "I get loud because…" **Aisha:** I get loud because I think you have already left. **Therapist (to Sam):** Sam, stay with that. What is happening for you hearing it?

Hazard. Primary emotion must be reached, not demanded. If you push a client to feel something in front of you before safety, you will get a performance or a shutdown.

Week 7 — Observed couple assessment

Forty minutes: contract, both accounts, cycle map, risk screen (in the individual sessions if scheduling permits), strengths, and feedback. Assessed on safety, structure, balance (did each partner get equal airtime?), and formulation quality.

Week 8 — EFT II: restructuring

Stage 2 (sessions 9–20). Withdrawal re-engagement; pursuer softening; enactment of new responses.

Softening event — the classic sequence.

1. The pursuer accesses the primary attachment fear and expresses it non-accusatorially.
2. The withdrawer hears it, stays present, and responds with care.
3. The pursuer risks reaching again from a position of vulnerability rather than protest.
4. Both experience a new pattern in the room.

Therapist tasks. Keep the withdrawer engaged ("stay with me, this is the hard part"); block the pursuer's escalation without silencing the need; and slow everything down.

Stage 3 (consolidation). Integrate the new pattern, problem-solve practical issues, and rehearse recovery from relapse: "When the cycle comes back — and it will — what will you do in the first hour?"

Week 9 — Behavioural couple therapy

The behavioural view. Distress is maintained by (a) a low rate of positive exchange, (b) poor communication under conflict, and (c) attributions that make neutral behaviour hostile.

Core interventions.

1. **Behavioural exchange.** Each partner lists 15–20 small caring behaviours; select three each; do them daily without being asked. Not transactional bookkeeping — an increase in the density of positive behaviour.
2. **Communication training.** Speaker–listener with a floor card, then problemsolving.
3. **Attribution retraining.** "When he is late, what do you assume? What else could be true?"
4. **Acceptance work (IBCT).** Some differences will not change; the work is to soften the emotional reactivity to them through empathic joining and tolerance building.

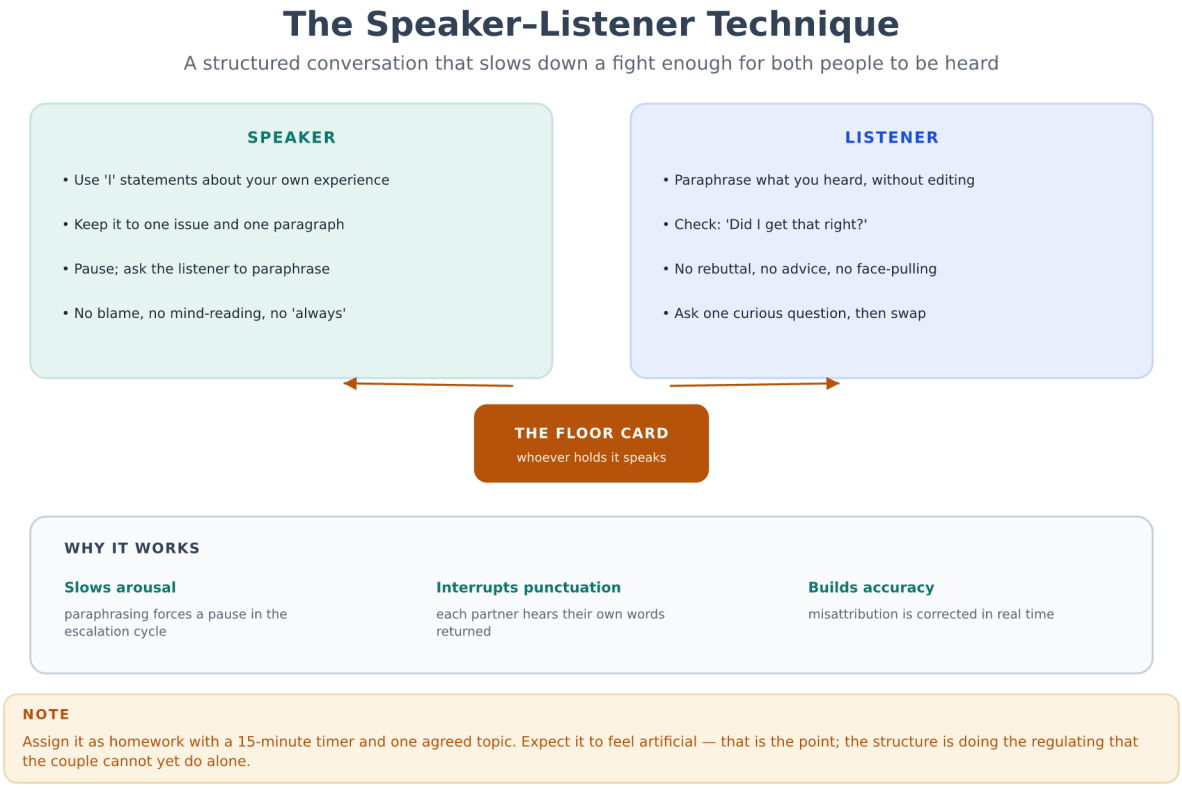
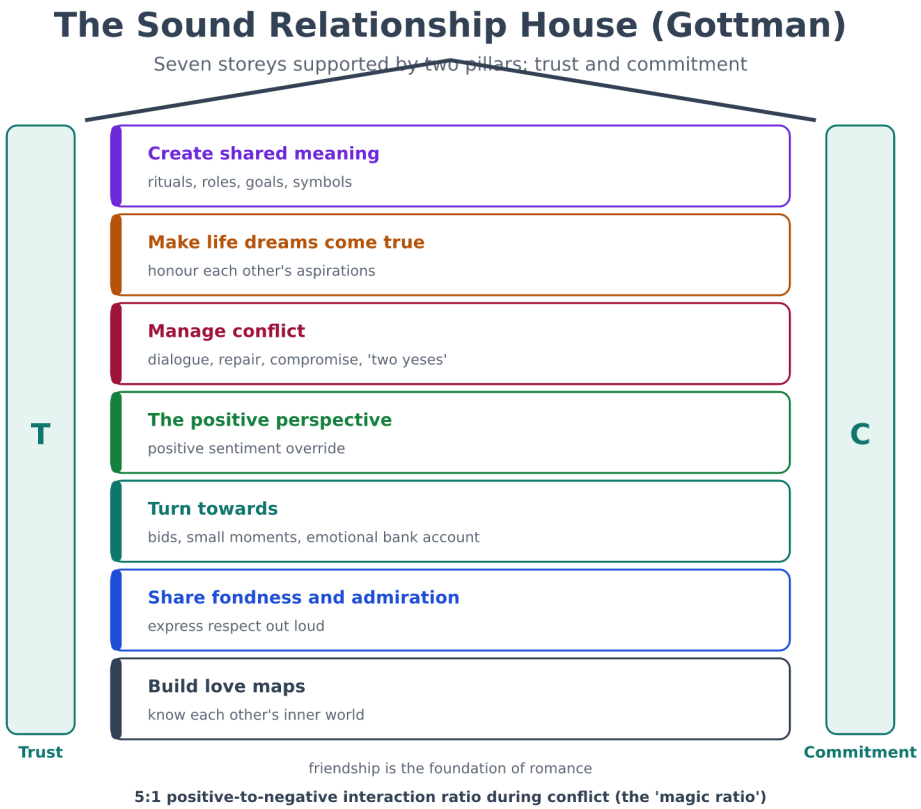


Figure 9.1 — The speaker–listener technique

Sequencing. Positives first (exchange), then communication, then problem-solving. Couples cannot solve problems they cannot discuss.

Week 10 — The Gottman research programme



NOTE

Adaptation caution: Gottman's norms were derived largely from U.S. samples. Use the structure of the house as a clinical map; do not treat the 5:1 ratio as a universal law.

Figure 10.1 — The Sound Relationship House

Findings worth using.

- **The four horsemen** — criticism, contempt, defensiveness, stonewalling — predict dissolution (Figure 14). Contempt is the strongest predictor.
- **The 5:1 ratio** of positive to negative interaction during conflict.
- **Bids for connection** — small moments of "turning towards" build the emotional bank account.
- **Love maps** — detailed knowledge of the partner's inner world.
- **Repair attempts** — the capacity to de-escalate mid-conflict is more predictive than the absence of conflict.
- **Physiological flooding** — above roughly 100 bpm, people cannot listen; a break is a skill, not an escape.

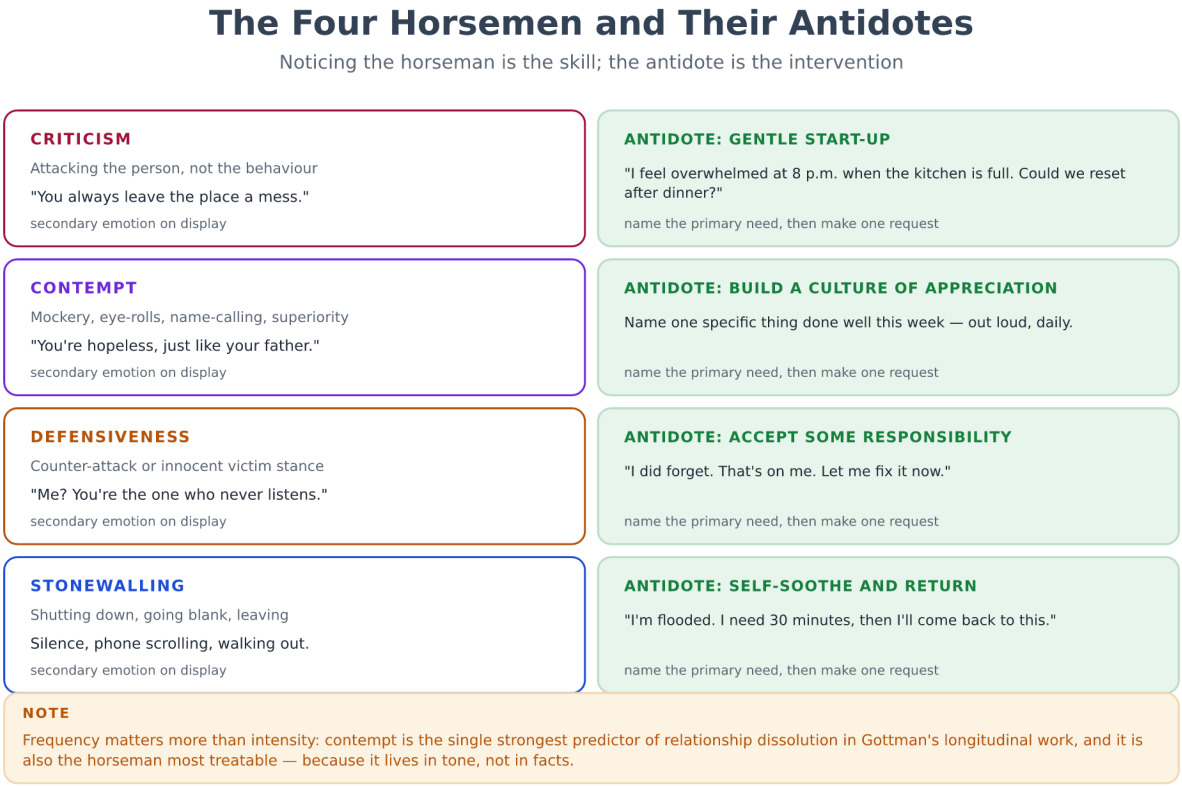


Figure 10.2 — The four horsemen and their antidotes

Adaptation caution. The norms come largely from US samples; treat the structure as a clinical map, not as a universal law. A couple who argue loudly and repair well is not in trouble because of the volume.

Week 11 — Infidelity and betrayal trauma

Two clinical events, often confused.

1. **The disclosure** — an acute crisis requiring containment, safety and structure.
2. **The betrayal trauma** — an injury to the betrayed partner's capacity to trust and to make sense of reality, with symptoms resembling PTSD (intrusion, hypervigilance, dissociation).

Phase 1: stabilise (weeks 1–4).

- Stop the affair (or the contact) — this is usually a precondition, not a negotiation.
- Establish safety: no violence, no coercion, agreed boundaries on phone and whereabouts.
- Contain the disclosure: the betrayed partner has a right to the facts, not to every detail; agree what will be asked and answered.
- Address STI testing and any financial/legal exposure.
- Individual sessions to assess each partner's capacity to continue.

Phase 2: meaning (weeks 4–12).

- What did the affair do for the unfaithful partner? (Not "why did you do it?" but "what was it in service of?")
- What was missing that could have been spoken? (Careful: explanation is not justification.)
- What does the betrayed partner need in order to consider rebuilding — transparency, remorse, time, consistency?

Phase 3: rebuilding or separating. Not every couple should stay together. Your task is to help them decide with full information, not to preserve the marriage at all costs.

> **CAUTION**
> Do not accept a couple for affair work if the unfaithful partner will not end the affair and will not disclose anything; you will be running a therapy that cannot progress and colluding in a lie.

Week 12 — Sex and intimacy

Start with permission, then information, then specific suggestions (Annon's PLISSIT model).

Assessment. Desire, arousal, orgasm, pain; frequency and satisfaction for each partner; the last time it was good; what changed; medical factors (medication, illness, menopause, postpartum); the couple's beliefs; and the meaning of sex in the relationship.

Common presentations and the couple-level formulation.

Presentation	Individual factor	Relational factor
Low desire in one partner	Fatigue, medication, depression, pain	Pursuit and pressure; unresolved resentment
Erectile difficulty	Vascular, hormonal, anxiety	Performance focus; fear of judgement
Painful intercourse	Medical cause must be excluded	Fear, poor communication, coercion
Mismatched desire	Cycles differ normally	Negotiation failure; rejection sensitivity

Intervention basics. Psychoeducation (desire discrepancy is the most common sexual complaint, not a sign of incompatibility); sensate focus (graded, non-goal-oriented touch); scheduling; reducing performance pressure; addressing the relational context; and medical referral when indicated.

Cultural and religious framing. Sexual expectations are among the most culturally loaded topics you will discuss. Ask what each partner was taught, what they believe is permitted, and whether they can say no to each other. Consent within marriage is consent.

Week 13 — Violence, control and contraindications

The distinction that matters (from MFT 704):

Situational couple violence	Coercive control
Mutual, escalating under stress	One-directional domination
No fear outside conflict	Chronic fear, surveillance, isolation
Couple therapy possible after assessment	Couple therapy contraindicated
Safety plan for both	Specialist advocacy for the victim

If a couple presents with violence. See each partner individually first. Ask about fear, injury, sexual coercion, threats, and escalation. Do not use conjoint sessions to "work through" violence; do not ask the victim to say in front of the perpetrator what he did, unless she chooses and it is safe.

REFLECT — Return to your Week 1 reflection. Where in this material are you most likely to lose balance — with the victim, with the perpetrator, with the unfaithful partner, with the one who wants to leave?

Week 14 — Separation, divorce and stepfamilies

Discernment counselling. Some couples are not deciding whether to repair the relationship but whether to end it. A structured, time-limited process (about 4–6 sessions) helps a couple reach a clear decision rather than drifting. Both outcomes — a renewed commitment or a respectful ending — are successes.

When the decision is to separate.

- Help the couple tell the children together, with a simple, blame-free message.
- Refer for legal advice on custody, maintenance, property and any customary or religious processes.
- Address co-parenting: the relationship continues as a parenting partnership.
- Manage the grief: divorce is a loss, even when it is a relief.

Stepfamilies. The central error is treating the stepfamily as a nuclear family that arrived late. It is a different structure with different tasks: the couple bond must be built before stepparent authority is imposed; children need permission to love both households; and the "insider/outsider" split (biological parent and child versus stepparent) must be named.

Week 15 — Culture, faith and family of origin

Questions to ask every couple.

- What did each of your families teach you about marriage, money, sex, and in-laws?
- Who has authority in your household, and who decided that?

- What does your faith require of each of you?
- Which of your families is closer, and what does that closeness cost the other?
- If your parents disagreed with a decision you made, what would happen?

Common cultural configurations. Bride price and its obligations; polygyny; co-residence with parents; gender-role expectations; religious courts and counselling; migration and split households; inter-ethnic and inter-faith marriages.

The rule. Do not treat the cultural pattern as the problem, and do not treat it as beyond discussion. Ask the couple what they want within it.

Week 16 — Consolidation and ending

Write the couple a one-page **relapse prevention plan**: their cycle in their own words, three early warning signs, two repair moves that work for them, and the agreement about when to return. Review the gains explicitly, name the risk of recurrence without frightening them, and end on time.

Glossary

Attachment injury — a betrayal or abandonment that damages trust and requires explicit repair. **Betrayal trauma** — trauma-like symptoms following the discovery of a partner's deception. **Bids for connection** — small attempts to gain a partner's attention or affection. **Coercive control** — a pattern of domination and isolation, distinct from situational conflict. **Negative cycle** — the recurrent pattern of interaction that organises couple distress. **Pursue–withdraw** — a cycle in which one partner escalates while the other distances. **Softening** — in EFT, the pursuer's shift from protest to vulnerable expression. **Sentiment override** — the tendency for the relationship's overall tone to colour the interpretation of specific events. **Sensate focus** — graded, non-goal-oriented touch exercises used in sex therapy.

Self-assessment

Multiple choice

1. The first step in assessing a couple is: (a) communication training (b) assessment including individual sessions and IPV screening (c) homework (d) diagnosis

2. In EFT, criticism is usually understood as: (a) abuse (b) protest at perceived abandonment (c) personality (d) a communication deficit

3. Couple therapy is contraindicated when: (a) the couple argues (b) there is coercive control (c) there is low desire (d) one partner is depressed

4. The strongest predictor of relationship dissolution in Gottman's research is: (a) conflict frequency (b) contempt (c) low income (d) different interests

5. A 5:1 ratio refers to: (a) sessions per month (b) positive to negative interactions during conflict (c) partners to therapist (d) years of marriage

6. In affair work, the precondition for progress is usually: (a) forgiveness (b) ending the affair and establishing transparency (c) a legal separation (d) a new sexual contract

7. "Softening" in EFT refers to: (a) lowering the volume (b) the pursuer expressing primary attachment emotion (c) the therapist's tone (d) reducing homework

8. Physiological flooding is significant because: (a) it indicates anger (b) above a threshold, listening becomes physiologically difficult (c) it predicts divorce (d) it requires medication

9. A key task in stepfamilies is to: (a) impose stepparent authority quickly (b) build the couple bond before stepparent authority (c) treat all children equally in discipline (d) avoid the ex-partner

10. Discernment counselling helps couples to: (a) stay together (b) reach a clear decision about the relationship (c) divorce quickly (d) avoid decisions

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Give the three-session structure of a couple assessment and the purpose of each session.
2. Write your confidentiality policy for secrets in couple therapy, in six sentences.
3. Describe the pursue–withdraw cycle in attachment terms.
4. Name three contraindications to conjoint couple therapy.
5. What are the phases of infidelity work, and what happens in each?

Essay (choose one, 2,000 words)

1. Compare EFT and behavioural couple therapy for a highly distressed couple with a pursue–withdraw cycle.
2. "Infidelity is a symptom and an injury." Discuss the treatment implications.
3. Critically evaluate the cultural assumptions embedded in the concept of "differentiation" in couple therapy.

Further reading

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- Epstein, N. B., & Baucom, D. H. (2002). Enhanced Cognitive-Behavioral Therapy for Couples. APA.
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- Lebow, J., & Snyder, D. K. (2022). Clinical Handbook of Couple Therapy (6th ed.). Guilford.

Next: MFT 711 — Child and Adolescent Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 11 OF 28

MFT 711

Child and Adolescent Therapy

15 teaching weeks · 3 practicals · 1 figure

SEMESTER TWO

1.71 units · Prerequisite: MFT 703, 706

> “Children rarely refer themselves and rarely benefit from an adult conversation.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 711 — Child and Adolescent Therapy

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

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Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	**Child and Adolescent Therapy**	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Children rarely refer themselves and rarely benefit from an adult conversation. This course teaches you to work with children as agents in a family system: how to speak to them, play with them, assess developmentally, and intervene through the adults who live with them.

How to use this book

Each week has a **developmental frame** (what is normal at this age), a **clinical frame** (what is different when it goes wrong), and a **practice frame** (what you actually do in the room). The play and activity protocols are written so you can use them next week, not after you qualify.

1. Course description

Child and adolescent work in family therapy is not a specialism bolted on to adult work; it is a distinct skill set. Children communicate through play, behaviour and the body; adolescents communicate through testing, silence and peers; and both live inside systems that can either support or undermine change.

The course covers: developmental assessment, engaging children and adolescents, family-based interventions, common presentations (anxiety, behaviour problems, school refusal, trauma, self-harm, eating difficulties), divorce and blended families, safeguarding, and the interface with schools and paediatrics.

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a developmental and systemic assessment of a child or adolescent	Observed session
LO2	Engage a child using play, drawing and conversation appropriate to age	Skills assessment
LO3	Formulate a child's presentation relationally and developmentally	Case study
LO4	Deliver a family-based intervention for at least two common presentations	Case study
LO5	Work with schools, paediatricians and child protection systems	Report
LO6	Recognise and respond to safeguarding concerns	Exam
LO7	Manage the ethical complexities of consent, confidentiality and divided loyalty	Viva

3. Assessment

Component	Weight	Due
Child assessment observed (30 min with child + 20 min feedback to carers)	30%	Week 8

Component	Weight	Due
Family-based intervention case study (2,500 words)	40%	Week 13
Examination	30%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Children in context: why family therapy is child therapy	Observation of a child at play
2	Developmental assessment and red flags	Milestone interview
3	Engaging children: language, play, drawing, behaviour	Drawing interview
4	Engaging adolescents: alliance, autonomy, honesty	Adolescent interview
5	Consent, confidentiality and divided loyalty	Ethics scenario work
6	Anxiety and school refusal	Family-based exposure plan
7	Behaviour problems and parenting interventions	Parent session rehearsal
8	**Child assessment observed**	Assessed
9	Trauma and maltreatment in children	Safety and stabilisation
10	Self-harm and suicidality in adolescents	Risk assessment with a young person
11	Divorce, separation and blended families	Telling-the-children protocol
12	Eating difficulties and body image	Family-based approach outline
13	**Case study due**; working with schools and paediatric services	Report writing
14	Neurodivergence in family context	Environmental adaptation
15	Endings with children; integration	Termination play session

Week 1 — Children in context

The paradox of child therapy. A child is referred because of behaviour. The behaviour occurs in a context. If you treat the child alone, you have removed them from the context that maintains the behaviour and asked them to change it on their own. Family-based work is therefore not an ideological preference; it is a practical inference.

Children as agents. Children are not passive recipients of family dynamics. They notice, interpret, protect, sacrifice and resist. A nine-year-old who refuses school may be keeping a depressed mother company; a thirteen-year-old who fights may be absorbing a father's rage to protect a younger sibling.

The evidence. Family-based treatments have strong evidence for: conduct problems (parent management training, functional family therapy, multisystemic therapy),

adolescent substance misuse, eating disorders (family-based treatment for anorexia nervosa is first-line), anxiety in children (parent-involved CBT), and psychosis relapse prevention (family intervention).

> **PRACTICAL**
> Observe a child (with consent) for 15 minutes in free play. Record only behaviour: what they did, in what order, what they returned to. No interpretation for the first ten minutes.

Week 2 — Developmental assessment and red flags



Figure 2.1 — Developmental milestones from 0 to 18

The four-category check (from MFT 703): normal variation, delay, regression, symptom.

Red flags requiring referral, not therapy alone.

Domain	Red flag
Language	Loss of words; no words at 24 months with no comprehension
Motor	Loss of motor skills; persistent asymmetry; regression
Social	Loss of eye contact and social interest; loss of pretend play

Domain	Red flag
Behaviour	Sudden change in personality; new aggression after a specific event
School	Marked decline in a previously competent child
Medical	Seizures, severe headaches, weight loss, unexplained bruising
Mental state	Hearing voices; fixed beliefs; severe withdrawal; self-harm
Safeguarding	Disclosure, injuries, sexualised behaviour, fear of a specific adult

Assessment with children. Use several modes: interview, drawing, play observation, parent report, teacher report, and standardised measures (SDQ, RCADS). Children's self-report and parents' report diverge systematically — collect both, report both.

Week 3 — Engaging children

Rules of engagement.

1. **Explain the frame to the child, not just the adult.** "We will meet four times. We will talk and sometimes draw. You can say 'pass' to any question."
2. **Get on their level.** Sit on the floor, use their name, match their pace.
3. **Use a medium.** Drawing, puppets, sand, figures, or a simple game. The medium is not a distraction; it is the language.
4. **Do not interrogate.** One question, then follow. "Tell me about this house" beats "who lives here?"
5. **Be honest.** Children detect adult pretence and are damaged by it. If you cannot answer, say so.

Three reliable techniques.

- **The three wishes.** "If you had three wishes, what would they be?" (One is usually for the child, one for the family, one impossible.)
- **Draw your family.** Then ask: "Who is this? What are they doing? Who is closest to whom?" Note who is omitted, who is drawn largest, who is at the edge.
- **The magic wand / miracle.** "If you woke up and the problem was gone, what would be different at breakfast?"

Avoid interpretation. "That must be your father" is a projection. Ask instead: "Tell me about this person."

> **PRACTICAL**
> 20-minute drawing interview with a peer playing an 8-year-old. Use the three techniques. Debrief: how many questions did you ask, and what did you learn without asking?

Week 4 — Engaging adolescents

What adolescents need from a therapist. Honesty, respect for autonomy, a genuine alliance with them (not with their parents), and confidentiality that is real except where safety requires otherwise.

The first five minutes. Speak to the adolescent first. Ask their version before the parents'. Explain confidentiality in their language: "What you tell me stays here, unless I think you or someone else is in danger. If that happens, I will tell you before I tell anyone else."

Techniques that work.

- **Third-person questions.** "What do teenagers your age usually say about this?" Lowers the exposure.
- **Scaling.** "On 0–10, how much does this bother you? How much does it bother your mum?"
- **Future questioning.** "What do you want your life to look like in two years? What would have to change first?"
- **Silence tolerance.** Do not fill every pause.
- **Negotiated goals.** "What would make this worth your time?"

What fails. Lectures, moralising, siding with parents, pretending to be young, and asking "why".

Week 5 — Consent, confidentiality and divided loyalty

Consent. For minors, a parent or guardian consents; the young person **assents**. Both matter. Record both. Where a competent adolescent seeks help without parental knowledge, follow your jurisdiction's law and your agency policy — and do not improvise.

Confidentiality with children.

Situation	Approach
Child discloses something minor	Keep it; help them tell the parent if they want
Child discloses self-harm	Assess risk; agree a plan; inform carers with the child's knowledge
Child discloses abuse	Follow safeguarding procedure; tell the child what will happen next
Parent asks what the child said	Give themes, not transcripts; protect the alliance
Parent demands a report for court	Refer; a treating therapist should not provide forensic opinion

Divided loyalty. The most common clinical trap: the child tells you something that would hurt the parent, or the parent asks you to "fix" the child's attitude. Name the dynamic and refuse to be recruited: "I am not going to be the one who tells your mother. I will help you do it."

Week 6 — Anxiety and school refusal

Anxiety in children. Developmentally normal fears (separation, darkness, monsters, social evaluation) become disorders when they are persistent, impairing and out of proportion.

The family-based approach.

1. **Psychoeducation.** Anxiety is a false alarm; avoidance reduces anxiety now and increases it later.
2. **Map the accommodation.** What does the family do to prevent the child's distress? (Answering questions for them, sleeping in their bed, allowing school avoidance, checking repeatedly.) Accommodation is usually maintained by love, and naming it must be done without blame.
3. **Graded exposure.** Together, build a ladder from easiest to hardest; start at the bottom; reward approach, not the absence of anxiety.
4. **Parental coaching.** Parents become coaches, not rescuers: "I know this is hard. I will be here at 3 o'clock."
5. **The school plan.** One named adult, a quiet space, a gradual return timetable, and no negotiation at the school gate.

School refusal — a differential. Anxiety, separation difficulty, bullying, learning difficulty, family crisis or a bid to protect a parent. The treatment differs for each. Assess by asking what happens on Sunday night, and what the child's absence makes possible at home.

> **PRACTICAL**
> Build an exposure ladder with a role-played parent for a 10-year-old with separation anxiety. Ten rungs, specific, graded, with a reward and a review date.

Week 7 — Behaviour problems and parenting

The evidence base. Parent management training (PMT) and its variants have the strongest evidence for child conduct problems. The mechanism is consistent: increase positive attention, give clear instructions, use consistent consequences, reduce harsh discipline, and rebuild the parent–child relationship.

The core skills (teach, model, rehearse, review).

Skill	What it looks like
Special time	10 minutes daily of child-led play, no correction
Praise	Specific, immediate, frequent ("thank you for putting your shoes away")
Instructions	Get attention, get close, one instruction, allow 5 seconds, praise compliance
Planned ignoring	For minor attention-seeking; not for dangerous behaviour

Skill	What it looks like
Consequences	Consistent, proportionate, predictable, delivered calmly
Routine	Predictable structure reduces conflict

Why parenting programmes fail. Too much content per session; the parent is depressed or exhausted; the child has an untreated condition; another adult undermines the plan; or the family's material circumstances make consistency impossible. Address these before adding more technique.

Adaptation. In households where several adults care for the child (grandmother, aunt, house help), bring them all in and agree one plan. A plan implemented differently by four adults is not a plan.

Week 8 — Observed child assessment

Thirty minutes with a child (role-played by a trained peer/actor), then twenty minutes of feedback to the carers. Assessed on: engagement, developmental appropriateness, information gathering without interrogation, risk screen, and the quality of the feedback.

Week 9 — Trauma and maltreatment

Principles (developed in MFT 714). Safety first; stabilise before processing; the caregiver's own regulation is the child's best medicine; and never force a narrative.

What trauma looks like in children.

Age	Presentation
0–2	Feeding and sleep disruption, startled response, withdrawal
3–6	Regression, repetitive play, nightmares, clinging
7–11	Aggression, school problems, somatic complaints, guilt
12–18	Risk-taking, self-harm, dissociation, substance use, precocious sexuality

Family work after maltreatment. The non-offending caregiver's belief and protection is the single strongest predictor of recovery. Work with them first. Where the perpetrator remains in the home, safety planning precedes everything.

Safeguarding. Know your reporting route, document verbatim disclosures without leading questions, and never promise a child secrecy about abuse.

Week 10 — Self-harm and suicidality in adolescents

Ask directly. "Sometimes when people feel this bad, they think about hurting themselves or ending their life. Has that happened for you?" Direct questions do not plant ideas.

Assess: frequency, method, function (relief, punishment, communication, control), intent, plan, means, protective factors, and the family's response.

Manage the family. Parents often respond with panic, punishment or secrecy. Help them respond with calm supervision and connection: remove means, increase monitoring without surveillance, and seek help immediately if risk rises.

Confidentiality. A young person's request for secrecy must yield to safety. Tell them what you will disclose and why, and involve them in how.

Week 11 — Divorce, separation and blended families

Telling the children. Both parents together, a simple message with no blame, permission to love both parents, and reassurance that the adults' decision is not the child's fault. No details about the affair, the money or the courts.

What harms children. Exposure to conflict (more than separation itself), being used as a messenger, being asked to choose, hearing a parent denigrated, and instability.

What protects them. A cooperative co-parenting relationship, structure, a relationship with both parents, and adults who can manage their own distress.

Blended families. Build the couple bond first; stepparents take authority gradually and through the biological parent; children need permission to love both households; and the "insider/outsider" split must be named and worked with.

Week 12 — Eating difficulties

Family-based treatment for anorexia nervosa (FBT, "Maudsley") is a first-line intervention for adolescents. It is agnostic about cause and externalises the illness: the family is recruited to defeat the eating disorder, not blamed for it.

Three phases. 1) Parents take charge of refeeding; 2) control is returned to the adolescent, age-appropriately; 3) identity and developmental issues are addressed.

Medical safety. Eating disorders carry the highest mortality of any mental disorder. Weight, vital signs and bloods are medical matters; do not work in isolation.

Avoid. Explaining the illness as a consequence of family dynamics; power struggles over food in session; and colluding with the idea that the young person can simply "decide to eat".

Week 13 — Schools and paediatric services

The school as a system. Teachers see the child in a different context and hold different data. Ask for specifics, not impressions: attendance, incidents, academic trajectory, peer relationships, and what strategies have been tried.

Writing a useful report for a school. One page: what you observed, what it means, three concrete recommendations, and who to contact. Avoid diagnosis-led recommendations the school cannot implement.

Paediatric liaison. Distinguish functional from organic symptoms without dismissing either. A referral that names what you want excluded is more likely to be answered (see MFT 707).

> **REPORT**
> Write a one-page school communication for a child with school refusal, including a graded return plan with named adults.

Week 14 — Neurodivergence in family context

Reframe. Autism and ADHD are not behavioural choices and not caused by parenting. But the environment determines how disabling they are.

Practical family work.

- Psychoeducation and demystification for siblings and grandparents.
- Environmental adaptation: predictability, visual schedules, sensory accommodations, transition warnings.
- Parental coaching: reduce the demand–avoidance cycle; increase specific praise; build in recovery time.
- Couple work: neurodivergence in a partner or child strains the parental alliance; address it explicitly.
- School liaison: accommodations are usually more effective than consequences.

Avoid. Behavioural contracts for skills the child does not yet have; blaming parents; and treating a diagnosis as an explanation for everything.

Week 15 — Endings with children

Children end differently. Use a session to review: what we did, what changed, what you will remember, and what you will do when you need help again. Use the medium you used (drawing, play) to close the loop. Give the child something concrete (a certificate, a drawing, a letter) and make sure the carers know when and how to return.

Glossary

Accommodation (of anxiety) — family behaviours that reduce a child's distress in the short term and maintain the anxiety. **Assent** — a minor's agreement to treatment, alongside guardian consent. **Externalising (the illness)** — separating the child from the disorder, as in FBT. **Family-based treatment (FBT)** — a manualised family approach to adolescent anorexia nervosa. **Parent management training (PMT)** — behavioural skills training for parents of children with conduct problems. **School refusal** — difficulty attending school owing to emotional distress, distinct from truancy. **Separation anxiety** — developmentally excessive distress about separation from attachment figures.

Self-assessment

Multiple choice

1. The strongest evidence for child conduct problems is for: (a) individual play therapy (b) parent management training (c) family sculpture (d) medication only

2. In anxiety work, family accommodation: (a) is helpful (b) maintains anxiety despite reducing short-term distress (c) is irrelevant (d) should be punished

3. Asking an adolescent directly about suicide: (a) increases risk (b) does not increase risk and is required (c) should be avoided (d) requires parental consent

4. FBT for anorexia: (a) blames parents (b) recruits parents to defeat the illness (c) focuses on insight only (d) is individual therapy

5. School refusal differs from truancy in that it involves: (a) deception without distress (b) emotional distress about attending (c) peer pressure (d) academic failure

6. A child's disclosure of abuse should be: (a) kept confidential (b) documented verbatim and reported (c) discussed with the alleged abuser (d) ignored if vague

7. The strongest predictor of recovery after maltreatment is: (a) the child's temperament (b) a protective, believing caregiver (c) the severity of abuse (d) therapy length

8. Neurodivergence is best addressed by: (a) behavioural contracts (b) environmental adaptation and skills-based support (c) blaming parents (d) exclusion

9. Divided loyalty refers to: (a) the therapist's loyalty to the agency (b) the child's conflict between therapist and parent (c) sibling rivalry (d) custody disputes

10. Telling children about divorce should: (a) include details of the affair (b) be done by both parents together without blame (c) be left to the courts (d) wait until they ask

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List six red flags requiring referral in child assessment.

2. Describe the three drawing/play techniques and what each reveals.
3. How would you explain confidentiality to a 14-year-old?
4. Give the five core skills of parent management training.
5. What are the three phases of FBT and what happens in each?

Essay (choose one, 2,000 words)

1. "There is no such thing as child therapy without the family." Discuss with evidence.
2. Using a case, show how a developmental formulation changed the treatment of a behaviour problem.
3. Analyse the ethical tensions in working with a 15-year-old whose parents disagree with her treatment goals.

Further reading

- Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.
- Kazdin, A. E. (2018). Parent Management Training. OUP.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford.
- Sroufe, L. A. et al. (2005). The Development of the Person. Guilford.
- Siegel, D. J., & Bryson, T. P. (2011). The Whole-Brain Child. Delacorte.

Next: MFT 712 — Crisis Intervention Strategies

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 12 OF 28

MFT 712

Crisis Intervention Strategies

15 teaching weeks · 3 practicals · 2 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 704

> “A crisis is not a bigger version of an ordinary session.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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Course leader's summary. A crisis is not a bigger version of an ordinary session. It is a different clinical state with its own rules: safety before insight, structure before exploration, and short contacts with clear follow-up. This course will make you useful in the first hour.

How to use this book

The course is built around the **first hour**: what you do in the ten minutes after a disclosure of suicide, a violent incident, a sudden death, or a family that arrives in chaos. Read the scripts aloud. Under stress, you will use the sentences you have rehearsed, not the theory you remember.

1. Course description

Crises are the point at which families enter services most often and at which therapists feel least prepared. This course covers: crisis theory and the phases of a crisis; assessment of lethality and danger; suicide and self-harm; domestic violence; child protection emergencies; acute grief and bereavement; family crises (separation, disclosure, runaway, arrest); disasters and collective trauma; telephone and remote crisis work; and the organisational context of crisis services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess a crisis using a structured model, and prioritise safety	Observed assessment
LO2	Conduct a suicide risk assessment and co-create a safety plan	Skills test
LO3	Manage acute family conflict and violence safely in session	Simulation
LO4	Respond to acute grief, shock and disorganisation	Case study
LO5	Work within the limits of a single session and a crisis team	Viva
LO6	Document crisis work defensibly	Portfolio
LO7	Protect yourself from vicarious trauma in crisis work	Reflective portfolio

3. Assessment

Component	Weight	Due
Simulated crisis assessment (30 min)	30%	Week 6
Safety plan portfolio (3 plans with rationale)	25%	Week 10
Written examination (case-based)	25%	Week 16
Crisis log and reflection (10 entries)	20%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What makes a crisis a crisis	Map a crisis in phases
2	The first ten minutes: safety, contact, structure	Opening drill
3	Crisis assessment and triage	Triage exercise
4	Suicide I: assessment	Observed risk interview
5	Suicide II: safety planning and means restriction	Safety plan lab
6	**Simulated crisis assessment**	Assessed
7	Self-harm and repeated crisis	Containment planning
8	Domestic violence and danger assessment	Safety planning with a victim
9	Child protection emergencies	Reporting rehearsal
10	**Safety plan portfolio due**; acute grief and bereavement	Death notification role-play
11	Family crises: runaway, arrest, disclosure, eviction	Multi-agency simulation
12	Disasters and collective trauma	PFA demonstration
13	Telephone, text and remote crisis work	Call simulation
14	The crisis team, handover and follow-up	Handover practice
15	Vicarious trauma and sustainable crisis work	Personal plan

Week 1 — What makes a crisis a crisis

Definition. A crisis is a state of psychological disequilibrium in which a person's usual coping methods fail and their functioning is impaired, usually following a precipitating event that is perceived as a threat. It is **time-limited** (typically 4–6 weeks), **dangerous** (risk of harm), and **an opportunity** (people are more open to help than at any other time).

The four phases (Caplan).

1. **Impact / shock** — disorganisation, disbelief, numbness.
2. **Tension and disorganisation** — attempts to cope fail, distress rises, help-seeking begins.
3. **Mobilisation** — active attempts to solve, often chaotic.
4. **Resolution or breakdown** — adaptation, or chronic dysfunction.

The precipitant is not the whole story. A crisis occurs when a stressor meets a vulnerability. Two families can experience the same event and only one present in crisis. Assess both.

Family crisis is systemic. One member's crisis reorganises the household: roles shift, secrets surface, and previously stable patterns break. Your assessment is of the family's equilibrium, not only the identified person's.

> **PRACTICAL**
> Take a crisis you have witnessed (disguised). Map it onto the four phases and identify where intervention would have changed the trajectory.

Week 2 — The first ten minutes

Crisis work has a protocol. In the first ten minutes you do five things, in this order:

1. **Safety.** Is anyone in immediate danger? Are you safe? Is there a weapon, a person at the door, an overdose already taken?
2. **Contact.** Your voice, your pace, your presence. Lower your tone, slow your speech, sit down, say your name and your role.
3. **Structure.** Tell them what is going to happen in the next hour. "I am going to ask you some difficult questions. Then we will make a plan together."
4. **Story.** Let them tell it, but with a boundary: "Start from the beginning; we have twenty minutes."
5. **Mobilise help.** Who is available? Who can come? What has helped before?

Sentences that work.

- "I am glad you told me. You are not in trouble."
- "I need to ask you some direct questions. They may feel intrusive. I am asking because I need to know how to keep you safe."
- "You do not have to solve this tonight. We need to get through the next twenty-four hours."

Sentences that do not work. "Everything will be fine." "You should not feel that way." "At least…" "Do you really mean that?"

Week 3 — Crisis assessment and triage

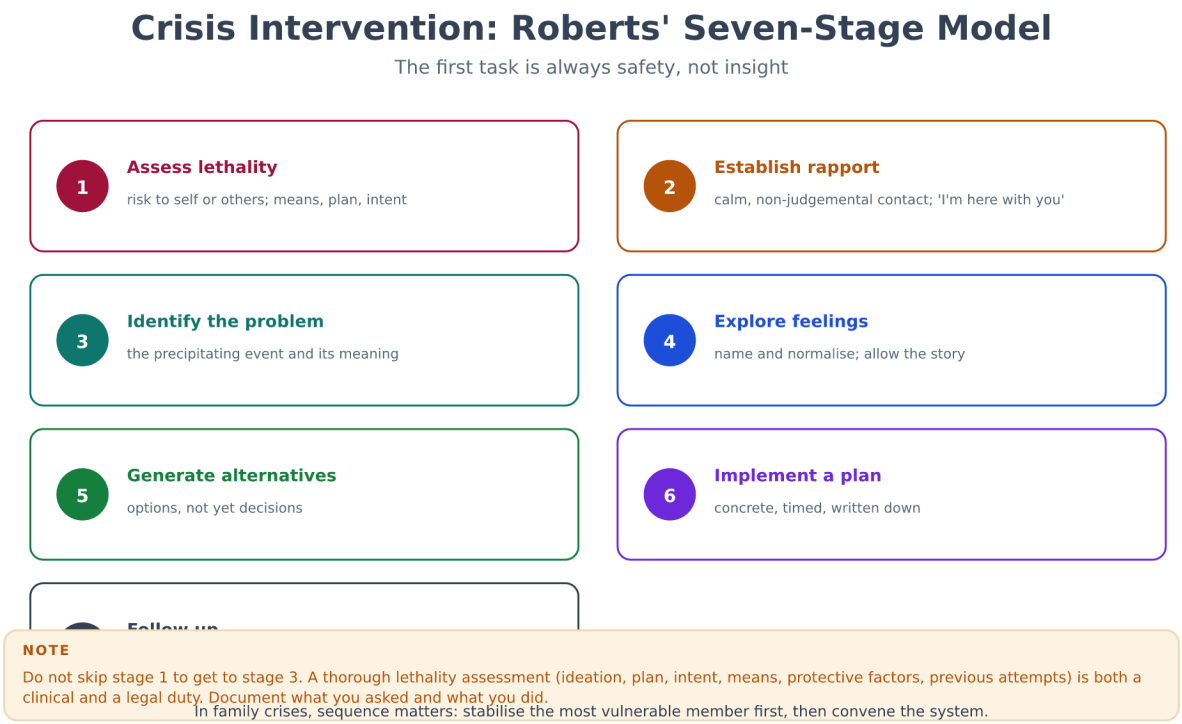


Figure 3.1 — Roberts' seven-stage crisis intervention model

The seven stages (Roberts) provide a structure that survives chaos: assess lethality → establish rapport → identify the problem and precipitant → explore feelings → generate alternatives → implement a plan → follow up.

Triage: three questions.

1. **Is there immediate danger?** (Weapon, overdose, assault in progress, child alone with a dangerous adult.)
2. **What is the appropriate level of care?** (Emergency department, crisis team, same-week appointment, routine.)
3. **Who else needs to know?** (Partner, school, social services, police, GP.)

Do not provide long-term therapy in a crisis. Your task is stabilisation, safety and connection to ongoing care. A crisis service that becomes the client's therapy creates dependence and denies the client the long-term work.

Week 4 — Suicide I: assessment

The epidemiology you need. Suicide is among the leading causes of death in young people; men die more often, women attempt more often; previous attempt is the strongest predictor; access to means matters; most people who die by suicide have communicated something to someone.

The structured assessment (ten domains). Ideation, plan, intent, means, previous attempts, protective factors, substance use, impulsivity, family history, current stressors — plus a direct question about what has kept you alive so far.

Asking.

> "When you say you cannot go on, I need to be clear: are you thinking about ending your life?" "Have you thought about how you would do it?" "Do you have [the means] at home?" "Have you ever tried before?" "What has kept you going until now?"

Formulation and risk level.

Level	Picture	Action
Low	Ideation without plan, strong protective factors, engaged	Safety plan, follow-up within 1 week, means counselling
Moderate	Ideation with plan, limited intent, some protective factors	Same-day mental health review, means restriction, daily contact
High	Plan, intent, means, recent attempt, agitation, intoxication	Do not leave alone; emergency department or crisis team; consider involuntary route

Documentation. Record the questions and answers verbatim where possible, the risk level, the rationale, the plan, who was informed, and the review. If you judged risk lower than others might, state why.

Week 5 — Suicide II: safety planning

A safety plan is not a no-suicide contract. Contracts have no evidence base and can increase guilt. A safety plan is a written, collaborative, stepwise coping plan.

Six components (Stanley & Brown).

1. **Warning signs** — the person's own early signals ("I stop answering my phone").
2. **Internal coping** — things they can do alone ("walk to the mosque", "cold shower", "music").
3. **People and places that distract** — social settings that reduce isolation.
4. **People to ask for help** — names and numbers.
5. **Professionals and agencies** — crisis line, clinic, emergency number.
6. **Making the environment safe** — means restriction: who holds the medication, the keys, the pesticide.

Means restriction is the most effective single intervention. Ask directly: "What is in the house?" Ask the family to take control of it, and be specific about who, what, and where.

With families. The family's response matters: reduce blame, increase supervision without surveillance, and give the person a role in their own plan.

PRACTICAL

Co-create a safety plan with a role-played client. The assessor checks: was it collaborative? Were means addressed specifically? Was a follow-up date agreed?

Week 6 — Simulated crisis assessment

Thirty minutes, unannounced scenario (suicide risk, domestic violence, or acute grief). Assessed on: safety first, contact and pace, structured assessment, formulation, plan, and documentation quality.

Week 7 — Self-harm and repeated crisis

Self-harm is often a regulation strategy, not a suicide attempt. Distinguish: function (relief, control, punishment, communication, feeling something), lethality, frequency, and hidden versus disclosed.

Working with repeated crises. Frequent attenders are often described as "manipulative" — a term that should be banned. The pattern usually reflects: an untreated disorder, chronic trauma, an unstable relationship system, or a service that rewards crisis and does not provide continuity.

The intervention is a plan, not a lecture. A written crisis protocol agreed with the person (what to do, who to call, when to come to hospital) reduces both crisis frequency and staff burnout.

Therapist boundaries. You cannot be available 24 hours. State your availability clearly, give the crisis route for out-of-hours, and do not give your personal number unless your service requires it and you have thought through the consequences.

Week 8 — Domestic violence and danger assessment

Danger assessment questions (adapted from the Danger Assessment).

- Has violence increased in frequency or severity in the last year?
- Has the partner ever used a weapon or threatened one?
- Has the partner ever tried to strangle or choke you?
- Has the partner threatened to kill you or the children?
- Is the partner violently or constantly jealous and controlling?
- Is there a separation in progress (the highest-risk period)?
- Does the partner have access to firearms?
- Has the partner ever forced sex?
- Is the partner using substances heavily?
- Has the partner threatened suicide?

Safety planning with a victim. Identify a safe place, a code word, documents and money, a bag prepared, children's needs, transport, and the number of a specialist service. Do not tell her to leave as though leaving were simple; leaving is the most dangerous period, and she is the expert on her own risk.

Do not conduct conjoint sessions where coercive control is present. Do not inform the abuser of the disclosure. Know your mandatory reporting obligations — and their limits (in many jurisdictions, adult IPV is not mandatorily reportable, but child exposure may be).

Week 9 — Child protection emergencies

Signs requiring immediate action.

- A disclosure of abuse (physical, sexual, emotional, neglect).
- Unexplained injury inconsistent with the explanation.
- A child who is frightened of a specific adult.
- A child left alone or in the care of someone unable to protect them.
- Sexualised behaviour beyond developmental norms.
- A parent who cannot or will not protect the child.

Procedure.

1. Respond calmly; do not interview in detail; use open prompts only ("tell me what happened").
2. Record verbatim, promptly, with the child's words and your questions.
3. Do not promise secrecy.
4. Report to the designated safeguarding lead or the appropriate authority, following your service's procedure and your jurisdiction's law (in Nigeria, the Child Rights Act and state child protection structures; the police family unit; the state ministry responsible for social welfare).
5. Tell the child what will happen next and who will be involved.
6. Document everything, including the advice you received and the time you acted.

> **REHEARSAL**
> Practise the first four minutes of a disclosure, and the phone call to report it.

Week 10 — Acute grief and bereavement

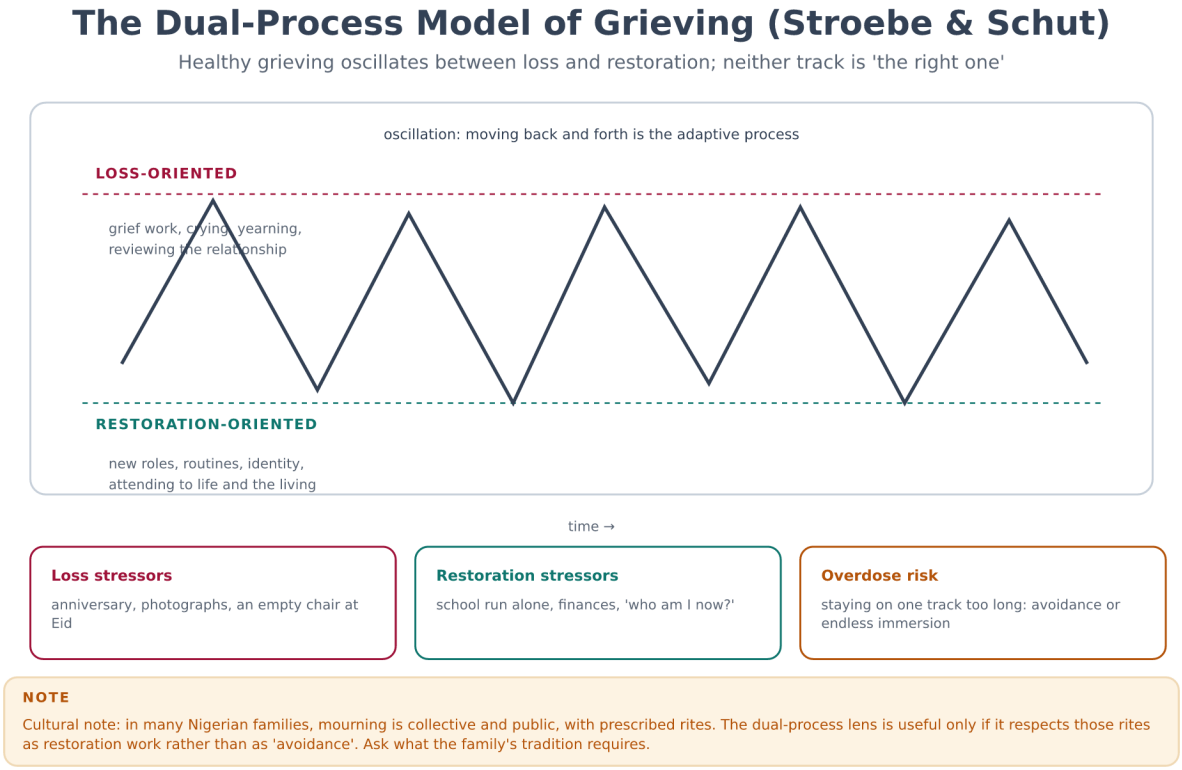


Figure 10.1 — The dual-process model of grieving

Normal acute grief includes waves of yearning, disbelief, anger, guilt, intrusive images, and a sense of unreality. It is not a disorder. It becomes complicated when it is prolonged, severe, and impairing (prolonged grief disorder in ICD-11/DSM-5-TR).

In the first weeks: accompany, do not treat. Practical help, information, permission to grieve, and continuity. Do not rush to meaning-making; do not say "he is in a better place" unless you know the family's faith.

Sudden and traumatic deaths (accident, homicide, suicide) add shock, anger, stigma and, for suicide, a family burden of blame and secrecy. Address the stigma explicitly.

Children and grief. Children grieve in bursts, return to play, and ask direct questions. Be honest, use clear language ("died", not "lost"), and reassure them that they are not to blame.

The dual-process frame. Healthy grieving oscillates between loss-oriented and restoration-oriented activity. Two errors: staying only on the loss track (rumination, avoidance of life) and staying only on the restoration track (avoidance of grief). The family's culture prescribes which is expected; work within it.

Week 11 — Family crises

Crisis	Immediate priorities	Common errors
Adolescent runaway	Safety, whereabouts, who they trust, why they left	Interrogating the parents first; treating the adolescent as the problem
Arrest or imprisonment	Practical information, children's needs, shame, finances	Moralising; ignoring the children
Disclosure of a secret (affair, paternity, HIV, debt)	Containment, safety, who needs to know and when	Forcing immediate disclosure; colluding in concealment
Eviction or financial collapse	Practical referral; safety; children's schooling	Psychologising material reality
Sudden separation	Safety, children's stability, legal referral	Assuming reconciliation is the goal
Death of a child	Presence, not technique; couple's survival	Interpreting; taking sides in blame

Week 12 — Disasters and collective trauma

Psychological first aid (PFA) — five elements: safety, stabilisation, connection, self-efficacy, hope. It is not debriefing. Mandatory psychological debriefing has been shown to be ineffective and possibly harmful; do not do it.

Principles in collective events.

- Attend to basic needs first (shelter, food, information, family reunification).
- Do not pathologise normal distress.
- Reconnect people to their own networks and rituals; community and faith resources are usually more powerful than clinical intervention in the acute phase.
- Identify the vulnerable: unaccompanied children, the injured, the bereaved, those with prior trauma, and responders.
- Plan for the medium term: most distress resolves; a minority need treatment at 4–12 weeks.

Week 13 — Telephone, text and remote crisis work

Advantages. Access, immediacy, anonymity, reach into areas with no services.

Risks. Loss of non-verbal information, difficulty verifying location, inability to intervene physically, and boundary erosion.

Protocol for a remote crisis contact.

1. Verify identity and location at the start ("where are you right now?").
2. Establish whether anyone is with them.
3. Assess risk verbally, more slowly than you would in person.

4. Agree a concrete plan for the next hour.
5. Record the number they called from and a safe contact.
6. If risk is high, keep them on the line while arranging help.

Week 14 — The crisis team and continuity

Handover. A good handover is structured: presenting problem, risk level and basis, what has been done, what is planned, who is responsible, and when the next contact is. Use a standard format (e.g. SBAR: Situation, Background, Assessment, Recommendation).

Follow-up is the intervention. Crisis care that does not follow up fails. Agree a date before the person leaves, and make the contact proactive.

Working with other agencies. You will share information; know the legal basis, share the minimum necessary, and record what you shared and why.

Week 15 — Vicarious trauma and sustainable practice

Crisis work takes a physiological toll. Warning signs: intrusive images, sleep disruption, cynicism, numbness, dread before shifts, irritability at home, and loss of pleasure in work.

Protective practices. Debrief structurally (not emotionally) after critical incidents; use supervision; keep caseload diversity; maintain boundaries on availability; sleep; exercise; and have your own therapist.

The organisational dimension. Individual self-care cannot compensate for an unsafe service. If you are carrying 40 crisis cases with no supervision, that is a management problem, and saying so is part of professional practice.

Glossary

Containment — the provision of structure and safety so that overwhelming material can be held. **Crisis** — a time-limited state of disequilibrium in which usual coping fails. **Danger assessment** — structured assessment of lethality risk in domestic violence. **Means restriction** — reducing access to methods of self-harm; the most effective suicide prevention measure. **Psychological first aid (PFA)** — a five-element supportive intervention after a critical event. **Safety plan** — a written, collaborative, stepwise plan for managing a suicidal crisis. **Triage** — the prioritisation of care according to urgency and risk.

Self-assessment

Multiple choice

1. The first task in a crisis is: (a) insight (b) safety (c) diagnosis (d) history

2. Roberts' model begins with: (a) rapport (b) lethality assessment (c) plan (d) follow-up

3. A no-suicide contract is: (a) evidence-based (b) not evidence-based; a safety plan is preferred (c) legally required (d) sufficient alone

4. The highest-risk period in domestic violence is: (a) during pregnancy (b) separation (c) the first year (d) after counselling

5. Psychological debriefing after disasters is: (a) recommended for all (b) not recommended and potentially harmful (c) legally required (d) equivalent to PFA

6. Means restriction is important because: (a) it is easy (b) it reduces the probability of a lethal attempt (c) it is legally required (d) it satisfies families

7. Conjoint therapy is contraindicated when: (a) there is conflict (b) there is coercive control (c) there is grief (d) there is low income

8. A child's disclosure of abuse should first be: (a) investigated by you (b) recorded verbatim and reported (c) discussed with the parents (d) kept confidential

9. Prolonged grief disorder is distinguished from normal grief by: (a) crying (b) duration, severity and impairment (c) cultural expression (d) family conflict

10. A good crisis handover includes: (a) only the diagnosis (b) situation, background, assessment, recommendation, and next contact (c) your opinion of the family (d) the full case notes

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List the five things you do in the first ten minutes of a crisis.
2. Give six components of a safety plan.
3. Name three questions from a danger assessment.
4. What are the five elements of psychological first aid?
5. Why is follow-up the intervention in crisis work?

Essay (choose one, 2,000 words)

1. "Crisis is both danger and opportunity." Discuss with a case, including what you did and what you would change.
2. Analyse the ethical tensions in responding to a suicidal adolescent whose parents refuse hospital admission.
3. Critically evaluate the evidence on psychological debriefing and its implications for crisis services.

Further reading

- Roberts, A. R. (2005). Crisis Intervention Handbook (3rd ed.). Oxford University Press.
- James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage.
- Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256–264.
- Hobfoll, S. E. et al. (2007). Five essential elements of immediate and mid-term mass trauma intervention. Psychiatry, 70(4), 283–315.
- Campbell, J. C. (2002). Danger Assessment.

Next: MFT 713 — Substance Abuse and Family Dynamics

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MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	**Substance Abuse and Family Dynamics**	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Addiction is a family-organising process. This course teaches you to work with the user, the family, and the household system — using motivational, behavioural, systemic and recovery-oriented approaches — without moralising, colluding or abandoning hope.

How to use this book

Read the case at the start of each week before the theory. Addiction work punishes abstraction: families are exhausted, professionals are frustrated, and the evidence is often misrepresented by both. Hold two principles throughout: **the person is not the substance**, and **the family is not the cause**.

1. Course description

The course covers: models of addiction (disease, learning, biopsychosocial, systemic); assessment; the family cycle of use, secrecy and shame; motivational interviewing; relapse prevention; behavioural couples therapy for substance use; CRAFT (for concerned relatives); the interface with mental illness ("dual diagnosis"); recovery as a family process; medication-assisted treatment; and policy and cultural context.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain and compare models of addiction, including their clinical implications	Exam
LO2	Conduct a substance-use assessment, including readiness to change	Observed assessment
LO3	Use motivational interviewing skills with fidelity	Skills test
LO4	Formulate the family's role in maintaining and resolving use	Case study
LO5	Deliver CRAFT or behavioural couples work for substance use	Case study
LO6	Work with dual diagnosis and coordinate with medical and recovery services	Viva
LO7	Address stigma, culture and shame in your own practice	Reflective portfolio

3. Assessment

Component	Weight	Due
Motivational interviewing observed (25 min)	25%	Week 5
Family formulation and intervention plan (2,500 words)	35%	Week 11
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio (6 entries)	15%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Models of addiction and their consequences	Model comparison debate
2	The family cycle: use, secrecy, enabling, crisis, shame, promise	Cycle mapping
3	Assessment: quantity, function, consequences, readiness	Structured interview
4	Motivational interviewing I: OARS and the spirit	OARS drill
5	**MI observed**; MI II: change talk and discord	Change-talk practice
6	The stages of change and the family's stage	Stage-matched planning
7	Working with the user: CBT and relapse prevention	Relapse prevention plan
8	Working with the couple: behavioural couples therapy	Daily recovery plan
9	Working with relatives: CRAFT and boundaries	CRAFT role-play
10	Co-dependency, enabling and the limits of the concept	Boundary-setting lab
11	**Case plan due**; dual diagnosis	Integrated plan
12	Medication-assisted treatment and the medical interface	Referral exercise
13	Recovery, relapse and the family's long haul	Relapse response plan
14	Culture, faith, stigma and the community	Community resource map
15	Integration: what works, for whom	Case presentations

Week 1 — Models of addiction

Model	Core claim	Clinical implication	Critique
Moral	Use is a choice; user is weak or bad	Punishment, exclusion	Shame increases use; ignores biology
Disease / medical	Addiction is a chronic brain disease	Treatment, medication, compassion	Can reduce agency; "once an addict always" fatalism
Learning / behavioural	Use is reinforced behaviour	Skills, contingency management	Can ignore neurobiology and context
Biopsychosocial	Biology × psychology × social context	Multimodal treatment	Descriptive rather than prescriptive
Systemic / family	Use is embedded in and organises relationships	Family-inclusive treatment	Risk of blaming the family

Model	Core claim	Clinical implication	Critique
Recovery / social	Recovery is a social process, not only clinical	Peer support, community, identity	Needs availability of recovery capital

The position used here. Addiction is a **chronic, relapsing condition** shaped by biology, learning, trauma, relationship, culture and economics. The person retains agency but not unlimited agency. Families are neither causes nor cures; they are the environment in which recovery either becomes possible or does not.

> **DEBATE**
> "Addiction is a disease" versus "addiction is a choice". Take a side, then argue the other side, then write the practical position you will hold with clients.

Week 2 — The family cycle

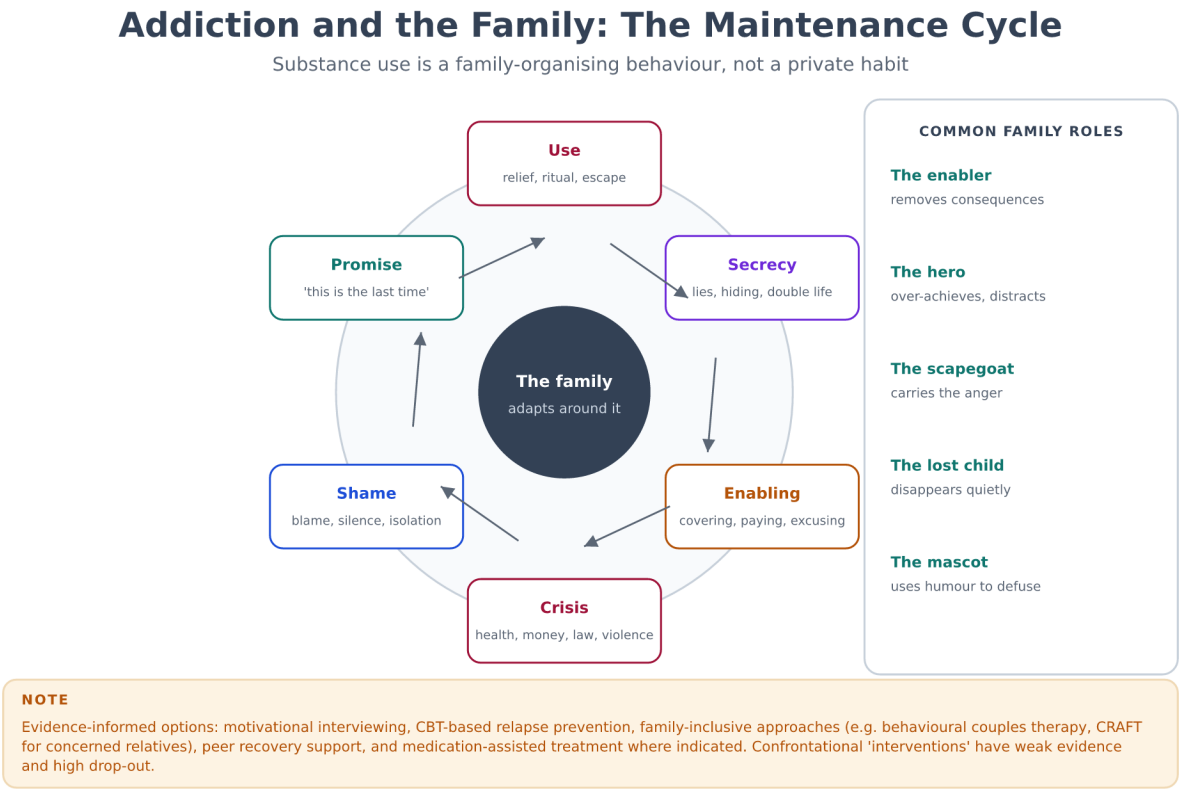


Figure 2.1 — The addiction and family maintenance cycle

Six stations that repeat: use → secrecy → enabling → crisis → shame → promise → use.

Enabling is not weakness. It is a set of behaviours that reduce immediate consequences because the alternative (the person losing their job, being arrested, or dying) feels unbearable: paying a debt, calling in sick, hiding the bottles, giving money "for food".

Family roles (an imperfect but useful map): the enabler, the hero, the scapegoat, the lost child, the mascot. Use them as questions, never as labels: "Who in the family keeps the peace? Who takes the blame? Who disappears?"

The systemic point. The cycle is maintained by both the using and the accommodating. This is not a statement of blame; it is a statement of leverage. Change the accommodating behaviour and the cycle must change.

> **PRACTICAL**
> Map a family cycle from a disguised case. At each station, write: who does what, what they feel, and what they fear. Then identify two points of leverage.

Week 3 — Assessment

Substance history.

Domain	Questions
Substances	What, how much, how often, by what route, for how long
Pattern	Time of day, alone or social, binge or daily
Function	What does it do for you? (sleep, courage, relief, numbness, belonging)
Consequences	Health, money, work, relationships, legal, children
Control attempts	Have you tried to stop? What happened? How long?
Withdrawal	Symptoms, previous seizures, previous medically assisted withdrawal
Dependence	Tolerance, withdrawal, compulsion, time spent, continued despite harm
Readiness	Do you want to change? Why now? What would it take?
Risk	Suicide, violence, driving, overdose, child safety

Validated tools. AUDIT (alcohol), DUDIT (drugs), ASSIST (multiple substances), CRAFFT (adolescents). Use them as a structure for conversation, not a verdict.

Assess the family separately. With the user absent: what has been tried, what has been covered up, what each member fears, and who is at risk.

Week 4 — Motivational interviewing I

The spirit (Miller & Rollnick): partnership, acceptance, compassion, evocation. You are not the expert on what they should do; you are the expert on a conversation that helps them say it themselves.

The four core skills (OARS).

Skill	What it is	Example
Open question	Invites elaboration	"What is a typical evening like?"
Affirmation	Names a strength, specifically	"You kept going to work through all this."
Reflection	Returns content or feeling	"You are worn out by the effort it takes to keep this hidden."
Summary	Collects and connects	"So on one hand it helps you sleep; on the other it is costing you your marriage."

What to avoid. The righting reflex (the urge to fix), confronting, warning, advising without permission, and arguing. When you argue for change, the client argues against it — and you have made change less likely.

Reflection is the active ingredient. Aim for two reflections for every question, and for complex reflections (which add meaning) over simple ones (which repeat content).

> **PRACTICAL**
> OARS drill: 8 minutes, no advice, no questions after the first one. Observer counts reflections, and rates how many were complex.

Week 5 — Motivational interviewing II

Change talk — client statements in favour of change. Four types: desire ("I want to"), ability ("I could"), reasons ("because…"), need ("I have to"). Also mobilising talk: commitment ("I will"), activation ("I am ready"), taking steps ("I bought the medication").

Discord is not resistance; it is a signal about the relationship. It appears as arguing, interrupting, discounting, ignoring. The response is not confrontation but **rolling with it**: "You are not sure this is worth it. Tell me more about that."

The MI-consistent techniques.

- **Elicit–provide–elicit:** "What do you already know about what alcohol does to the liver?" → brief, neutral information → "What does that mean for you?"
- **The decisional balance:** "What are the good things about using? And the less good things?" (Ask in that order, always.)
- **Looking forward:** "If nothing changes, where do you imagine you will be in five years?"
- **Readiness ruler:** "On 0–10, how important is it to change? How confident are you that you could? What would move you from 4 to 6?"

Avoid the readiness trap: a score of 3 is an argument for a smaller intervention, not for discharge.

ASSESSED

25-minute MI session with a trained role-player. Rated on: spirit, OARS fidelity, ratio of reflections to questions, and handling of discord.

Week 6 — Stages of change and the family

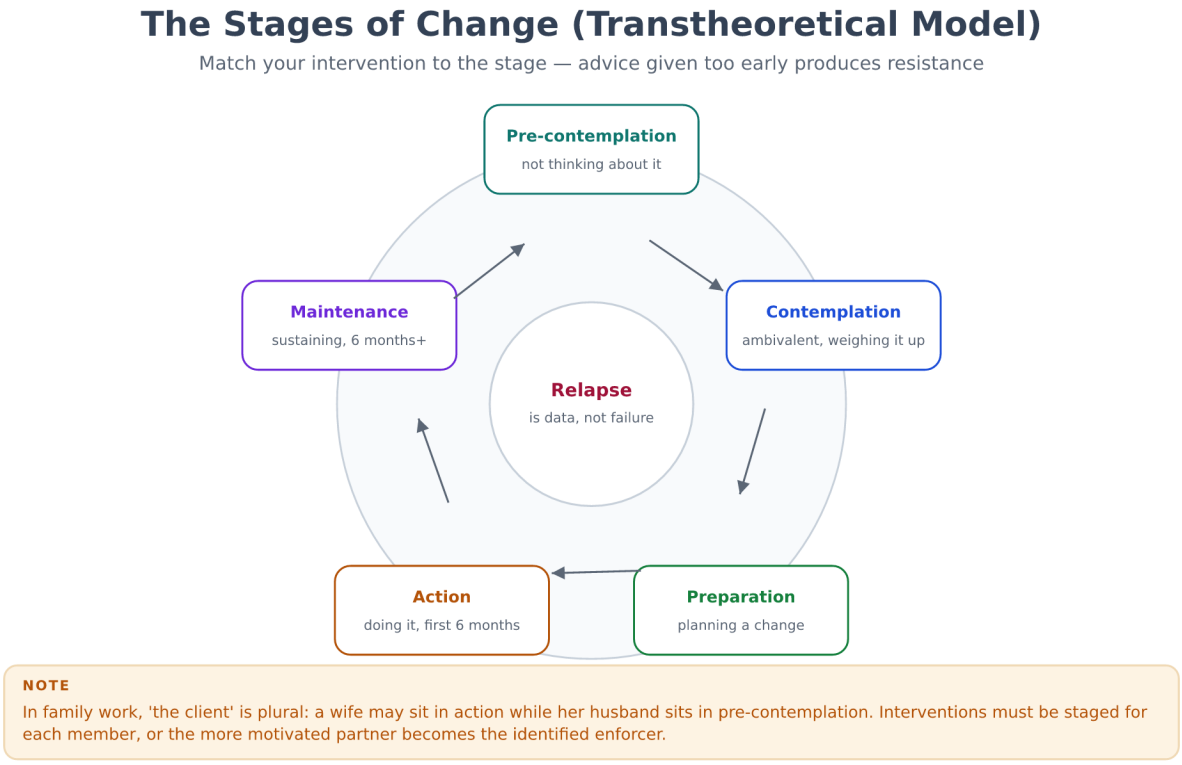


Figure 6.1 — The stages of change, including relapse

Stage	The person	The family member	Your task
Pre-contemplation	Not thinking about change	Frustrated, angry, exhausted	Raise doubt gently; do not push
Contemplation	Ambivalent	Hopeful and suspicious	Tip the balance; build confidence
Preparation	Planning	Mobilising	Concrete plan; remove barriers
Action	Changing	Adjusting, testing trust	Support, skills, relapse prevention
Maintenance	Sustaining	Rebuilding	Protect gains; plan for risk
Relapse	Using again	Devastated, "I told you"	Reframe as data; re-plan fast

The family's stage often differs from the user's. A wife in action married to a husband in pre-contemplation becomes the "enforcer", which damages the relationship and does not work. Stage-matched intervention means working separately where necessary.

Week 7 — Relapse prevention

The model (Marlatt & Gordon). Relapse is a process, not an event: high-risk situations → poor coping → lapse → the abstinence violation effect ("I've blown it, so I may as well continue") → relapse.

The plan.

1. **High-risk situations** — people, places, times, feelings (loneliness, anger, celebration, payday).
2. **Coping strategies** for each.
3. **A lapse protocol** — what to do in the first 24 hours, and whom to call.
4. **Lifestyle balance** — the "shoulds" versus the "wants"; boredom and resentment are relapse risks.
5. **The family's role** — agreed, non-punitive responses to a lapse, decided in advance.

With the family. A relapse response agreed in advance prevents the cycle of crisis, shame and secrecy from restarting. Write it down: "If I use again, we will…"

Week 8 — Behavioural couples therapy for substance use

The evidence. Behavioural couples therapy (BCT) for alcohol and drug problems has good evidence, particularly where the partner is non-using and the relationship is not violent.

The "daily recovery plan" (O'Farrell). Each day: a shared trust discussion in which the abstinent partner states their intention and the other expresses support, then a specified rewarding activity together. It is behavioural, daily, and builds the relationship as a recovery resource.

Why couples work helps. Partners are often the strongest motivator and the most reliable monitoring system — but only if the relationship becomes a source of reward rather than a site of surveillance.

Contraindications. Current violence, coercive control, a partner with active untreated addiction, or a relationship the client is being coerced to remain in.

Week 9 — Working with relatives: CRAFT

CRAFT (Community Reinforcement and Family Training) works with the concerned significant other to change the contingencies around the user's behaviour, without confrontation. It has better evidence for treatment entry than traditional "interventions" or Al-Anon-style support alone.

Three goals. 1) Improve the relative's own wellbeing; 2) reduce the user's substance use; 3) increase the chance the user enters treatment.

Core skills taught to the relative.

- **Functional analysis:** what triggers use, what consequences follow, what the relative does.
- **Positive communication:** brief, specific, warm.
- **Allow natural consequences:** stop removing consequences where it is safe.
- **Reward non-use:** notice and reinforce sober days.
- **Domestic safety:** never ask a relative to accept violence.
- **Self-care and social support:** the relative's life must not be entirely about the user.
- **Invitation to treatment:** a script, rehearsed.

> **PRACTICAL**
> CRAFT role-play: a mother of a 24-year-old cannabis user. Teach positive communication and one natural consequence. Assessor rates warmth, specificity and safety.

Week 10 — Enabling, co-dependency and boundaries

"Co-dependency" is a contested term. Used carefully it describes patterns of over-functioning and self-neglect in relationships with an addicted person. Used carelessly it blames the relative — usually a woman — for the addiction.

Better language: accommodation and boundary. Ask what the relative does, what it costs them, and what they would like to do differently.

Boundary setting that works. Specific, behavioural, enforceable, and about the relative's own actions: "I will not lend money. If you come home intoxicated, I will sleep in the other room and we will talk in the morning." Not: "You must stop drinking or I will leave." (An unenforceable ultimatum teaches the family that threats are empty.)

Week 11 — Dual diagnosis

The overlap is the rule. Anxiety, depression, PTSD, ADHD and psychosis commonly co-occur with substance use. Direction of causality varies; the treatment must be integrated rather than sequential.

Principles.

1. Treat both, at the same time, by the same team where possible.
2. Do not require abstinence before treating mental illness, and do not require remission of mental illness before treating addiction.
3. Be cautious with medication (benzodiazepines, stimulants) and coordinate with the prescriber.
4. Distinguish substance-induced symptoms from independent disorders (timing and persistence).
5. Trauma is common; assume it until assessed.

CASE PLAN (35%)
A family case with dual diagnosis: assessment, formulation, integrated plan, coordination with medical and recovery services, risk plan, and outcome measures.

Week 12 — Medication-assisted treatment

Opioid agonist therapy (methadone, buprenorphine) reduces mortality and improves retention; it is evidence-based treatment, not "substituting one drug for another". **Acamprosate** and **naltrexone** reduce alcohol relapse; **disulfiram** works only with supervision. Withdrawal management alone is not treatment.

The family therapist's role. Support adherence, address stigma within the family, challenge the belief that medication is cheating, and coordinate with the prescriber. Do not advise on doses or discontinuation.

Week 13 — Recovery as a family process

Recovery capital (Cloud & Granfield): social (relationships, support), physical (health, housing), human (skills, education, health literacy), cultural (values, belonging). Treatment that builds recovery capital predicts better outcomes than treatment that focuses only on symptoms.

The family's long haul. Early recovery is often called "the pink cloud"; the harder work is at six to eighteen months, when the family must renegotiate roles that were organised around the addiction. A partner who has run everything may not want to hand back control; a child who has been the carer may not want to be a child. Name this, and plan for it.

Relapse is data. The question is not "why did you fail?" but "what happened, and what does the plan need?"

Week 14 — Culture, faith, stigma and community

- **Substance use in context:** alcohol is culturally embedded in many Nigerian settings (burial rites, festivals, business); kola, cannabis, tramadol and codeine have specific local patterns.
- **Faith:** deliverance and prayer can be powerful resources and can also delay medical care. Coordinate, do not compete.
- **Stigma:** "a disgrace to the family" keeps use hidden and prevents early help. Your language matters: "person with a substance use disorder", not "addict".
- **Community resources:** mutual aid (AA/NA and their local equivalents), faith-based recovery groups, community health workers — map what actually exists near your clinic.

Week 15 — Integration

What works: screening and brief intervention; MI; CBT-based relapse prevention; BCT and CRAFT; medication where indicated; integrated treatment for dual diagnosis; recovery support; and continuity over time. What does not: confrontation, "tough love" as a single strategy, sequential treatment for dual diagnosis, and asking a family to do the work alone.

Glossary

Abstinence violation effect — the spiral of guilt and continued use after a lapse. **CRAFT** — Community Reinforcement and Family Training, for concerned significant others. **Change talk** — client statements favouring change. **Dual diagnosis** — co-occurring substance use and mental disorder. **Enabling** — behaviours that remove the natural consequences of use. **Harm reduction** — interventions that reduce harm without requiring abstinence. **Motivational interviewing** — a collaborative, directive method for strengthening intrinsic motivation. **Recovery capital** — the resources a person can draw on to sustain recovery. **Relapse** — return to problematic use after a period of change.

Self-assessment

Multiple choice

1. The righting reflex refers to: (a) the client's resistance (b) the clinician's urge to fix (c) a CBT technique (d) a legal duty

2. CRAFT is aimed primarily at: (a) the user (b) concerned relatives (c) employers (d) courts

3. Change talk includes: (a) sustain talk (b) desire, ability, reasons, need (c) only commitment (d) only questions

4. Medication-assisted treatment is: (a) substituting one drug for another (b) evidence-based treatment that reduces mortality (c) only for the wealthy (d) discouraged

5. The "abstinence violation effect" describes: (a) legal consequences (b) the spiral after a lapse (c) withdrawal (d) family conflict

6. A well-formed boundary is: (a) an unenforceable ultimatum (b) specific, behavioural and about the relative's own actions (c) a threat (d) a diagnosis

7. In dual diagnosis, treatment should be: (a) sequential (b) integrated (c) only psychiatric (d) only addiction-focused

8. Enabling is best understood as: (a) malice (b) reducing immediate consequences out of care (c) a diagnosis (d) irrelevant

9. Relapse should be treated as: (a) failure (b) data for re-planning (c) grounds for discharge (d) proof of character

10. Recovery capital includes: (a) only abstinence (b) social, physical, human and cultural resources (c) only income (d) only family support

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Compare the disease and systemic models of addiction and name one clinical implication of each.
2. Map the six stations of the family cycle and give one intervention at each.
3. What are OARS, and what is the recommended ratio of reflections to questions?
4. Describe the daily recovery plan in behavioural couples therapy.
5. Name the three goals of CRAFT.

Essay (choose one, 2,000 words)

1. "The family is neither the cause nor the cure, but it is the context." Discuss with a case.
2. Critically evaluate the concept of co-dependency and its gendered assumptions.
3. Design a family-inclusive service for adolescents with substance use problems in a Nigerian city.

Further reading

- Miller, W. R., & Rollnick, S. (2013). Motivational Interviewing (3rd ed.). Guilford.
- O'Farrell, T. J., & Fals-Stewart, W. (2006). Behavioral Couples Therapy for Alcoholism and Drug Abuse. Guilford.
- Smith, J. E., & Meyers, R. J. (2004). Motivating Substance Abusers to Enter Treatment: Working with Family Members. Guilford.
- Marlatt, G. A., & Donovan, D. M. (2005). Relapse Prevention (2nd ed.). Guilford.
- Cloud, W., & Granfield, R. (2008). Conceptualizing recovery capital. Substance Use & Misuse, 43(12–13).

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	**Trauma-Informed Care**	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Trauma changes the body, the memory, the attachment system and the family. This course teaches the physiology, the phased treatment models, and the organisational stance — and it teaches you to avoid the two commonest errors: doing too much too soon, and doing nothing at all.

How to use this book

This is the most personally demanding book in the programme. Announce it in advance to your peers; know your own history; use supervision; and do not read the trauma chapters late at night. The material is presented in the order treatment unfolds: safety, then processing, then integration.

1. Course description

Trauma-informed care means organising practice around the prevalence and impact of trauma; **trauma- specific** care means delivering interventions designed to treat it. You need both.

The course covers: definitions and types of trauma; the Adverse Childhood Experiences evidence; the neurobiology of threat and regulation; dissociation; attachment and developmental trauma; the phased model; specific modalities (TF-CBT, EMDR, narrative exposure, somatic approaches); family and couple work with trauma; intergenerational transmission; re-traumatisation in services; and practitioner self-protection.

2. Learning outcomes

#	Outcome	Assessed
LO1	Define trauma and distinguish types, including complex and developmental trauma	Exam
LO2	Explain the neurobiology of threat, regulation and memory	Exam
LO3	Conduct a trauma-informed assessment without re-traumatising	Observed assessment
LO4	Apply a phased treatment model and justify the sequencing	Case study
LO5	Use at least two trauma-specific interventions under supervision	Skills test
LO6	Work systemically with trauma in couples and families	Case study
LO7	Design trauma-informed organisational practices	Report

3. Assessment

Component	Weight	Due
Trauma-informed assessment observed (40 min)	25%	Week 7

Component	Weight	Due
Phased treatment plan (2,500 words)	35%	Week 12
Trauma-informed service audit report (2,000 words)	20%	Week 14
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	What trauma is: single, complex, developmental, collective	Typology exercise
2	The ACE evidence and its limits	ACE discussion (with care)
3	Neurobiology I: threat, arousal and the window of tolerance	Regulation skills lab
4	Neurobiology II: memory, dissociation and the body	Grounding practice
5	Attachment and developmental trauma	Attachment-informed formulation
6	Trauma-informed assessment	Structured interview
7	**Observed assessment**	Assessed
8	The phased model: safety, processing, integration	Phase planning
9	Trauma-focused CBT with children and families	TF-CBT component practice
10	EMDR, narrative exposure and somatic approaches	Comparison and critique
11	Couples and trauma: betrayal, violence, and the repair of trust	Couple session design
12	**Treatment plan due**; intergenerational transmission	Genogram of trauma
13	Re-traumatisation in services; the four R's	Service audit
14	**Service audit due**; vicarious trauma and supervision	Personal resilience plan
15	Integration: what trauma-informed care is not	Case presentations

Week 1 — What trauma is

Definition (DSM-5/ICD-11, paraphrased). Exposure to actual or threatened death, serious injury or sexual violence, whether experienced directly, witnessed, learned about in relation to a close person, or through repeated occupational exposure.

Types.

Type	Description	Example
Single-incident	One event, adult onset	Road traffic collision, armed robbery
Complex	Repeated, prolonged, interpersonal, inescapable	Domestic abuse, childhood abuse

Type	Description	Example
Developmental	Occurs during maturation; shapes personality and attachment	Chronic neglect, chaotic caregiving
Collective	Shared by a community	War, displacement, massacre, epidemic
Vicarious / secondary	Exposure through another's trauma	Therapist, first responder, family carer

What makes an event traumatic is not only the event. It is the event plus the meaning, the interpersonal context (was it caused by someone who was supposed to protect you?), the aftermath (was the person believed? supported?), and prior vulnerability.

Complex PTSD (ICD-11) comprises three additional clusters beyond classic PTSD: affect dysregulation, negative self-concept, and disturbances in relationships. This matters clinically: classic PTSD responds well to exposure-based treatment; complex PTSD usually requires a longer stabilisation phase.

> **PRACTICAL**
> Classify five disguised cases by type, and note which additional features each has.

Week 2 — The ACE evidence

The original study. Adverse Childhood Experiences (abuse, neglect, household dysfunction) were found to have a graded relationship with later health outcomes, including depression, addiction, suicide attempts, heart disease and early death.

What it established. Early adversity is common, cumulative, and biologically embedded. It shifted the field from asking "what is wrong with you?" to "what happened to you?"

What it does not establish. That ACE scores are destiny; that any individual's future is determined; or that a high score is a diagnosis. Roughly a third of people with high ACE scores have no significant impairment — protective relationships matter enormously.

The clinical use. ACE enquiry is a conversation, not a score: "Was there anything difficult in your childhood that we should know about? You do not have to give details."

> **CAUTION**
> Asking about adversity can activate distress. Ask when you have time to respond, offer a choice, and have a grounding plan ready.

Week 3 — Neurobiology I: the window of tolerance

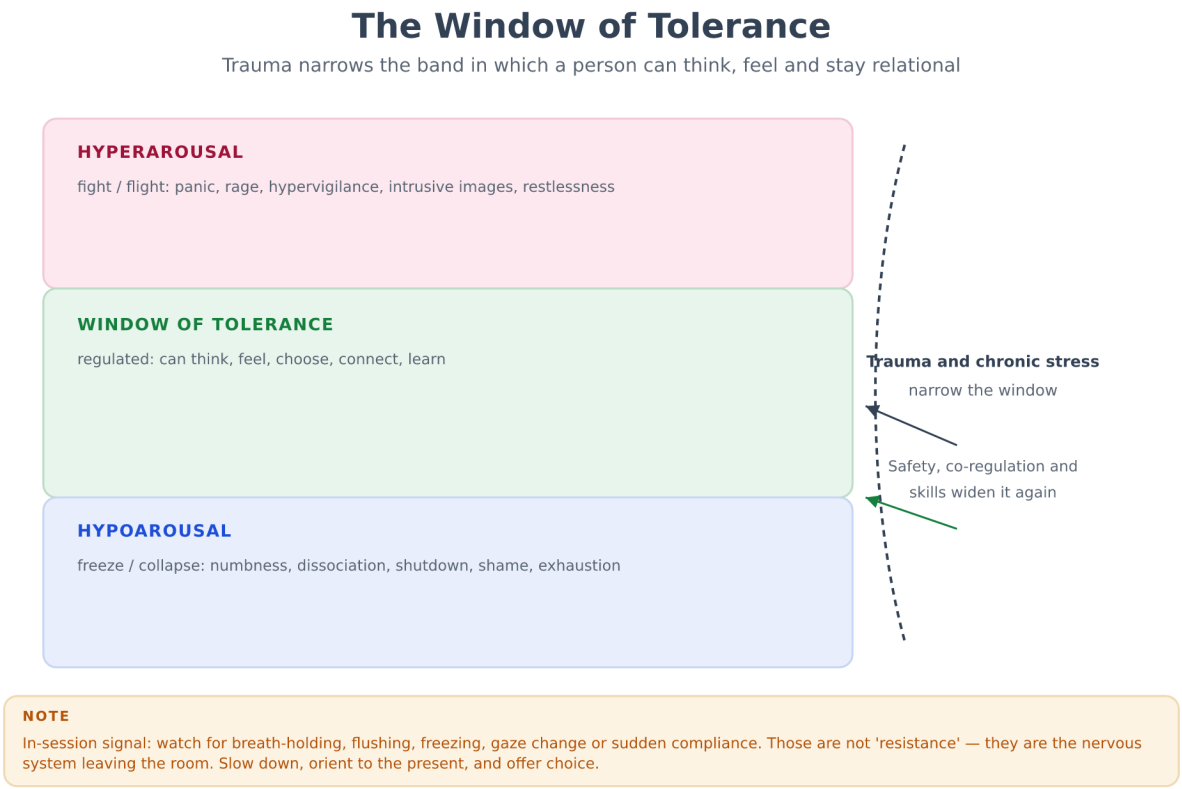


Figure 3.1 — The window of tolerance

The three states.

- **Within the window:** the person can think, feel, choose, connect and learn.
- **Hyperarousal:** fight/flight — panic, rage, hypervigilance, intrusive images, restlessness.
- **Hypoarousal:** freeze/collapse — numbness, dissociation, shutdown, shame, exhaustion.

Trauma narrows the window. Small triggers produce large reactions; the person's capacity to stay relational shrinks. This is not "over-reacting"; it is a nervous system doing what it learned.

Regulation skills to teach (and use in session).

1. **Paced breathing** — exhale longer than inhale (4 in, 6 out) for 90 seconds.
2. **Orienting** — look around the room and name five things you can see.
3. **Grounding** — feet on the floor, weight in the chair, describe the texture of an object.
4. **Movement** — stand, walk, stretch; discharge the mobilised energy.
5. **Temperature and touch** — cold water, a heavy blanket, pressure.
6. **Naming** — "I am having a memory, not the event."

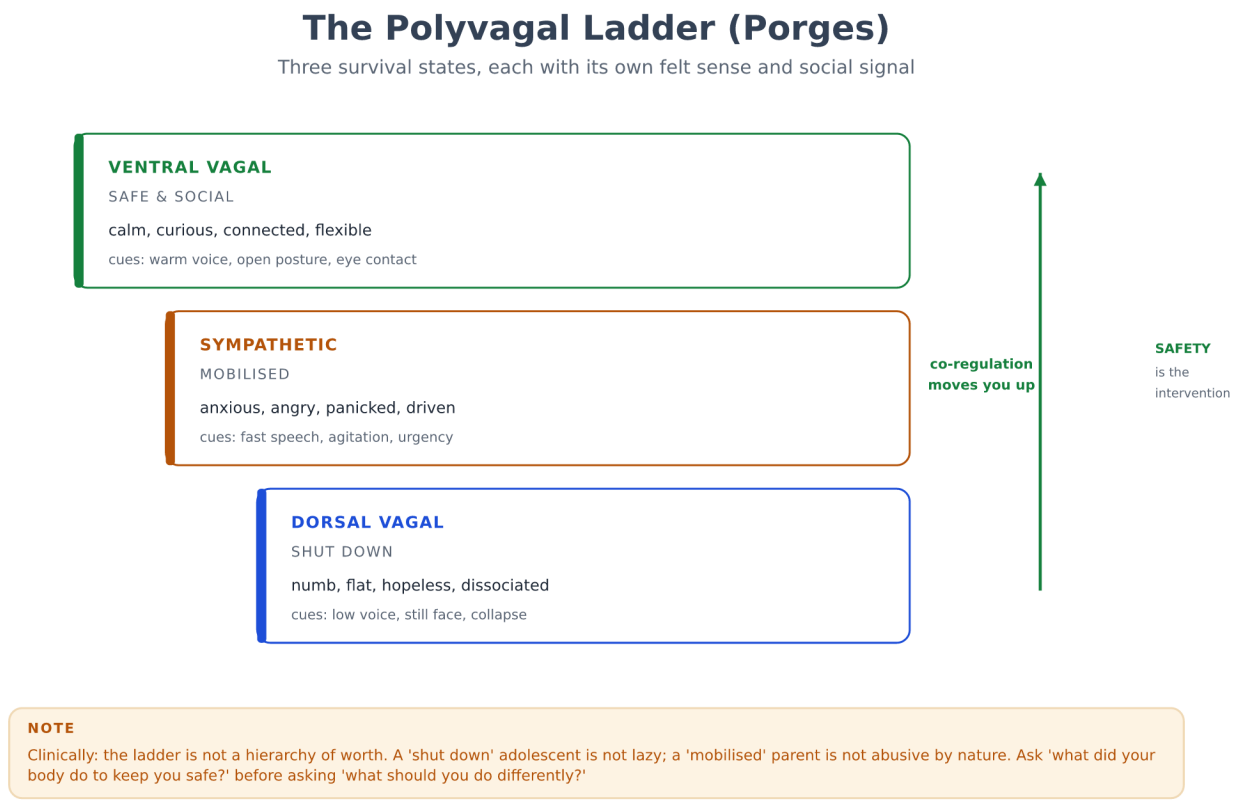


Figure 3.2 — The polyvagal ladder

In family sessions. If one member leaves the window, the session has ended for them. Pause, regulate, and return. Teaching a couple to recognise each other's early signals is itself an intervention.

Week 4 — Neurobiology II: memory, dissociation, the body

Traumatic memory is different. It is often implicit, sensory and fragmented: smells, images, body sensations, without a coherent narrative or a sense of "pastness". This is why "just talk about it" can flood a person and why containment matters.

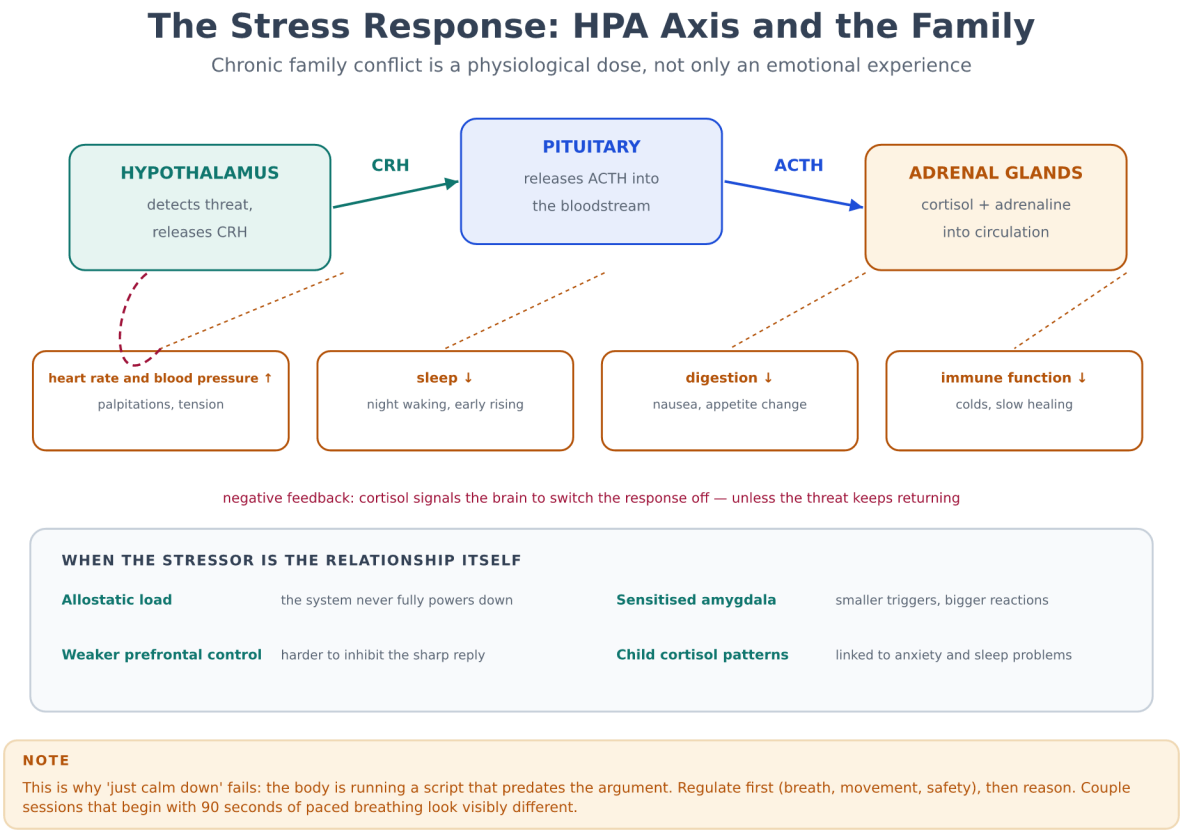


Figure 4.1 — The HPA axis and chronic stress

Dissociation — a disruption of consciousness, memory, identity or perception. Mild forms are common (going blank, feeling unreal); severe forms include depersonalisation, derealisation and dissociative amnesia. Assess gently: "Do you ever feel like you are watching yourself from outside? Do you lose time?"

The clinical rule. Do not interpret a dissociative response as resistance. If a client's eyes glaze, stop the exercise, orient them to the room, and slow down. Then shorten the next segment.

Week 5 — Attachment and developmental trauma

Developmental trauma (sometimes "complex relational trauma") occurs when the person who should provide safety is the source of danger. The child adapts with strategies that later look like personality: vigilance, people-pleasing, aggression, emotional shutdown, compulsive caregiving.

The internal working model. "I am unlovable; others are dangerous or unreliable; I must manage alone (or never let go)." These beliefs are not irrational; they were accurate in the original environment.

Clinical implications.

1. Expect the therapeutic relationship to be tested — idealisation, devaluation, testing, sudden withdrawal.
2. Predictability and reliability matter more than technique.
3. Repair after rupture is the treatment, not a distraction from it.
4. Do not interpret the transference early; be the relationship first.

Family application. Parents with their own unresolved trauma often misread their children's ordinary behaviour as threat. Psychoeducation about trauma and parenting can reduce harsh discipline dramatically.

Week 6 — Trauma-informed assessment

The four R's (SAMHSA).

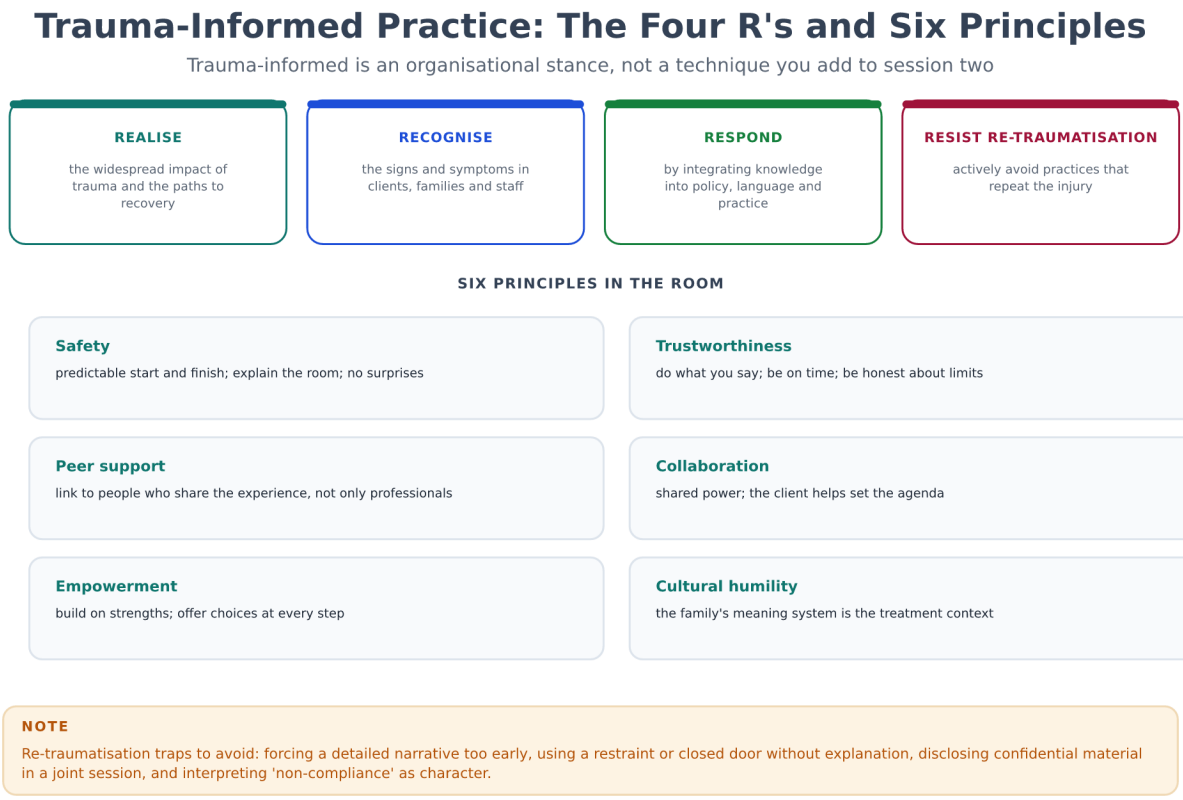


Figure 6.1 — The four R's and six principles of trauma-informed practice

1. **Realise** the widespread impact of trauma.
2. **Recognise** the signs and symptoms.
3. **Respond** by integrating knowledge into practice and policy.
4. **Resist re-traumatisation.**

Assessment principles.

- Explain why you are asking, and offer choices ("you can skip any question").

- Ask about the event only at the level of detail needed for formulation; detail belongs in processing.
- Ask about **strengths, survival strategies and meaning**, not only symptoms.
- Screen for dissociation before any trauma-focused work.
- Screen for current safety (violence, self-harm, substance use).

Instruments. PCL-5 (PTSD symptoms), IES-R, CTQ (childhood trauma; use with care), DES-II (dissociation), PHQ-9, AUDIT. Always follow up a positive screen with a conversation.

Week 7 — Observed trauma-informed assessment

Forty minutes. Assessed on: safety and choice throughout, pacing, grounding as needed, formulation without premature interpretation, and accurate risk screening.

Week 8 — The phased model

Phase 1: Safety and stabilisation. Establish safety, build the alliance, teach regulation, address practical dangers (housing, violence, substance use), and develop what Herman called "the capacity to contain".

Phase 2: Processing. Work through the traumatic material using a validated method (TF-CBT, EMDR, narrative exposure, prolonged exposure) — only when the person can tolerate it and has stabilisation skills.

Phase 3: Integration and reconnection. Rebuild identity, relationships, work, and meaning. This phase is often the longest and the most neglected.

Why sequence matters. Processing without stabilisation produces flooding, drop-out, and sometimes retraumatisation. Stabilisation without processing can produce a stable but constrained life. The judgment is clinical, not clerical: some people need two sessions of stabilisation; some need twenty.

Family application. The family's safety and stability is Phase 1 at the system level. You cannot do trauma processing in a household where the perpetrator is still present.

Week 9 — Trauma-focused CBT with children

TF-CBT (Cohen, Mannarino & Deblinger) is the best-evidenced treatment for childhood trauma. Its PRACTICE components:

Component	Content
Psychoeducation and parenting skills	Trauma responses; consistent, non-punitive discipline
Relaxation	Breathing, muscle relaxation, grounding
Affect expression and modulation	Feelings identification, coping

Component	Content
Cognitive processing	Correcting trauma-related beliefs ("it was my fault")
Trauma narrative	Gradual, structured account in the child's medium (book, drawing)
In vivo mastery	Facing avoided, safe situations
Conjoint sessions	Child shares the narrative with the caregiver
Enhancing safety	Safety skills and planning for the future

The caregiver's role is essential. The non-offending caregiver's belief, protection and support is the strongest predictor of recovery. Work with them in parallel.

Week 10 — Other trauma-specific modalities

Modality	Mechanism	Evidence	Cautions
EMDR	Bilateral stimulation during memory recall	Strong for PTSD	Requires training; not a magic technique
Narrative exposure therapy	Chronological testimony of the whole life	Strong for multiple/collective trauma	Can be destabilising without stabilisation
Prolonged exposure	Repeated imaginal and in vivo exposure	Strong	Drop-out; requires strong alliance
Somatic approaches	Body-based regulation and discharge	Emerging evidence	Beware of unfounded claims
Yoga / mindful movement	Regulation, interoception	Good adjunctive evidence	Not a substitute for trauma processing

The professional duty. Use what you are trained and supervised to use. A half-learned trauma protocol is more dangerous than a well-delivered stabilisation phase.

Week 11 — Couples and trauma

Three presentations.

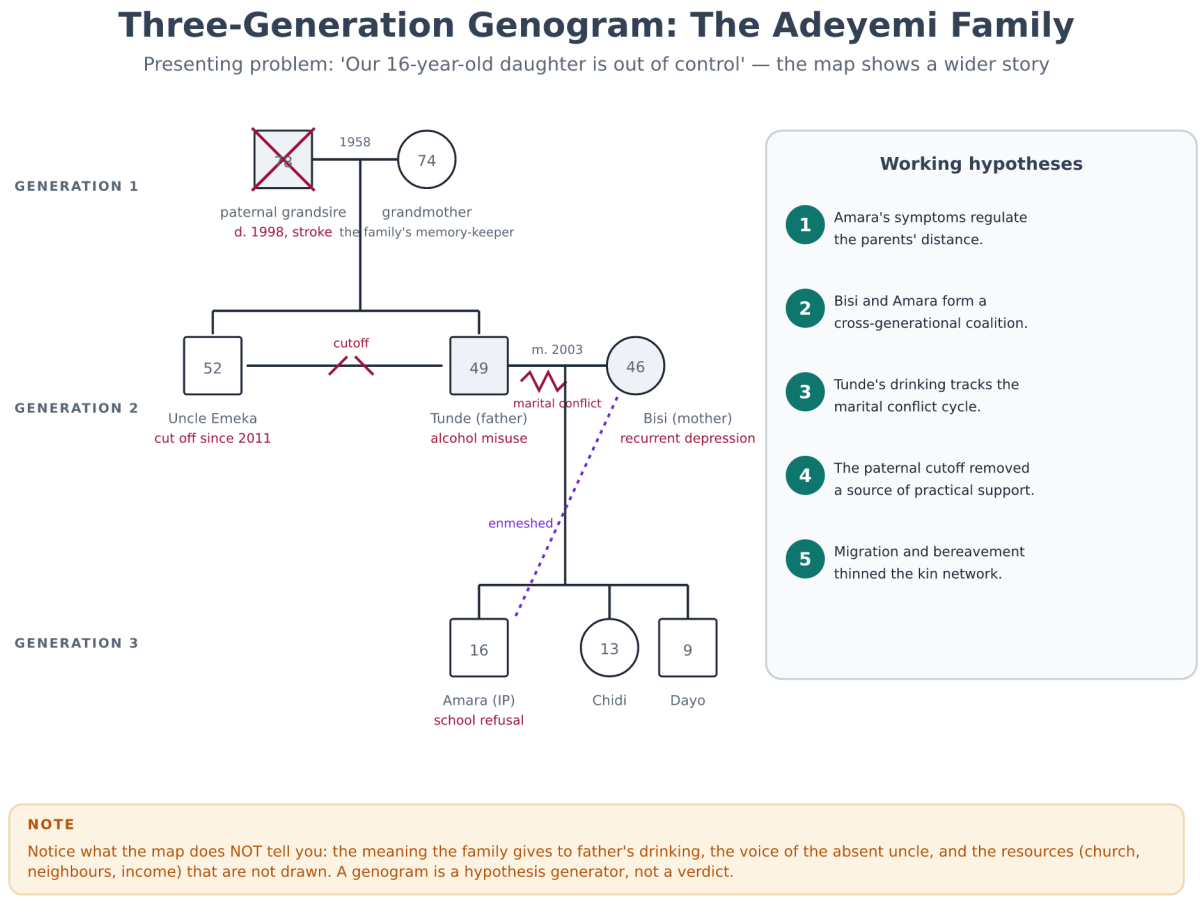
1. **One partner has a trauma history.** The other becomes a de facto carer; intimacy becomes a trigger; the couple's sexual relationship collapses. Work with both: psychoeducation for the partner, boundaries for the survivor, and a paced intimacy plan.
2. **The trauma happened inside the relationship** (betrayal, violence, coercion). This is betrayal trauma. Safety and accountability come first; joint work only where the offending partner acknowledges the harm and the victim chooses it.
3. **Both partners are trauma survivors.** Two dysregulated nervous systems in one room. The pace must be slower, the structure tighter, and the regulation explicit.

The couple-level formulation. "When she freezes, he experiences rejection and pursues; when he pursues, she freezes harder. The loop is a collision of two survival strategies, not a failure of love."

Week 12 — Intergenerational transmission

Mechanisms. Parenting behaviour (frightening, inconsistent, harsh), attachment patterns, family narrative, epigenetic and stress-physiology effects, and structural factors (poverty, displacement, community violence).

The genogram of trauma. Draw three generations and mark losses, violence, separation, migration, untimely deaths and secrets. Then ask: what was never spoken about? What did each generation have to survive?



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 12.1 — A three-generation genogram as a map of transmission

The hopeful finding. Transmission is probabilistic, not deterministic. A consistent, protective relationship at any point breaks the chain. Your therapy may be that relationship.

Week 13 — Re-traumatisation in services

How services re-traumatise, usually without meaning to.

Practice	Why it harms	Alternative
Restraint without explanation	Replicates powerlessness	Explain, offer choices, debrief
Closed doors, no warning	Replicates entrapment	Predictability, warnings, exits
Being asked to repeat the story to every professional	Replicates helplessness	Record once, share with consent
Long waits in public areas	Replicates exposure	Private, quiet space
Disclosing in front of a partner	Replicates danger	Always screen individually
"Non-compliance" language	Replicates blame	"What got in the way?"

The organisational standard. Trauma-informed care is a policy and management issue: staff training, physical environment, restraint policy, complaint routes, and supervision. A trauma-informed clinician in a trauma-organised service will burn out.

Week 14 — Vicarious trauma

Distinguish three concepts. Burnout (exhaustion from workload), compassion fatigue (reduced empathy), and vicarious trauma (changes to the therapist's beliefs and inner world from exposure to others' trauma).

Signs. Intrusion of clients' material, nightmares, cynicism, loss of hope, changed worldview ("the world is unsafe"), avoidance of trauma cases, over-involvement.

Protection. Supervision that addresses the work's emotional impact; caseload diversity; clear boundaries; your own therapy; sleep, exercise and relationships; and acknowledging meaning — many clinicians find that sustained trauma work deepens their sense of purpose when it is properly supported.

Week 15 — What trauma-informed care is not

It is not the avoidance of difficult material. It is not an excuse for never processing trauma. It is not a synonym for "be gentle". It is a disciplined sequencing of safety, processing and integration, delivered in a relationship and an organisation that do not repeat the injury.

Glossary

Adverse childhood experiences (ACEs) — potentially traumatic events in childhood, measured cumulatively. **Betrayal trauma** — trauma caused by someone on whom the person depends. **Complex PTSD** — PTSD plus affect dysregulation, negative self-concept and relational disturbance. **Dissociation** — disruption of consciousness, memory, identity or perception. **Grounding** — techniques that return attention to the present and the body. **Re-traumatisation** — repetition of the dynamics of the original trauma in the therapy or service. **Stabilisation** — the first phase of trauma treatment, focused on safety and regulation. **Window of tolerance** — the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Complex PTSD is distinguished by: (a) one event (b) affect dysregulation, negative self-concept, relational disturbance (c) psychosis (d) amnesia only

2. The first phase of trauma treatment is: (a) processing (b) safety and stabilisation (c) integration (d) exposure

3. Dissociation should first be: (a) interpreted (b) assessed and accommodated (c) ignored (d) challenged

4. TF-CBT's trauma narrative is: (a) forced in session one (b) gradual and structured, in the child's medium (c) written by the therapist (d) optional and never used

5. The strongest predictor of recovery in childhood trauma is: (a) the trauma's severity (b) a believing, protective caregiver (c) IQ (d) medication

6. Vicarious trauma is: (a) the same as burnout (b) changes to the therapist's beliefs and inner world from exposure (c) a diagnosis (d) avoidable by holidays

7. Re-traumatisation in services often occurs through: (a) privacy (b) predictability (c) restraint without explanation (d) choice

8. Processing trauma without stabilisation risks: (a) faster recovery (b) flooding and drop-out (c) improved alliance (d) no effect

9. Intergenerational transmission is: (a) deterministic (b) probabilistic and can be interrupted (c) genetic only (d) irrelevant

10. A trauma-informed organisation is characterised by: (a) only individual therapy (b) policy, environment, training and supervision aligned with trauma knowledge (c) shorter sessions (d) avoiding trauma discussion

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. Distinguish complex PTSD from classic PTSD.
2. Describe three regulation skills and when you would use each.
3. Name the four R's of trauma-informed care.
4. Give the three phases of trauma treatment with the task of each.
5. List four ways services re-traumatise clients and one alternative for each.

Essay (choose one, 2,000 words)

1. "Trauma-informed care is a stance, not a technique." Discuss with reference to your own practice.
2. Design a trauma-informed family service: policies, environment, staffing and clinical pathway.
3. Analyse the challenges of trauma work with a couple where the trauma occurred inside the relationship.

Further reading

- Herman, J. L. (1992/2015). Trauma and Recovery. Basic Books.
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- Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford.
- Porges, S. W. (2011). The Polyvagal Theory. Norton.
- SAMHSA. (2014). SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach.
- Felitti, V. J. et al. (1998). Relationship of childhood abuse and household dysfunction to many of the leading causes of death in adults. American Journal of Preventive Medicine, 14(4), 245–258.

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	**Family Systems Theory**	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the theoretical anchor of the programme. Where MFT 702 surveys schools, this course interrogates the theory beneath them: cybernetics, general systems thinking, structure, communication, constructionism and their critics. You will finish able to explain precisely what you mean when you say a family is "a system".

How to use this book

The word "system" is used loosely in clinical settings. This course tightens it. Each week takes one theoretical tradition, states its claims precisely, applies it to a case, and then asks what it cannot explain. You are expected to leave with arguments, not slogans.

1. Course description

Family systems theory is not one theory but a family of theories sharing a commitment: **the unit of analysis is the relationship, and the whole is more than the sum of its parts**. The course traces that commitment from von Bertalanffy's general systems theory and the Macy conferences on cybernetics, through the Palo Alto communication school, Minuchin's structuralism, Bowen's multigenerational theory, and the Milan group, to social constructionism, critical and feminist theory, and contemporary complexity and network perspectives.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the core propositions of general systems theory and cybernetics	Exam
LO2	Analyse a family using at least four theoretical traditions	Case analysis
LO3	Critique each tradition, including its cultural and political assumptions	Essay
LO4	Distinguish first- and second-order cybernetics and demonstrate reflexivity	Viva
LO5	Apply constructionist ideas to meaning-making in a family	Case analysis
LO6	Evaluate the evidence for systemic propositions	Exam
LO7	Construct a coherent theoretical position of your own	Position paper

3. Assessment

Component	Weight	Due
Theoretical analysis of a case using four traditions (2,500 words)	35%	Week 9
Essay: critique of a systemic concept (2,000 words)	25%	Week 13
Examination	25%	Week 16

Component	Weight	Due
Position paper and viva (1,000 words)	15%	Week 15

4. The fifteen-week plan

Week	Topic	Practical
1	The systemic turn: what it was and what it answered	Compare two accounts of one case
2	General systems theory: wholes, boundaries, equifinality	Systems vocabulary drill
3	Cybernetics I: feedback, control, homeostasis	Feedback mapping
4	Cybernetics II: first and second order; reflexivity	Reflexivity exercise
5	Communication theory: axioms, punctuation, paradox	Transcript analysis
6	Structural theory: organisation, hierarchy, boundaries	Structural mapping
7	Multigenerational theory: anxiety, triangles, differentiation	Triangle mapping
8	Milan and meaning: beliefs, premises, circularity	Circular questioning
9	**Theoretical analysis due**; constructivism and social constructionism	Meaning-making interview
10	Postmodern and narrative theory: knowledge and power	Texts and stories exercise
11	Critical, feminist and anti-racist systems theory	Power audit of a model
12	Attachment and neuroscience meet systems theory	Integration diagram
13	**Essay due**; critiques and limitations of systems thinking	Debate
14	Complexity, networks and the contemporary field	Network map of a family
15	Your theoretical position	Viva

Week 1 — The systemic turn

What changed. Before the systemic turn, the family was understood as the environment of an individual patient. After it, the family became the unit of treatment, and individual symptoms became functions of relationship patterns.

What it answered. Three problems the individual model struggled with: Why does a symptom appear in a particular child? Why does the family resist change (homeostasis)? Why does the same individual behave differently in different relationships?

What it risked. Dissolving the person into the system; treating "the family" as a homogeneous actor; ignoring power, gender, class and history; and producing descriptions that could not be falsified.

PRACTICAL

Take one case. Write it twice: once as an individual formulation (as a CBT or psychodynamic clinician would) and once as a systemic one. Then write a paragraph on what each version makes visible and invisible.

Week 2 — General systems theory

Von Bertalanffy's propositions, translated for clinicians:

Proposition	Clinical meaning
A system is a set of elements in interaction	The family is defined by its interactions, not its members
The whole is more than the sum of its parts	The family has properties no member has alone
Systems are open or closed to their environment	A family that refuses outside contact becomes closed and rigid
Systems have boundaries	Who is inside, who is outside, and how permeable
Systems are hierarchical	Subsystems nested within larger systems
Equifinality	Different starting points can reach the same end state
Equipotentiality	Similar starting points can reach different end states
Negative entropy	Systems import energy to counter disorder (information, support)

Equifinality and equipotentiality are the most useful and most neglected. Equifinality explains why two very different families produce the same symptom; equipotentiality explains why two similar families produce very different outcomes. Together they demolish simple causal stories.

Example. Two children are referred for school refusal: one in a family with high conflict and one in a family with high achievement pressure. Equifinality explains the identical presentation; equipotentiality explains why one family recovers in four sessions and the other takes two years.

Week 3 — Cybernetics I

Feedback. A system regulates itself by monitoring its own output and adjusting its input.

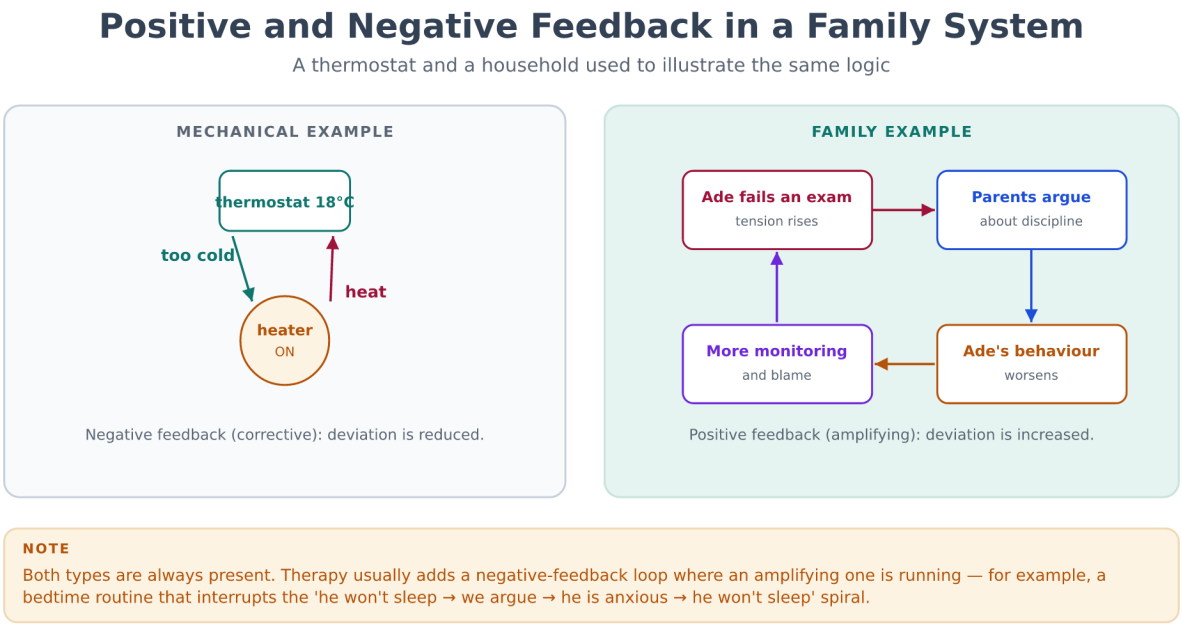


Figure 3.1 — Negative and positive feedback in a machine and in a family

- **Negative feedback** corrects deviation: the thermostat, the bedtime routine, the family that de-escalates when voices rise.
- **Positive feedback** amplifies deviation: the microphone screech, the escalation, the panic spiral.

Homeostasis. The system's tendency to maintain its organisation. **Morphostasis** maintains; **morphogenesis** changes. A family can be stably unhappy: the symptom is one of the mechanisms that holds the system together.

The clinical insight. Attempts to change a system are themselves inputs. If the family's equilibrium depends on the symptom, your intervention will produce a counter-move. Predicting the counter-move — and sometimes prescribing it — is the strategic tradition's contribution.

Calibration example. A family's daughter has "tantrums" that end a 45-minute argument between her parents. The tantrum is negative feedback for the marital system (it restores calm) and positive feedback for the child (it works). Two feedback systems, one event.

> **PRACTICAL**
> Map both feedback loops in a disguised case: one for each "layer" (the couple system and the child's behaviour). Show how the same event serves both.

Week 4 — Cybernetics II: first and second order

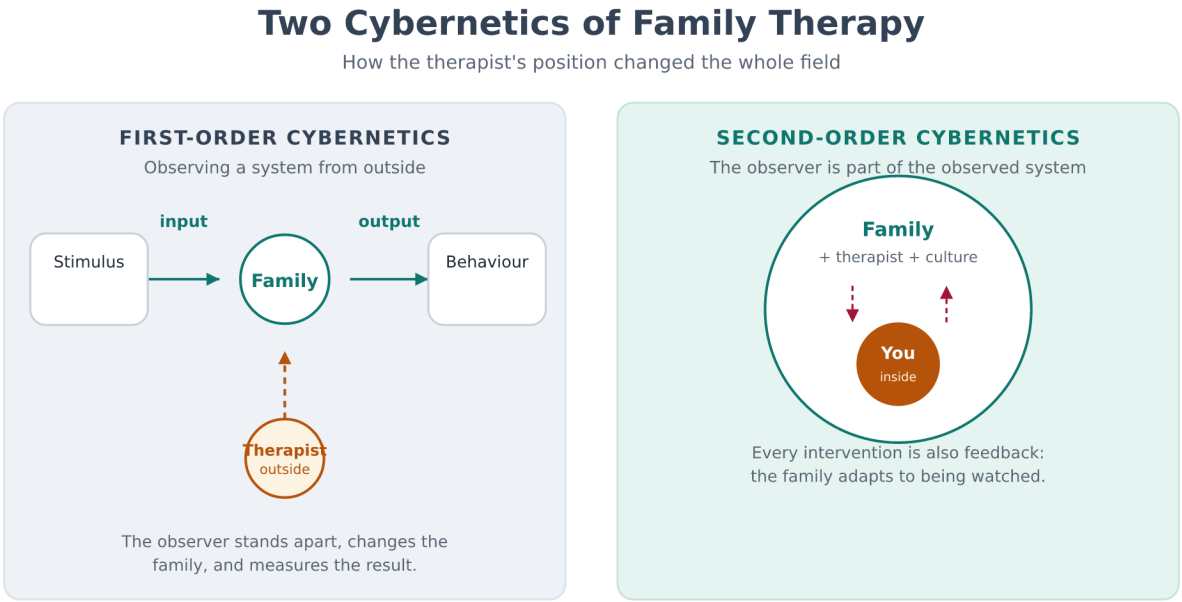


Figure 4.1 — First- and second-order cybernetics: where the therapist stands

First-order cybernetics. The observer is outside the system; interventions are inputs; the aim is control of the system's output. This produced the heroic era: directive, strategic, clever.

Second-order cybernetics. The observer is part of the observed system; distinctions are drawn by an observer; knowledge is provisional. This produced curiosity, the reflecting team, and the therapist's accountability for their own effect.

The clinical consequences.

First order	Second order
"What intervention will fix this family?"	"What am I noticing, and what am I blind to?"
The family is the object of change	The family and the therapist co-construct change
Neutrality is possible	Position is unavoidable; transparency is required
Expertise is the therapist's	Expertise is distributed

Reflexivity is not self-indulgence. It is the discipline of accounting for your own contribution to the clinical picture: which family member you find easier to like, which story you believed first, which data you stopped collecting once you had a hypothesis.

Week 5 — Communication theory



Figure 5.1 — Watzlawick's five axioms of communication

The five axioms (Watzlawick, Beavin & Jackson).

1. One cannot not communicate.
2. Every message has a content and a relationship level.
3. Communication is punctuated into sequences by participants.
4. Messages are digital (words) and analogical (analogue).
5. Relationships are symmetrical or complementary.

Double bind hypothesis (Bateson): a person receives contradictory messages at different levels, cannot comment on the contradiction, and cannot leave the relationship. The hypothesis was weakened as an explanation of schizophrenia, but its clinical descendant — recognising contradictory, un-commentable communications in families — remains useful.

Critique. Communication theory could become mechanistic (families as machines exchanging messages) and could pathologise families of people with psychosis. It also tended to describe "systems" without the power differences that determine who gets to define what is communicated.

> **PRACTICAL**
> Transcript analysis: take ten lines of a couple's argument and identify punctuation, relationship-level messages, and symmetrical/complementary moves.

Week 6 — Structural theory

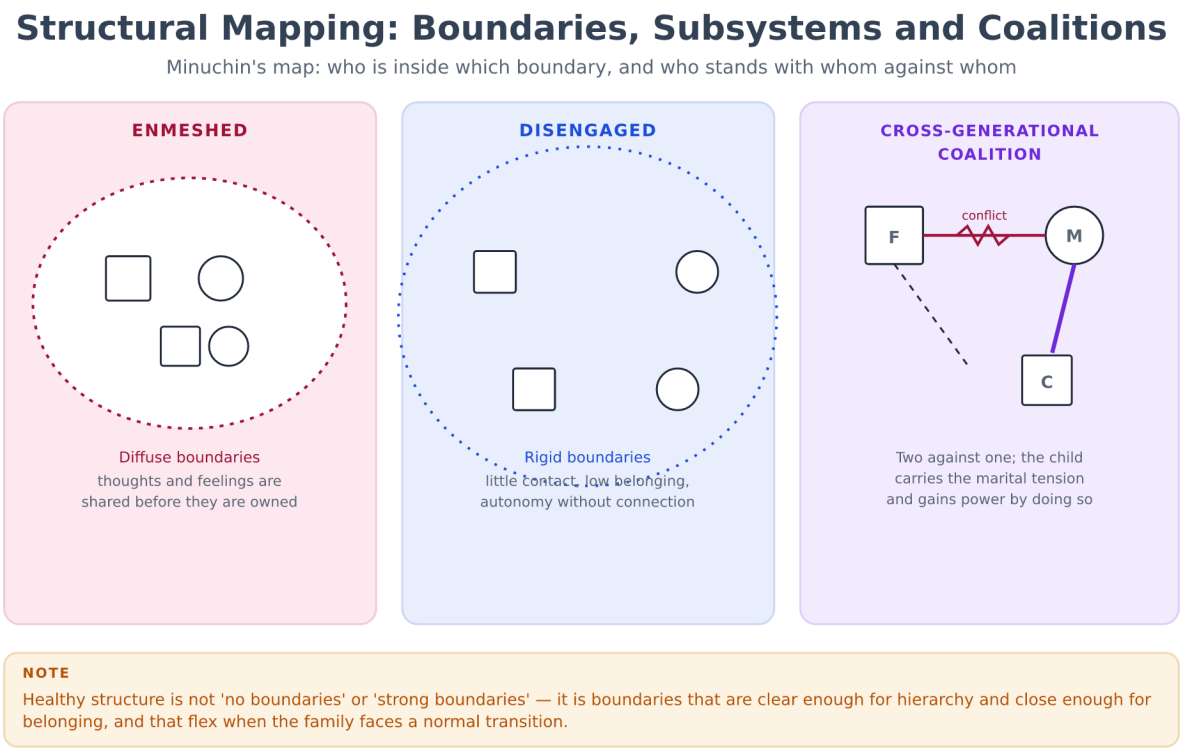


Figure 6.1 — Structural mapping: enmeshed, disengaged and cross-generational coalition

Core propositions (Minuchin).

1. The family's structure is the invisible set of rules that govern transactions.
2. The structure organises the family into subsystems (spousal, parental, sibling) separated by boundaries.
3. Symptoms are best understood as the family's response to its structure, and sometimes as the only way the structure can remain stable.
4. Change in structure produces change in experience.

The evaluation criteria for a structure. Boundaries must be clear (not diffuse, not rigid); hierarchy must be appropriate to stage and culture; and subsystems must have enough autonomy to function and enough connection to belong.

Power and structure. Structural theory was criticised for assuming a normative hierarchy (executive parents, subordinate children) and for not asking how that hierarchy is upheld. Contemporary structural practice inserts the questions: whose authority is legitimate in this family, and who bears the cost of it?

Week 7 — Multigenerational theory

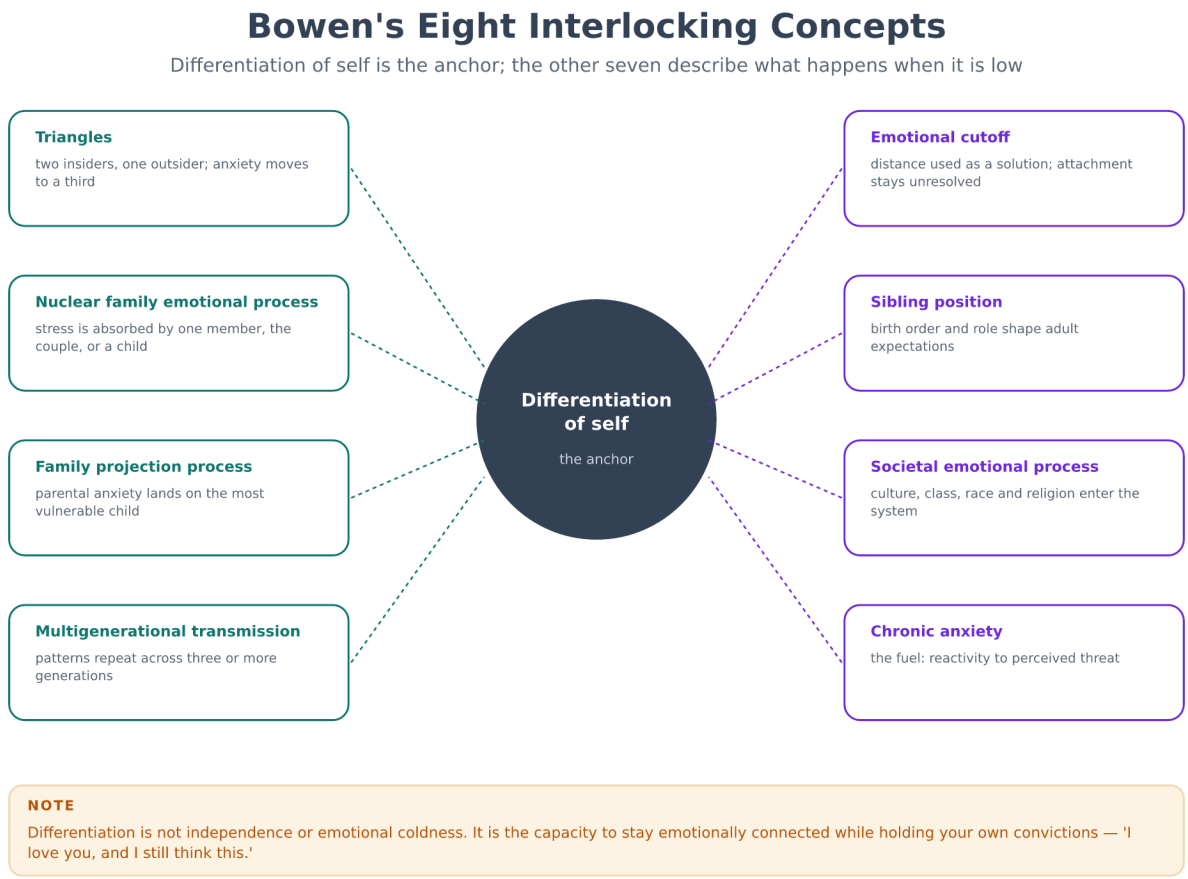


Figure 7.1 — Bowen's eight interlocking concepts

Core propositions (Bowen).

1. The family is an emotional unit; anxiety is its most important variable.
2. Differentiation of self is the capacity to remain connected while thinking independently.
3. Anxiety is managed in one of four ways: conflict, distance, projection to a child, or dysfunction in one member.
4. Triangles distribute anxiety and stabilise dyads.
5. Patterns are transmitted across generations; symptomatic children are often the most triangulated.

The theory's strength is time: it offers an account of why now, by locating the present pattern in several generations of adaptation to war, migration, loss and religion.

The theory's weakness is its insufficient account of power, material constraint and culture. A woman cannot always "differentiate" out of an arranged marriage; a family cannot always "stay out of triangles" when migration has severed the network.

Week 8 — Milan and meaning

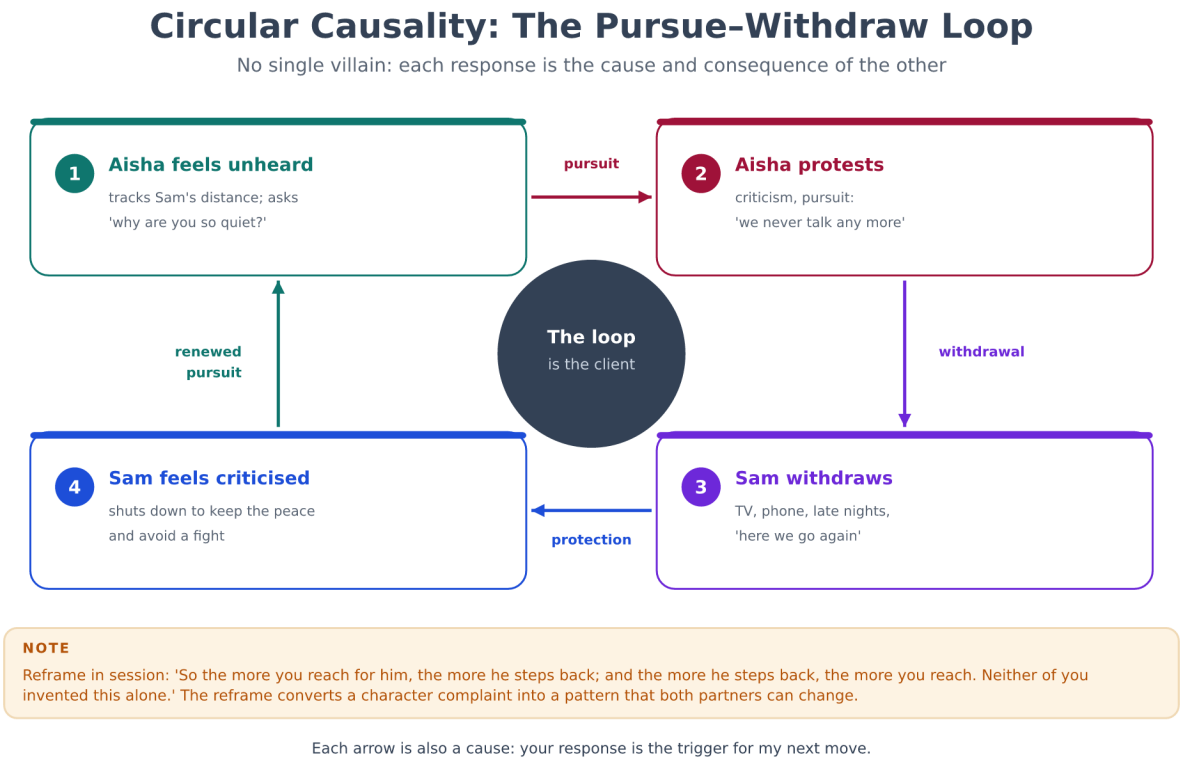


Figure 8.1 — Circular causality in the consulting room

Core propositions (Palazzoli, Boscolo, Cecchin, Prata).

1. The family is a system of meanings (premises) that organise behaviour.
2. The therapist forms hypotheses and tests them with circular questions.
3. Neutrality is a stance of curiosity towards all members, not indifference.
4. Positive connotation frames all behaviour as serving the system's coherence.
5. Interventions disturb the system by changing premises, not by demanding behaviour change.

The evolution. Classical Milan used teams, rituals and directives; later Milan (and the reflecting team) abandoned directive intervention and emphasised multiple perspectives and the therapist's own openness.

Critique. Neutrality became indefensible in cases of violence and abuse; circular questioning can become an elegant evasion; and the intellectual register excluded many families.

Week 9 — Constructivism and social constructionism

Constructivism (Maturana, von Glasersfeld): each person constructs a model of reality from their own experience; we cannot access an objective reality directly. Clinically: take each family member's account seriously, and stop trying to establish "the truth".

Social constructionism (Gergen, Shotter): meanings are produced between people, in language and in culture, and change with context. Clinically: the problem, the roles and the identities are made in talk, and so they can be made differently.

The distinction matters. Constructivism risks relativism and individualism (everything is "just your view"); social constructionism locates meaning in relationships and power, which makes it compatible with the critical and cultural traditions.

Implication for practice. The therapist's formulation is one description among several. Offering it humbly — "here is a story I am beginning to see; how does it look to you?" — is both a constructionist technique and a mark of respect.

Week 10 — Narrative and postmodern theory

Narrative (White & Epston). Stories organise experience; dominant stories make alternatives invisible; the task is to externalise the problem and thicken a preferred story.

Postmodern influences. Foucault on discourse and power-knowledge; Derrida on deconstruction. The clinical translation: question the "official" account, ask who benefits from it, and look for what it silences.

The reflexivity of narrative therapy is unusual: the therapist is "decentred but influential", and the client is the author. This is an ethical stance as much as a technique.

Critique. The linguistic emphasis can under-serve families with limited literacy or in acute crisis; the evidence base, while growing, is smaller than the cognitive-behavioural tradition's.

Week 11 — Critical, feminist and anti-racist systems theory

The feminist critique (Hare-Mustin 1978; Goldner; Walters) made four claims that changed the field:

1. "Circular" descriptions conceal power asymmetries.
2. "Neutrality" in the face of violence colludes with the perpetrator.
3. The gendered division of labour, not "the family system", is often the object of the wife's complaint.
4. Models that prize individuation encode a male norm.

Anti-racist and postcolonial theory extend the critique: whose family structures are treated as healthy? Which knowledge counts as evidence? How do services reproduce the state's relationship to marginalised families?

Clinical translation.

- Ask about power explicitly, in every assessment.

- Never use systemic language to make a victim responsible for the violence she receives.
- Place race, migration, class and religion inside the system, not outside it.

Week 12 — Attachment and neuroscience in systems theory

Two imports that have reshaped the field.

1. **Attachment theory** gives systemic thinking a developmental mechanism: the couple and parent–child systems are organised by attachment strategies, and those strategies are formed in relationships.
2. **Neuroscience** gives systemic thinking a physiology: co-regulation, the window of tolerance, mirroring and stress physiology explain how relationships change bodies.

The integration. "Systems" are not abstract patterns; they are embodied patterns between nervous systems. A couple's pursue–withdraw cycle is simultaneously a communication pattern, an attachment strategy, and a sequence of autonomic states.

Critique of the imports. Neuroscience is often oversold in clinical texts; "mirror neurons" and "the right brain" are frequently invoked without evidence. Use physiology where it is well established (stress, regulation, sleep, trauma) and keep the metaphors labelled as metaphors.

Week 13 — Critiques and limitations

Critique	Substance	Response
The system has no agent	"The family did it" hides who actually did what	Locate action; ask who decided
Power is under-theorised	Systemic descriptions can neutralise abuse	Explicit power analysis
Culture is externalised	"The system" was a Western two-parent family	Cultural and critical turns
Evidence is thin	Many systems concepts are unfalsifiable	Operationalise and test
The therapist disappears	Neutral observer stance	Second-order reflexivity
Complexity obscures action	Endless circular description delays intervention	Use the simplest map that guides action

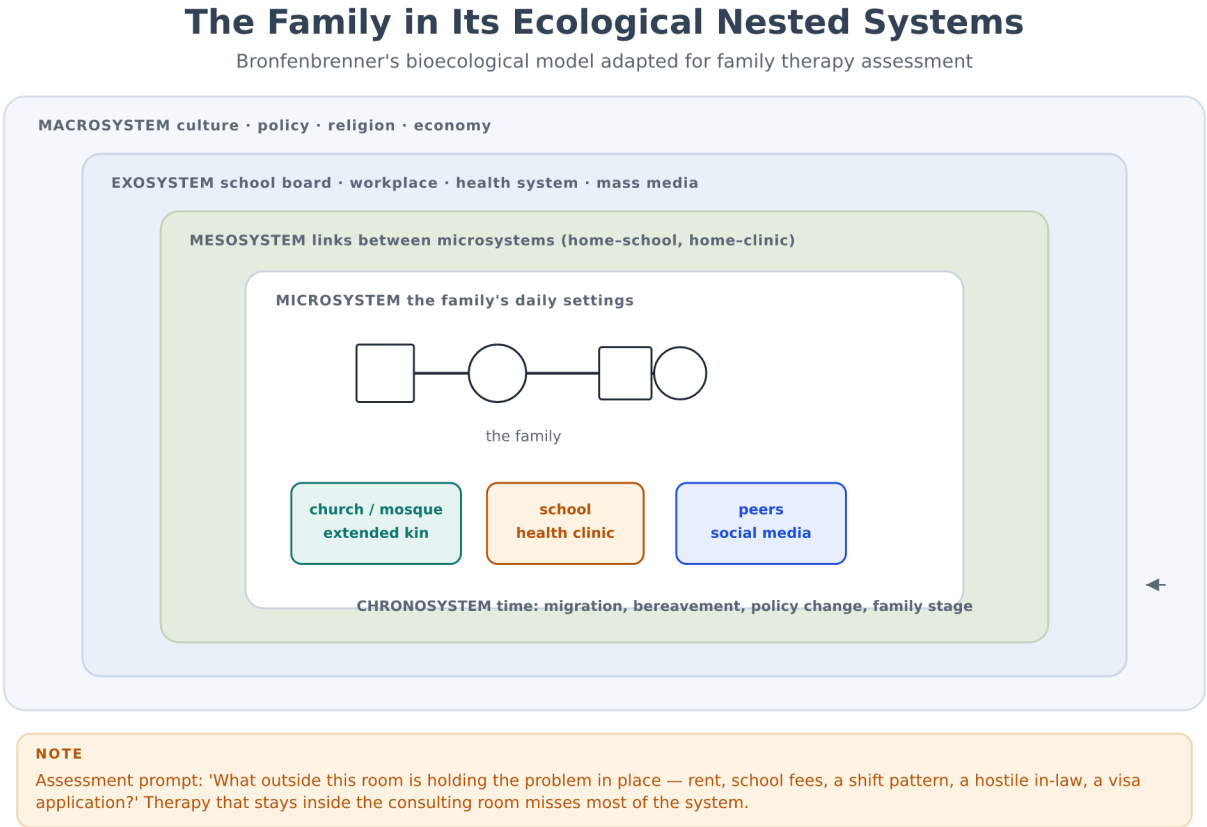
The mature position. Systems theory is a disciplinary sensibility, not a complete theory. It tells you where to look (between people, across generations, in context) and what to resist (single villains and simple causes). It does not, by itself, tell you what to do.

Week 14 — Complexity, networks and the contemporary field

Complexity theory. Families are non-linear, self-organising systems: small differences can produce large divergence; cause and effect are not proportional; and feedback loops are the rule. This fits clinical experience better than linear models, but it resists prediction.

Network thinking. Contemporary research treats families as networks of relationships, with density, centrality and clustering measurable. Who is central? Who is isolated? Who is the only link between two clusters? This is a formalised genogram, and it is improving our understanding of family structure and resilience.

Ecological models. Bronfenbrenner's nested systems (Figure 7 in MFT 701) place the family inside institutions, culture and time — a corrective to the field's tendency to treat the family as self-contained.



NOTE

Assessment prompt: 'What outside this room is holding the problem in place — rent, school fees, a shift pattern, a hostile in-law, a visa application?' Therapy that stays inside the consulting room misses most of the system.

Figure 14.1 — The family inside nested ecological systems

Week 15 — Your theoretical position

Write 1,000 words and defend it in ten minutes:

1. What is the unit of analysis you will work with, and why?
2. What do you believe about causation in families?

3. What is the therapist's position, and how will you manage your own power?
4. Which systemic concept do you find least defensible, and what do you use in its place?
5. What would make you abandon your position?

Glossary

Autopoiesis — the self-producing organisation of living systems. **Boundary** — the rules governing participation and exchange between systems. **Circularity** — reciprocal causation within a system. **Cybernetics** — the study of control and communication in systems. **Differentiation of self** — see Bowen. **Equifinality** — different paths leading to the same end state. **Equipotentiality** — similar starting points leading to different end states. **Homeostasis / morphostasis** — the maintenance of system organisation. **Morphogenesis** — change in system organisation. **Punctuation** — the marking of a sequence's beginning; the source of blame. **Reflexivity** — the therapist's accountability for their own participation. **Second-order cybernetics** — cybernetics of observing systems. **Structure** — the repetitive patterns of transaction that organise a family.

Self-assessment

Multiple choice

1. Equifinality means: (a) same cause, same outcome (b) different paths, same outcome (c) same paths, different outcomes (d) no causation

2. Homeostasis in a family refers to: (a) change (b) the maintenance of existing organisation (c) hierarchy (d) communication

3. Second-order cybernetics holds that the observer: (a) is outside the system (b) is part of the observed system (c) is irrelevant (d) is objective

4. "One cannot not communicate" is: (a) axiom 1 (b) axiom 3 (c) axiom 5 (d) a double bind

5. Punctuation is clinically important because it: (a) describes memory (b) assigns blame for a sequence (c) measures cohesion (d) is a diagnostic criterion

6. The feminist critique of family therapy argued that: (a) systems are too complex (b) circular descriptions conceal power asymmetries (c) hierarchy is unnecessary (d) therapy should be individual

7. Positive connotation is associated with: (a) structural theory (b) Milan (c) Bowen (d) behavioural therapy

8. A closed system in family terms is one that: (a) has clear boundaries (b) has limited exchange with its environment and becomes rigid (c) is disengaged (d) is enmeshed

9. Social constructionism locates meaning in: (a) the individual brain (b) relationships, language and culture (c) genetics (d) behaviour

10. The most defensible use of systems theory is as: (a) a complete predictive model (b) a disciplinary sensibility that shapes where you look (c) a diagnosis (d) a legal framework

Answer key

1 b · 2 b · 3 b · 4 a · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Define equifinality and equipotentiality and give one clinical example of each.
2. Distinguish negative and positive feedback with a family example.
3. What are the three criteria for evaluating a family's structure?
4. State the feminist critique of "circular" formulations in four sentences.
5. What is reflexivity, and why is it a clinical skill?

Essay (choose one, 2,000 words)

1. "Systems theory tells you where to look but not what to do." Discuss.
2. Critically evaluate the concept of homeostasis in families where violence is present.
3. Compare Bowen's and Minuchin's accounts of how problems develop, and assess which is better supported.

Further reading

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End of Semester Two. Next: MFT 716 — Communication Skills in Therapy

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MFT 716

Communication Skills in Therapy

15 teaching weeks · 3 practicals · 3 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 706

> “Every therapeutic effect passes through communication.”

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A note on the case material

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The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
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MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	**Communication Skills in Therapy**	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
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MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Every therapeutic effect passes through communication. This course refines the therapist's own instrument — listening, responding, questioning, silence, feedback, and the management of difficult conversations — and teaches the protocols you will hand to families.

How to use this book

Skills are built by repetition under observation. Each week has a **micro-skill** to practise, a **protocol** to teach to families, and a **transcript** to analyse. Record yourself weekly; the gap between what you think you said and what you said is the curriculum.

1. Course description

The course covers therapeutic communication at three levels: the **micro** (attending, reflecting, summarising, questioning), the **meso** (structuring difficult conversations, giving feedback, managing conflict), and the **macro** (written communication, interprofessional communication, and communication across language and culture).

It also teaches the **structured communication protocols** used with couples and families — speaker–listener, the Imago dialogue, negotiation formats, family meetings — and requires you to deliver them with fidelity.

2. Learning outcomes

#	Outcome	Assessed
LO1	Demonstrate micro-skills with fluency and intention	Observed session
LO2	Analyse communication at content, relationship and process levels	Transcript analysis
LO3	Teach two structured communication protocols with fidelity	Skills test
LO4	Conduct a difficult conversation (bad news, disclosure, confrontation)	Simulation
LO5	Adapt communication for children, interpreters and low-literacy contexts	Viva
LO6	Write clinical communication that is accurate and usable	Report portfolio
LO7	Identify and modify your own characteristic communication patterns	Reflective portfolio

3. Assessment

Component	Weight	Due
Transcript analysis (1,500 words)	20%	Week 6
Observed micro-skills session (30 min)	25%	Week 8
Protocol teaching test (speaker–listener or Imago)	25%	Week 11

Component	Weight	Due
Difficult conversation simulation (20 min)	30%	Week 14

4. The fifteen-week plan

Week	Topic	Practical
1	Communication as the medium of therapy	Self-recording baseline
2	Attending and non-verbal communication	Non-verbal drill
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Empathy: affective, cognitive, and its limits	Empathy accuracy exercise
5	Questioning and prompting	Question taxonomy drill
6	**Transcript analysis due**; content, relationship, process	Three-level analysis
7	Feedback: giving it, receiving it, teaching families to give it	Feedback lab
8	**Observed micro-skills session**	Assessed
9	Conflict communication and repair	Repair rehearsal
10	Speaker–listener and Imago dialogue	Protocol fidelity test prep
11	**Protocol teaching test**	Assessed
12	Difficult conversations: bad news, disclosure, confrontation	Simulation
13	Communication with children, interpreters and across literacy	Adaptation lab
14	**Difficult conversation assessment**	Assessed
15	Written and interprofessional communication	Report portfolio

Week 1 — Communication as the medium

Three levels of every message.

1. **Content** — what is said.
2. **Relationship** — what it implies about the relationship ("you are the kind of person who…").
3. **Process** — what is happening between the people while it is said.



Figure 1.1 — Five axioms of communication

The therapist's communication is an intervention. Every reflection selects something from the client's account and returns it amplified. There is no neutral way to summarise.

Baseline recording. Record ten minutes of your own session (consented) or a role-play. Count: questions, reflections, advice statements, interruptions, and seconds of silence you initiated. Most beginners are surprised by the advice count.

Week 2 — Attending and non-verbal communication

The SOLER posture (Egan): **S**it squarely, **O**pen posture, **L**ean slightly forward, **E**ye contact (culturally calibrated), **R**elax.

What families read. Pace, volume, facial expression, seating, who you look at, how you respond to distress, and whether you check your phone. A therapist who says "take your time" while glancing at the clock has said something else.

Cultural calibration. Eye contact signals respect in some cultures and challenge in others; physical distance varies; silence may indicate deference rather than resistance. Ask, or observe what the family does with each other.

Working with more than two people. Your attention is a resource that structures the family. Distribute it deliberately; narrate your choices ("I am going to come back to you, I have not forgotten"). Do not let the most articulate person become the narrator of the family.

> **DRILL**
> Triads. The therapist must convey three different messages using only non-verbal behaviour (welcome, "I need to interrupt", "take your time"). The client guesses each.

Week 3 — Listening

Paraphrase (content), **reflect** (feeling), **summarise** (pattern across people and time).

Reflection levels.

Level	Example	Effect
Simple	"You are angry about the money."	Shows attention; may stall
Complex	"You are angry about the money, and underneath that, frightened that you are the only one thinking about the future."	Adds meaning; deepens
Double-sided	"You want him to be more involved, and you also do not trust him to do it right."	Holds ambivalence

The 2:1 rule. Two reflections for every question, until it becomes automatic.

What blocks listening. Formulating your next question; evaluating; wanting to reassure; your own emotional reaction; diagnostic thinking that closes the account prematurely.

Week 4 — Empathy

Three components.

1. **Affective empathy** — feeling something in response.
2. **Cognitive empathy** — understanding the other's perspective.
3. **Empathic communication** — conveying that understanding so the other feels understood.

The therapist needs all three, and needs to keep the first from driving. Affective empathy without boundaries produces over-involvement; cognitive empathy without warmth produces technique; communication without either produces scripts.

Empathy is not agreement. "I understand why you would want to leave" is empathy. "You are right to want to leave" is agreement. The first opens exploration; the second closes it and creates a coalition.

Empathic accuracy is testable. Watch a recorded session and pause before each therapist response. Write what you think the client felt. Then check with the client (or with the recording's subsequent cues).

> **DRILL**
> In pairs, one person describes a minor grievance. The listener must respond with an empathic statement that contains no advice, no question and no story of their own. Three rounds each.

Week 5 — Questioning and prompting

Six families of question (from MFT 706), refined for communication work:

- **Open** — "What was that like?"
- **Circular** — "Who noticed first?"
- **Clarifying** — "When you say 'disrespectful', what did he actually do?"
- **Reflexive** — "If your daughter were listening, what would she say you both do?"
- **Scaling** — "How close are you, 0–10?"
- **Confronting (gently)** — "You say it does not matter, and you have mentioned it four times."

The prompt instead of the question. "Tell me more about that" often produces more than any question. Use prompts when the client is in flow; questions when you need to change direction.

Week 6 — Content, relationship, process

Transcript analysis exercise. Take fifty lines of a session transcript and mark:

- every **content** statement (topic, fact)
- every **relationship-level** message ("you always…", "you are the kind of…")
- every **process** moment (something audible happening between speakers: interruption, softening, joke, silence)

Then ask: what did the therapist respond to? Beginners respond to content and miss process. Advanced practitioners respond to process and use content as evidence.

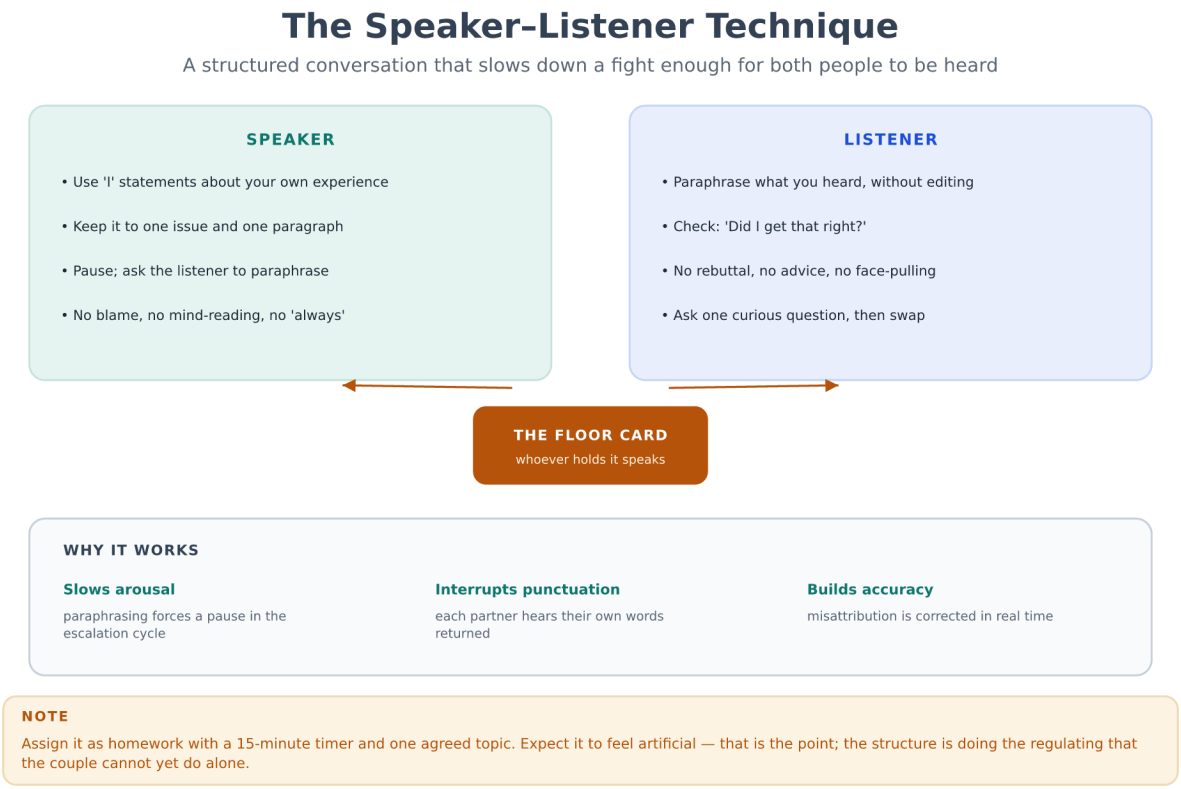


Figure 6.1 — The speaker–listener protocol

Week 7 — Feedback

Feedback to clients should be behavioural, specific, timely and owned: "I noticed your voice dropped when you said that" (not "you are passive-aggressive").

Feedback between family members is often the missing skill. Teach the four-part structure:

1. "When you ___" (behaviour, no adjectives)
2. "I feel ___" (feeling)
3. "Because ___" (meaning)
4. "What I would like is ___" (specific request)

Feedback to colleagues and supervisees: use the same discipline. "What I saw / what it did to me / what to keep / what to try" — never the sandwich, which trains people to hear praise as a warning.

Week 8 — Observed micro-skills session

Thirty minutes. Assessed on: attending; reflection ratio; complex reflections; question variety; one deliberate silence; one summary; and a clean close.

Week 9 — Conflict communication and repair

The escalation sequence: complaint → criticism → contempt → defensiveness → stonewalling.

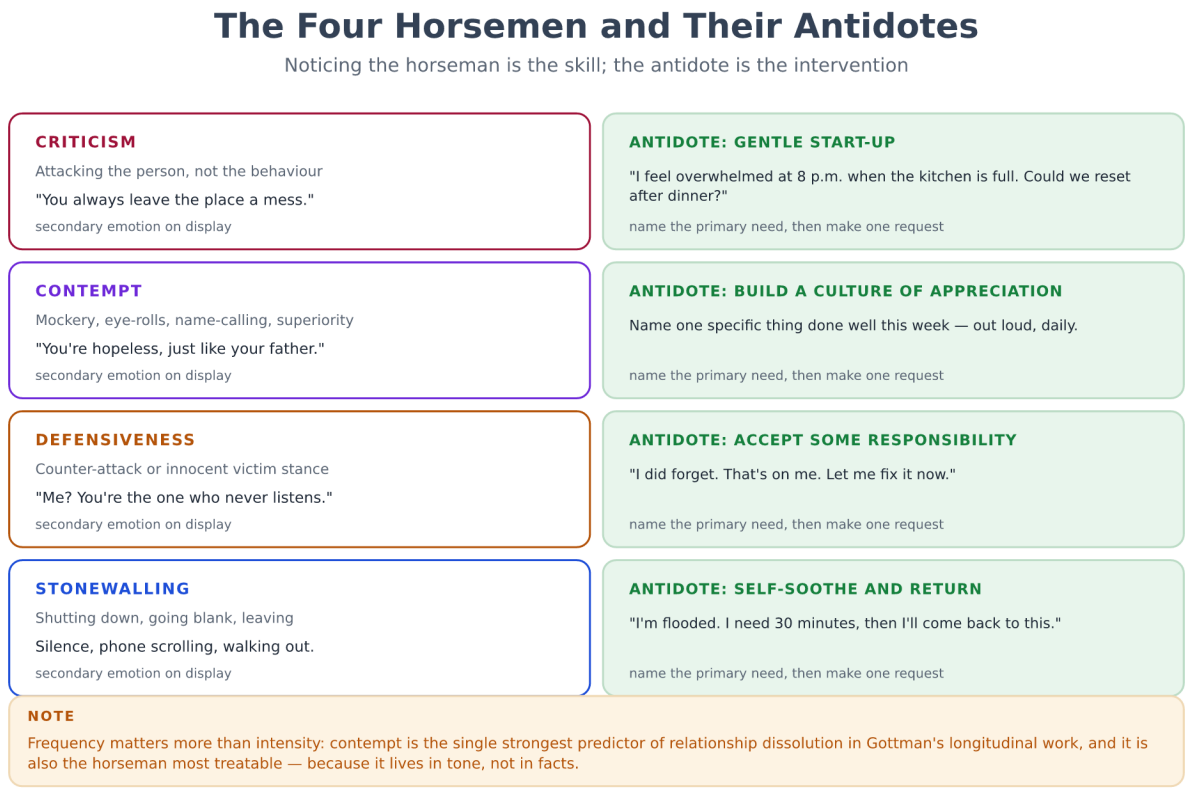


Figure 9.1 — The four horsemen and their antidotes

Repair is the skill that matters. Teach couples to use a repair phrase: "Can we start again?", "I am flooded, give me twenty minutes", "That came out wrong."

The therapist's repair. You will make mistakes: mis-summarise, miss a disclosure, take a side. Repair in the moment: "I got that wrong. Let me try again." Modelling repair is more powerful than describing it.

Week 10 — Protocols

Speaker–listener. A floor card, a timer, paraphrase before responding, then swap. Teach it, rehearse it in session, and set it as homework with one agreed topic.

Imago dialogue. Three steps: mirror ("so what I heard you say is…"), validate ("that makes sense because…"), empathise ("I imagine you feel…"). More demanding than speaker–listener, and more powerful with practised couples.

Negotiation format. Define one issue; each states a position; generate three options; choose one to trial for two weeks; review.

Fidelity matters. A protocol delivered loosely is a protocol that fails, and the couple will conclude that "communication exercises do not work". Practise until you can teach it in ten minutes without notes.

> **ASSESSED**
> Teach speaker–listener to a role-played couple in twelve minutes, then coach them through one round. Assessed on clarity, demonstration, coaching, and management of a partner who will not follow the rules.

Week 11 — Protocol teaching test

As above, assessed formally, with feedback.

Week 12 — Difficult conversations

Four types and their structures.

1. **Bad news** (a diagnosis, a death, a safeguarding outcome). Structure: warning shot ("I have some difficult news") → information in small chunks → check understanding → silence → emotional response → plan → follow-up. Do not fill the silence after the news.
2. **Disclosure** (a secret that must be shared, an affair, a risk). Structure: prepare the person → agree the content → be present → manage the response → plan.
3. **Confrontation** (a boundary violation, a safety concern, a colleague's conduct). Structure: describe the behaviour → its impact → the expectation → the consequence → the support available.
4. **Ending therapy when it is not working** (referral, rupture, incompetence). Structure: name the difficulty → own your part → offer a route → document.

The common failure is rushing to reassurance. "At least…" and "it will be fine" terminate the conversation the client needs to have.

Week 13 — Adaptation

Children. Short sentences, concrete language, check understanding by asking them to explain it back, and use a medium.

Interpreters. Speak to the family, first-person interpretation, brief and debrief (see MFT 708).

Low literacy. Avoid written-only instructions; use drawings, demonstration, and teach-back ("show me how you would do it").

Hearing or cognitive impairment. Reduce background noise, face the person, one idea per sentence, and confirm comprehension.

Second-language sessions. Slow down, avoid idiom, check meaning ("when you say 'respect', what does that include?").

Week 14 — Difficult conversation assessment

Twenty-minute simulation drawn from a bank (bad news, disclosure, confrontation, ending). Assessed on: structure, pacing, emotional attunement, clarity of the message, and a documented follow-up.

Week 15 — Written and interprofessional communication

Clinical notes. Behavioural, dated, factual, and signed. Avoid adjectives about people; avoid speculation.

Referral letters. One page: reason, relevant history, what you have tried, what you are asking for, and who to contact.

Reports for courts and schools. Answer the question asked; state the limits of your expertise; separate fact from opinion.

Interprofessional communication. Use SBAR (Situation, Background, Assessment, Recommendation). It is short, it survives interruption, and it is what other professions are trained to receive.

Glossary

Complex reflection — a reflection that adds meaning or feeling beyond the client's words. **Double-sided reflection** — a reflection that holds both sides of an ambivalence. **Empathic accuracy** — the degree to which a therapist correctly infers a client's experience. **Mirroring** — repeating back the other's words as the first step of a structured dialogue. **Process** — the relational layer of communication, distinct from content. **Repair** — an attempt to de-escalate and reconnect after a rupture. **SBAR** — Situation, Background, Assessment, Recommendation: a handover format. **SOLER** — a posture mnemonic for attending behaviour.

Self-assessment

Multiple choice

1. The recommended ratio of reflections to questions for beginners is: (a) 1:1 (b) 2:1 (c) 1:2 (d) 5:1

2. "You are right to want to leave him" is: (a) empathy (b) agreement (c) a reflection (d) a summary

3. A complex reflection: (a) repeats content (b) adds meaning or feeling (c) asks a question (d) gives advice

4. The main error in delivering bad news is: (a) pausing (b) rushing to reassurance (c) checking understanding (d) using plain language

5. SBAR stands for: (a) Situation, Background, Assessment, Recommendation (b) Summary, Behaviour, Action, Review (c) Safety, Boundaries, Alliance, Repair (d) Session, Block, Ask, Respond

6. In Imago dialogue the three steps are: (a) mirror, validate, empathise (b) ask, tell, repeat (c) listen, advise, conclude (d) reflect, question, summarise

7. Fidelity in teaching a communication protocol matters because: (a) it is required by codes (b) a loosely delivered protocol fails and discredits the method (c) it is faster (d) clients prefer it

8. Eye contact should be: (a) constant (b) culturally calibrated (c) avoided (d) used only with adults

9. The 2:1 rule exists to: (a) reduce interrogation and enrich material (b) fill time (c) avoid emotion (d) demonstrate technique

10. A referral letter should include: (a) the full case file (b) reason, relevant history, what you tried, the request, contact (c) only a diagnosis (d) your opinion of the family

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 a · 7 b · 8 b · 9 a · 10 b

Short answer

1. Describe the three levels of a message with one example of each.
2. Give three examples of a complex reflection on the same client statement.
3. What are the four steps of the feedback structure for family members?
4. Outline the structure for delivering bad news.
5. Name four adaptations for a family with limited literacy.

Essay (choose one, 1,800 words)

1. Analyse a transcript of your own session: what did you respond to, and what did you miss?
2. "Repair, not avoidance, is the mark of a skilled communicator." Discuss with clinical examples.
3. How should communication protocols be adapted for a family in which the couple does not share a first language?

Further reading

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Next: MFT 717 — Psychopathology in Families

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MFT 717

Psychopathology in Families

15 teaching weeks · 1 practical · 1 worked case

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1.71 units · Prerequisite: MFT 707, 715

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Course leader's summary. Severe mental distress is not caused by families, and it is not treated without them. This course builds the knowledge and the nerve to work with psychosis, mood disorders, severe anxiety, personality-level difficulties and eating disorders in a relational frame — alongside medication and psychiatry, not instead of them.

How to use this book

The course deliberately reverses a historical mistake. For decades, family therapy implied that families caused severe mental illness; that legacy still harms families who arrive expecting to be blamed. Each week therefore begins with what is known about aetiology, then asks what a family therapist actually contributes.

1. Course description

The course covers the major categories of severe psychopathology as they present in family life: psychotic disorders, bipolar disorder, major depression, severe anxiety and OCD, personality-level difficulties, eating disorders, and the interface with substance use. It emphasises **family psychoeducation**, **expressed emotion**, **family accommodation**, **relapse prevention**, **caregiver burden**, the **recovery model**, and respectful collaboration with psychiatric services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the aetiology and family impact of major disorders	Exam
LO2	Deliver family psychoeducation for psychosis or bipolar disorder	Skills test
LO3	Assess and reduce expressed emotion and family accommodation	Case study
LO4	Work with caregiver burden and family adaptation	Case study
LO5	Collaborate with psychiatric services and understand medication	Viva
LO6	Avoid both blaming and neglecting the family	Reflective portfolio
LO7	Apply a recovery-oriented frame	Position paper

3. Assessment

Component	Weight	Due
Family psychoeducation session observed (40 min)	30%	Week 8
Case study: family intervention for severe mental illness (2,500 words)	35%	Week 12
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Historical legacy: from schizophrenogenic mothers to family partnership	Timeline and critique
2	Psychosis I: what it is and what families see	Symptom and experience mapping
3	Psychosis II: family intervention and expressed emotion	EE assessment
4	Family psychoeducation in practice	Session delivery
5	Bipolar disorder and the family	Relapse signature mapping
6	Depression in relational context	Couple-based depression work
7	Severe anxiety, OCD and family accommodation	Accommodation reduction plan
8	**Psychoeducation observed**	Assessed
9	Personality-level difficulties and the family	Boundary and validation work
10	Eating disorders and the family	FBT principles
11	Caregiver burden, siblings and the household	Carer support plan
12	**Case study due**; medication, psychiatry and collaboration	Medication interface
13	Risk, hospitalisation and the family	Admission and discharge planning
14	Recovery, stigma and the family's future	Recovery plan
15	Integration: partnership without blame	Case presentations

Week 1 — The historical legacy

The mistake. Mid-twentieth-century theories blamed mothers for schizophrenia ("schizophrenogenic mother"), attributed psychosis to family communication ("double bind"), and described "families of origin" as pathogenic. These ideas were poorly evidenced, deeply damaging, and are still remembered by families.

The correction. Contemporary research shows that families are neither the cause nor the cure of psychotic disorders, but they are among the strongest predictors of outcome — through support, monitoring, accommodation and emotional climate.

What family therapy offers now:

- psychoeducation (reliable information, in plain language)
- communication and problem-solving training

- reduction of criticism, hostility and over-involvement where they are high
- relapse prevention and early-warning planning
- support for carers
- help with the ordinary relational life that the illness interrupts

> **PRACTICAL**
> Read one historical text and one contemporary family-intervention trial. Write 400 words on what changed, and what harm the earlier framing did.

Week 2 — Psychosis

What it involves. Hallucinations, delusions, disorganised thinking, negative symptoms (flat affect, anhedonia, withdrawal), and cognitive impairment. Onset is typically late adolescence or early adulthood, which means it lands in the middle of a family's launching stage.

What families experience. Fear, grief, confusion, exhaustion, stigma, violence in a minority of cases, and a chronic state of vigilance. Siblings often report invisibility; parents often report marital strain.

What helps (evidence summary). Antipsychotic medication is the foundation. Family intervention (typically 10–12 sessions of psychoeducation plus communication and problem-solving) reduces relapse and improves functioning. Supported employment, cognitive remediation and long-term follow-up matter.

What to avoid. Blaming; interpreting delusions symbolically; colluding with the illness ("he cannot help it, so nothing is required of him"); and treating the family as the patient.

Week 3 — Expressed emotion and family intervention

Expressed emotion (EE) comprises criticism, hostility and emotional over-involvement. High EE predicts relapse in schizophrenia and other conditions, though the effect is smaller than early research suggested and is bidirectional (illness severity also raises EE).

Assessing EE without blaming. Ask about the last month: how many arguments, what triggered them, how the carer responded, how much time is spent managing symptoms, and whether the carer feels able to have a life of their own.

The compassionate reframe. High criticism is usually exhaustion; over-involvement is usually love and fear. Naming that — before teaching any skill — is what makes the intervention acceptable.

Script: "You have been managing something terrifying for four years. Of course you are exhausted and sometimes sharp. The aim of these sessions is not to make you nicer; it is to give you back some room."

Communication training then teaches: making positive requests, active listening, expressing negative feelings safely, and problem-solving in structured steps.

Week 4 — Family psychoeducation

The format. 6–12 sessions, ideally including the person with the diagnosis and their carers, with a combination of information, communication training and problem-solving.

A single-session structure.

Segment	Content
1. What the family already knows	Elicit their model of the illness
2. What is known	Symptoms, course, causes (multi-factorial), treatment, prognosis
3. What helps	Medication adherence, sleep, stress reduction, early warning signs, routines
4. What the family can do	Communication, problem-solving, reducing accommodation, protecting their own life
5. What to do in a crisis	Numbers, triggers, what to say, who to call
6. Questions	Ten minutes of honest answers

Language. Plain, non-technical, and hopeful without being false: "This is a serious condition that can be managed. It is not anyone's fault, and it is not a life sentence."

Week 5 — Bipolar disorder

What families see. Periods of elevated mood, reduced sleep, grandiosity, irritability, risk-taking, and then depression. The family's experience is often of a person they recognise only intermittently.

Family tasks. Recognising the **relapse signature** (early signs of an episode in this person), agreeing a plan in advance, protecting sleep, managing money during highs, and responding to risk.

The relapse signature plan.

1. List the person's own early signs (e.g. sleeping 4 hours and feeling fine; starting three projects).
2. Agree what the family will do (e.g. call the psychiatrist; take over the bank card; reduce stimulation).
3. Agree what the family will not do (argue about whether an episode is real; wait for a crisis).
4. Review after each episode.

Psychoeducation plus family-focused treatment has good evidence for relapse prevention in bipolar disorder.

Week 6 — Depression in relational context

Two directions. Depression is influenced by relationships (criticism, conflict, isolation, caregiving burden), and it influences them (withdrawal, irritability, reduced libido, helplessness).

Couple-based interventions have good evidence for depression, particularly where relationship distress is present. Key mechanisms: reducing criticism and hostility, increasing positive exchange, addressing attributional blame, and supporting behavioural activation with a partner's help.

What not to do. Treat the depressed person as the sole problem; ask the partner to become a therapist; ignore the couple's sexual and practical reality; or reassure prematurely.

> **CASE**
> A husband's depression follows a job loss. The couple's cycle: he withdraws, she takes over the household, he feels useless, she feels abandoned. Intervention: behavioural activation with agreed tasks that restore his role, plus work on her anxiety about relinquishing control.

Week 7 — Severe anxiety, OCD and accommodation

Family accommodation is the family's participation in the anxiety: answering the same question forty times, checking locks, avoiding supermarkets, providing reassurance, taking over tasks.

Why it persists. It reduces distress immediately, which reinforces it for both parties.

Reducing accommodation without cruelty. Build a hierarchy with the family; reduce one item at a time; agree what the person will do instead; expect a temporary increase in distress; and rehearse the script: "I love you, and I am not going to answer that again. I will sit with you for ten minutes instead."

For OCD specifically: psychoeducation about the function of compulsions; family agreement not to participate in rituals; ERP as the treatment of choice, with the family as allies.

Week 8 — Observed psychoeducation session

Forty minutes: eliciting the family's model, delivering information in plain language, teaching one communication skill, agreeing a crisis plan, and answering questions honestly.

Week 9 — Personality-level difficulties

Caution with language. "Personality disorder" is a contested and stigmatising label. Many clinicians describe "personality-level difficulties" or "complex relational patterns" and focus on the pattern rather than the category.

What families experience. Emotional storms, self-harm, idealisation and devaluation, fear of abandonment, and often years of contact with services that have not helped.

What helps.

1. **Validation before change:** "Given what you have survived, this reaction makes sense."
2. **Consistency:** the same rules, the same times, the same responses.
3. **Boundary clarity:** "I will call you back at six" — and doing it.
4. **Family psychoeducation** about emotion regulation and invalidation.
5. **Coordination between services**, because splitting between professionals is part of the pattern.

What harms. Moralising; interpreting the behaviour as manipulation; inconsistency; and treating every crisis as a new emergency rather than part of a known pattern.

Week 10 — Eating disorders

The relational reality. Eating disorders reorganise the household: meals become battlegrounds, siblings become collateral, and parents oscillate between control and helplessness.

Family-based treatment (FBT) for adolescent anorexia is first-line. The stance is **agnostic about cause** and **externalising of the illness**: the family defeats anorexia, and no one is blamed for it.

Phase 1: parents take charge of refeeding, with support, and are absolved of guilt. **Phase 2:** control is returned to the adolescent as developmentally appropriate. **Phase 3:** identity and adolescent issues.

Medical safety is primary. Eating disorders have the highest mortality of any mental disorder. Weight, electrolytes and cardiac status are medical matters; do not work alone.

For adults with longstanding eating disorders, the work is often individual and relational: shame, identity, control, and the couple's accommodation.

Week 11 — Caregiver burden and the household

Assess the carers, not only the patient. Burden is measurable and treatable: use a brief carer measure and ask directly about sleep, work, finances, social life, and their own health.

Siblings. Often invisible, often parentified, often resentful and guilty about the resentment. Give them a session, language for their experience, and permission to have their own life.

The household system. Who is doing the caring? Who has given up what? Is there a "designated carer" whose own life has been suspended? Naming this is often the most useful intervention of the month.

Carer support plan. Respite, information, peer support groups, practical help, and their own GP.

Week 12 — Medication and collaboration

What you need to know (not prescribe).

Class	Common uses	Common side effects to ask about
Antipsychotics	Psychosis, bipolar	Weight gain, sedation, tremor, sexual dysfunction
Antidepressants (SSRIs)	Depression, anxiety, OCD	Nausea, sexual dysfunction, initial agitation
Mood stabilisers (lithium, valproate)	Bipolar	Thirst, tremor, weight, thyroid and renal monitoring
Benzodiazepines	Short-term anxiety	Dependence, sedation, falls

Your role. Support adherence without policing it; explore ambivalence about medication as you would any other ambivalence; report side effects; never advise stopping or changing a dose; and know the prescriber's name.

Collaboration. Write a short letter; attend a case conference; use SBAR on the phone; agree who is responsible for what. Families are often caught between two professionals who have never spoken.

Week 13 — Risk, hospitalisation and the family

Admission is a family event: relief, guilt, fear, and often secrecy. Prepare the family, explain the process, agree visiting, and plan from the first day for discharge.

Discharge is the highest-risk period. Plan: medication supply, follow-up appointment, who is at home, what the early warning signs are, and what to do at night.

When the family is afraid. Take the fear seriously; it is usually based on experience. Build a written crisis plan that the whole household knows.

Week 14 — Recovery, stigma and the future

The recovery model emphasises hope, identity, meaning and self-determination, not only symptom reduction. For families, this means asking: what does a good life look like for this person, and what would support it?

Stigma operates inside the family too: shame, secrecy, "we do not tell anyone", blame of the mother. Addressing internalised stigma is a clinical task.

Planning the future. Housing, work, relationships, and what happens when the parents can no longer care. This conversation is difficult and usually deferred; the therapist's job is to make it possible.

Week 15 — Integration

Four principles.

1. **No blame.** Families are partners, not pathogens.
2. **Information is an intervention.** Accurate, plain-language psychoeducation changes outcomes.
3. **Emotion is contagious.** Reducing criticism and over-involvement helps — and it is achieved by supporting carers, not by instructing them.
4. **Collaboration beats isolation.** Medication, psychiatry, community services and family work together.

Glossary

Accommodation — family behaviours that participate in and maintain symptoms. **Expressed emotion (EE)** — criticism, hostility and emotional over-involvement in carers. **Family-focused treatment (FFT)** — a manualised programme of psychoeducation, communication and problem-solving, with good evidence in bipolar disorder. **Family psychoeducation** — structured information and skills for families living with a disorder. **Prodrome** — the early signs preceding a full episode. **Recovery model** — an orientation emphasising hope, identity, meaning and self-determination. **Relapse signature** — an individual's characteristic early warning signs of an episode.

Self-assessment

Multiple choice

1. Contemporary research on families and psychosis shows that families: (a) cause schizophrenia (b) can significantly influence outcome (c) are irrelevant (d) should be excluded

2. High expressed emotion comprises: (a) criticism, hostility, over-involvement (b) warmth, humour (c) avoidance (d) grief

3. Family accommodation in OCD: (a) reduces long-term symptoms (b) maintains symptoms despite short-term relief (c) is irrelevant (d) should be punished

4. FBT for adolescent anorexia is: (a) blame-based (b) agnostic about cause and externalises the illness (c) individual therapy (d) medication-only

5. A relapse signature is: (a) a diagnosis (b) an individual's early warning signs (c) a blood test (d) a medication

6. The highest-risk period after psychiatric admission is: (a) admission (b) discharge (c) the first year (d) none

7. The therapist's role regarding medication is to: (a) prescribe (b) adjust doses (c) support adherence and report side effects (d) advise stopping

8. Siblings of a person with severe mental illness commonly report: (a) no impact (b) invisibility and guilt about resentment (c) greater support (d) reduced burden

9. The recovery model emphasises: (a) symptom removal only (b) hope, identity, meaning, self-determination (c) institutional care (d) family blame

10. The most damaging historical legacy for families was: (a) too much medication (b) theories blaming families for causing psychosis (c) short admissions (d) family therapy

Answer key

1 b · 2 a · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. State the evidence for family intervention in psychosis in four sentences.
2. What is expressed emotion, and how do you assess it compassionately?
3. Outline a single-session family psychoeducation structure.
4. Give three ways to reduce family accommodation without cruelty.
5. What should a discharge plan contain?

Essay (choose one, 2,000 words)

1. "Families are partners, not pathogens." Trace the history of this claim and its clinical implications.
2. Design a family intervention for a young person with first-episode psychosis, including the first four sessions.
3. Critically discuss the ethics of using the diagnosis "personality disorder" with a family in the room.

Further reading

- Leff, J., & Vaughn, C. (1985). Expressed Emotion in Families. Guilford.

- Kuipers, E., Leff, J., & Lam, D. (2002). Family Work for Schizophrenia (2nd ed.). Gaskell.
- Miklowitz, D. J. (2008). Bipolar Disorder: A Family-Focused Treatment Approach (2nd ed.). Guilford.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa (2nd ed.). Guilford.
- National Institute for Health and Care Excellence. (2014/2020). Psychosis and Schizophrenia in Adults (CG178).

Next: MFT 718 — Intimacy and Relationship Dynamics

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MFT 718

Intimacy and Relationship Dynamics

15 teaching weeks · 1 practical · 1 worked case · 1 figure

SEMESTER THREE

1.71 units · Prerequisite: MFT 710

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MFT 718 — Intimacy and Relationship Dynamics

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	**Intimacy and Relationship Dynamics**	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Intimacy is not a single thing, and sex is not the whole of it. This course integrates attachment, desire, communication, culture and the body — and gives you language for the conversations most couples cannot start.

How to use this book

The material is intimate by design. You will be asked to examine your own beliefs about sex, bodies, gender and obligation, because those beliefs will shape what you notice and what you avoid. Supervision is essential for this course; several exercises should be processed there.

1. Course description

The course covers the psychology and physiology of intimacy and desire, the major sexual concerns presenting in couple therapy, the relational dynamics of pursuit and distance, betrayal and repair, the effects of culture and religion, and the practical skills of talking about sex.

It is explicitly **not** a sex therapy certification. It is the level of competence a family therapist needs to assess sexual concerns confidently, intervene relationally, and refer appropriately.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess intimacy across multiple domains	Observed assessment
LO2	Explain desire, arousal and response, including discrepancy	Exam
LO3	Conduct a respectful sexual history	Skills test
LO4	Formulate sexual concerns relationally and identify medical rule-outs	Case study
LO5	Deliver sensate focus and communication interventions	Skills test
LO6	Address culture, religion and consent	Reflective portfolio
LO7	Recognise the limits of your competence and refer	Viva

3. Assessment

Component	Weight	Due
Sexual history observed (30 min)	30%	Week 6
Case formulation and intervention plan (2,500 words)	40%	Week 11
Examination	20%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What intimacy is: eight domains	Domain assessment
2	Attachment and intimacy	Attachment-informed formulation
3	Desire: responsive and spontaneous	Desire mapping
4	The sexual response and its variation	Physiology teaching practice
5	Taking a sexual history	Observed practice
6	**Sexual history assessed**; talking about sex with couples	Language lab
7	Desire discrepancy and negotiation	Negotiation rehearsal
8	Common concerns I: erectile and arousal difficulties	Biopsychosocial formulation
9	Common concerns II: pain, orgasm, low desire	Referral decision-making
10	Betrayal, infidelity and the repair of trust	Disclosure planning
11	**Case plan due**; culture, religion and sexual scripts	Cultural genogram of sex
12	Consent, coercion and obligation	Boundary and consent work
13	Sexuality across the lifespan and in special populations	Adaptation planning
14	Sensate focus and intimacy exercises	Exercise design
15	Integration: the therapist's own comfort	Viva

Week 1 — What intimacy is

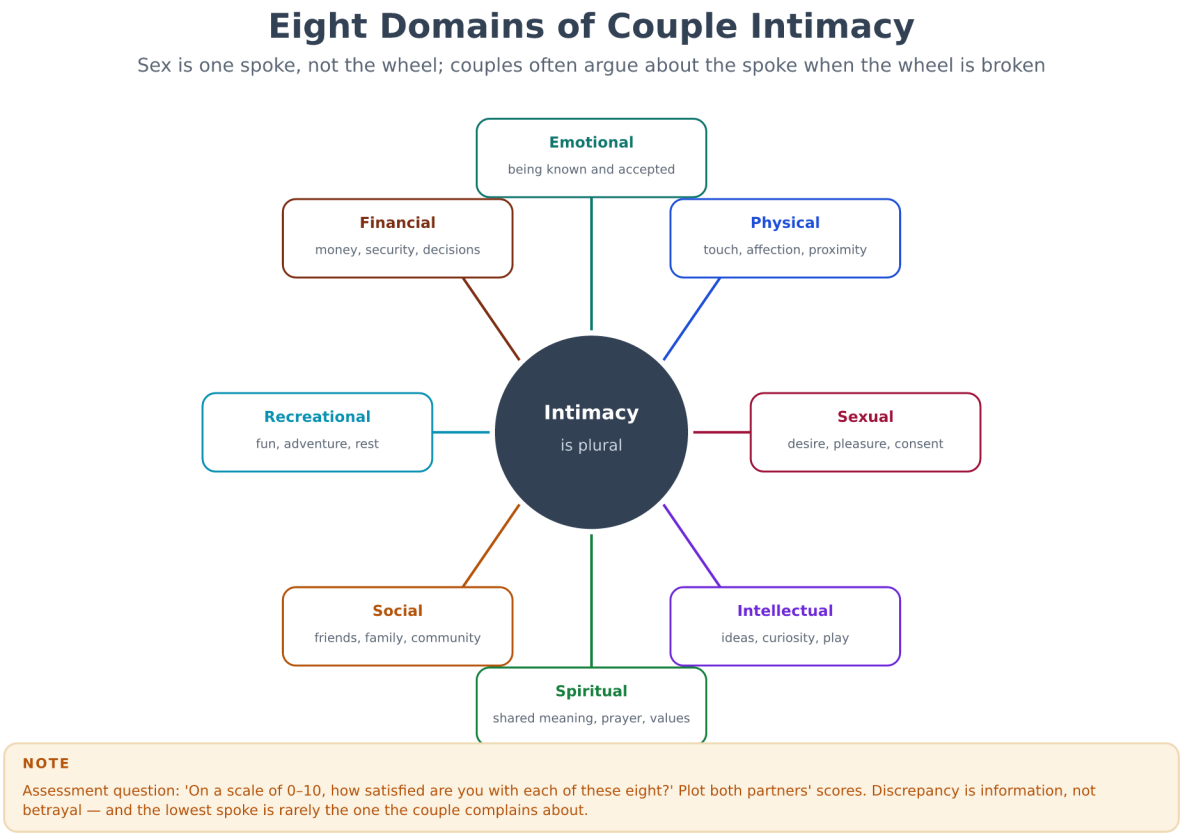


Figure 1.1 — Eight domains of couple intimacy

Couples rarely complain about "intimacy". They complain about sex, or loneliness, or a partner who is always on the phone. Your task is to locate the complaint in the wider system of intimacy domains: emotional, physical, sexual, intellectual, spiritual, social, recreational, financial.

The common pattern. A couple presents with a sexual problem, and assessment reveals that the emotional domain has been starved for years. Treating the sexual symptom alone usually fails; rebuilding the other domains usually helps the sexual one.

Assessment question. "On a scale of 0 to 10, how satisfied are you with each of these eight areas?" Plot both partners' answers. The gap between them, and the gap between the couple's stated problem and their lowest domain, is the clinical map.

Week 2 — Attachment and intimacy

Secure attachment allows both closeness and separateness: the person can ask for comfort and can tolerate a partner's independence.

Anxious attachment produces hypervigilance: sex may be used to secure reassurance, and any distance feels like rejection.

Avoidant attachment produces discomfort with dependency: sex may be easier than emotional closeness, and post-coital distance is common.

Disorganised attachment produces approach–avoidance: desire and fear in the same moment.

Clinical application. "What does sex mean to each of you?" often reveals the attachment function: proof of love, proof of adequacy, a way to avoid conversation, a way to end a conflict, a rare moment of safety.

Week 3 — Desire

Two models.

- **Spontaneous desire**: desire appears before arousal, seemingly unbidden. More commonly reported by men in many samples, and by people in new relationships.
- **Responsive desire**: desire emerges after arousal, in response to context, closeness, relaxation or stimulation. More commonly reported by women, by parents of young children, and by people under stress.

The clinical error. Treating responsive desire as "low desire" or "dysfunction". If a person's desire is responsive, the intervention is context — time, privacy, relaxation, non-sexual affection — not pressure to "want it more".

The dual control model (Bancroft & Janssen): sexual response depends on both excitation and inhibition. Stress, shame, performance anxiety, religious guilt and fear of pregnancy all raise inhibition. Lowering inhibition is often more effective than increasing stimulation.

Week 4 — The sexual response

The classic model (Masters & Johnson; Kaplan): desire → excitement → plateau → orgasm → resolution. **The circular model** (Basson) is more accurate for many people, especially women: intimacy, closeness, context and stimulation interact repeatedly, without a fixed sequence.

What clinicians need to know.

- Arousal is not desire, and lubrication is not consent or interest.
- Arousal non-concordance is normal: the body can show arousal without desire, and desire without arousal.
- Orgasm is not the goal; pleasure and connection are.
- Pain is always a medical question first (vaginismus, vulvodynia, endometriosis, infection, scarring, menopause).
- Medication (especially SSRIs) commonly affects desire, arousal and orgasm.
- Ageing changes speed and reliability, not the capacity for pleasure.

Week 5 — Taking a sexual history

Principles. Ask permission; explain why; use the person's language; be matter-of-fact; do not react with surprise, approval or disapproval; and take a history from each partner separately at least once.

A workable sequence.

1. Permission: "I ask everyone about this because it is often part of what brings couples here. Is that all right?"
2. Current: "How is intimacy between you at the moment? How often, and how satisfying, from 0 to 10?"
3. History: "Was there a time it was good? What was different then?"
4. Meaning: "What does sex mean to you — closeness, relief, duty, play, reassurance?"
5. Body: "Any pain, any difficulty with arousal or orgasm, any medical condition or medication?"
6. History of harm: "Has anyone ever pressured or forced you sexually?" (Ask individually.)
7. Culture and faith: "What were you taught about sex growing up? What do you believe is allowed?"
8. Expectations: "What would you like to be different — and what would your partner say you want?"

Language. Avoid euphemism and avoid clinical jargon. Use the words the couple uses, and offer a plain alternative if they have none.

> **ASSESSED**
> A 30-minute sexual history with a role-played client. Assessed on permission, pacing, breadth, non-judgemental tone, and the identification of medical or safety issues requiring referral.

Week 6 — Talking about sex

Why couples cannot. Shame, fear of hurting, fear of being judged, previous rejection, cultural prohibition, and the belief that "if you loved me you would know".

The therapist's job is to make the conversation normal, structured and safe. Techniques:

- **The third object.** Talk about "the sex thing" as an object between you, rather than about the people.
- **Scaling.** "How satisfied are you, 0–10?" Numbers are less exposing than adjectives.
- **Written disclosure.** Each writes three sentences about what they want more of; you read them aloud.
- **The permission sentence.** "I am going to ask something direct. You can say pass."

Scripts to give couples.

> "I want to talk about us, and I am nervous. Can we take twenty minutes on Saturday?" "I miss being close to you. I am not asking for sex tonight; I am asking whether we can talk about it this week." "When you turn away, I tell myself you do not want me. Is that what you mean?"

Week 7 — Desire discrepancy

The most common sexual complaint in couple therapy. It is a normal difference, not a disorder, until it produces distress.

The pattern. The higher-desire partner pursues; the lower-desire partner avoids; the pursuit increases pressure; the avoidance increases; both feel rejected, and the couple stops touching altogether.

Intervention.

1. **Normalise.** "Desire differences are the norm, not the exception. The problem is not the difference; it is the cycle around it."
2. **De-pressurise touch.** Reintroduce non-sexual affection with an explicit ban on sex for a period.
3. **Negotiate.** What is acceptable to both? Frequency is a poor target; quality, initiation and affection are better ones.
4. **Address the context.** Sleep, children, work, resentment, body image, medication.
5. **Reject the scoreboard.** Sex is not a debt; "I did it last time" is a symptom of the problem, not a solution.

Week 8 — Erectile and arousal difficulties

Always rule out medical causes: vascular disease, diabetes, hypertension, medication, hormonal factors, alcohol and smoking, neurological conditions. Refer to a physician; do not treat as purely psychological.

The psychological pattern. Performance anxiety → monitoring → arousal drops → confirmation of the fear → increased anxiety. The partner's disappointment (real or imagined) intensifies it.

Relational intervention. Reduce performance focus; reintroduce sensate focus; involve the partner in non-demanding intimacy; address the couple's broader pattern; and refer for medical assessment and, where appropriate, medication.

Arousal difficulties in women follow a similar logic, with a stronger contribution from context, pain, relationship quality and hormonal change.

Week 9 — Pain, orgasm and low desire

Presentation	First question	Relational work
Painful intercourse	Has a medical cause been excluded?	Pace, consent, fear, communication
Difficulty reaching orgasm	Is this new? Is it situational?	Reduce goal focus; explore what works alone
Absent or low desire	Is it global or partner-specific?	Context, resentment, responsive desire model
Aversion	Is there a history of coercion or trauma?	Safety, trauma-informed pacing, referral

Referral is not failure. Persistent pain, unexplained physical symptoms, severe trauma responses and psychiatric complexity belong with specialists. The relational work continues alongside.

Week 10 — Betrayal and repair

Betrayal trauma (developed in MFT 710). The betrayed partner's symptoms mimic PTSD: intrusion, hypervigilance, intrusive images, and a shattered assumption that the partner is safe.

Repair requires four things, in order.

1. **Safety** — the behaviour has stopped; boundaries are in place; no violence.
2. **Disclosure** — enough truth to end the lying, not so much detail that it becomes a weapon. Agree what will be asked and answered.
3. **Remorse that is not self-pity** — "I did this, it was my choice, and I see what it cost you."
4. **Consistency over time** — transparency, reliability, and patience with the betrayed partner's cycling.

The cycling is normal. Recovery is not linear: the betrayed partner may be calm for two weeks and then devastated by a song. Warn both partners, so the cycle does not become a second betrayal.

Sex after betrayal. Often the couple either avoids sex entirely or uses it to short-circuit the conversation. Both are understandable and neither is repair. Rebuild slowly, with explicit consent and no expectation.

Week 11 — Culture, religion and sexual scripts

Sexual scripts are culturally learned instructions about who should want what, when, and with whom.

Questions for the couple (each individually first).

- What were you taught about sex before marriage? By whom?
- What did you learn about your body?
- What is permitted, and what is forbidden? What happens if you break the rule?
- Is sex a duty, a gift, a right, a sin, a reward?
- Who initiates? Who is allowed to refuse? What happens when someone refuses?

Clinical themes in Nigerian and similar contexts.

- Pre-marital abstinence teaching that produces guilt after marriage.
- The expectation that sex is a marital obligation, which can silence refusal.
- Female sexual pleasure as unspoken or stigmatised.
- Bride price and the sense of "ownership" it can create.
- Family planning, fertility pressure, and the stigma of childlessness.
- Religious counselling that frames desire as sin.
- Polygyny, and the sexual dynamics between co-wives.

The clinical stance. You are not there to adjudicate the script; you are there to make it speakable, and to ensure that consent is real for both people.

> **CASE PLAN (40%)**
> A couple with a desire discrepancy and a history of religious guilt. Includes sexual history, relational formulation, medical rule-outs, intervention plan, and referral criteria.

Week 12 — Consent, coercion and obligation

Consent is ongoing, specific and freely given. It can be withdrawn at any point, including during sex, and including in marriage.

Coercion includes pressure, sulking, threats, guilt, economic withholding, and the use of religion. It does not have to be physical to be harmful.

Assess individually: "Can you say no to your partner without fear of what will happen? What does happen when you say no?"

When there is coercion, the work is with the person experiencing it first: safety, validation, and options. Couple work is contraindicated until safety is established.

Week 13 — Lifespan and special populations

Group	Considerations
Parents of young children	Exhaustion, touch saturation, interrupted privacy; plan rather than wait for spontaneity
Midlife	Hormonal change, body image, teenagers in the house, career strain

Group	Considerations
Later life	Assumption of asexuality by families and clinicians; health, medication, bereavement, new partners
Chronic illness and disability	Access, pain, fatigue, body image, and the partner's carer role
LGBTQ+ couples	Minority stress, disclosure, family rejection, and the absence of affirming services
Asexual people	Absence of sexual attraction is not pathology; negotiate intimacy on other terms
Post-cancer	Body change, fertility, fear, and the couple's avoidance

The clinician's assumption to avoid: that everyone wants sex, that everyone should, and that a relationship without sex is deficient. Ask what this couple wants.

Week 14 — Sensate focus

Purpose. To reduce performance anxiety and rebuild non-demanding physical intimacy.

Progression.

1. **Non-genital touch, no sex.** Twenty minutes, one partner touches, the other receives; then swap. Feedback only on what felt pleasant, not on what was missing.
2. **Genital touch, still no intercourse.** Continue the same structure.
3. **Intercourse permitted but not required.** Often the couple is advised to continue the exercises first.
4. **Generalisation.** Gradually return to the couple's own sexual repertoire.

Rules. No goal, no intercourse during the first stages, no negative feedback, and a clear opt-out at any time. Couples who "fail" at sensate focus are usually still performing.

Week 15 — The therapist's comfort

Your own sexual history, beliefs and discomfort will shape this work. Write 500 words on: what you find hardest to ask; what judgements you notice; and what you will do when you are out of your depth. Then defend it in a viva, and take it to supervision.

Glossary

Arousal non-concordance — mismatch between physiological arousal and subjective desire. **Desire discrepancy** — a difference between partners in the frequency or intensity of sexual desire. **Dual control model** — sexual response as the balance of excitation and inhibition. **Responsive desire** — desire that emerges in response to arousal or context. **Sensate focus** — graded, non-demanding touch exercises. **Sexual script** — culturally learned expectations about sexual behaviour and meaning. **Spontaneous desire** — desire experienced before arousal, without an obvious trigger.

Self-assessment

Multiple choice

1. Lubrication indicates: (a) consent (b) physiological arousal, which may not match desire (c) desire (d) orgasm

2. Responsive desire is best supported by: (a) pressure to initiate (b) context, relaxation and non-sexual affection (c) performance goals (d) frequency targets

3. The most common sexual complaint in couple therapy is: (a) infidelity (b) desire discrepancy (c) pain (d) infertility

4. Sensate focus requires: (a) intercourse from stage one (b) no goal and no intercourse initially (c) feedback on what is missing (d) daily practice

5. Erectile difficulties should first prompt: (a) psychodynamic exploration (b) medical rule-out (c) couple blame (d) medication advice

6. Consent in marriage is: (a) automatic (b) ongoing, specific and revocable (c) implied by the ceremony (d) not a clinical matter

7. Betrayal trauma symptoms resemble: (a) depression only (b) PTSD (c) psychosis (d) nothing

8. Asexual identity: (a) is a disorder (b) is not pathology; intimacy may be negotiated otherwise (c) requires treatment (d) means no relationships

9. Painful intercourse is first: (a) a relational problem (b) a medical question (c) a sign of avoidance (d) untreatable

10. The therapist should refer when: (a) the couple mentions sex (b) persistent pain, trauma complexity or issues beyond competence arise (c) the couple is young (d) culture differs

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the eight domains of intimacy and explain why the lowest is not always the complaint.
2. Distinguish spontaneous and responsive desire and give the clinical implication.
3. Write six questions from a sexual history.
4. What are the four stages of sensate focus?
5. How do you assess for coercion without leading the client?

Essay (choose one, 2,000 words)

1. "Desire discrepancy is a relationship problem, not a disorder." Discuss with a case.

2. Analyse the influence of religious teaching on sexual intimacy in couple therapy.
3. Discuss the ethical and clinical issues in working with sex in a couple where coercion is suspected.

Further reading

- Basson, R. (2001). Human sex response cycles. Journal of Sex and Marital Therapy, 27(1), 33–43.
- Bancroft, J., & Janssen, E. (2000). The dual control model of male sexual response. Neuroscience & Biobehavioral Reviews, 24(5), 571–579.
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- Perel, E. (2006). Mating in Captivity. Harper.
- Nagoski, E. (2019). Come as You Are (rev. ed.). Simon & Schuster.

Next: MFT 719 — Family Life Cycle and Transitions

MSC MARRIAGE AND FAMILY THERAPY

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MFT 719

Family Life Cycle and Transitions

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MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	**Family Life Cycle and Transitions**	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate. This course teaches you to locate the symptom in time: at which stage is this family, which transition is it facing, and what must be renegotiated.

How to use this book

The life-cycle framework is a diagnostic instrument, not a moral timetable. Each week applies it to a different stage and a different kind of transition, including the ones that do not fit the classic sequence — migration, polygyny, foster care, sudden death, disability. Culture is not an exception to the model; it is the material from which the timetable is made.

1. Course description

The course covers the Carter–McGoldrick stage model; the distinction between predictable (normative) and unpredictable (non-normative) transitions; horizontal and vertical stressors; the family's need to renegotiate roles, rules and boundaries at each transition; rituals; intergenerational legacies; and the transitions particular to contemporary African and diasporic family life.

2. Learning outcomes

#	Outcome	Assessed
LO1	Place a family within the life-cycle model and identify its transition	Case analysis
LO2	Distinguish normative from non-normative transitions and their clinical implications	Exam
LO3	Analyse horizontal and vertical stressors in a family	Case analysis
LO4	Design a transition plan with a family	Observed session
LO5	Use rituals and marking to consolidate change	Skills test
LO6	Adapt the model for migration, polygyny, fosterage and non-linear trajectories	Essay
LO7	Work with loss and unfinished transitions	Reflective portfolio

3. Assessment

Component	Weight	Due
Life-cycle case analysis (2,000 words)	30%	Week 7
Transition plan observed session (40 min)	30%	Week 11

Component	Weight	Due
Essay on cultural adaptation (1,500 words)	20%	Week 13
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why transitions produce symptoms	Transition timeline
2	The six stages and their tasks	Stage identification
3	Normative and non-normative transitions	Stressor mapping
4	Horizontal and vertical stressors	Genogram + timeline
5	Stage 1–2: leaving home and joining	Case analysis
6	Stage 3: young children and the couple's reorganisation	Case analysis
7	**Case analysis due**; Stage 4: adolescents	Family session practice
8	Stage 5: launching and the empty nest	Couple re-negotiation
9	Stage 6: later life, caregiving and loss	Caregiver family meeting
10	Migration and displacement as family transitions	Migration genogram
11	**Transition plan observed**	Assessed
12	Divorce, remarriage and blended families	Structural adaptation
13	**Essay due**; births, deaths and unfinished transitions	Ritual design
14	Rituals, anniversaries and marking	Ritual design lab
15	Integration: the family in time	Case presentations

Week 1 — Why transitions produce symptoms

The thesis. Symptoms appear at points of transition because the family's existing organisation is no longer adequate to the new demands, and the renegotiation has not yet happened.

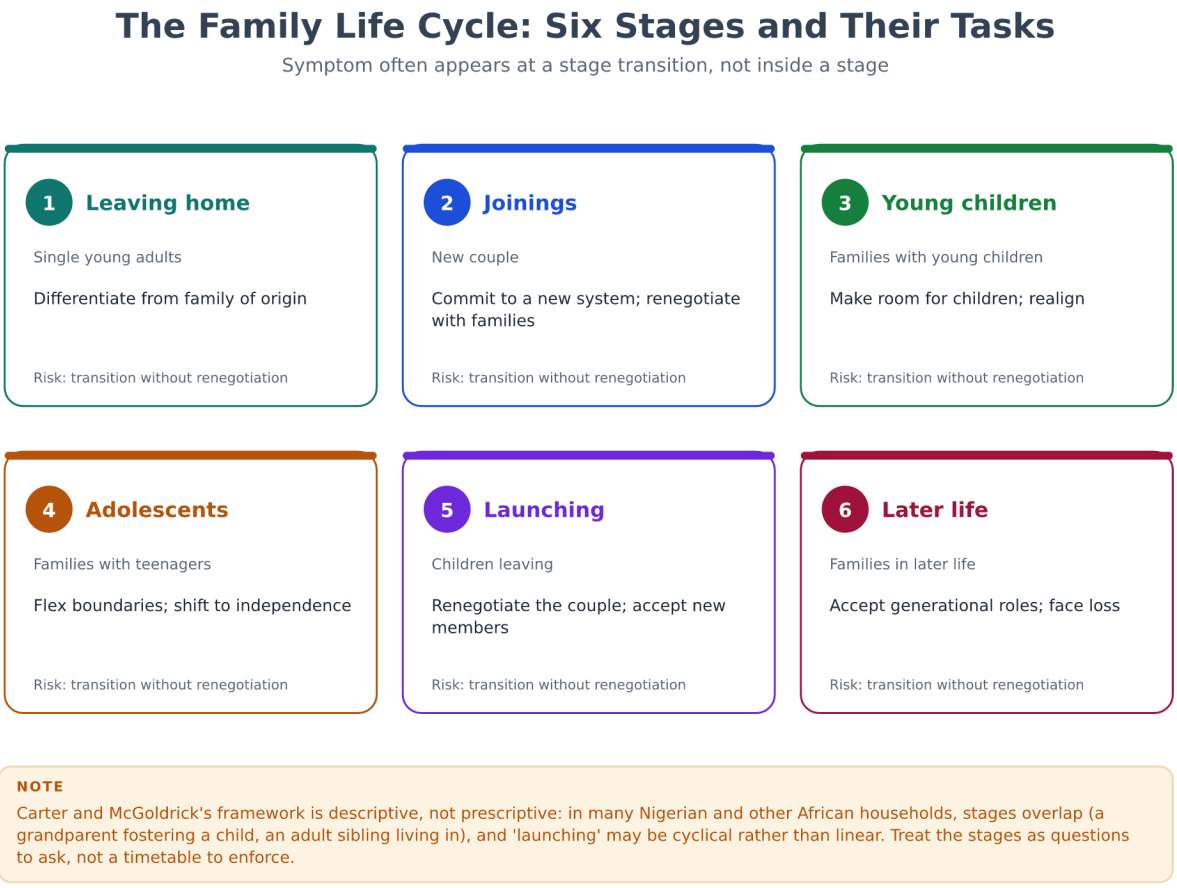


Figure 1.1 — Six stages of the family life cycle and their tasks

Three mechanisms.

1. **Role vacuum.** A role disappears (the working mother, the eldest child at home) and no one has been assigned the new one.
2. **Boundary mismatch.** A boundary that suited the previous stage is now too rigid or too diffuse (parents who cannot let an adolescent have privacy; an adolescent who has become the mother's confidante).
3. **Unfinished business.** The previous transition was never completed (a father who never really left home; a family that never mourned a stillbirth), so the new one cannot be navigated.

Clinical question. "What changed in the last 6–18 months?" almost always produces the precipitant.

> **PRACTICAL**
> Draw a timeline of your family of origin marking moves, births, deaths, marriages, migrations, and job changes. Mark where symptoms or conflicts appeared. Note the pattern.

Week 2 — The six stages

Stage	Emotional task	Second-order change required
1. Leaving home	Accept emotional and financial responsibility	Differentiate from family of origin; develop intimate peer relationships
2. Joining	Commit to a new system	Form a couple; realign relationships with extended families
3. Young children	Accept new members	Adjust the couple system; co-parent; realign with grandparents
4. Adolescents	Increase flexibility	Shift parent–child boundaries; refocus on midlife marriage and careers
5. Launching	Accept many entries and exits	Renegotiate the marriage as a dyad; include in-laws and grandchildren
6. Later life	Accept shifting roles	Deal with loss, disability, and the next generation's centrality

Two warnings. The model is **descriptive of a particular Western, middle-class trajectory**. And the stages overlap: in many families, launching and caregiving occur simultaneously, children return home, and grandparents raise grandchildren.

Week 3 — Normative and non-normative transitions

Normative transitions are expected: marriage, first child, school entry, adolescence, launching, retirement, death of elderly parents.

Non-normative transitions are unexpected or off-time: divorce, a child's disability, a death of a young parent, migration, redundancy, imprisonment, a pandemic, infertility, a child born outside marriage.

Why the distinction matters. Normative transitions have cultural scripts — rituals, advice, role models. Non-normative transitions usually do not, which leaves families improvising under stress, often with shame.

Clinical implication. For non-normative transitions, part of the work is script-making: helping the family construct a way to do something no one they know has done.

> **PRACTICAL**
> Take a non-normative transition (e.g. a child's diagnosis). List the scripts that exist, the scripts that do not, and what the family has had to invent.

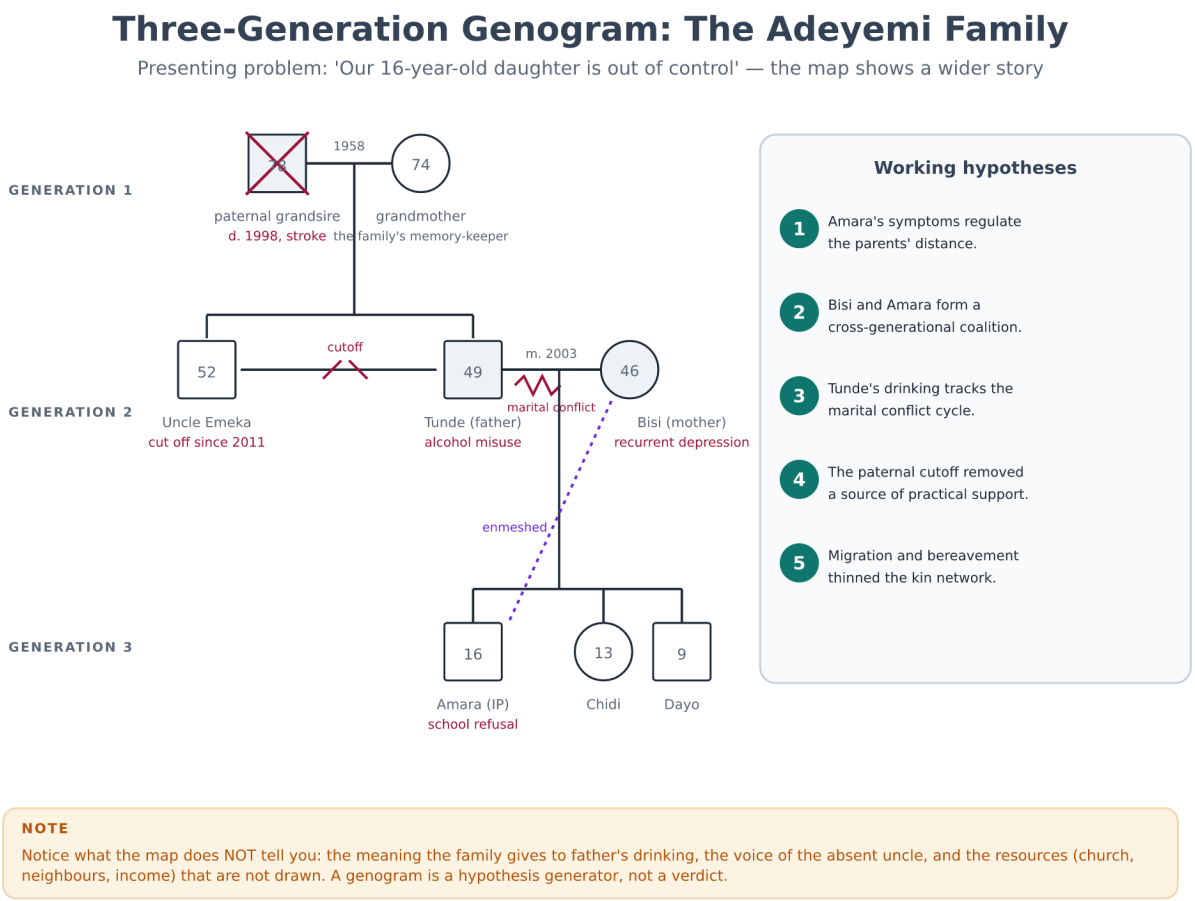
Week 4 — Horizontal and vertical stressors

Horizontal stressors — events moving through time: life-cycle transitions, illness, accidents, migration, poverty, discrimination.

Vertical stressors — legacies handed down: family myths, expectations, secrets, taboos, ethnic and religious beliefs, scripts about gender and success.

The clinical insight. A family can usually manage horizontal stressors or vertical ones; the crisis comes when both arrive at once.

Example: A family facing redundancy (horizontal) in which the father's family has a myth that "a man who cannot provide is worthless" (vertical). The financial problem is manageable; the collision is not.



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 4.1 — A genogram as a map of vertical stressors

Week 5 — Leaving home and joining

Leaving home. The task is differentiation, not departure. A young adult may live at home and be differentiated, or live abroad and be fused. The clinical markers: can they disagree with a parent without catastrophe? Can they ask for help without regression?

Joining. The couple must build a "we" while remaining connected to two families of origin. The most common failure is a boundary problem: a spouse who remains more loyal to a parent than to the partner.

Assessment questions.

- "When you disagree with your mother, what happens?"
- "Who did you consult first about this decision?"
- "How often do you see each family? Who decided that?"
- "Whose opinion matters most in this marriage?"

Week 6 — Young children

The reorganisation. Time, sleep, money, division of labour, sexuality, identity, and the relationship with grandparents all change at once. Research consistently shows a dip in relationship satisfaction after the first child, and that the quality of the co-parenting alliance predicts child outcomes more strongly than conflict.

The clinical presentations.

1. **The couple disappears into parents.** No couple time, no sexuality, no conversation that is not about the child.
2. **The grandmother becomes a third parent.** A cross-generational coalition forms; the father becomes peripheral.
3. **The child becomes the regulator.** A toddler's behaviour organises the household's emotional life.

Intervention. Protect the couple subsystem (time, privacy, an agreement about the division of labour); clarify the parental subsystem (a united front, agreed rules); and negotiate explicitly with grandparents about their role.

Week 7 — Adolescents

The family task is to increase flexibility: more autonomy with more accountability; less control and more influence; boundaries that permit privacy without permitting secrecy.

Two failure patterns.

Pattern	What it looks like	Consequence
Escalation	Parents tighten as the adolescent pushes	Conflict, running away, secrecy, risk
Abdication	Parents withdraw to avoid conflict	Unsupervised risk, no structure, anxiety

The middle path is negotiation with limits: clear non-negotiables (safety), negotiable domains (time, phone, clothes), and consequences agreed in advance.

Also at this stage: the parents' own midlife renegotiation, which often appears as an "adolescent problem" in the referral.

Week 8 — Launching and the empty nest

The task is to renegotiate the marriage as a dyad, and to accept new members and exits (partners, grandchildren, returns home).

The common presentations.

- **The couple who have nothing left to say.** They organised everything around the children; now there is silence.
- **The parent who cannot launch.** A mother who sabotages the child's independence because her role depends on being needed.
- **The child who will not leave.** Economic dependency, anxiety, or a parent's covert invitation.

Intervention. Rebuild the couple relationship deliberately; address the parent's identity and loss; and set a graduated plan for independence that neither humiliates the young adult nor abandons them.

Week 9 — Later life

The tasks. Accept generational role changes; deal with loss (parents, siblings, spouse, friends); support the next generation without taking over; prepare for disability and death; and make meaning of a life.

Clinical presentations. Caregiving disputes among siblings; a widowed parent's new relationship; the "responsible child" who carries everything; inheritance conflict; a grandparent raising grandchildren.

Intervention. Make the loyalty ledger explicit (who has given what, and what is owed); negotiate caregiving tasks concretely; and address the unspoken grief that usually sits under the conflict.

The dual-process frame for grief (Figure 22 in MFT 712) helps here: restoration-oriented activity (tasks, roles, legal arrangements) is not avoidance; it is part of grieving.

Week 10 — Migration and displacement

Migration is a family transition with all the features of a life-cycle stage and none of the rituals.

Phase	Family task	Common difficulty
Decision	Who goes, who stays, who pays	Coercion, debt, secrecy
Separation	Maintain attachment across distance	Children left with relatives; marital drift
Arrival	Establish a household	Status loss, housing, language

Phase	Family task	Common difficulty
Reunification	Re-form as a family	Children who do not know the parent; new authority conflicts
Settlement	Integrate without dissolving identity	Second-generation conflict; "you are not like us"

The "astronaut" and "parachute" patterns. A parent working abroad while the family remains; a child sent abroad for education. Both produce high achievement and high loneliness, and often a "satellite child" who is expected to justify the family's sacrifice.

Clinical questions. Who decided? Who was left? What was lost (status, language, community, identity)? What does the family owe the ones who stayed?

Week 11 — Observed transition plan session

Forty minutes with a role-played family at a transition (launching, caregiving, migration reunification). Assessed on: accurate location of the transition; renegotiation of roles and boundaries; inclusion of all members; and a concrete, reviewable plan.

Week 12 — Divorce, remarriage and blended families

Divorce as a transition, not an ending. The spousal relationship ends; the parental relationship continues in a different form. The family's task is to reorganise across two households while protecting the children from conflict.

The stages. Decision → telling the children → practical reorganisation → legal process → emotional adjustment → new equilibrium (often 2–4 years).

Remarriage and stepfamilies. A new structure with different tasks:

- Build the couple bond before the stepparent assumes authority.
- Negotiate household rules across two systems.
- Allow children to love both households without disloyalty.
- Expect 4–7 years to integrate, not months.
- Address the "insider/outsider" split explicitly.

Clinical error. Treating the stepfamily as a nuclear family that started late, and applying a nuclear family timetable.

Week 13 — Births, deaths and unfinished transitions

Unfinished transitions are those that were never marked, mourned or renegotiated: a stillbirth nobody mentions, a migration that was supposed to be temporary, a divorce that was never explained to the children, a death the family does not discuss.

Signs. Anniversary reactions, a name that cannot be spoken, a child named after the dead, a room kept unchanged, a recurring argument with no present cause.

Intervention. Name the unfinished transition gently; help the family construct a way to mark it; and allow the grief that the avoidance was managing.

Week 14 — Rituals

Rituals consolidate transitions. They mark the boundary between the old state and the new, assign new roles, and connect the family to its wider culture.

Types. Cultural and religious rites (naming, marriage, burial); family rituals (Sunday meals, holidays, anniversaries); and therapeutic rituals (a letter, a ceremony, a shared object, a family meeting).

Designing a therapeutic ritual.

1. **Purpose.** What transition needs marking?
2. **Participants.** Who must be present, and who must be absent?
3. **Symbol.** What object, place, word or action carries the meaning?
4. **Structure.** Beginning, middle, end.
5. **Follow-up.** How will the change be maintained?

Caution. Respect the family's religious and cultural boundaries; do not invent rites that displace their own; and never use a ritual to bypass a necessary conversation.

> **PRACTICAL**
> Design a ritual for a family that has never marked a bereavement. Present it in five minutes.

Week 15 — Integration: the family in time

The life-cycle lens gives you three questions that belong in every assessment:

1. **Where is this family in time?** (Stage, and the transition in progress.)
2. **What must be renegotiated?** (Roles, boundaries, loyalties, rules.)
3. **What has not been finished?** (Losses, transitions, legacies.)

Ask them in the first session and you will rarely be short of a focus.

Glossary

Horizontal stressors — events and transitions moving through time. **Vertical stressors** — legacies, myths and expectations transmitted across generations. **Normative transition** — an expected, culturally scripted life event. **Non-normative transition** — an unexpected or off-time event. **Second-order change** — change in the system's organisation, not only in its behaviour. **Unfinished transition** — a transition that was never marked or renegotiated. **Satellite child** — a child sent abroad or away for education, carrying the family's hopes.

Self-assessment

Multiple choice

1. Symptoms most often appear: (a) randomly (b) at transitions requiring renegotiation (c) only in adolescence (d) only after trauma

2. Vertical stressors are: (a) events in time (b) legacies and myths across generations (c) financial problems (d) illnesses

3. The task of "leaving home" is: (a) departure (b) differentiation while remaining connected (c) financial independence only (d) marriage

4. The dip in couple satisfaction after a first child is: (a) rare (b) a well-documented common pattern (c) caused by the child (d) unavoidable

5. The two adolescent failure patterns are: (a) escalation and abdication (b) fusion and cutoff (c) pursuit and withdrawal (d) enmeshment and disengagement

6. Blended family integration typically takes: (a) months (b) 4–7 years (c) weeks (d) one year

7. A "satellite child" is: (a) a child in orbit (b) a child sent away for education carrying family hopes (c) an only child (d) a foster child

8. Unfinished transitions often present as: (a) clear grief (b) anniversary reactions and unspoken names (c) financial problems (d) delinquency

9. Therapeutic rituals should: (a) replace the family's own rites (b) respect and complement them (c) be religious (d) be avoided

10. The life-cycle model is best used as: (a) a timetable to enforce (b) a descriptive diagnostic lens (c) a legal standard (d) a diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Distinguish normative from non-normative transitions with two examples each.
2. Explain horizontal and vertical stressors with a case.
3. What must be renegotiated when a first child is born?
4. Name four features of migration as a family transition.
5. Give five elements of a therapeutic ritual.

Essay (choose one, 2,000 words)

1. "The life cycle is a Western timetable." Critically evaluate and adapt the model for Nigerian families.

2. Analyse a family's presenting problem as a transition problem, and show what changed in your treatment plan.
3. Discuss the role of rituals in consolidating therapeutic change.

Further reading

- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- McGoldrick, M., Garcia-Preto, N., & Carter, B. (2016). The Multicultural Family Life Cycle. In The Expanding Family Life Cycle (5th ed.).
- Falicov, C. J. (2007). Working with transnational immigrants. Family Process, 46(2), 157–171.
- Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford.

Next: MFT 720 — Supervision and Consultation in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 20 OF 28

MFT 720

Supervision and Consultation in Therapy

15 teaching weeks · 1 figure

SEMESTER THREE

1.71 units · Prerequisite: MFT 706, 704

> “Supervision is the profession's quality-control mechanism and its main method of training.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 720 — Supervision and Consultation in Therapy

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 706, 704

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	**Supervision and Consultation in Therapy**	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Supervision is the profession's quality-control mechanism and its main method of training. This course prepares you to receive supervision well and to give it competently — with models, structure, feedback skills, attention to power, and the discipline of parallel process.

How to use this book

You are simultaneously a supervisee and a developing supervisor in this course. Each week you will analyse your own supervision (what worked, what did not, what you did not say) and practise supervising a peer. The two roles teach each other.

1. Course description

The course covers models of supervision (developmental, systemic, competency-based, reflective), the supervisory alliance, contracting and boundaries, live and recorded supervision, feedback, evaluation and gatekeeping, power and culture in supervision, parallel process, ethical and legal responsibilities, and consultation to organisations.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe and apply at least three supervision models	Exam
LO2	Contract clearly with a supervisee, including evaluation criteria	Contract document
LO3	Give specific, behavioural feedback that changes practice	Observed supervision
LO4	Identify and use parallel process	Case analysis
LO5	Manage power, culture and conflict in supervision	Reflective portfolio
LO6	Fulfil gatekeeping responsibilities ethically	Viva
LO7	Consult to a team or organisation	Consultation report

3. Assessment

Component	Weight	Due
Supervision contract and learning plan	15%	Week 4
Observed supervision session (40 min)	30%	Week 10
Case analysis of parallel process (1,500 words)	20%	Week 12
Consultation report (2,000 words)	25%	Week 14
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What supervision is for; supervision versus therapy versus mentoring	Role clarification
2	Models I: developmental and competency-based	Model application
3	Models II: systemic and reflective	Reflecting team in supervision
4	**Contract due**; contracting and boundaries	Contract negotiation
5	The supervisory alliance and rupture repair	Alliance repair rehearsal
6	Structure: agenda, case presentation formats, time	Structured case presentation
7	Feedback that changes practice	Feedback lab
8	Live supervision and recorded review	Live observation
9	Parallel process and the supervision triangle	Parallel process analysis
10	**Observed supervision session**	Assessed
11	Power, culture, gender and race in supervision	Power audit
12	**Parallel process analysis due**; conflict and incompetence	Difficult conversations
13	Evaluation, gatekeeping and fitness to practise	Evaluation interview
14	**Consultation report due**; consultation to organisations	Consultation simulation
15	Integration: the supervisor you want to be	Viva

Week 1 — What supervision is for

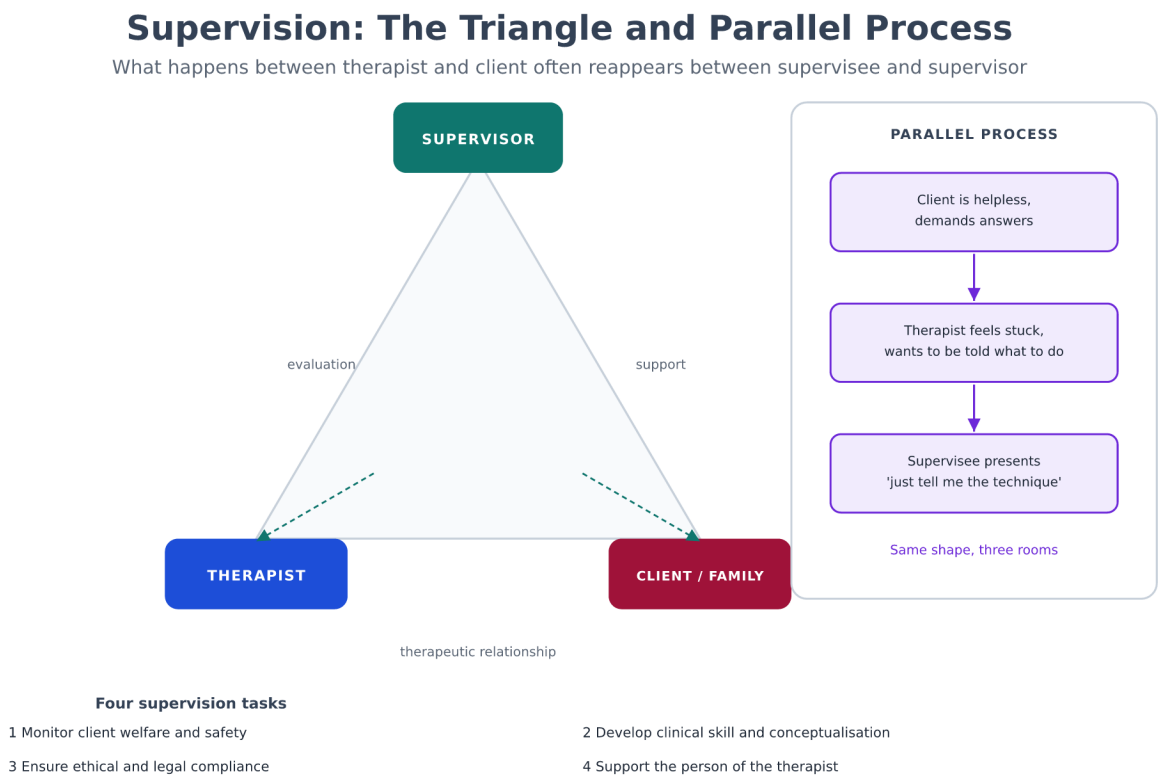


Figure 1.1 — The supervision triangle and parallel process

Four functions (Kadushin, adapted).

1. **Clinical** — ensuring the client receives good, safe care.
2. **Developmental** — building the supervisee's competence.
3. **Supportive** — sustaining the person doing difficult work.
4. **Gatekeeping** — protecting the public and the profession.

What supervision is not. It is not therapy, although it may be therapeutic. It is not mentoring, which is about career. It is not line management, although it overlaps. Confusing these roles is the most common source of boundary difficulty.

The client is the third party in the room. Whatever the supervisee needs, the question that governs the supervision is: is this client safe and well served?

Week 2 — Developmental and competency models

Developmental models (e.g. Stoltenberg) describe stages: the supervisee moves from dependence and anxiety, through dependence–autonomy conflict, to conditional autonomy, to mastery. The supervisor's style must change with the stage: more structure early, more consultation later.

Stage	Supervisee needs	Supervisor stance
Novice	Structure, direct guidance, safety	Teaching, modelling, clear expectations
Advanced beginner	Autonomy with support	Coaching, prompting, joint formulation
Competent	Challenge, complexity	Consulting, debating, opening alternatives
Proficient	Peer-level dialogue	Collegial, focusing on the system and self

Competency-based models specify observable skills and rate them, using frameworks such as the UCL competence frameworks. Their strength is transparency and defensibility; their risk is reducing supervision to a checklist.

The best use of both. Use developmental stage to decide how much structure, and competencies to decide what to look for.

Week 3 — Systemic and reflective models

Systemic supervision attends to the system of client–therapist–supervisor, and to the wider systems (agency, family, culture). The supervisor asks: what is the function of this presentation in this system, at this time?

Reflective supervision uses the supervisor's and supervisee's emotional responses as data, and models curiosity rather than expertise. Techniques include the reflecting team (in supervision), genograms of the case, and structured self-reflection.

Live supervision (behind a mirror, or in-room) gives immediate data. Formats: the phone-in, the "whispering" supervisor, the consultant who joins for ten minutes, and the reflecting team.

The advantage of live work is that it reveals what the therapist does, not what they remember doing. The disadvantage is anxiety, which must be managed explicitly in the contract.

Week 4 — Contracting

A supervision contract covers:

1. Purpose and scope; who else is involved (agency, university).
2. Frequency, duration, location, cancellation.
3. Case presentation format and preparation expectations.
4. Recording and observation policy, including consent from clients.
5. Confidentiality and its limits (client safety, fitness to practise, legal).
6. Evaluation: criteria, timing, who sees it, and consequences.
7. Boundaries: contact between sessions, personal disclosure, dual relationships.
8. Procedures for disagreement, complaint and termination.

Negotiate, do not impose. The contract should include the supervisee's goals: "By the end of this placement, I want to be able to…"

> **DOCUMENT (15%)**
> A supervision contract and learning plan for a first-placement supervisee.

Week 5 — The supervisory alliance

Three components (parallel to the therapeutic alliance): goals, tasks, and bond.

Ruptures are inevitable: a supervisee feels judged; a supervisor feels ignored; a disagreement about a case becomes personal. The repair is the model.

Repair script.

> "I noticed you went quiet when I suggested you had missed the risk. Did I get that wrong, or did it land badly?" "What I intended was a question, not a verdict. Let me say it again differently."

The unspoken curriculum. Supervisees learn more from how you behave than from what you teach. If you avoid difficult feedback, they will avoid it with clients. If you are curious about your own mistakes, they will be curious about theirs.

Week 6 — Structure

A 50-minute supervision session.

Minutes	Content
0–5	Check-in; agenda setting
5–15	Case 1: presentation, formulation, question
15–30	Case 1: exploration, recording review, options
30–40	Case 2 or a theme (risk, ethics, culture)
40–47	Learning points; tasks for the week
47–50	Administrative and evaluation notes

A case presentation format (five minutes, no notes).

1. Who is the family, and who is in the room?
2. What is the presenting problem, in their words?
3. What is my formulation and my uncertainty?
4. What did I do, and what happened?
5. What is my question for supervision?

The discipline matters: a supervisee who cannot present in five minutes is not yet thinking systemically.

Week 7 — Feedback

Principles. Behavioural, specific, timely, owned, and limited to what can be acted on.

The four-step structure.

1. **What I observed** ("you asked three questions in a row after she cried").
2. **What it did** (to the client, or to me as observer) ("she stopped elaborating").
3. **What to keep** ("your opening was excellent — you named the risk clearly").
4. **What to try** ("next time, try a reflection before the next question").

The banned practices. The sandwich (praise–criticism–praise), "you need to be more confident" (not behavioural), public criticism, and feedback given weeks after the event.

Receiving feedback is a skill too: listen, ask for an example, resist explaining, and decide what to use.

Week 8 — Live supervision and recording

Consent. Families must consent to recording and to observation, in writing, with a clear explanation of who will watch and how long the recording is kept.

In-session formats.

- **Phone-in:** the supervisor calls in with one or two suggestions. Risk: the therapist becomes a puppet.
- **Reflecting team:** the team talks in front of the family (or behind a mirror). The therapist chooses what to bring back. Strong evidence and strong engagement effects.
- **Consultant joins:** a senior clinician joins for ten minutes and then leaves.

Debriefing. Always debrief the supervisee's experience first, then the clinical content. The order matters: the person before the problem.

Week 9 — Parallel process

Definition. The pattern in the therapy reappears in the supervision: a helpless family produces a helpless therapist who produces a supervisor who takes over; a controlling family produces a therapist who complies and a supervisor who feels ignored.

How to use it.

1. Notice your own experience in supervision (boredom, irritation, the urge to solve).
2. Ask: is this mine, the supervisee's, or the system's?

3. Name it tentatively: "I notice I am doing a lot of the thinking here. I wonder whether that is also how it feels in the room."
4. Use it to generate a hypothesis about the family.

Caution. Parallel process is a hypothesis, not a law. Do not use it to explain away a supervisee's difficulty or your own poor supervision.

> **ANALYSIS (20%)**
> A 1,500-word analysis of a parallel process from your own supervision, with evidence and alternatives considered.

Week 10 — Observed supervision session

Forty minutes, supervising a peer with a real (anonymised) case. Assessed on: contract and structure; case presentation discipline; formulation support; feedback quality; attention to the supervisee's experience; and clear learning tasks.

Week 11 — Power, culture, gender and race

Power is structural. The supervisor evaluates, gatekeeps and holds institutional authority. It cannot be wished away; it can be made transparent.

Cultural dynamics. Differences in language, ethnicity, religion, gender, class and age shape what is said and what is withheld. A supervisee from a culture that does not openly disagree with elders may appear "unreflective" when they are being respectful.

Practical disciplines.

- Discuss difference explicitly at the start: "What should I know about how you prefer to receive feedback?"
- Invite disagreement in a form that is culturally possible: "What would you say if you disagreed?"
- Notice who speaks first, who is interrupted, and whose cases get the most time.
- Examine your own assumptions about competence.

Week 12 — Conflict, incompetence and impairment

Conflict. Address early, privately, behaviourally. Distinguish disagreement about a case (healthy) from disagreement about values (harder) from a relationship rupture (needs repair).

Incompetence. Define it against the competencies, in writing, with a development plan and a review date. Most incompetence is developmental and remediable.

Impairment. When the supervisee's own difficulties affect client care (substance use, untreated trauma, exhaustion), the response is support plus boundaries, not silence. Silence is the most common failure of supervisors and the most damaging to clients.

The conversation. "I am concerned about you, and I am also concerned about your clients. Let us talk about both."

Week 13 — Evaluation and gatekeeping

Evaluation criteria should be known in advance and behavioural.

Domain	Example indicator
Clinical competence	Can conduct a structured assessment and formulation
Professional conduct	Punctuality, record-keeping, boundaries
Reflective capacity	Uses feedback; identifies own contribution
Ethical practice	Recognises and escalates risk
Relational capacity	Builds alliances with all family members

Gatekeeping. If a supervisee is not safe, the supervisor must act, even when it is painful: extend supervision, restrict caseload, require therapy, or fail the placement with a documented rationale.

Failing well. Give clear reasons, a written plan, support, and a route to appeal. Document throughout, not only at the end.

Week 14 — Consultation to organisations

The shift. Supervision develops a person; consultation helps a system. Your client is the team, the school, the clinic or the agency.

A consultation cycle.

1. **Contract:** who is the client, what is the question, who holds authority, what will be shared?
2. **Assess the system:** structure, communication, roles, alliances, and the presenting problem's function.
3. **Feedback:** present the systemic picture, with hypotheses rather than verdicts.
4. **Intervention:** a workshop, a reflecting team, a policy change, a case conference structure.
5. **Exit and evaluate:** what changed, and how will it be sustained?

Common organisational dynamics. The identified "difficult team member" who carries the team's anxiety; the "impossible case" that holds a conflict between services; the over-worked manager who mirrors the over-worked family therapist.

> **REPORT (25%)**
> A consultation report for a team or organisation, with a systemic assessment and a proposal.

Week 15 — The supervisor you want to be

Write 700 words: your model of supervision; how you will handle power; what you will never do; the feedback you find hardest to give; and how you will know you are doing harm. Defend it in a viva.

Glossary

Competency-based supervision — supervision organised around observable, rated skills. **Gatekeeping** — the supervisor's responsibility to protect the public by preventing unsafe practice. **Live supervision** — observation of therapy as it happens, with in-session input. **Parallel process** — the reproduction of the therapy dynamic in the supervision. **Reflecting team** — a group that discusses the case in front of the family, with the therapist choosing what to bring back. **Supervisory alliance** — the goals, tasks and bond of the supervision relationship.

Self-assessment

Multiple choice

1. The client in supervision is: (a) the supervisee (b) the family receiving therapy (c) the agency (d) the university

2. Gatekeeping exists to: (a) protect the profession's reputation only (b) protect the public and the profession (c) increase pass rates (d) satisfy administrators

3. Parallel process is: (a) a proven law (b) a hypothesis about the reproduction of dynamics in supervision (c) transference (d) supervision of supervision

4. A good feedback statement is: (a) general and encouraging (b) behavioural, specific and owned (c) delayed (d) public

5. Live supervision requires: (a) no consent (b) written client consent and clear explanation (c) only supervisor consent (d) audio only

6. The sandwich technique is discouraged because: (a) it is too positive (b) it trains people to hear praise as a warning (c) it is fast (d) it is illegal

7. Developmental models suggest that novices need: (a) less structure (b) more structure and direct guidance (c) peer consultation only (d) no evaluation

8. Impairment in a supervisee should be met with: (a) silence (b) support plus clear boundaries (c) immediate dismissal (d) transfer without discussion

9. Consultation differs from supervision in that it: (a) develops a person (b) helps a system (c) evaluates students (d) is always clinical

10. A supervision contract should include: (a) only the times (b) purpose, frequency, confidentiality, evaluation and boundaries (c) the supervisor's fees only (d) the client's diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the four functions of supervision.
2. Describe a five-minute case presentation format.
3. What are the four steps of behavioural feedback?
4. Give three ways power operates in supervision and one way to make it transparent.
5. What should a supervisor do when a supervisee is unsafe?

Essay (choose one, 1,800 words)

1. "Supervision is the profession's most important ethical practice." Discuss.
2. Analyse a parallel process from your own experience and what it revealed about the case.
3. Discuss the management of cultural difference in supervision, with examples.

Further reading

- Hawkins, P., & Shohet, R. (2012). Supervision in the Helping Professions (4th ed.). Open University Press.
- Stoltenberg, C. D., & McNeill, B. (2010). IDM Supervision (3rd ed.). Routledge.
- Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson.
- Andersen, T. (1991). The Reflecting Team. Norton.
- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation Competence Frameworks. UCL.

Next: MFT 721 — Research Methodology

MSC MARRIAGE AND FAMILY THERAPY

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MFT 721

Research Methodology

14 teaching weeks · 2 practicals · 2 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 705

> “This is the advanced methods course that turns a proposal into a thesis.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 721 — Research Methodology

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 705

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MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
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MFT 710	Couples Therapy	Two	1.71
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MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	**Research Methodology**	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the advanced methods course that turns a proposal into a thesis. You will work with real data, choose and justify analytic strategies, defend your design against examiners, and learn to write findings that practitioners will actually read.

How to use this book

Every week's task contributes directly to your thesis. Bring your own dataset, or use the teaching dataset supplied with the course. Do not read statistics passively; run them, break them, and interpret them badly at least once.

1. Course description

The course covers advanced design (longitudinal, dyadic, single-case, participatory), analysis (regression, multilevel modelling, dyadic models, thematic and framework analysis, mixed-methods integration), measurement and psychometric evaluation, systematic reviewing, data management and reproducibility, writing for publication, and defence of a study.

2. Learning outcomes

#	Outcome	Assessed
LO1	Select and justify a design for a complex relational question	Thesis proposal
LO2	Conduct and interpret appropriate analyses for relational data	Analysis portfolio
LO3	Evaluate the psychometric quality of a measure in a new population	Psychometric report
LO4	Integrate quantitative and qualitative findings coherently	Mixed-methods report
LO5	Defend design decisions against alternative explanations	Viva
LO6	Write a publishable findings chapter	Thesis chapter
LO7	Manage data ethically, securely and reproducibly	Data plan and audit

3. Assessment

Component	Weight	Due
Analysis portfolio (4 exercises)	30%	Weeks 5, 8, 11, 13
Psychometric evaluation report (1,500 words)	20%	Week 9
Mixed-methods integration report (2,000 words)	20%	Week 12
Thesis methodology chapter + viva	30%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	From proposal to protocol; pre-registration and its limits	Protocol draft
2	Design for relational questions	Design decision log
3	Sampling, power and feasibility in clinical settings	Power calculation
4	Data preparation and the analysis plan	Data cleaning log
5	**Analysis 1:** regression and its assumptions	Run and interpret
6	Multilevel models for nested family data	Model comparison
7	Dyadic models: APIM in practice	APIM exercise
8	**Analysis 2:** thematic and framework analysis	Coding audit
9	Psychometrics: reliability, factor structure, invariance	Report
10	Systematic review and meta-analysis basics	Search and appraise
11	**Analysis 3:** single-case and small-N designs	SCED exercise
12	Mixed methods and integration	Joint display
13	**Analysis 4:** robustness, missing data, common pitfalls	Sensitivity analysis
14	Writing, publishing and defending	Chapter and viva prep

Week 1 — Protocol and pre-registration

The protocol is the study's constitution: question, design, sample, measures, analysis plan, ethics, and timeline. Write it before you collect data; revise it with dated amendments if you must change course.

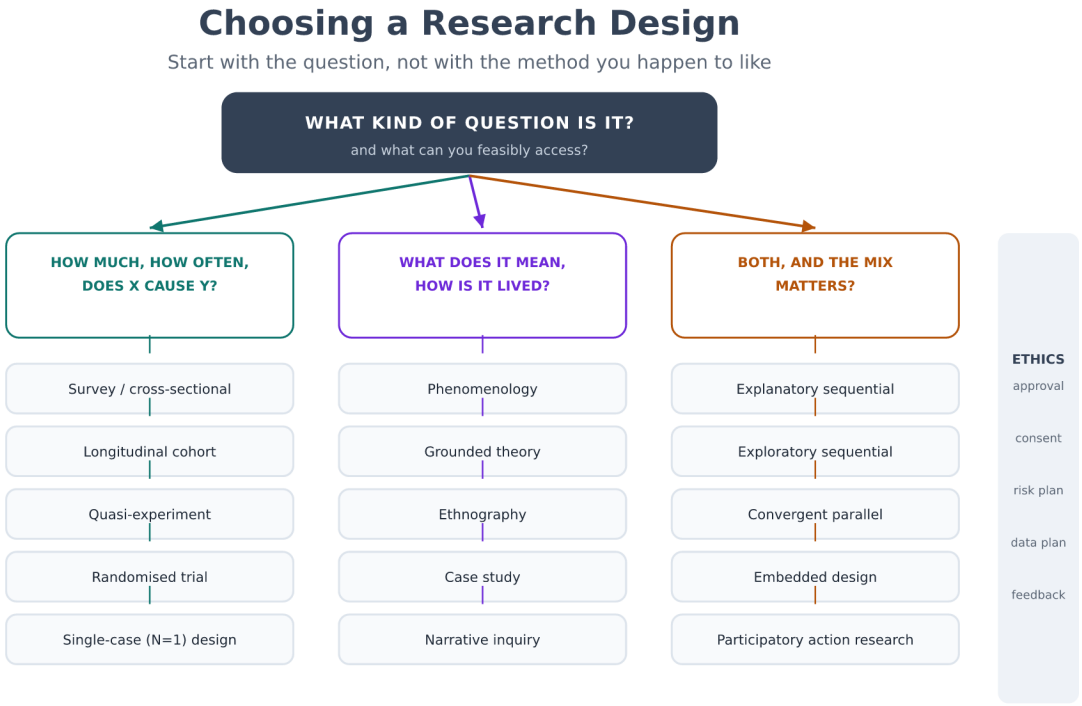
Pre-registration (e.g. on OSF) commits you publicly to a plan, which reduces the risk of unconscious analytical flexibility ("p-hacking"). It is standard in trials and increasingly expected elsewhere.

Honest limits. Exploratory qualitative work and community-based participatory research are often incompatible with rigid pre-registration. Declare your approach and be transparent about decisions made during the study.

A live analysis log — one page per week, dated — is the single most useful document in a thesis. It records what you did, what you decided, and why.

> **TASK**
> Draft a two-page protocol for your thesis. Include one paragraph on what you will do if recruitment is half of what you expect.

Week 2 — Design for relational questions



NOTE

In MFT the unit of analysis is the relationship, not only the individual — which creates real statistical problems: partners are non-independent. Plan for it (dyadic data analysis, actor–partner models, or a design that samples one person per household).

Figure 2.1 — Choosing a design from the question

Four designs especially suited to family questions.

Design	Question	Strength	Weakness
Longitudinal panel	Does X predict change in Y?	Temporal ordering	Attrition, cost
Dyadic (APIM)	How do partners affect each other?	Separates actor and partner effects	Requires both partners
Single-case experimental (SCED)	Does this intervention work for this family?	Clinical realism, causal inference within case	Generalisation
Participatory action research	What would improve this community's experience?	Validity, ethics, impact	Time, power negotiation

The design decision log. For each choice, record: the alternative considered, the reason for rejection, and the threat it leaves unresolved. Examiners reward this; it shows you know what your design cannot do.

Week 3 — Sampling, power and feasibility

Power. Do the calculation before you collect (GPower, R's `pwr`, or simulation). For dyadic and multilevel designs, power depends on the number of clusters* (couples, families), not the number of individuals.

Practical realities in clinical settings.

- Referral rates: how many families does the clinic actually see per week?
- Eligibility: what proportion will meet your criteria?
- Consent: what proportion will agree? (Often 30–60% in clinical samples.)
- Attrition: assume 20–30% at follow-up.
- Seasonality: school terms, holidays, religious calendars.

The feasibility sentence. "To recruit 60 couples in six months, I need 4 eligible referrals per week with a 60% consent rate; the clinic receives 9 per week; two colleagues have agreed to refer."

Week 4 — Data preparation

The unglamorous work that determines everything.

1. **Codebook** — every variable, its label, its values, its missingness convention.
2. **Raw data frozen** — never edit the original file after data entry.
3. **Cleaning log** — every decision (a value of 99 recoded to missing; one duplicate removed).
4. **Missing data** — describe the pattern; distinguish missing at random from missing not at random; consider multiple imputation rather than listwise deletion.
5. **Reverse scoring** — check; a mis-scored subscale invalidates the results.
6. **Outliers** — inspect, do not automatically delete.

> **TASK**
> Produce a codebook and cleaning log for your dataset (or the teaching dataset).

Week 5 — Regression

What regression does. Estimates the association between predictors and an outcome, adjusting for other predictors, and expresses it in the outcome's units.

Assumptions to check. Linearity, independence of errors, homoscedasticity, normality of residuals (for inference), and no severe multicollinearity. Report the checks; do not just assert them.

Reading output. Coefficient (direction, size, units), standard error, confidence interval, p-value, R^2, and model fit. Report effect sizes with confidence intervals, not only p-values.

Clinical translation. "Each additional point on the criticism scale was associated with a 0.3-point lower satisfaction score (95% CI −0.5 to −0.1), holding depression constant." That sentence is worth more than a table of asterisks.

Week 6 — Multilevel models

The problem. Individuals nested in couples, couples nested in therapists, repeated measures nested in people. Standard regression assumes independence and will produce confidence intervals that are too narrow.

The multilevel solution. Model the nesting explicitly:

- **Fixed effects** — the average relationship across the sample.
- **Random effects** — how intercepts (and slopes) vary across clusters.
- **ICC** — the proportion of variance attributable to the cluster. A high ICC means the clustering matters greatly and cannot be ignored.

Model-building sequence. Null model (ICC) → add level-1 predictors → add level-2 predictors → test random slopes → compare with likelihood ratio tests or information criteria (AIC/BIC).

Report. Always report the ICC, the number of clusters, and the estimation method.

Week 7 — Dyadic models

APIM in four numbers.

- **Actor effect (a):** my predictor on my outcome.
- **Partner effect (p):** my predictor on my partner's outcome.
- **Distinguishability:** whether partners are distinguishable (e.g. by gender or role) and whether effects can be constrained equal.
- **K:** the correlation between partners' residuals, which models the non-independence.

Interpretation example. In a study of 80 couples, criticism predicted lower satisfaction for the person giving it (actor effect) more strongly than for the person receiving it (partner effect). Clinical implication: the critic is also being harmed, which is a more useful reframe than "stop criticising your partner".

Software. R (`lme4`, `nlme`, `ape`), Mplus, SPSS MIXED.

Week 8 — Qualitative analysis

Two workable approaches.

Approach	Logic	Output
Reflexive thematic analysis (Braun & Clarke)	Themes constructed from coding, with the analyst's role explicit	Rich thematic account

Approach	Logic	Output
Framework analysis (Ritchie & Spencer)	Matrix of cases × themes, often for applied policy questions	Structured matrix, comparable across cases

Rigour in practice.

- Keep a **coding audit trail**: initial codes, revisions, discarded codes and why.
- Use **analytic memos** after each transcript.
- **Second-coder check** on at least 20% of data with a documented discussion of divergence.
- **Member reflection** where appropriate (not to validate themes, but to deepen them).
- **Reflexive journal** on your own reactions and assumptions.

The common error is presenting themes as "findings that emerged" from data, as though the analyst were not present. Themes are constructed; say so.

Week 9 — Psychometrics



Figure 9.1 — Validity, reliability and their limits

A psychometric evaluation of a measure in your population.

1. **Internal consistency** — Cronbach's α (and ω, which is more appropriate for congeneric items).

2. **Factor structure** — exploratory or confirmatory factor analysis; report fit indices (CFI, TLI, RMSEA, SRMR).
3. **Construct validity** — correlations with related and unrelated measures.
4. **Measurement invariance** — configural, metric, scalar, across groups (gender, language, age).
5. **Responsiveness** — change scores in a treated sample.

A local validation study is a perfectly good thesis, and often more useful to the field than another outcome study using an unvalidated instrument.

> **REPORT (20%)**
> A 1,500-word psychometric evaluation of one instrument, with recommendations.

Week 10 — Systematic review basics

Steps. Register the protocol (PROSPERO) → define the question (PICO) → search systematically (at least three databases; record the strategy) → screen (two reviewers; report agreement) → extract data → assess risk of bias (e.g. Cochrane RoB 2, or CASP for qualitative) → synthesise (narrative or meta-analytic) → assess certainty (GRADE).

Meta-analysis, briefly. Pool effect sizes with a random-effects model unless there is a justification for fixed effects; report heterogeneity (I^2, τ^2); explore it with subgroup analysis or meta-regression if there are enough studies; assess publication bias with a funnel plot and Egger's test, understanding their limitations.

What examiners look for. A search strategy that could be reproduced; a bias assessment that is not perfunctory; and a conclusion proportionate to the evidence.

Week 11 — Single-case and small-N designs

Why they matter in family therapy. Clinical samples are small, families are heterogeneous, and randomised trials are expensive. Single-case designs give causal inference within a case.

Core designs.

Design	Structure	Inference
A–B	Baseline, then intervention	Weak (confounded by time)
A–B–A	Baseline, intervention, withdrawal	Stronger if reversal occurs
Multiple baseline across behaviours	Staggered introduction	Strong; no withdrawal needed
Multiple baseline across participants	Staggered across families	Strong; realistic in clinics

Analysis. Visual analysis (level, trend, variability) plus a quantitative index (e.g. Tau-U) and, ideally, systematic replication across cases.

Reporting standard. Follow the SCED reporting guidelines (e.g. SCRIBE), including baseline stability and treatment fidelity.

Week 12 — Mixed methods integration

The integration question. What does the combination tell you that neither strand could alone?

Joint display format.

Quantitative finding	Qualitative finding	Convergence / divergence	Interpretation
Attendance predicted outcome (β = .32)	Families described "being known" by the therapist as decisive	Divergence in mechanism	Attendance may be a proxy for alliance
Drop-out 34%	Families cited transport and work shifts	Convergence	Structural barriers, not motivation

Avoid the two common failures. Reporting two parallel studies with no integration; and using qualitative data merely to illustrate quantitative findings.

> **REPORT (20%)**
> A 2,000-word integration report with at least three joint display rows.

Week 13 — Robustness and pitfalls

Pitfalls you will meet.

Pitfall	What it looks like	Fix
p-hacking	Many tests, reporting the significant ones	Pre-specify; adjust; report all
HARKing	Hypothesis stated after results known	Register the hypothesis
Overfitting	Model too complex for the sample	Simpler model; cross-validation
Ignoring clustering	Treating partners as independent	Multilevel or dyadic models
Selective reporting	Reporting one measure, ignoring another	Report all pre-specified outcomes
Causal language	"Caused" from cross-sectional data	Use associational language

Sensitivity analysis. Run your main analysis again with a different reasonable decision (e.g. excluding an outlier, using a different missing-data approach) and report whether conclusions change. If they do, say so.

Week 14 — Writing, publishing and defending

The findings chapter. Present, do not interpret (that is the discussion's job). Structure by question. Report descriptive statistics before inferential. Use tables for numbers and prose for meaning.

The discussion. What you found → how it compares with the literature → mechanisms → limitations (specific, not generic) → implications for practice and policy → future research.

Defending. Prepare for four questions: (1) Why this design? (2) What are the threats to validity and how did you address them? (3) Why these analyses? (4) What can and cannot be concluded? Rehearse short answers.

Publishing. Choose a journal that publishes practitioner research; follow the author guidelines exactly; expect a rejection and a revision; respond to reviewers point by point.

Glossary

APIM — Actor–Partner Interdependence Model. **Cluster** — a grouping unit (couple, family, therapist) whose members are not independent. **GRADE** — a system for rating the certainty of evidence. **ICC** — intraclass correlation; the share of variance due to clustering. **Measurement invariance** — equivalence of a measure's functioning across groups. **Pre-registration** — public commitment to a study plan before data collection. **SCED** — single-case experimental design. **Sensitivity analysis** — re-analysis under different assumptions to test robustness.

Self-assessment

Multiple choice

1. Treating partners as independent observations primarily affects: (a) external validity (b) standard errors and Type I error (c) reliability (d) sampling

2. The ICC indicates: (a) internal consistency (b) the proportion of variance due to clustering (c) effect size (d) power

3. A multiple baseline design is strong because: (a) it requires withdrawal (b) staggered introduction controls for time (c) it uses large samples (d) it is qualitative

4. Joint displays are used to: (a) present descriptive statistics (b) integrate quantitative and qualitative findings (c) assess bias (d) calculate power

5. Pre-registration mainly reduces: (a) attrition (b) analytical flexibility and p-hacking (c) cost (d) sample size

6. Measurement invariance concerns: (a) whether a measure behaves equivalently across groups (b) internal consistency only (c) sample size (d) randomisation

7. A sensitivity analysis tests: (a) whether conclusions hold under different assumptions (b) the sample's normality (c) the coding frame (d) the ethics

8. HARKing means: (a) hypothesising after results are known (b) random sampling (c) qualitative coding (d) data entry

9. In framework analysis the output is typically: (a) a case × theme matrix (b) a regression table (c) a factor loading matrix (d) a funnel plot

10. The most useful document in a thesis is: (a) the title page (b) a dated analysis log (c) the abstract (d) the reference list

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 a · 7 a · 8 a · 9 a · 10 b

Short answer

1. Explain why clustering matters for standard errors.
2. Distinguish actor and partner effects with an example.
3. List five elements of a systematic review protocol.
4. What is the difference between a sensitivity analysis and a subgroup analysis?
5. Name four threats to validity in a pre-post clinic study.

Essay (choose one, 2,000 words)

1. Defend (or attack) the claim that randomised trials are the gold standard for family therapy research.
2. Present a design for a dyadic study of criticism and satisfaction, justifying every choice.
3. Critically appraise a systematic review of family interventions, focusing on method and certainty.

Further reading

- Kenny, D. A., Kashy, D. A., & Cook, W. L. (2006). Dyadic Data Analysis. Guilford.
- Hox, J., Moerbeek, M., & van de Schoot, R. (2017). Multilevel Analysis (3rd ed.). Routledge.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Higgins, J. P. T. et al. (Eds.). (2023). Cochrane Handbook for Systematic Reviews of Interventions.
- Kazdin, A. E. (2021). Single-Case Research Designs (3rd ed.). OUP.

Next: MFT 770 — Master Thesis

MSC MARRIAGE AND FAMILY THERAPY

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MFT 722

Professional Development and Self-Care

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MFT 722 — Professional Development and Self-Care

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	**Professional Development and Self-Care**	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is not a wellbeing add-on at the end of training. It is the course that decides whether you are still practising in fifteen years, and whether the practising is any good. It treats the therapist's own life as the primary instrument, and professional development as a discipline rather than an ambition.

How to use this book

Every week has a **professional** task and a **personal** one. Do both. The course is assessed largely on a portfolio, and a portfolio written retrospectively in the final week will be visible and weak. Keep a weekly entry; it takes eight minutes.

1. Course description

The course covers: the person of the therapist; burnout, compassion fatigue and vicarious trauma; boundaries and availability; personal therapy; reflective practice and supervision; continuing professional development; career pathways and private practice fundamentals; money, contracts and pricing; professional identity and belonging; and sustainable practice in resource-limited settings.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess your own burnout and vicarious trauma risk using structured tools	Portfolio
LO2	Design a personal and professional self-care plan that survives a busy month	Plan
LO3	Set and maintain boundaries with clients, colleagues and institutions	Viva
LO4	Plan continuing professional development against a competency framework	CPD plan
LO5	Explain the fundamentals of ethical private practice (contracts, fees, records)	Exam
LO6	Use reflective practice and personal therapy appropriately	Portfolio
LO7	Articulate a five-year professional trajectory	Presentation

3. Assessment

Component	Weight	Due
Reflective portfolio (10 entries)	35%	Fortnightly
Personal and professional sustainability plan (1,500 words)	25%	Week 10
Practice readiness viva (20 min)	25%	Week 13
CPD and career presentation (10 min)	15%	Week 14

4. The fourteen-week plan

Week	Topic	Task
1	The person of the therapist	Therapy autobiography (private)
2	Burnout: what it is and how it develops	Maslach self-assessment
3	Compassion fatigue and vicarious trauma	Symptom inventory
4	Boundaries: time, contact, disclosure, gifts	Boundary audit
5	Personal therapy and its place in training	Decide and plan
6	Reflective practice: models and habits	Weekly reflection structure
7	Supervision as a career-long practice	Supervision plan
8	CPD and competency frameworks	CPD gap analysis
9	Career pathways in MFT	Pathway map
10	**Sustainability plan due**; money, fees and contracts	Fee and contract design
11	Private practice fundamentals	Practice blueprint
12	Professional identity, community and belonging	Professional network plan
13	**Practice readiness viva**	Assessed
14	**CPD and career presentation**	Assessed

Week 1 — The person of the therapist

The premise. You cannot separate the clinician from the person. Your history shapes what you notice, what you avoid, whom you like, and where you lose your balance.

The "therapy autobiography". Write, privately, about: your family of origin; the rules you absorbed; the losses; the roles you played; what you learned about conflict, intimacy, money and authority; and why you chose this work. Do not submit it. Do keep it, and revisit it in three years.

Common motivations and their risks.

Motivation	Strength	Risk
"I want to help people"	Compassion	Rescuing; over-functioning
"I want to understand my family"	Insight	Using clients to solve your own history
"People tell me I'm easy to talk to"	Alliance skill	Difficulty setting boundaries
"I survived something"	Credibility	Over-identifying; avoidance of certain material
"I want status/expertise"	Drive	Distance; difficulty with not-knowing

The professional requirement. Know your history well enough that you can tell the difference between your client's material and your own reaction. Personal therapy and supervision exist for this, not because something is wrong with you.

Week 2 — Burnout

Definition (Maslach): a syndrome of **emotional exhaustion**, **depersonalisation/cynicism**, and **reduced sense of accomplishment**, arising from chronic occupational stress.

The six drivers (Maslach & Leiter) — all of them organisational:

1. Workload
2. Control (or lack of it)
3. Reward (financial, social, intrinsic)
4. Community
5. Fairness
6. Values (mismatch between what you believe and what the system asks)

The clinically important point. Burnout is not a personal failing and cannot be solved by yoga. If the caseload is impossible, the supervision absent and the values compromised, self-care is a distraction from the real problem.

Self-assessment. Use the Maslach Burnout Inventory or the freely available Copenhagen Burnout Inventory. Score yourself honestly; write one paragraph on which of the six drivers is most active in your current situation.

Week 3 — Compassion fatigue and vicarious trauma

Three distinct concepts.

Concept	Definition	Signature
Burnout	Exhaustion from workload	"I am too tired to care"
Compassion fatigue	Reduced empathy and satisfaction from caring	"I used to feel for people; now I don't"
Vicarious trauma	Changes to beliefs and inner world from exposure to others' trauma	"The world is not safe; people are dangerous"

Signs to monitor in yourself. Intrusion of client material; nightmares; avoidance of certain cases; cynicism; irritability at home; numbness; loss of hope; over-involvement; boundary erosion; sleep and appetite change; increased alcohol use.

Protective factors with evidence. Supervision that addresses the emotional impact; caseload diversity; control over scheduling; peer support; meaning-making; sleep and exercise; personal therapy; and organisational support.

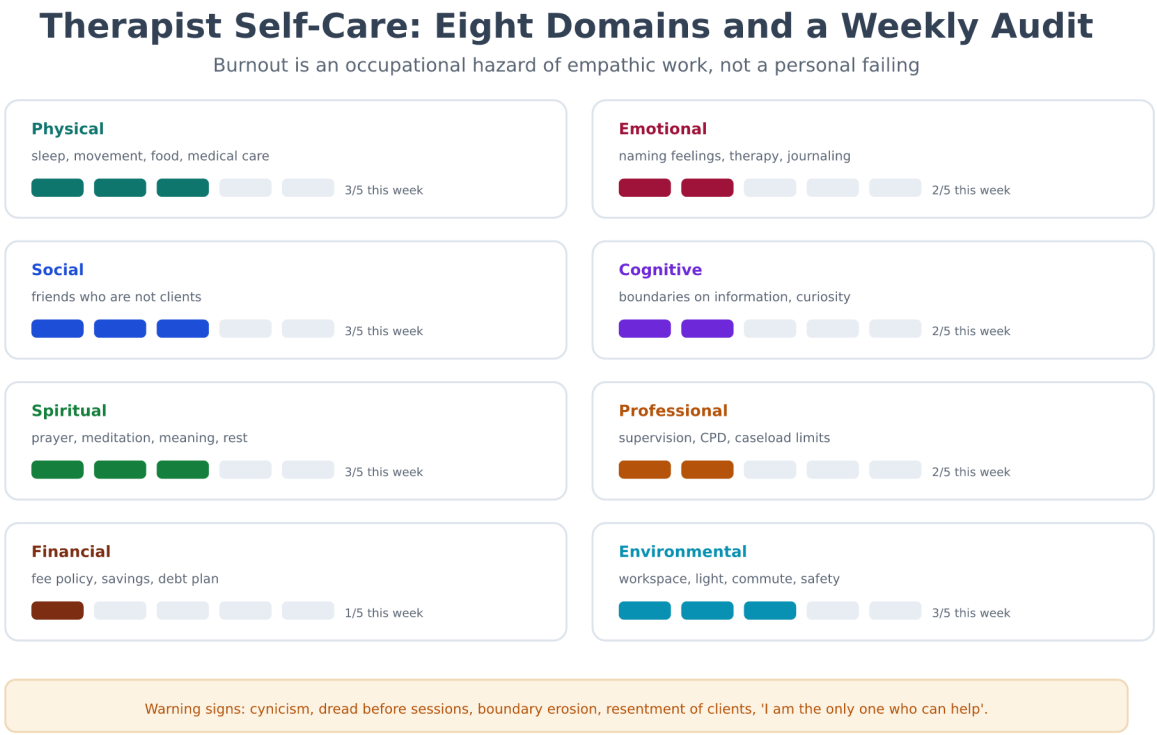


Figure 3.1 — Self-care audit across eight domains

TASK

Complete the eight-domain audit. Rate each domain 0–5 for last week. Identify the lowest two and write one change for each that is small enough to happen on a Tuesday.

Week 4 — Boundaries

The boundaries that matter most in early practice.

Boundary	The trap	A workable standard
Time	Sessions over-run; you arrive early, leave late	Fixed session length; 10 minutes between clients
Contact	Clients text at night; you reply	Written policy; emergency route; no personal number
Disclosure	You share to reassure	Disclose for the client's benefit, rarely, and briefly
Gifts	Refusing feels rude	Accept small cultural gifts; discuss significant ones
Social media	Client requests; you see their posts	Clear policy; never search for clients
Dual roles	Church, community, family ties	Disclose, discuss, refer if unmanageable
Emotional labour	You become the family's friend	Warmth plus frame; endings are explicit

The test. Not "was it kind?" but "whose need did it serve, and would I be comfortable explaining it in supervision?"

Week 5 — Personal therapy

Requirements vary. Many training programmes require a minimum number of personal therapy hours. Treat the requirement as a floor, not a target.

What it is for. Not to become "fixed", but to know your patterns well enough to notice them in the room; to have a place to process work that is not your supervisor's job; and to experience being a client.

What to look for in a therapist. A different orientation from yours (to see another way of working); a recommendation from someone you trust; a clear contract; and someone you can be honest with.

The cost problem. Therapy is expensive, and trainees are often poorly paid. Options: reduced-fee schemes, group therapy, university counselling services, peer consultation groups, and budgeting it as a professional expense rather than a luxury.

Week 6 — Reflective practice

Reflection is a method, not a mood. Use a structure.

The Gibbs cycle, applied to a session: description → feelings → evaluation → analysis → conclusion → action plan.

Six questions to answer weekly.

1. What happened that surprised me?
2. What did I feel, and where in my body?
3. What did I avoid?
4. What did I do that worked?
5. What would I do differently?
6. What do I need to bring to supervision?

Writing. Keep one page per week. Do not write case details in a personal journal that is not secure; use initials or pseudonyms, and never store client-identifying material outside the clinical record.

Week 7 — Supervision as a career-long practice

The norms. One hour of supervision for every five hours of clinical work is the common standard; more for complex cases and for trainees.

How to use supervision well.

- Arrive with an agenda and one clear question.
- Bring the case you least want to bring.
- Say what you felt, not only what you did.
- Ask for observation, not only opinion ("what would you have said?").
- Record what you will do differently, and report back next time.

Peer consultation groups. A complement, not a substitute: 3–5 clinicians, monthly, structured, with a confidentiality agreement and a rotating facilitator.

Week 8 — Continuing professional development

Frameworks. Most professional bodies require annual CPD across domains: clinical practice, supervision, ethics, research, teaching, management. Map your year against the domains rather than collecting certificates.

A CPD plan that works.

1. Identify three gaps from your competency self-assessment.
2. For each, specify an activity, a date, and an evidence of completion.
3. Include one activity that stretches you (a new modality, a research skill, a leadership role).
4. Review every six months; adjust.

Beware CPD theatre. Ten webinars on topics you already know count for nothing. One supervised course in an area of genuine weakness counts for a great deal.

Week 9 — Career pathways

Pathway	What it needs	What it gives	What it costs
Clinical specialist	Depth in a modality; supervision; certification	Expertise, referrals	Narrowing
Public sector clinician	Systems navigation; resilience	Security, breadth	Bureaucracy, heavy caseloads
Private practice	Business skills, capital, network	Autonomy, income potential	Isolation, income volatility
Academic/teaching	Research, publication, teaching	Influence, stability	Bureaucracy, distance from clinic
Supervision/consultation	Experience, accreditation	Leverage, variety	Responsibility for others' practice
NGO/humanitarian	Flexibility, languages, resilience	Meaning, breadth	Insecurity, exposure to trauma
Policy/advocacy	Writing, networks, patience	Scale of impact	Slowness, compromise

The honest question. Which of these fits the life you actually want — including your family, your health, and your finances — not only the career you imagined at 22?

Week 10 — Money, fees and contracts

Fees are clinical decisions. Set them deliberately; publish them; apply them consistently; review annually.

A fee policy covers: the fee; session length; payment method and timing; late cancellation; non-attendance; sliding scale criteria (decided in advance); annual review; and what happens to outstanding balances.

Contracts. Even a one-page agreement prevents most disputes: parties, purpose, confidentiality and limits, fees, cancellation, contact policy, complaints route, and signatures.

The uncomfortable truths.

- Charging is not exploitation; under-charging until you resent clients is.
- A sliding scale with no criteria becomes arbitrary and unfair.
- Chasing unpaid fees is a clinical conversation, not an administrative one.
- You will need to budget for tax, insurance, supervision, CPD and sick leave.

Week 11 — Private practice fundamentals

The blueprint. Location and room (privacy, sound, waiting area, accessibility); legal structure and registration; professional indemnity insurance; record-keeping and data protection; booking and payment systems; referral network; risk and emergency protocols; supervision arrangement; and a marketing approach that is ethical and honest.

Starting safely. Keep a part-time employed post while you build; take a business course or find a mentor; decide your niche only after 50 cases, not before; and never practise without supervision, however experienced you are.

Isolation is the occupational hazard. Build a peer group before you need one.

Week 12 — Professional identity and belonging

Belonging matters. Clinicians who are connected to a professional community report less burnout, better decision-making, and greater persistence.

How to build it. Join a professional association; attend one conference a year; contribute to a peer group; teach one workshop; write one short piece; mentor one junior colleague.

Identity in a contested field. Family therapy sits between psychology, psychiatry, social work and counselling. Be clear about who you are and what you offer, without disparaging

others: "I am a systemic therapist; my expertise is in relationships. I work alongside your psychiatrist, not instead of them."

Week 13 — Practice readiness viva

Twenty minutes. Questions drawn from: boundary scenarios, burnout scenarios, ethical dilemmas, fee conversations, supervision difficulties, and career planning. Assessed on judgement, honesty, and the ability to name your own limits.

Week 14 — CPD and career presentation

Ten minutes on where you are, where you are going, what you need, and what you will stop doing. Include one commitment to your own sustainability that is specific, dated, and verifiable.

Glossary

Burnout — exhaustion, cynicism and reduced efficacy from chronic occupational stress. **Compassion fatigue** — reduced empathy and satisfaction resulting from prolonged caring. **Compassion satisfaction** — the positive counterpart: meaning and pleasure derived from helping. **CPD** — continuing professional development. **Reflective practice** — structured examination of one's own work and reactions. **Vicarious trauma** — transformation of the therapist's inner world through exposure to others' trauma.

Self-assessment

Multiple choice

1. Burnout is primarily caused by: (a) personal weakness (b) chronic occupational stressors (c) too much supervision (d) client resistance

2. Vicarious trauma is characterised by: (a) tiredness (b) changes to beliefs and inner world (c) low fees (d) poor timekeeping

3. A workable contact boundary is: (a) giving your personal number (b) a written policy with an emergency route (c) replying at night (d) no policy

4. Personal therapy in training is best understood as: (a) a punishment (b) a professional requirement and an investment (c) unnecessary (d) a substitute for supervision

5. The recommended supervision ratio is roughly: (a) 1 hour per 20 client hours (b) 1 hour per 5 client hours (c) 1 hour per year (d) no standard

6. A useful CPD plan starts from: (a) available webinars (b) a competency gap analysis (c) cheap courses (d) colleagues' choices

7. Fee policies should be: (a) improvised (b) written, published and applied consistently (c) secret (d) identical everywhere

8. Peer consultation groups: (a) replace supervision (b) complement supervision (c) are unnecessary (d) are illegal

9. Reflection is best done: (a) once a year (b) weekly with a structure (c) only in crisis (d) verbally only

10. Private practice should ideally begin: (a) with no supervision (b) alongside employed work with supervision in place (c) with maximum debt (d) without insurance

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the three dimensions of burnout and the six organisational drivers.
2. Distinguish burnout, compassion fatigue and vicarious trauma.
3. List five boundaries and the standard you will use for each.
4. What are the six questions of a weekly reflection?
5. Outline the components of a fee policy.

Essay (choose one, 1,500 words)

1. "Self-care is a professional obligation, not a luxury." Discuss with reference to the evidence on burnout.
2. Design a sustainable practice model for a newly qualified family therapist in a low-resource setting.
3. Analyse your own family-of-origin patterns and their likely effect on your clinical work.

Further reading

- Norcross, J. C., & Guy, J. D. (2019). Leaving It at the Office (2nd ed.). Guilford.
- Figley, C. R. (Ed.). (1995). Compassion Fatigue. Brunner/Mazel.
- Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience. World Psychiatry, 15(2), 103–111.
- Skovholt, T. M., & Trotter-Mathison, M. (2014). The Resilient Practitioner (3rd ed.). Routledge.
- Bolton, G. (2018). Reflective Practice (5th ed.). Sage.

Next: MFT 723 — Special Populations in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 24 OF 28

MFT 723

Special Populations in Therapy

ELECTIVES

1.71 units · Prerequisite: MFT 708, 714

> “"Special populations" is a compromise term.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 723 — Special Populations in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 708, 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	**Special Populations in Therapy**	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. "Special populations" is a compromise term. It names groups whom services tend to miss, misread or harm — and it trains the clinician to adapt the frame without lowering the standard of care.

How to use this book

Each week takes one population and works through four questions: what is the specific stressor, what does the evidence say helps, what must be adapted, and what should I not do? The aim is competence in adaptation, not expertise in every group.

1. Course description

The course covers families affected by forced displacement; LGBTQ+ clients and their families; people with disabilities and chronic illness; older adults; families involved with the criminal justice system; veterans and uniformed services; single-parent and grandparent-headed households; childless and infertile couples; families affected by HIV; and street-connected children.

Each carries a specific combination of stigma, legal precarity, service exclusion and clinical complexity that requires a specific adaptation of the systemic frame.

2. Learning outcomes

#	Outcome	Assessed
LO1	Identify the specific stressors and strengths of each population	Exam
LO2	Adapt assessment and intervention without diluting standards	Case study
LO3	Recognise and interrupt the ways services exclude these groups	Reflective portfolio
LO4	Work with interpreters, advocates and community organisations	Skills test
LO5	Address legal, safeguarding and ethical complexities	Viva
LO6	Evaluate the evidence for culturally adapted interventions	Essay
LO7	Design an accessible service pathway	Proposal

3. Assessment

Component	Weight	Due
Case study with adaptation plan (2,500 words)	40%	Week 9
Service pathway proposal (1,500 words)	25%	Week 12
Viva on legal and ethical complexity (20 min)	15%	Week 13
Examination	20%	Week 14

4. The fourteen-week plan

Week	Population	Adaptation focus
1	Families affected by forced displacement	Safety, loss, limbo
2	Refugees and asylum-seeking families	Legal precarity and the "second trauma"
3	LGBTQ+ clients and their families	Risk, disclosure, acceptance
4	Families with a member with a disability	Access and the social model
5	Chronic illness and palliative care families	Illness as a family member
6	Older adults and their families	Ageing, loss, elder abuse
7	Criminal justice involvement and imprisonment	Absence, stigma, re-entry
8	Families of uniformed services	Deployment, injury, silence
9	**Case study due**; single-parent and grandparent-headed households	Structure and load
10	Infertility, childlessness and reproductive loss	Stigma, grief, gender
11	Families affected by HIV	Stigma, secrecy, transmission anxiety
12	**Pathway proposal due**; street-connected children and youth	Safety, trust, systems
13	**Viva**; intersectionality and service design	Integrated planning
14	Integration and examination	Review

Week 1 — Forced displacement

The specific stressors. Loss (home, status, language, network, identity); prolonged uncertainty; trauma before, during and after flight; family separation; role reversal; and the "limbo" of waiting for legal status.

The clinical tasks. Safety and basic needs first; normalising distress without pathologising; restoring roles; supporting the family's own rituals and faith; and addressing the grief of the life left behind — often unmourned because the family is still in survival mode.

Adaptation.

- Do not require a full trauma narrative in early sessions.
- Use practical problem-solving alongside emotional work; both are therapeutic.
- Involve community and faith organisations.
- Ask about the family members who are missing — literally and emotionally.

Week 2 — Asylum-seeking families

The "second trauma" is the legal process: interviews, credibility assessments, detention, dispersal, uncertainty, and the requirement to tell the worst story repeatedly to strangers.

Clinical realities.

- Status anxiety dominates every decision: whether to report violence, whether to seek medical care, whether to disclose sexuality.
- Children may be the family's cultural and linguistic bridge.
- Court and immigration reports may be requested; the report and the therapy must be kept distinct.

What helps. Concrete assistance with documentation and access; clear, honest explanation of what you will write and to whom; liaison with legal representatives (with consent); and a therapeutic space where the person is not required to justify their deserving.

Week 3 — LGBTQ+ clients and their families

Minority stress. Prejudice, expectation of rejection, internalised stigma and concealment produce chronic stress and elevated rates of anxiety, depression, self-harm and suicide.

Family work. Families often present in crisis after disclosure, with religious and cultural conflict. The work is to: create safety for the young person; hear the parents' fear without endorsing rejection; slow the timetable (rejection, like acceptance, is a process); and set limits on harm — verbal abuse, expulsion, forced "corrective" interventions.

What not to do.

- Do not promise to change the young person, or to change the family.
- Do not require the client to come out as a condition of care.
- Do not collude with secrecy that is unsafe, or with disclosure that is premature.
- Do not refer to "the lifestyle"; use the client's language.

The clinical stance. You are not there to adjudicate theology or politics. You are there to reduce harm, improve relationships where possible, and support the person's dignity.

Week 4 — Disability

The two models. Medical (the body is the problem) and social (society's barriers are the problem). Clinically, both matter.

What families need.

- Accurate information about the condition and its trajectory.
- Help with the burden of care and the sibling experience.
- Support for the parental relationship, which is often absorbed by the caregiving.

- Advocacy with schools, employers and health services.
- Attention to the disabled person's own voice and autonomy, which families often override in the name of protection.

Adaptation. Accessible rooms; longer sessions; communication support; written materials in accessible formats; and a willingness to conduct sessions in the home when necessary.

Week 5 — Chronic illness and palliative care

The illness as a third member of the family. It takes a role, demands attention, reorganises routines and relationships.

Clinical tasks. Rebalance the couple relationship when one partner becomes a carer; address the children's understanding and fears; manage the family's beliefs about illness and fate; support decision-making about treatment; and prepare for death without abandoning hope.

Palliative and end-of-life work. Advance care planning; legacies; unfinished business; the family's conflicts about treatment; and bereavement support afterwards.

The clinician's own mortality. This work touches your own fears. Supervision is essential.

Week 6 — Older adults

Specific stressors. Retirement, bereavement, physical decline, loss of role, relocation, dependence, and the assumption by others that life is over.

Clinical presentations. Depression misdiagnosed as ageing; anxiety; cognitive change (which must be differentiated from delirium, depression, hearing loss and dementia); family conflict about care; elder abuse; and grief.

Adaptation. Slower pace; larger print; hearing support; sessions at home; involving the family in practical decisions; and taking the older person's goals as primary, not the family's convenience.

Elder abuse. Ask directly and separately: "Has anyone hurt you, taken your money, or stopped you seeing people?" Know your safeguarding route.

Week 7 — Criminal justice involvement

The absence. Imprisonment removes a person from the household while keeping them psychologically present. Children are often told a story; partners carry shame and practical burden.

Clinical tasks. Support the remaining household; address the children's understanding and stigma; prepare for release and re-entry; manage the couple's renegotiation; and address the family's history (which may include the behaviour that led to imprisonment).

Re-entry. The returning member often expects to resume an old role, while the family has reorganised without them. Naming this mismatch early prevents a second rupture.

Adaptation. Sessions in prison settings; rules about contact; careful confidentiality (the prison is not a confidential setting); and coordination with probation or social services.

Week 8 — Uniformed services

The specific culture. Stoicism, mission focus, hierarchy, and a strong norm against help-seeking.

Clinical presentations. Deployment and reintegration; moral injury; PTSD; substance use; domestic violence; and the family's secondary traumatisation.

Adaptation. Frame treatment in terms the culture respects (performance, mission readiness, the family's stability); involve the unit where appropriate; and address the couple's shift in roles during absence and return.

Week 9 — Single-parent and grandparent-headed households

The specific stressors. Role overload; financial strain; the absent parent's shadow; stigma; and children's questions.

Strengths. Many single-parent households function very well; treat the structure as a structure, not a deficit.

Grandparent-headed households. Grief (for the parent, often due to death or incapacity); the generation gap; legal status (custody, maintenance); health limitations; and the children's loyalty conflicts.

Adaptation. Practical support first; a clear parenting plan; support for the child's contact with the absent parent where safe; and attention to the carer's own health.

Week 10 — Infertility and childlessness

The cultural weight. In many settings, childlessness carries severe social stigma, blamed disproportionately on women, with consequences for marriage stability and family standing.

Clinical themes. Grief (often disenfranchised); shame; blame and the couple's conflict; pressure from extended family; religious meaning; treatment decision-making and its cost; and the couple's sexual relationship, which becomes medicalised.

The clinical stance. Do not assume that parenthood is the goal for every couple. Ask what a good life looks like for them. Name the social pressure as external, not personal failure.

Week 11 — Families affected by HIV

The clinical realities. Stigma and secrecy; disclosure decisions (to partner, children, family, church); medication adherence; transmission anxiety; bereavement and orphanhood; and the intersection with poverty and gender inequality.

Disclosure work. Help the person decide who, when and how; rehearse; prepare for different responses; and ensure the children's questions are answered at their developmental level.

Adaptation. Confidentiality is paramount in small communities; coordinate with HIV services carefully; address the family's beliefs about illness and blame; and support the "worried well" partner as well.

Week 12 — Street-connected children and youth

The specific realities. Survival needs first (food, safety, shelter); mistrust of adults and institutions; peer networks that function as family; substance use; exposure to violence and exploitation; and legal vulnerability.

What helps. Outreach and continuity; a trusted adult who does not disappear; harm reduction; family reunification work only where it is safe; and collaboration with community organisations, not only clinics.

What harms. Coercive removal; moralising; short-term projects that end abruptly; and treating the peer group as pathology when it is often the only protection available.

Week 13 — Intersectionality and service design

The design questions.

1. Who does not come to our service, and why?
2. What are the access barriers — cost, transport, language, hours, stigma, documentation, disability?
3. Who is the first point of contact, and are they trained?
4. What does the physical environment communicate?
5. What happens when a client needs an interpreter, a wheelchair, a child-friendly space, or a private entrance?
6. How do we measure whether these groups are actually being served?

A pathway that works typically includes: community outreach; a named link worker; flexible appointments; interpreters; transport support; plain-language materials; and a feedback loop with the communities served.

> **PROPOSAL (25%)**
> A service pathway for one under-served population, with access analysis, staffing, and outcome measures.

Week 14 — Integration

The generalizable skills. Ask rather than assume; adapt the frame, not the standard; work with community organisations; attend to legal precarity; protect confidentiality in small communities; and examine your own discomfort.

The generalizable caution. "Special population" can become a way of othering. Every family is specific; the skill is in the specificity, not in the label.

Glossary

Asylum seeker — a person seeking protection who has not yet been granted refugee status. **Disenfranchised grief** — grief that is not socially recognised or supported. **Minority stress** — chronic stress from stigma, prejudice and concealment. **Moral injury** — psychological distress from acting against one's values in service. **Social model of disability** — disability located in societal barriers rather than the body.

Self-assessment

Multiple choice

1. The "second trauma" for asylum seekers refers to: (a) the original event (b) the legal process (c) the journey (d) therapy

2. Family work after a young person's coming out should first: (a) change the young person (b) create safety and hear the parents' fear (c) enforce acceptance (d) require celibacy

3. The social model of disability locates disability in: (a) the body (b) societal barriers (c) genetics (d) the family

4. Elder abuse should be: (a) assumed unlikely (b) asked about directly and separately (c) ignored (d) reported without consent always

5. On release from prison, families commonly: (a) resume old roles easily (b) have reorganised without the returning member (c) refuse contact (d) need no support

6. Childlessness in many settings is: (a) neutral (b) severely stigmatised and blamed on women (c) rare (d) a legal matter

7. For street-connected youth, the first priority is: (a) family therapy (b) survival needs and a trusted adult (c) diagnosis (d) schooling only

8. Minority stress explains elevated rates of: (a) income (b) anxiety, depression and self-harm (c) physical fitness (d) employment

9. Effective pathways for under-served groups begin with: (a) more clinics (b) access analysis and community outreach (c) longer waiting lists (d) more diagnosis

10. The risk of the term "special populations" is: (a) inaccuracy (b) othering and stereotyping (c) legality (d) cost

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name four specific stressors of forced displacement and one clinical task for each.
2. What are three "do nots" in family work after a coming-out disclosure?
3. How does the social model of disability change your assessment?
4. What must a service pathway include for under-served populations?
5. Why is confidentiality harder in small communities, and what do you do about it?

Essay (choose one, 2,000 words)

1. "Adaptation without dilution." Discuss with reference to one special population.
2. Critically evaluate the evidence for culturally adapted family interventions.
3. Design an accessible family therapy service for one excluded group in your context.

Further reading

- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Papadopoulos, R. K. (Ed.). (2002). Therapeutic Care for Refugees. Karnac.
- Laing, L., & Topping, K. (2019). Family Therapy with Older Adults. Routledge.
- Lev, A. I., & Sennott, S. L. (2019). Gender and Sexuality in Family Therapy. Routledge.
- Miller, G. (2011). Management of Alcohol and Drug Problems. (For harm-reduction principles in outreach work.)

Next: MFT 724 — Family Law and Advocacy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 25 OF 28

MFT 724

Family Law and Advocacy

14 teaching weeks · 2 figures

ELECTIVES

1.71 units · Prerequisite: MFT 704

> “Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 724 — Family Law and Advocacy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 704

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How to cite this volume

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Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	**Family Law and Advocacy**	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship. This course gives you the legal literacy to be useful, the boundaries to stay in your lane, and the advocacy skills to change the systems your clients pass through.

How to use this book

This is orientation, not legal advice. Statutes, case law and state practice vary and change. Every week names the questions you should ask a lawyer, not the answers you should give a client. The final weeks turn from individual cases to systemic advocacy.

1. Course description

The course covers: the Nigerian marriage and family law framework (statutory, customary, Islamic and cohabitation); divorce and dissolution; custody, access and maintenance; the welfare principle; child rights and protection law; domestic violence legislation; mediation and alternative dispute resolution; report writing and expert testimony; the therapist's role in court; and advocacy for policy and service change.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the four marriage systems and their dissolution routes	Exam
LO2	Describe the legal framework for custody, maintenance and child protection	Exam
LO3	Distinguish therapy, mediation, expert assessment and legal advice	Viva
LO4	Write a defensible court-facing report within your competence	Report
LO5	Prepare a client for legal processes without becoming an advocate for a version of events	Case study
LO6	Conduct safe mediation or refer appropriately	Skills test
LO7	Design an advocacy campaign or policy proposal	Proposal

3. Assessment

Component	Weight	Due
Legal framework examination	25%	Week 7
Court-facing report (2,000 words)	30%	Week 10
Mediation skills assessment (30 min)	20%	Week 12
Advocacy proposal (1,500 words + presentation)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why family therapists need legal literacy	Role boundary mapping
2	Four marriage systems	Case classification
3	Divorce and dissolution	Route mapping
4	Custody, access and the welfare principle	Custody scenario
5	Maintenance and financial provision	Calculation exercise
6	Child rights and protection law	Reporting scenario
7	**Examination**; domestic violence law	Protection order pathway
8	Mediation and ADR	Mediation demonstration
9	The therapist in court: reports and testimony	Report drafting
10	**Report due**; expert versus treating roles	Role conflict analysis
11	Working with lawyers and courts	Interprofessional simulation
12	**Mediation assessment**	Assessed
13	Advocacy: from case to policy	Campaign design
14	**Advocacy proposal**; integration	Presentations

Week 1 — Why legal literacy matters

Three reasons.

1. **Your clients are inside legal processes.** Divorce, custody, maintenance, protection orders, immigration, inheritance. If you do not know the terrain, you cannot help them navigate it.
2. **You will be asked for opinions you should not give.** "Will I get custody?" is a legal question with a clinical component. Confusing them harms clients.
3. **You may be a witness or a report-writer.** The quality of your documentation determines whether your evidence is useful or damaging.

The boundary map.

Activity	Who does it	Your role
Legal advice	Lawyer	Refer; explain the difference
Mediation	Trained mediator	Conduct only if trained and if safe; otherwise refer
Therapeutic work	You	Your core role
Expert assessment (custody)	Independent expert	Generally not your own client

Activity	Who does it	Your role
Court testimony	Witness	Fact witness; opinion only within expertise
Advocacy	You, as a professional	Legitimate, within your competence and evidence

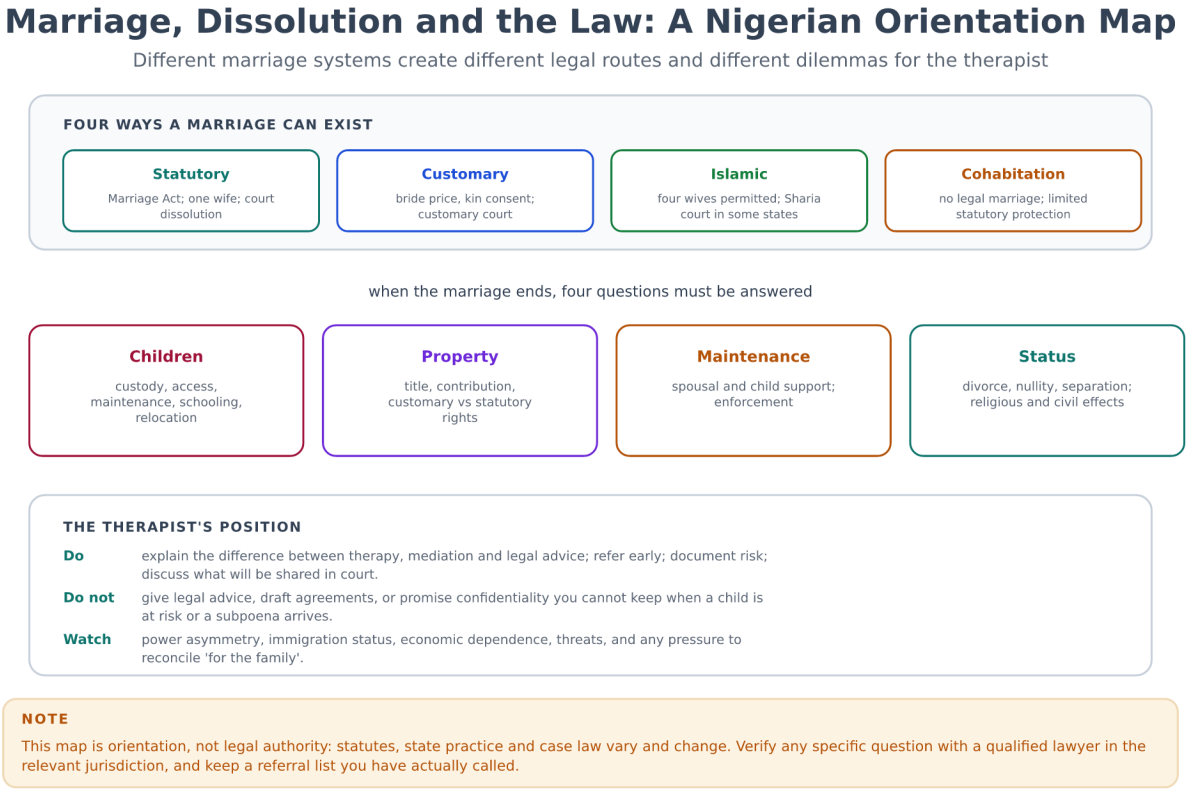


Figure 1.1 — Marriage systems, dissolution questions and the therapist's position

Week 2 — Four marriage systems

System	Legal basis	Form	Dissolution route
Statutory	Marriage Act	Monogamous; registered	High Court petition under the Matrimonial Causes Act
Customary	Customary law; state customary courts	Bride price, kin consent; potentially polygynous	Customary court; customary arbitration
Islamic	Islamic law; Sharia courts in relevant states	Up to four wives	Sharia court; talaq, khul', or judicial dissolution
Cohabitation	No marriage	—	No dissolution; limited statutory protection

Clinical implications of the overlap. A couple may be married under two systems; the customary marriage may not be registered; the religious ceremony may have legal consequences; and children's status may depend on which system applies. Do not assume.

Ask: "How was your marriage formalised? Was it registered? Is there a certificate? Which court would you go to if you needed to?" Then refer to a lawyer for the answer's consequences.

Week 3 — Divorce and dissolution

Statutory divorce (orientation). Governed by the Matrimonial Causes Act: a petition to the High Court; a two-year minimum duration (with exceptions); a recognised ground (adultery, unreasonable behaviour, desertion, separation, or living apart); and provision for custody, maintenance, property and costs.

Customary and Islamic dissolution. Different processes, different courts, different evidentiary rules, different outcomes for women in practice. Know where to refer.

The therapist's role.

- Help the couple/clients clarify what they are deciding (discernment).
- Explain that therapy is not a legal process and does not produce evidence for court.
- Support the emotional work alongside the legal process.
- Refer for legal advice early, not at the point of crisis.
- Warn clients that therapy records may be requested in proceedings.

Week 4 — Custody, access and the welfare principle

The welfare principle. Decisions about children are governed by the child's best interests, not the parents' preferences or the mother's automatic entitlement.

What courts consider (broadly). The child's wishes (weighted by age and understanding); continuity of care; the capacity of each parent; the child's relationship with each parent; siblings; education and health; and any risk of harm.

The therapist's contribution. You can describe the child's relationships, needs and views (carefully, and with appropriate methods); you can support the parents to reduce conflict; and you can help them construct a parenting plan.

What you should not do. Offer a custody recommendation for your own client. That is an expert assessment requiring independence, a defined methodology and often a court appointment. Role conflict invalidates the opinion and harms the family.

The parenting plan. Residence, contact, holidays, schooling, health decisions, communication, transitions, handovers, new partners, and a dispute-resolution clause. Concrete plans reduce conflict.

Week 5 — Maintenance and financial provision

Child maintenance. Both parents have an obligation to maintain their children, under statutory and customary law. Amounts are determined by need, capacity and standard of living; enforcement is a practical problem.

Spousal maintenance. Available in some statutory proceedings; less so in customary and religious frameworks and highly variable in practice.

The therapist's role. Money is clinical material: it carries meaning about fairness, power and abandonment. Help the couple separate the amount from the meaning; encourage full financial disclosure; and refer for legal advice on entitlements.

Practical support. Referral to legal aid, mediation services, and women's rights organisations where appropriate.

Week 6 — Child rights and protection

The Child Rights Act (2003) — adopted in many but not all Nigerian states — sets out the rights of the child, including protection from abuse and neglect, the right to be heard, and provisions on child marriage, child labour, and alternative care. State child protection laws and structures operationalise it.

Your duties.

1. Recognise signs of abuse and neglect.
2. Respond without leading or investigating.
3. Record verbatim.
4. Report through the designated route (state ministry responsible for social welfare; police family unit; designated safeguarding lead).
5. Document the advice received and the action taken.
6. Support the child through the process.

What not to do. Investigate yourself; promise secrecy; confront the alleged abuser; or delay the report while you "gather more information".

Week 7 — Domestic violence law

The Violence Against Persons (Prohibition) Act 2015 applies in the FCT and in states that have adopted it; other states have their own laws or rely on criminal provisions. Protection orders, duties of the police and support for victims vary substantially by jurisdiction.

The clinician's knowledge. Where the nearest police family unit is; what a protection order requires; which shelters and legal aid organisations exist; and what happens procedurally to a woman who reports.

The clinician's limits. You are not the police, the lawyer, or the judge. Your role is safety planning, documentation, referral and support — and not making the client's decisions for her.

Week 8 — Mediation and alternative dispute resolution

What mediation is. A structured, voluntary process in which a neutral third party helps the parties reach their own agreement. It is not therapy and not arbitration.

When it is suitable. Both parties voluntary and safe; no coercive control; reasonably balanced power; an issue that can be negotiated; and willingness to disclose information.

When it is not. Violence or coercion; a severe power imbalance (including literacy, language or economic dependency); untreated substance use or mental illness that prevents participation; and safeguarding concerns.

Structure of a mediation session. Opening and ground rules → each party's account → identification of issues → exploration of interests → generation of options → negotiation → agreement documented → review.

The therapist's temptation. To slip into therapy mid-mediation (exploring feelings) or into advice. Explain the frame at the start and hold it.

Week 9 — Reports and court

The report structure.

1. Your role, qualifications and the basis of your opinion.
2. Who you saw, when, and with whose consent.
3. Methodology (interviews, observation, measures).
4. Findings, described behaviourally.
5. Opinion, clearly separated and limited to your expertise.
6. Limitations, explicitly stated.
7. Recommendations, if requested and within competence.

Language. "It is my opinion that…" not "the court should". No adjectives about character. No speculation about events you did not observe.

Testimony. Answer the question asked; do not volunteer; distinguish fact from opinion from hypothesis; say "I don't know" when you don't; and remain courteous under challenge.

Records. Assume that anything you write may be read in court. Write accordingly.

Week 10 — Expert versus treating roles

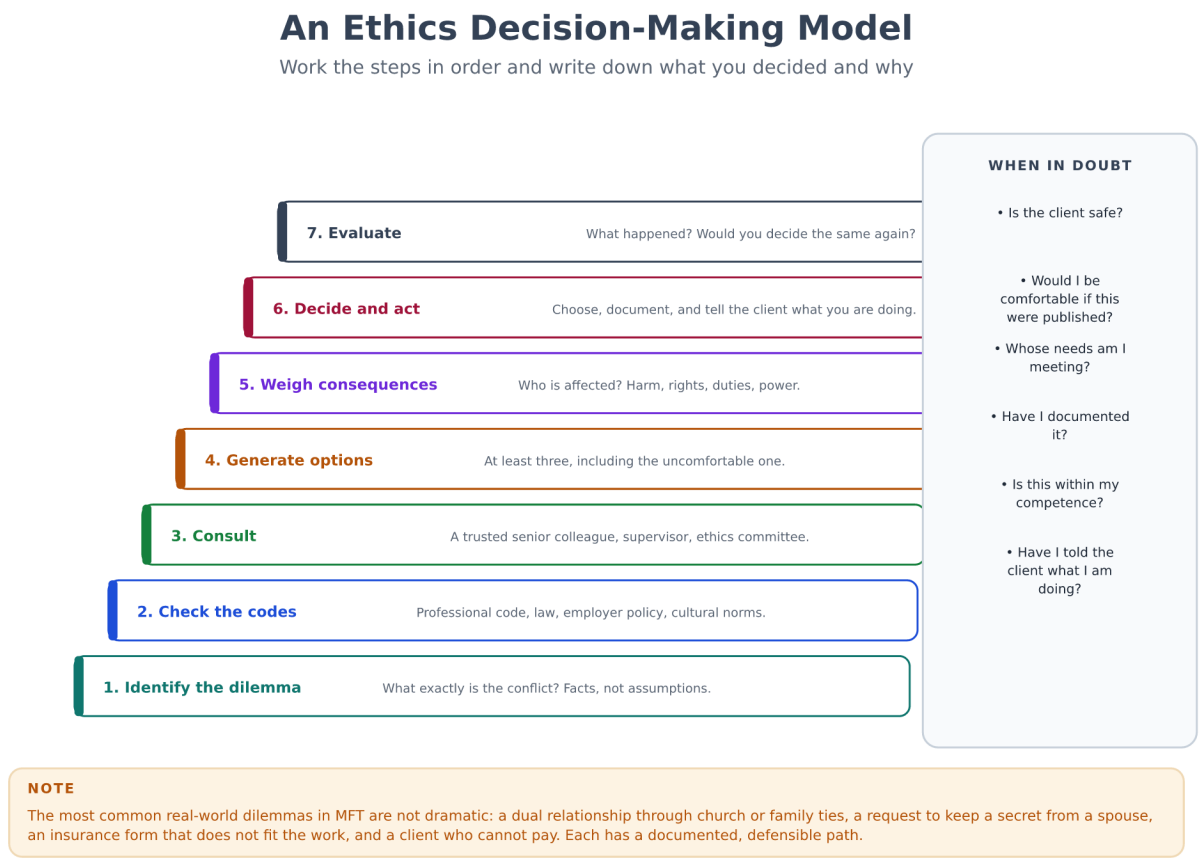


Figure 10.1 — An ethics decision model for role conflicts

Treating clinician	Expert assessor
Paid by/for the client; therapeutic duty	Paid by the court; duty to the court
Confidential	Limited confidentiality; report disclosed
Advocates for the client's wellbeing	Neutral; reports what the evidence shows
Continuous relationship	Time-limited, defined assessment
Cannot be objective about custody	Can be objective if independent

The conflict is structural, not a matter of willpower. Accepting both roles compromises the therapy and the evidence. If asked, explain and refer.

Where you can help. Provide factual records with consent and a court order as required; support the client through the process; and help parents reduce conflict.

Week 11 — Working with lawyers and courts

What lawyers need from you. Clear, dated, factual information; a timely response; an opinion you can defend; and an understanding of your limits.

What you need from them. Clarity about the purpose of the report; the legal questions to address; the deadline and format; and confirmation of consent arrangements.

Interprofessional etiquette. Do not tell a lawyer their job; do not let a lawyer tell you your clinical findings. Negotiate the boundary explicitly and in writing.

Week 12 — Mediation skills assessment

Thirty minutes, role-played. Assessed on: framing and ground rules; neutrality; screening for safety; interest-based exploration; option generation; documentation; and referral decisions.

Week 13 — Advocacy

From case to policy. Individual work reveals systemic failures: no shelter, no legal aid, no interpreter, no protection order enforcement, no follow-up. Advocacy is the professional response.

Levels.

1. **Case advocacy** — with one client: writing a letter, making a call, accompanying to a service (with care).
2. **Service advocacy** — improving your own organisation's pathway.
3. **Policy advocacy** — submissions, research briefs, media, coalitions.
4. **Community advocacy** — working with community organisations and faith groups.

The evidence base for advocacy. Documented case patterns, service data, and research. Anecdote persuades sympathetic listeners; data persuades sceptical ones.

Ethical guardrails. Consent before using a client's story; never identify a client; separate your professional opinion from your personal politics; and do not compromise clinical relationships for a campaign.

Week 14 — Advocacy proposal and integration

Present a 1,500-word proposal: the problem, the evidence, the target, the ask, the allies, the timeline, and how you will know whether it worked. Then, in the final discussion, return to the boundary map from Week 1 and revise it with what you have learned.

Glossary

ADR — alternative dispute resolution. **Coercive control** — a pattern of domination and isolation that makes mediation unsafe. **Guardian ad litem** — a person appointed to represent a child's interests in proceedings. **Maintenance** — financial support owed to a spouse or child. **Matrimonial Causes Act** — the Nigerian statute governing statutory divorce. **Protection order** — a court order restraining a person from specified conduct. **Welfare principle** — the rule that the child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. A therapist asked by a client "will I get custody?" should: (a) predict the outcome (b) explain the welfare principle and refer for legal advice (c) refuse to speak (d) advise on strategy

2. Conducting a custody evaluation of your own client is: (a) acceptable with consent (b) a role conflict (c) required (d) ideal

3. Mediation is contraindicated when: (a) parties disagree (b) there is coercive control (c) there are children (d) money is involved

4. The welfare principle concerns: (a) the parents' rights (b) the child's best interests (c) the court's convenience (d) the family's income

5. A child protection concern should first be: (a) investigated by you (b) recorded and reported through the designated route (c) discussed with the family (d) held until you are certain

6. In mediation the therapist should: (a) explore childhood (b) hold the mediation frame (c) give legal advice (d) take sides

7. An expert report should separate: (a) facts from opinions (b) parents from children (c) fees from findings (d) nothing

8. Advocacy with a client's story requires: (a) no consent (b) informed consent and anonymisation (c) media involvement (d) the client's name

9. The four marriage systems include all except: (a) statutory (b) customary (c) Islamic (d) corporate

10. Therapy records in litigation should be treated as: (a) permanently confidential (b) potentially disclosable (c) irrelevant (d) destroyed immediately

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 a · 8 b · 9 d · 10 b

Short answer

1. Distinguish therapy, mediation, expert assessment and legal advice.
2. What are the grounds for refusing to mediate?
3. Outline a court-facing report structure.
4. List the steps when you suspect child abuse.
5. Give three examples of legitimate professional advocacy.

Essay (choose one, 2,000 words)

1. "The therapist's duty is to the family, the expert's duty is to the court." Analyse the consequences.

2. Critically examine the protection available to victims of domestic violence in your jurisdiction.
3. Design an advocacy campaign to improve one aspect of family justice in your context.

Further reading

- Matrimonial Causes Act (Nigeria), Cap M7 LFN 2004.
- Child Rights Act (Nigeria), 2003.
- Violence Against Persons (Prohibition) Act, 2015.
- Stahl, P. M. (2011). Conducting Child Custody Evaluations (2nd ed.). Sage.
- Emery, R. E. (2012). Renegotiating Family Relationships (2nd ed.). Guilford.
- Barsky, A. E. (2016). Clinicians in Court (2nd ed.). Guilford.

Next: MFT 725 — Integrative Approaches in Therapy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 725

Integrative Approaches in Therapy

14 teaching weeks · 1 figure

ELECTIVES

1.71 units · Prerequisite: MFT 702, 715

> “Most experienced clinicians are integrative, whether or not they say so.”

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MFT 725 — Integrative Approaches in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 702, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	**Integrative Approaches in Therapy**	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Most experienced clinicians are integrative, whether or not they say so. The question is whether the integration is disciplined or accidental. This course builds a principled framework for combining models without producing incoherence, and teaches you to justify every combination.

How to use this book

Integration is a design problem, not a mood. Each week adds a dimension to a framework you will use for the rest of your career. By Week 14 you will have a written integrative framework, tested on three cases, and defended in a viva.

1. Course description

The course covers: the varieties of integration (assimilative, theoretical, sequential, pluralistic); common factors and their implications; the major dimensions of family therapy (systemic, attachment, behavioural, experiential, cognitive, narrative, biological); decision frameworks for matching model to client and phase; feedback-informed treatment; staged and stepped care; and the risks of eclecticism.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish the forms of integration and their assumptions	Exam
LO2	Explain common factors and use them to justify technique selection	Exam
LO3	Construct a coherent integrative formulation for a complex case	Case study
LO4	Sequence models across phases with an explicit rationale	Treatment plan
LO5	Use routine outcome monitoring and feedback to guide integration	Portfolio
LO6	Avoid incoherence and demonstrate coherence to a supervisor	Viva
LO7	Articulate a personal integrative framework	Framework paper

3. Assessment

Component	Weight	Due
Integrative case study (2,500 words)	35%	Week 8
Treatment plan with sequenced models (1,500 words)	25%	Week 11
Outcome monitoring portfolio	15%	Week 13
Framework paper and viva (2,000 words)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why integrate: evidence, clients, and clinicians	Integration audit of your own practice
2	Forms of integration	Mapping four forms onto a case
3	Common factors and their clinical use	Common-factors observation
4	The systemic dimension	Systemic formulation
5	The attachment dimension	Attachment-informed plan
6	The behavioural and cognitive dimensions	Skills module design
7	The experiential and narrative dimensions	Emotion and meaning work
8	**Case study due**; the biological dimension	Medication-aware plan
9	Matching, sequencing and staging	Sequence design
10	Outcome monitoring and feedback-informed treatment	ORS/SRS practice
11	**Treatment plan due**; comorbidity and complexity	Complex case lab
12	Culture, values and the ethics of integration	Values audit
13	**Portfolio due**; coherence and supervision	Coherence defence
14	**Framework paper and viva**	Assessed

Week 1 — Why integrate

Three pressures.

1. **Evidence.** No single model is superior for everything. Different models have stronger evidence for different problems (FBT for adolescent anorexia, EFT for couple distress, PMT for conduct problems, family psychoeducation for psychosis).
2. **Clients.** Real families present with multiple, interacting problems: trauma plus addiction plus couple distress plus a child's school refusal.
3. **Clinicians.** Most experienced therapists drift towards integration as their caseload diversifies.

The danger. Integration without discipline becomes eclecticism: a technique for every mood, no coherent theory of change, and a client who cannot tell what is expected of them.

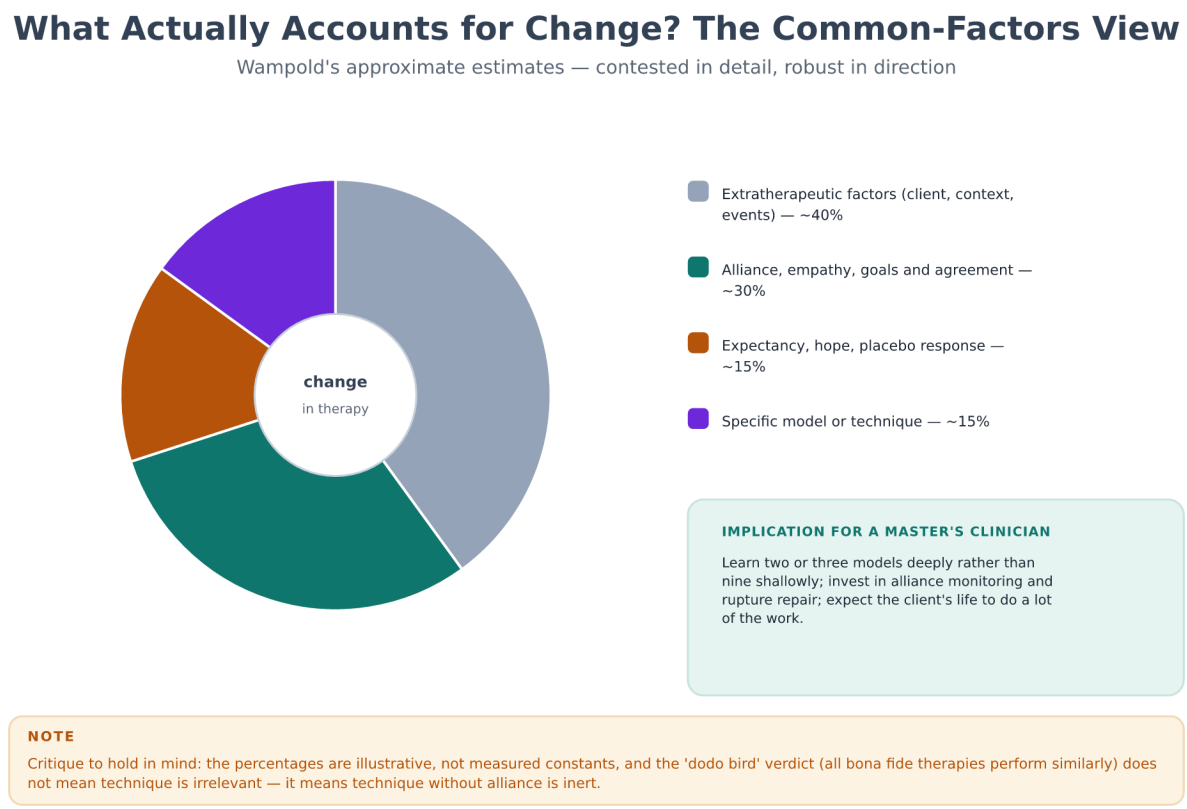


Figure 1.1 — What accounts for change: a common-factors view

> **AUDIT**
> List the models you actually use. For each, name the client problem you use it for and the theory of change you rely on. Identify any technique you use without being able to explain why.

Week 2 — Forms of integration

Form	Definition	Example	Risk
Assimilative	A home model, enriched by techniques from elsewhere	EFT therapist borrowing a behavioural task	Drift from the core theory
Theoretical	A new model synthesising several	Integrative behavioural couples therapy	Complexity, training burden
Sequential	Different models at different phases	Structural stabilisation, then EFT	Discontinuity for the family
Parallel	Different models for different subsystems	Couple work + individual trauma work	Splitting between clinicians
Pluralistic	Collaborating with the client on method	"Which approach fits you?"	Requires an informed client

Choosing a form. Assimilative integration is the most realistic for a newly qualified clinician: it keeps coherence while allowing flexibility. Sequential and parallel integration are appropriate for complexity. Pluralistic practice is an ethical stance as much as a technique.

Week 3 — Common factors

The claim. Much of psychotherapy's effect comes from factors shared across models: the alliance, agreement on goals, therapist empathy, client expectancy, and extratherapeutic factors.

The clinical use — without relativism.

1. **Monitor the alliance** in every session, with every family member.
2. **Repair ruptures** quickly; the repair is often more powerful than the original intervention.
3. **Agree goals explicitly**; disagreement about goals predicts drop-out.
4. **Engender hope** honestly (not false reassurance).
5. **Then use techniques** — which matter most for specific problems and for structuring the work.

The critique. Percentages attributed to common factors are illustrative, not measured; "all models are equal" is over-stated; and technique does matter for specific disorders. The defensible position: the alliance is necessary but not sufficient; technique without alliance is inert; alliance without technique is often insufficient.

Week 4 — The systemic dimension

What it contributes. Attention to pattern, structure, triangles, context and power. It tells you where to look and what maintains the problem.

What it lacks alone. Specific techniques for symptom change; guidance for individual emotion and trauma; and a sufficient account of biology.

Integration in practice. Systemic formulation + any intervention. You can use a behavioural task, an EFT enactment, or a narrative externalising conversation, provided the systemic formulation justifies it.

Week 5 — The attachment dimension

What it contributes. A developmental account of why bonds matter; a map of strategies under threat; a mechanism for change (corrective emotional experience, enacted in the room).

Integration with systemic work. Systemic thinking tells you the loop; attachment tells you why the loop matters so much and what each person is asking for.

Integration with behavioural work. Behavioural tasks build positive exchange; attachment work deepens the meaning of the exchange. Alone, either can be shallow.

Week 6 — The behavioural and cognitive dimensions

What they contribute. Specific, testable, teachable skills; measurable change; homework; structure; and a strong evidence base for many problems.

Integration cautions. Behavioural work can feel mechanical when the emotional bond is damaged; cognitive restructuring can be premature in acute trauma; and skills practice without meaning can become performance.

A practical sequence. Stabilise and structure (behavioural) → access emotion and meaning (experiential, attachment) → consolidate with skills and plans (behavioural, cognitive).

Week 7 — The experiential and narrative dimensions

What they contribute. Access to primary emotion; meaning-making; identity; and the language of the family's own story.

Integration. Experiential work deepens the emotional field; narrative work organises it into a story the family can hold. Together they answer "what do we feel?" and "what does it mean?".

Caution. Emotional work without containment can flood; narrative work without emotion can become intellectual. The integration is in the pacing.

Week 8 — The biological dimension

What it contributes. Sleep, medication, substances, hormonal change, illness, and the physiology of stress and trauma.

Integration. A formulation that ignores biology will fail in predictable ways: the depressed client who does not respond to therapy because of untreated hypothyroidism; the couple whose sexual problem is a medication side effect; the adolescent whose sleep deprivation mimics ADHD.

Your role. Screen, refer, coordinate, and never advise on medication.

Week 9 — Matching, sequencing and staging

Decision framework — five questions.

1. **What is the most dangerous thing here?** (Risk governs sequence.)
2. **What is the most changeable thing here?** (Start where change is possible; hope is built on early wins.)

3. **What must be in place before other work can happen?** (Safety, stabilisation, attendance, sobriety.)
4. **Which model has the best evidence for the primary problem?** (Use it, or refer.)
5. **What does this family want and believe in?** (Engagement is a precondition, not a luxury.)

Staging example. A couple with active violence, alcohol misuse, and a child's school refusal: Phase 1 — safety and sobriety (individual, specialist referral); Phase 2 — couple de-escalation and parenting plan; Phase 3 — child's return to school and family consolidation.

Week 10 — Feedback-informed treatment

The instruments. The Outcome Rating Scale (ORS) and Session Rating Scale (SRS) are brief, free, and practical: four visual analogue items each, completed every session.

How to use them.

1. Introduce them as a way of making sure the therapy is working.
2. Look at the scores before you speak.
3. Ask about a drop: "Your score fell by three points. What happened?"
4. Ask about the alliance: "Is there anything about how we work that is not fitting?"
5. Act on the answer in the session, not next month.

The evidence. Routine outcome monitoring with feedback improves outcomes and reduces deterioration, particularly for clients who are not progressing. It is one of the most robust findings in the field.

> **PORTFOLIO (15%)**
> Ten sessions with ORS/SRS data, your interpretation, and the changes you made.

Week 11 — Comorbidity and complexity

Complexity is the norm. Typical community caseloads include trauma plus substance use plus couple distress plus child behaviour plus financial strain.

Principles.

1. Formulate the whole system, not a list of diagnoses.
2. Identify the keystone: the one change that would make others possible.
3. Sequence: safety → stabilisation → skills → processing → consolidation.
4. Share the load: refer where specialist input is needed; coordinate rather than compete.
5. Review: complexity requires more frequent review, not longer sessions.

Week 12 — Culture, values and the ethics of integration

Culture shapes which models fit. An approach that prizes emotional expressiveness may feel exposing; one that prizes autonomy may feel cold; one that requires homework may be impractical.

Values shape technique choice. Your beliefs about gender, family duty, forgiveness and individual autonomy will show up in what you emphasise. Audit them.

Ethical integration. Do not use a technique you are not trained in; do not combine models to appear sophisticated; and do not use integration as a reason to avoid referring.

Week 13 — Coherence

The coherence test.

Question	If the answer is no…
Can I state the formulation in three sentences?	You do not have one yet
Does every intervention follow from the formulation?	You are improvising
Can I name the model I am using at this moment?	You are drifting
Can I predict what will happen next, and be wrong productively?	Your formulation is too vague
Would a supervisor understand my reasoning without me explaining it?	Document it

The coherence defence. Present a case to peers and answer: what are you doing, why, and how will you know whether it worked?

> **COHERENCE DEFENCE**
> Ten minutes with a case, followed by hostile questioning from peers.

Week 14 — Your integrative framework

Write 2,000 words:

1. Your home model and its theory of change.
2. The dimensions you integrate (systemic, attachment, behavioural, experiential, narrative, biological).
3. Your decision rules for matching and sequencing.
4. Your use of feedback and outcome data.
5. Your limits: what you do not do, and to whom you refer.
6. How you will know you are drifting.

Defend it in a viva.

Glossary

Assimilative integration — a home model enriched by techniques from elsewhere. **Common factors** — mechanisms shared across therapies (alliance, empathy, expectancy, goals). **Feedback-informed treatment** — using session-by-session outcome and alliance data to adjust therapy. **Keystone change** — a change that makes other changes possible. **Pluralistic practice** — collaborating with the client to choose methods. **Sequential integration** — using different models at different phases of treatment. **Stepped care** — matching intensity of intervention to need.

Self-assessment

Multiple choice

1. The main risk of undisciplined integration is: (a) cost (b) incoherence for the family (c) slow progress (d) paperwork

2. Assimilative integration means: (a) equal blending (b) a home model enriched by borrowed techniques (c) alternating models by session (d) refusing a model

3. Common factors research suggests: (a) technique is irrelevant (b) alliance matters greatly and technique matters for specific problems (c) all models are identical (d) diagnosis is useless

4. The first question in sequencing is: (a) what is most interesting (b) what is most dangerous (c) what is cheapest (d) what the therapist prefers

5. Feedback-informed treatment uses: (a) long questionnaires (b) brief session-by-session measures such as ORS/SRS (c) only diagnosis (d) supervisor ratings

6. A keystone change is: (a) the first session (b) a change that makes others possible (c) a diagnosis (d) a referral

7. The dimensional approach includes all except: (a) systemic (b) attachment (c) behavioural (d) astrological

8. Integration is unethical when it involves: (a) borrowing a technique (b) using techniques outside your competence (c) sequencing models (d) monitoring outcomes

9. Coherence is demonstrated by: (a) using many techniques (b) a formulation from which interventions follow (c) longer sessions (d) confidence

10. Complexity should lead to: (a) longer sessions (b) more frequent review and coordination (c) more diagnoses (d) abandoning formulation

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 d · 8 b · 9 b · 10 b

Short answer

1. Distinguish four forms of integration with an example of each.

2. Give three clinical uses of common-factors research.
3. State the five sequencing questions.
4. How do you use ORS/SRS data in a session?
5. What are three ethical limits on integration?

Essay (choose one, 2,000 words)

1. "Integration without a theory of change is improvisation." Discuss with a case.
2. Design an integrative treatment plan for a family with trauma, addiction and couple distress.
3. Critically evaluate feedback-informed treatment as a professional standard.

Further reading

- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Duncan, B. L., Miller, S. D., & Sparks, J. A. (2004). The Heroic Client. Jossey-Bass.
- Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). Handbook of Psychotherapy Integration (3rd ed.). OUP.

Next: MFT 726 — Neurobiology of Relationships

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 27 OF 28

MFT 726

Neurobiology of Relationships

14 teaching weeks · 4 figures

ELECTIVES

1.71 units · Prerequisite: MFT 714

> “Neuroscience does not replace systemic thinking; it supplies the mechanism.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 726 — Neurobiology of Relationships

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

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How to cite this volume

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A note on the case material

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The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
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MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	**Neurobiology of Relationships**	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Neuroscience does not replace systemic thinking; it supplies the mechanism. This course teaches what is well established, what is contested, and what is oversold — so that you can use physiology to help families regulate, without pretending to be a neuroscientist.

How to use this book

The field is full of confident claims that outrun the evidence. Each week distinguishes three levels: **robust** (well replicated), **plausible** (reasonable but uncertain), and **popular** (widely repeated, weakly supported). You are expected to be able to say which is which.

1. Course description

The course covers the autonomic nervous system and co-regulation; the social brain; stress physiology and the HPA axis; attachment as a biological system; memory and trauma; the physiology of couple conflict; sleep, exercise and substances; adolescent and ageing brains; neuroplasticity; and the responsible use of neuroscience in clinical communication.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the autonomic, endocrine and central mechanisms relevant to relationships	Exam
LO2	Use physiological knowledge to pace and structure sessions	Observed session
LO3	Teach regulation skills accurately and without over-claiming	Skills test
LO4	Explain the physiology of couple conflict and repair	Case study
LO5	Distinguish robust from popular claims in the literature	Critical appraisal
LO6	Integrate physiological and systemic formulations	Case study
LO7	Communicate neuroscience to families in accessible language	Viva

3. Assessment

Component	Weight	Due
Critical appraisal of a "neuro" claim (1,500 words)	20%	Week 5
Regulation skills teaching (20 min)	25%	Week 8
Integrative case study (2,500 words)	35%	Week 12
Examination	20%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why physiology belongs in family therapy	Physiology in a case
2	The autonomic nervous system	Vagal braking demonstration
3	Co-regulation and the dyad	Co-regulation observation
4	Polyvagal theory: use and critique	Ladder mapping
5	**Appraisal due**; stress and the HPA axis	Stress mapping
6	Attachment as biology	Attachment formulation
7	The social brain	Neuroanatomy for clinicians
8	**Regulation skills teaching**	Assessed
9	Memory, trauma and the body	Trauma-informed pacing
10	Couple conflict physiology	Flooding and repair
11	Sleep, exercise, substances and mood	Lifestyle screening
12	**Case study due**; adolescent and ageing brains	Developmental adaptation
13	Neuroplasticity and the limits of the claim	Critical reading
14	Integration and examination	Review

Week 1 — Why physiology belongs in family therapy

Four clinical facts that physiology explains.

1. Why a person cannot "just calm down" mid-argument (autonomic arousal; prefrontal resources offline).
2. Why a parent's calm voice regulates a child (co-regulation before self-regulation).
3. Why the same conversation is possible at 10 a.m. and impossible at 11 p.m. (fatigue, cortisol, blood sugar).
4. Why trauma changes memory and reaction (implicit memory, sensitised threat detection).

What physiology does not explain. Meaning, culture, power, loyalty and choice. A physiological account is a mechanism, not a formulation. It tells you how the system works; it does not tell you what the family's behaviour means.

The rule of use. Use physiology to (a) pace the session, (b) teach regulation, (c) reduce shame ("your body is doing its job"), and (d) justify referral. Do not use it to explain away relational responsibility.

Week 2 — The autonomic nervous system

Two branches, three states (simplified).

- **Sympathetic** — mobilisation: heart rate up, pupils dilate, digestion down, muscles tense. Fight or flight.
- **Parasympathetic** — rest and digestion, plus (via the ventral vagal pathway) social engagement.
- **Dorsal vagal shutdown** — immobilisation: numbness, collapse, dissociation.

Robust findings. The autonomic system responds rapidly and largely outside awareness; arousal affects cognition; chronic stress alters baseline functioning; breathing and movement can shift arousal.

Plausible but contested. The precise mapping of "ventral vagal" states onto social behaviour is a theoretical proposal (Porges) with partial empirical support; the popular version often overstates what is known.

Clinical translation. Teach simple, reliable regulation: slower exhale, orienting, movement, grounding, cool water. These work regardless of which theory explains them.

Week 3 — Co-regulation

The concept. Regulation develops in relationships: a calm adult's voice, face, breathing and touch regulate an infant's arousal before the infant can regulate alone.

Robust. Infants synchronise with caregivers on multiple channels; caregiver depression and unresponsiveness affect infant physiology and behaviour; parental calm reduces child distress in experimental settings.

Clinical use.

1. In session, your own regulated state is an intervention: slow your speech, lower your volume, breathe out slowly.
2. Teach parents to co-regulate before reasoning: "First calm the body, then talk about the behaviour."
3. In couple work, teach partners to be a regulating presence rather than a stimulus.

The asymmetry to remember. Co-regulation is not always available. A dysregulated parent cannot lend a calm nervous system. Support the adult first.

Week 4 — Polyvagal theory: use and critique

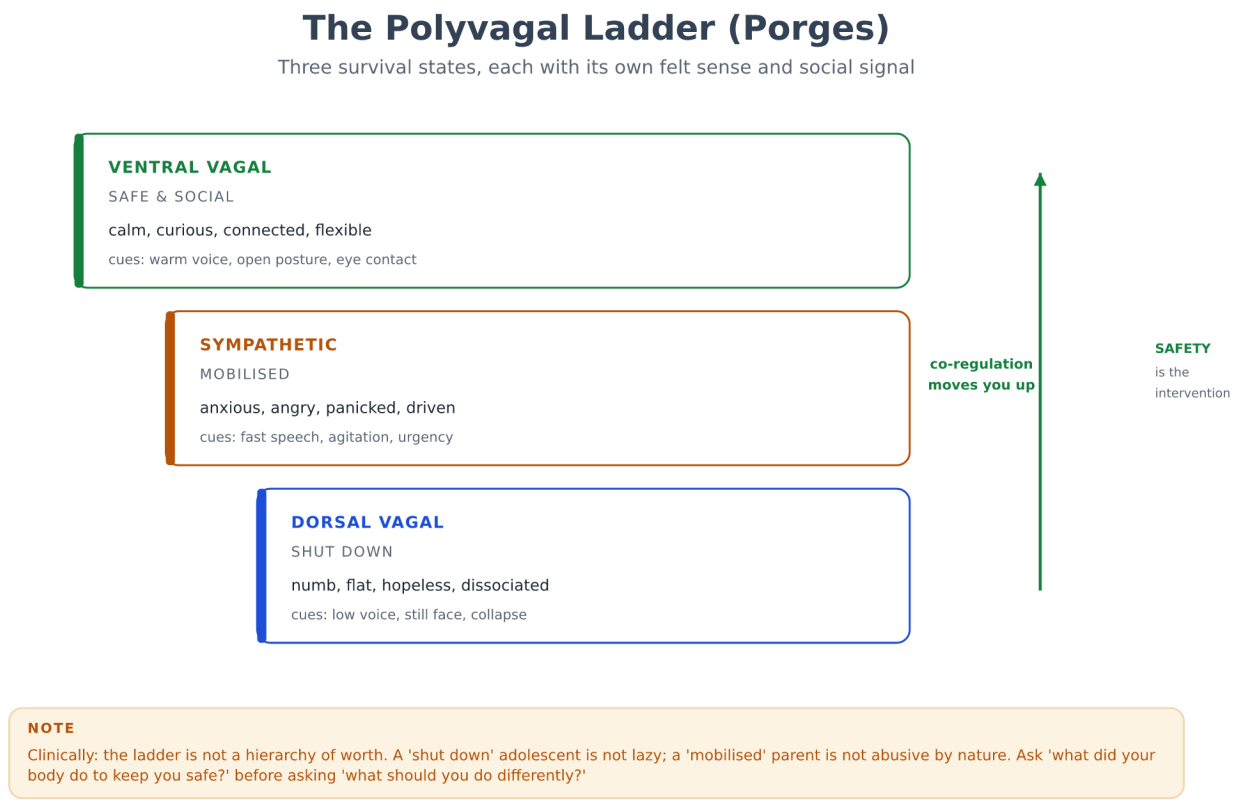


Figure 4.1 — The polyvagal ladder

What the theory claims. Three phylogenetically ordered states — ventral vagal (safe/social), sympathetic (mobilised), dorsal vagal (shutdown) — with the ventral vagal system enabling social engagement via myelinated pathways.

What is useful clinically. The ladder metaphor helps clients recognise state; "neuroception" (unconscious threat detection) explains reactions that seem disproportionate; and the emphasis on safety and co-regulation is good practice.

What is contested. The evolutionary claims, the strict correspondence between anatomy and behaviour, and the tendency to treat the three states as discrete and universal. Critics note the evidence is thinner than the popularity suggests.

The professional stance. Use the metaphor, label it as a metaphor, and do not present contested claims as established fact. Clients benefit from the map, not from a lecture on vagal anatomy.

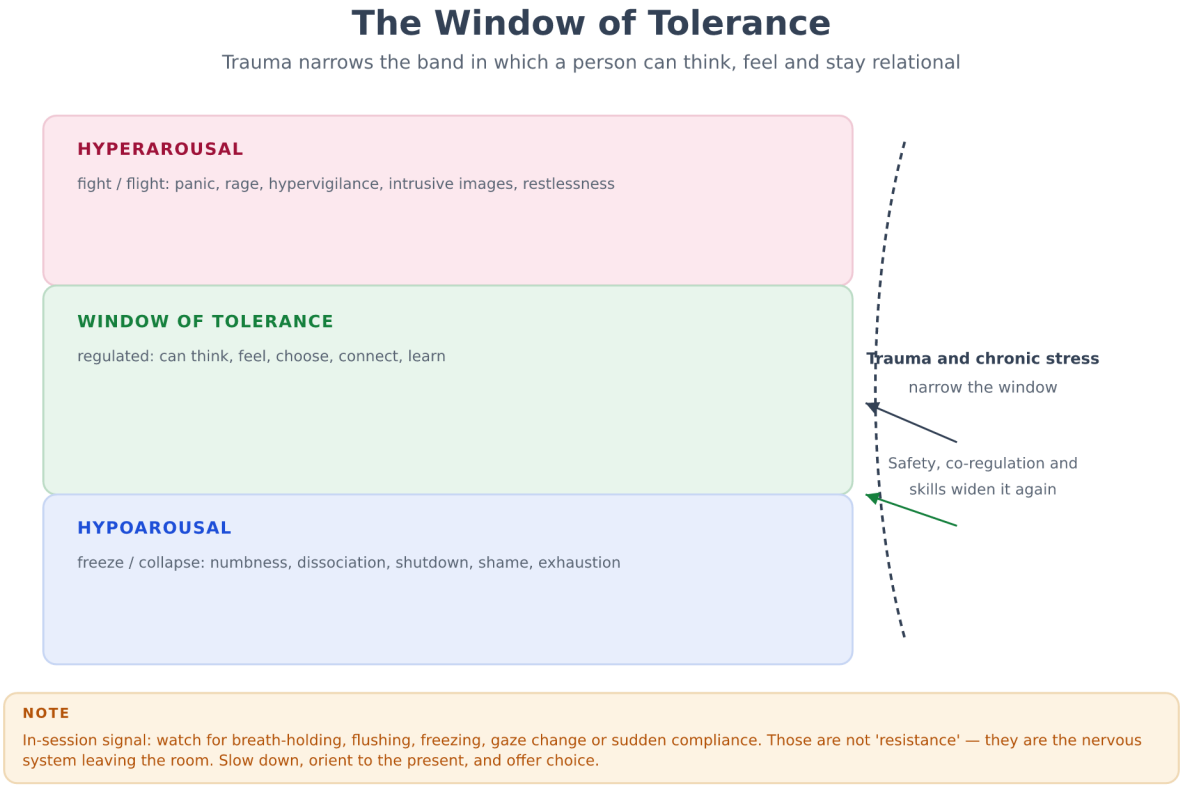


Figure 4.2 — The window of tolerance

Week 5 — Stress and the HPA axis

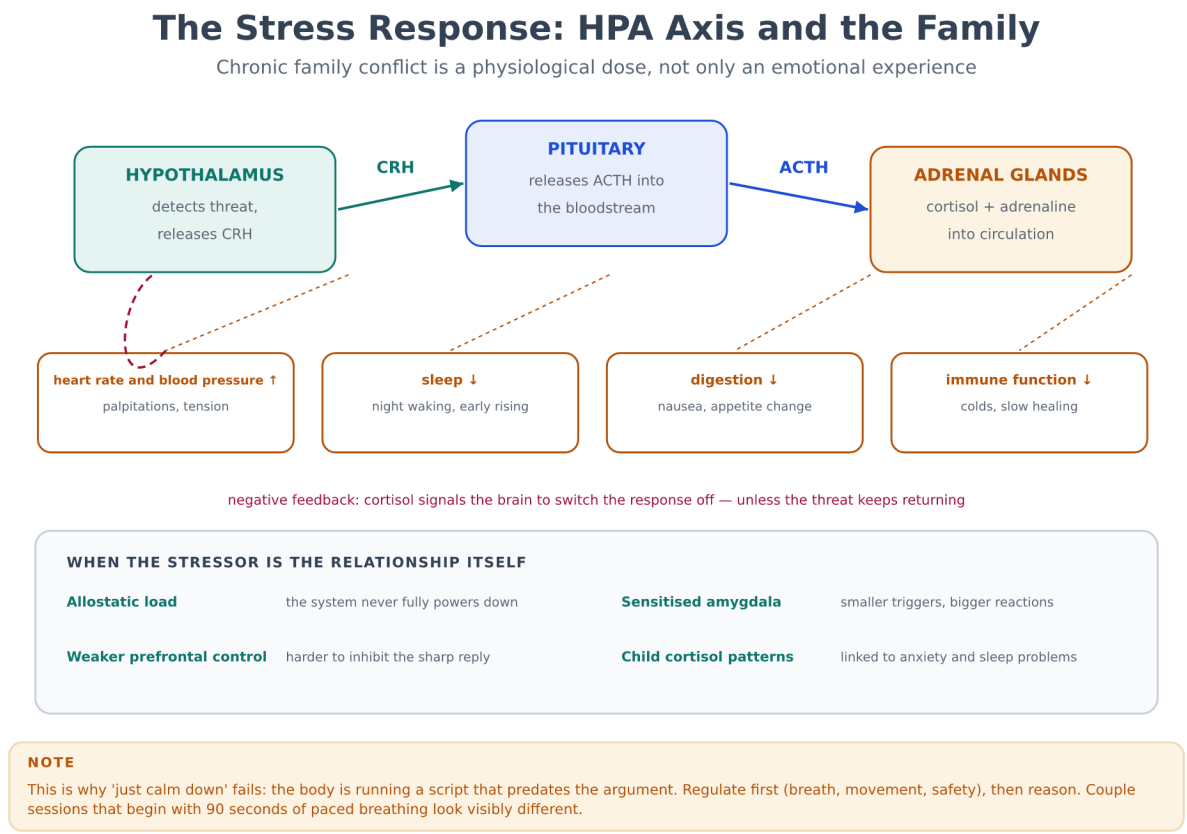


Figure 5.1 — The HPA axis and chronic family stress

The pathway. Hypothalamus (CRH) → pituitary (ACTH) → adrenal glands (cortisol) → feedback to the brain. Acute stress is adaptive; chronic stress produces allostatic load.

Robust findings. Chronic stress is associated with altered cortisol patterns, impaired immune function, sleep disruption, cardiovascular risk, and (in children) altered stress reactivity.

Plausible. Chronic relational conflict functions as a chronic stressor with measurable physiological effects. The size of the effect, and the direction of causality, remain debated.

Clinical translation.

- Screen for sleep, exercise, caffeine, alcohol, and medical conditions.
- Teach that physiological state affects the conversation: "You are not bad at communicating; you are tired at 9 p.m."
- Justify referral: persistent fatigue, weight change, palpitations, unexplained pain.

Week 6 — Attachment as biology

Robust. Attachment behaviours are observable and cross-culturally recognisable; caregiver responsiveness predicts later social and emotional outcomes; severe early deprivation has lasting effects (Romanian orphan studies).

Plausible. Attachment patterns are encoded as expectations and physiological set-points, and can be revised by later relationships.

Popular but overstated. Claims that specific brain regions "store" attachment, or that a single relationship "rewires" the brain in a defined number of weeks.

Clinical use. The attachment frame explains why a partner's tone matters more than their argument, and why therapy must provide a predictable relational experience before it can provide insight.

Week 7 — The social brain

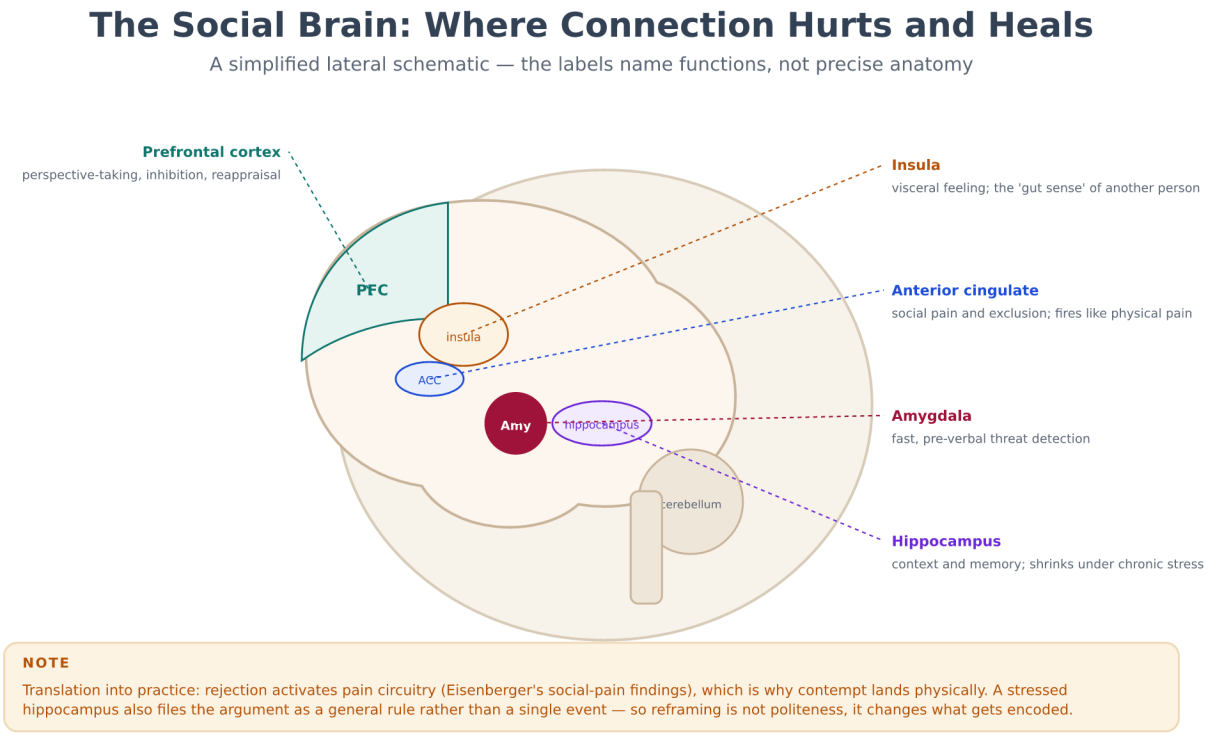


Figure 7.1 — A simplified social brain map

Regions and functions (functionally, not precisely).

Region	Function relevant to therapy
Prefrontal cortex	Perspective-taking, inhibition, reappraisal — offline under stress
Amygdala	Rapid threat detection, pre-verbal alarm
Insula	Interoception; the "gut sense" of another person
Anterior cingulate	Social pain, exclusion, conflict monitoring
Hippocampus	Context and memory; vulnerable to chronic stress
Mirror system (contested)	Imitation and resonance; overstated in popular accounts

Robust. Social exclusion activates pain-related circuitry (Eisenberger); reappraisal changes emotional response; stress impairs prefrontal function.

Contested. "Mirror neurons" as an explanation of empathy; the "right brain/left brain" attachment story; "the amygdala hijack" as a precise mechanism.

Clinical translation. Rejection hurts physically — say so, because it reduces shame. Reappraisal works but requires a regulated brain — so regulate first.

Week 8 — Teaching regulation

Assessed: teach a regulation skill in 20 minutes so that a family can use it at home.

A teaching sequence.

1. **Explain briefly** why the body matters (30 seconds, no lecture).
2. **Demonstrate** the skill yourself.
3. **Practise together** in the session.
4. **Troubleshoot** ("what would make this hard at home?").
5. **Anchor it** to a specific moment ("when you feel your jaw tighten").
6. **Write it down** and review next session.

Four reliable skills.

Skill	How	When
Extended exhale breathing	In for 4, out for 6–8, for 90 seconds	Before a difficult conversation
Orienting	Slowly look around, name five things	Dissociation, panic
Grounding	Feet, weight, texture, temperature	Trauma intrusion
Movement	Stand, walk, stretch, shake out	After conflict; restlessness

---

Week 9 — Memory, trauma and the body

Robust. Traumatic memories are often fragmented and sensory; stress hormones affect memory consolidation; avoidance maintains fear; exposure-based treatments work.

Plausible. Implicit memory contributes to reactions without a narrative; the body "remembers" in the sense that physiological responses are conditioned.

Popular but false. That traumatic memories are stored "in the muscles" and released by massage; that the body has a literal memory independent of the brain; that a single technique can "release" trauma.

Clinical translation.

- Do not require a narrative before stabilisation.
- Expect sensory fragments and treat them as memory, not madness.
- Use grounding to establish "now" versus "then".
- Refer for trauma-specific therapy when indicated; do not improvise.

Week 10 — Couple conflict physiology

Robust. Physiological arousal during conflict predicts poor problem-solving; high arousal narrows attention and reduces empathy; recovery after conflict predicts relationship outcomes.

Gottman's flooding threshold (roughly 100 bpm) is a clinical heuristic rather than a precise cut-off, but the principle is sound: above a certain arousal, listening is physiologically difficult.

Clinical use.

1. Teach a **break protocol**: agree a signal, separate for 20–30 minutes, self-regulate, return.
2. Explain that the break is not abandonment; it is a condition for a useful conversation.
3. Do not hold difficult conversations late at night, when tired, hungry, or after alcohol.
4. In session, watch for signs of flooding and pause before continuing.

Week 11 — Sleep, exercise, substances and mood

Sleep. Robustly linked to mood, irritability, attention and emotion regulation. Ask about it in every assessment, and treat it as a clinical target, not a lifestyle issue.

Exercise. Modest but reliable effects on mood and anxiety; a useful adjunct.

Alcohol and cannabis. Disrupt sleep architecture, increase impulsivity, and reduce inhibition — all relevant to couple conflict and parenting.

Caffeine and screens. Contribute to sleep disruption; worth asking about without moralising.

The clinical frame. "Let us look at what your body is doing before we ask it to have a better conversation."

Week 12 — Adolescent and ageing brains

Adolescence. Robust: increased reward sensitivity, heightened peer influence, gradual maturation of regulatory systems. Popularly overstated: "the adolescent brain is not fully formed until 25" is a simplification that can infantilise young people.

Ageing. Robust: processing speed and some memory functions decline; crystallised knowledge and emotional regulation often improve; social selectivity increases. Popular: "the brain stops changing" — neuroplasticity continues.

Clinical use. Match expectations to capacity without lowering them; scaffold rather than lecture; and design sessions around the person's best time of day.

Week 13 — Neuroplasticity and its limits

Robust. Experience changes the brain; learning involves synaptic change; rehabilitation after injury exploits plasticity.

Oversold. "You can rewire your brain in 21 days"; "neuroplasticity means anything is possible"; specific protocols claiming to rebuild neural pathways in defined sessions.

The clinical use. Neuroplasticity is the reason therapy can work and the reason it takes time. Say that; it is both hopeful and honest.

Week 14 — Integration

A physiological formulation answers: what is the person's current state, what regulates them, what dysregulates them, and what does the family do that helps or harms. Add it to the systemic formulation, do not replace it.

The communication standard. If you cannot explain it to a 12-year-old and to a sceptical colleague, you do not understand it well enough to use it.

Glossary

Allostatic load — cumulative physiological cost of chronic adaptation to stress. **Co-regulation** — regulation of one person's arousal by another's presence and behaviour. **Cortisol** — the primary glucocorticoid stress hormone. **HPA axis** — hypothalamic–pituitary–adrenal axis, the endocrine stress pathway. **Interoception** — perception of internal bodily states. **Neuroception** — unconscious evaluation of safety and threat (Porges). **Polyvagal theory** — a theory linking autonomic states to social behaviour. **Window of tolerance** — the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Above a certain level of physiological arousal, a person: (a) listens better (b) finds listening and empathy physiologically difficult (c) becomes more rational (d) is unaffected

2. Co-regulation means: (a) self-soothing alone (b) one person's calm regulating another's arousal (c) medication (d) breathing only

3. The HPA axis ends with: (a) the amygdala (b) cortisol release from the adrenal glands (c) the hippocampus (d) the vagus nerve

4. Social exclusion: (a) is emotionally neutral (b) activates pain-related circuitry (c) improves resilience (d) has no physiological correlate

5. "The brain is fully formed at 25" is: (a) a precise finding (b) a simplification that can infantilise young people (c) false in every sense (d) irrelevant

6. Polyvagal theory is best described as: (a) fully established (b) clinically useful and partly contested (c) disproven (d) unrelated to therapy

7. Neuroplasticity means that: (a) change is instant (b) experience shapes the brain, over time (c) therapy is unnecessary (d) the brain stops changing at 25

8. A break protocol in couple conflict works because: (a) it avoids the issue (b) it allows arousal to fall before the conversation continues (c) it punishes the partner (d) it is recommended by law

9. Sleep should be assessed because it: (a) is not clinical (b) is robustly linked to mood and regulation (c) only matters for children (d) is a lifestyle preference

10. Physiological explanations should be used to: (a) replace formulations (b) pace sessions, teach regulation and reduce shame (c) assign blame (d) impress families

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name three robust and one contested claim in the neuroscience of relationships.
2. Describe the HPA axis in five steps.
3. Give a four-step teaching sequence for a regulation skill.
4. Why is "regulate before reason" clinically important in couple conflict?
5. What is the difference between a physiological account and a formulation?

Essay (choose one, 2,000 words)

1. "Neuroscience has been oversold to therapists." Critically evaluate with three examples.
2. Integrate a physiological and a systemic formulation for a couple with escalating conflict.
3. How should a family therapist talk about the brain to a sceptical 15-year-old and a religious grandmother?

Further reading

- Porges, S. W. (2011). The Polyvagal Theory. Norton.

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Next: MFT 727 — Play Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

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MFT 727

Play Therapy Techniques

14 teaching weeks · 3 practicals · 1 figure

ELECTIVES

1.71 units · Prerequisite: MFT 711

> “Play is the child's language and the therapist's instrument.”

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MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	**Play Therapy Techniques**	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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- Essay (choose one, 2,000 words) . . . 13

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Course leader's summary. Play is the child's language and the therapist's instrument. This course trains you to set up a room, hold a session, use directed and non-directed methods, involve parents as partners, and recognise when play is not enough.

How to use this book

Play therapy is learned in a room, not in a book — but a badly equipped room produces bad therapy. Work through the weeks in order: room, stance, skills, methods, family application, limits. Practise every technique on a peer before you use it with a child.

1. Course description

The course covers the history and theory of play therapy; the child-centred (non-directive) model and its core skills; directive and cognitive-behavioural play; sandtray; art and puppetry; filial therapy (teaching parents to be therapeutic agents); family play therapy; assessment through play; trauma and play; and the evidence base.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the theory and evidence for play therapy	Exam
LO2	Set up a safe, well-equipped playroom	Room plan and rationale
LO3	Conduct a non-directive session using core skills	Observed session
LO4	Use at least three directive techniques appropriately	Skills portfolio
LO5	Teach parents filial skills	Parent session observed
LO6	Assess through play without over-interpreting	Case study
LO7	Recognise limits and refer	Viva

3. Assessment

Component	Weight	Due
Playroom plan with item rationale (1,200 words)	15%	Week 4
Observed non-directive session (30 min)	30%	Week 7
Techniques portfolio (five techniques, with reflection)	20%	Week 10
Case study including filial work (2,500 words)	25%	Week 13
Examination	10%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	What play is for; history and theory	Free play observation
2	The therapeutic powers of play	Session structure mapping
3	The room and the toys	Room setup
4	**Room plan due**; limit setting	Limit-setting rehearsal
5	Non-directive skills I: tracking and reflecting	Tracking drill
6	Non-directive skills II: returning responsibility, esteem-building	Skills drill
7	**Observed non-directive session**	Assessed
8	Directive techniques: art, puppets, role-play, stories	Technique practice
9	Sandtray	Sandtray practice
10	**Portfolio due**; assessment through play	Assessment exercise
11	Trauma and play; dissociation in the playroom	Pacing and grounding
12	Filial therapy: parents as agents	Parent coaching
13	**Case study due**; family play therapy	Family session design
14	Limits, evidence and integration	Viva

Week 1 — What play is for

Theory. Play is children's natural medium for expression, mastery, and meaning-making. Axline's eight principles (1947) remain the most quoted statement of the non-directive stance: the therapist develops a warm relationship; accepts the child as they are; establishes permissiveness; recognises and reflects feelings; respects the child's capacity to solve problems; does not direct; does not hurry; and sets only those limits that are necessary.

Why it works (proposed mechanisms). Play allows symbolic expression of experience that cannot be verbalised; it provides a sense of control and mastery; it allows rehearsal of difficult situations; and the relationship with the therapist provides a corrective relational experience.

Evidence. Play therapy has moderate support for a range of child problems, with stronger effects when parents are involved (filial therapy) and when the approach is matched to the problem (e.g. CBPT for anxiety). It is not a panacea, and it is not a substitute for trauma-specific treatment where indicated.

> **PRACTICAL**
> Observe a child in free play for 15 minutes. Record behaviours only: what they chose, what they returned to, what they avoided, how they ended.

Week 2 — The therapeutic powers of play

Power	What it looks like	How the therapist supports it
Communication	The child enacts the problem	Track and reflect
Emotional regulation	Repetitive calming play	Allow repetition; contain
Mastery	Replaying a frightening event with a different ending	Do not interpret; note the shift
Symbolic distance	The "monster" rather than the father	Stay in the metaphor
Relationship	The child tests and then trusts	Consistency, warmth, limits
Imagination	New possibilities	Follow, do not lead

Session shape for a child. Opening ritual (same each time), free play, a focused activity, a closing ritual. Predictability is the intervention.

Week 3 — The room and the toys

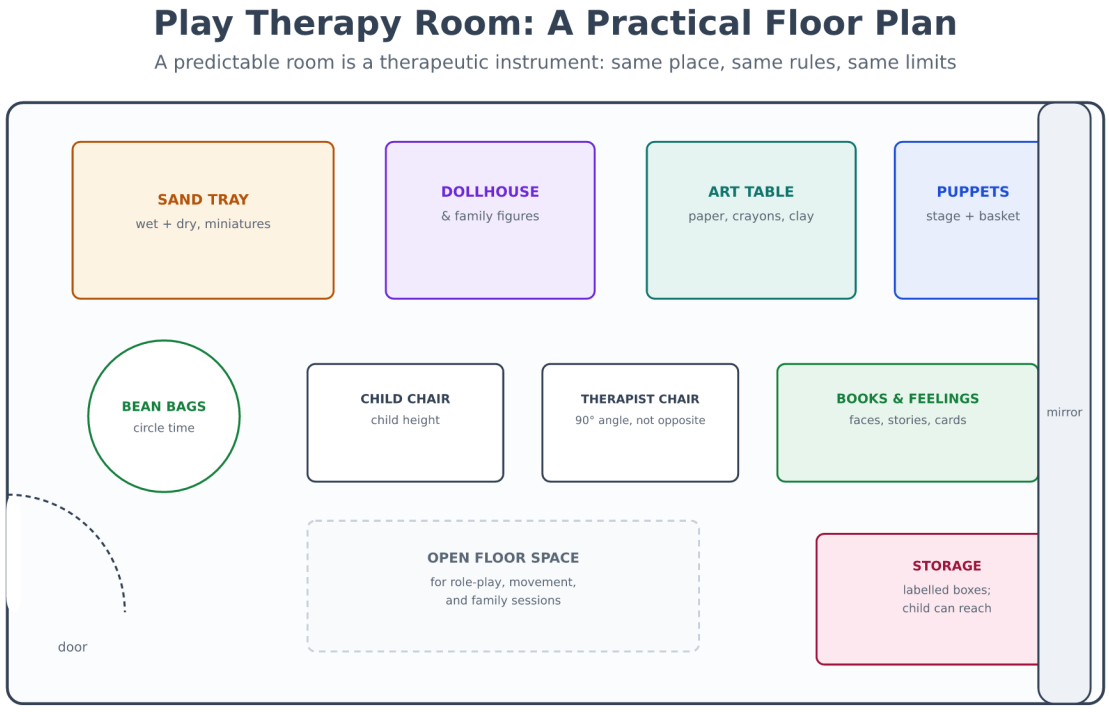


Figure 3.1 — A play therapy room: layout, furniture and sightlines

Principles. Safety, predictability, accessibility, and privacy. The child should be able to reach what they need; the therapist should be able to see everything.

Toy categories (Landreth's classification).

1. **Real-life toys** — dolls, family figures, cars, a cash register, a phone, cooking utensils.
2. **Acting-out/aggressive toys** — toy soldiers, a punch bag, wild animals, a toy gun, dinosaurs.
3. **Creative/expressive toys** — crayons, paint, clay, sand, blocks, musical instruments.

What to include. Sand tray, water, dolls and family figures, animal families, puppets, art materials, blocks, a dollhouse, a medical kit, a small house, blankets, and a soft toy.

What to avoid. Toys that require the therapist's help to operate; toys with one "right" answer; electronic toys that dominate; and anything that could be used to hurt without a limit.

Room rules posted at child height. "We stay in the room. We do not hurt ourselves. We do not hurt each other. We do not hurt the room. We put things away when we finish."

Week 4 — Limit setting

Why limits matter. Limits create safety, teach self-control, and protect the relationship. Permissiveness without limits is not acceptance; it is abandonment.

The ACT model (Landreth).

- **A — Acknowledge the feeling:** "You are really angry with me."
- **C — Communicate the limit:** "But I am not for hitting."
- **T — Target an acceptable alternative:** "You can hit the cushion, or you can tell me with words."

Sequence. State the limit calmly, once; repeat with the alternative; if it continues, offer a choice ("you can choose to put the toy down or I will put it away"); then follow through without anger.

What not to do. Negotiate; explain at length; threaten consequences you will not enforce; or shame the child.

> **REHEARSAL**
> Role-play a child who throws a toy at you, three times. The third attempt should be delivered with the same calm as the first.

Week 5 — Non-directive skills I

Tracking — narrating behaviour without evaluation: "You are putting the blue block on top. It wobbles."

Reflecting content — "You want the mother doll to go away."

Reflecting feeling — "You look pleased with that." / "That made you angry."

Rules of thumb.

- Describe, do not question.
- Name feelings tentatively ("you seem…"), allowing correction.
- Follow, do not lead.
- Stay in the metaphor; do not translate ("so the monster is your father" is an interpretation, not a reflection).

> **DRILL**
> Ten minutes with a peer playing a 6-year-old. You may only track and reflect. No questions. The observer counts interpretations (target: zero).

Week 6 — Non-directive skills II

Returning responsibility — handing the problem back: "That is something you can decide." / "You can try it."

Esteem-building — attributing effort and choice, not talent: "You kept trying until it stood up."

Facilitating self-direction — "You know what you want to do next."

Enlarging the meaning — offering a slightly wider frame: "You are the one who decides what happens in this story."

The therapist's own regulation. Children notice your body. Breathe slowly, sit at their level, keep your face open, and speak more slowly than you think you should.

Week 7 — Observed non-directive session

Thirty minutes with a trained child actor (or a filmed role-play marked against criteria). Assessed on: room and structure; tracking and reflection; returning responsibility; one limit set well; warmth; and the closing ritual.

Week 8 — Directive techniques

Directive play is not instruction. It is the deliberate use of a medium to address a specific issue, always following the child's lead within the structure.

Technique	Use	How
Drawing (family, house, fears, "worst day")	Assessment and expression	"Draw your family doing something." Then ask them to tell you about it

Technique	Use	How
Puppets	Safe displacement	Let the child choose a puppet; interview the puppet, not the child
Role-play	Rehearsal and mastery	"Show me what happens at the school gate"
Bibliotherapy	Normalising and skills	Read a story about a similar problem; discuss the character
Feelings cards / thermometer	Emotional vocabulary	Sort into "big/little", "now/then"
CBPT	Anxiety, trauma	Psychoeducation, coping skills, graded exposure in play

Sequence for a directive activity. Explain briefly → demonstrate → let the child lead → observe → reflect → close.

Week 9 — Sandtray

What it is. A tray of dry and wet sand with miniature figures, used to create a world.

Two traditions. Non-directive (the child builds, the therapist witnesses and reflects) and directive (therapist suggests a theme, e.g. "make a world where you feel safe").

Practicalities. Trays with a water-resistant base; dry sand and water; a wide range of miniatures (people, animals, buildings, vehicles, natural objects, fantasy, symbols of spirituality); a camera to record the scene (with consent); and a clear policy about dismantling.

Interpretation discipline. Do not impose symbolic meanings. Ask: "Tell me about this world." "What happens next?" "Who is this?" The child is the authority on their world.

With older children and adults, sandtray can be used as a non-verbal way of representing a family system — who is placed near whom, what is buried, what is outside the tray.

Week 10 — Assessment through play

What you can learn. Developmental level (language, symbolic play, attention, motor control); emotional expression; themes (abandonment, violence, rescue, repetition); relational capacity (can they play with you?); and the impact of specific events.

What you cannot learn reliably. Specific causation; the identity of an abuser; or a diagnosis from a single drawing. Any assessment claim must be cautious and triangulated with other data.

Signs warranting attention. Play that is repetitive, narrowed and joyless; sexualised play beyond developmental norms; play involving harm to self or others; a sudden change in play after an event; and a child who cannot play at all.

Documentation. Describe behaviour and the child's own words; avoid interpretation in the record.

Week 11 — Trauma and play; dissociation

Principles (from MFT 714). Safety and stabilisation first; the caregiver's regulation is the child's best medicine; do not force the narrative; and expect repetition.

In the playroom. Watch for signs of dissociation (stillness, glazed eyes, going blank, sudden compliance). Slow down, orient to the room, offer a grounding object, and return to safe play.

Post-traumatic play is repetitive and lacks resolution. The therapeutic task is not to stop it but to help it move — by reflecting feeling, introducing a small element of mastery, and involving the caregiver.

When play is not enough. Severe trauma symptoms, self-harm, dissociation, and complex abuse histories need trauma-specific treatment (e.g. TF-CBT) alongside or after stabilisation.

Week 12 — Filial therapy

What it is. Training parents to conduct child-centred play sessions at home, with supervision. Developed by Guerney; extensively researched; strong evidence for improving parent–child relationships and reducing child problems.

Why it works. It increases parental empathy and reflective function, changes the parent's view of the child, and provides a structured positive interaction that most distressed families lack.

The structure (typically 10 sessions).

1. Teach the principles and skills (tracking, reflecting, limit setting).
2. Demonstrate a play session with the child while the parent observes.
3. The parent conducts a session while the therapist coaches.
4. The parent conducts sessions at home (30 minutes weekly) and reports back.
5. Review and generalise.

Practical adaptations. Limited space at home; other children; a parent who works nights; a parent with their own trauma history (support first).

> **PARENT SESSION**
> Coach a parent through a 15-minute play session using tracking, reflection and one limit.

Week 13 — Family play therapy

Why involve the family. The child's problem exists in a system; play can make the system visible.

Techniques.

- **Family drawing and sculpture** — who sits where, who is close, who is left out.
- **Co-constructed play** — the family builds one world together; observe negotiation, hierarchy and exclusion.
- **Role reversal** — the child directs the parents in a scene; useful for revealing the child's experience.
- **Structured family games** — cooperative tasks that require negotiation.

Caution. Family play can expose conflict. Have a structure for containing it, and be ready to end the activity and debrief.

Week 14 — Limits, evidence and integration

The limits of play therapy. Not a substitute for medical assessment, safeguarding action, trauma-specific treatment or family work where the family system is the problem. Play therapy with a child in a family that continues to harm them is not treatment; it is containment.

The evidence summary. Moderate support across problems; stronger when parents are involved; strongest when the approach is matched to the problem and delivered with fidelity.

Integration. Play therapy sits comfortably within a systemic frame when it is used as the child's channel of communication within a family assessment, and when parents are brought into the work.

Glossary

ACT model — Acknowledge, Communicate the limit, Target an alternative. **Child-centred play therapy** — non-directive approach following the child's lead (Axline, Landreth). **CBPT** — cognitive-behavioural play therapy. **Filial therapy** — training parents to conduct child-centred play sessions. **Post-traumatic play** — repetitive, unresolving play following trauma. **Sandtray** — a tray of sand with miniatures used for symbolic expression. **Therapeutic powers of play** — the mechanisms through which play produces change.

Self-assessment

Multiple choice

1. The ACT model stands for: (a) Acknowledge, Communicate, Target (b) Ask, Calm, Tell (c) Assess, Contain, Treat (d) Attend, Connect, Teach

2. In non-directive play therapy the therapist: (a) leads the play (b) follows the child's lead (c) interprets symbols (d) teaches skills

3. Telling a child "the monster is your father" is: (a) a reflection (b) an interpretation and generally to be avoided (c) tracking (d) limit setting

4. Filial therapy involves: (a) the child alone (b) training parents to conduct child-centred sessions (c) medication (d) group therapy

5. Post-traumatic play is typically: (a) joyful (b) repetitive and without resolution (c) absent (d) directed by the therapist

6. Sandtray interpretation should be: (a) imposed by the therapist (b) led by the child's own account (c) avoided entirely (d) symbolic decoding

7. Limits in play therapy exist to: (a) control the child (b) create safety and teach self-control (c) save toys (d) satisfy parents

8. A child's drawing alone: (a) diagnoses abuse (b) cannot diagnose or identify an abuser (c) is irrelevant (d) replaces assessment

9. Play therapy is contraindicated as a substitute for: (a) warmth (b) safeguarding action and trauma-specific treatment (c) structure (d) toys

10. The strongest play therapy effects are found when: (a) the child is seen alone for a year (b) parents are involved and the approach matches the problem (c) the therapist interprets (d) sessions are longer

Answer key

1 a · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State Axline's principles in your own words (five of the eight).
2. List the three categories of toys with two examples of each.
3. Give the ACT sequence with a scripted example.
4. What are the five steps of a child-centred session structure?
5. When is play therapy not enough, and what do you do?

Essay (choose one, 2,000 words)

1. "Play is the child's language." Discuss the implications for assessment and treatment.
2. Design a play therapy service for children in a Nigerian primary school, including the room, the toys, the referral pathway and the evidence.

3. Analyse a play session (yours or a recorded one) using the core skills, and identify what you would do differently.

Further reading

- Axline, V. M. (1947/1969). Play Therapy. Ballantine.
- Landreth, G. L. (2012). Play Therapy: The Art of the Relationship (3rd ed.). Routledge.
- VanFleet, R., Sywulak, A. E., & Sniscak, C. C. (2010). Child-Centered Play Therapy. Guilford.
- Bratton, S. C., Ray, D., Rhine, T., & Jones, L. (2005). The efficacy of play therapy with children: A meta-analytic review. Professional Psychology: Research and Practice, 36(4), 376–390.
- Gil, E. (2017). Play in Family Therapy (2nd ed.). Guilford.

End of the elective sequence. Return to the Programme Handbook.

MSC MARRIAGE AND FAMILY THERAPY

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MFT 770

Master Thesis

14 teaching weeks · 2 practicals

SEMESTER THREE

1.71 units · Prerequisite: MFT 705, 721

> “The thesis is not a bigger essay.”

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MFT 770 — Master Thesis

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	**Master Thesis**	Three	1.71

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Course leader's summary. The thesis is not a bigger essay. It is a supervised, ethical, original inquiry that a professional body would accept as evidence that you can generate knowledge, not only consume it. This book is the project management manual plus the academic guide.

How to use this book

Work the milestones in order; they are dated to protect you from the two classic disasters — a brilliant plan that cannot be completed, and a completed study that answers nothing. Bring the weekly artefact to supervision even if it is imperfect. Progress beats polish.

1. What the thesis is

Element	Requirement
Length	15,000–20,000 words (excluding appendices), or the institutional equivalent
Contribution	Original, however modest: new data, new analysis, a local validation, a qualitative account, a service evaluation
Methods	Quantitative, qualitative or mixed; must be defensible and feasible
Ethics	Institutional approval before any data collection
Supervision	Minimum fortnightly contact with a named supervisor
Output	Bound thesis, oral defence, and (encouraged) a publishable manuscript

What it is not. A literature review alone (that is a different degree component). A clinical case series without a systematic method. A replication of a manual with no question. A project that requires resources you do not have.

2. The fourteen structured weeks

Week	Milestone	Artefact due in supervision
1	Topic refinement and scope	One-page problem statement
2	Literature search strategy	Search log and 20 key references
3	Critical literature review draft	2,000-word draft
4	Research question and aims	PICO/SPIDER and aims table
5	Design selection	Design decision log
6	Measurement and materials	Instrument pack
7	Ethics application	Completed ethics form
8	Sampling and recruitment plan	Recruitment protocol and letters
9	Analysis plan	Pre-specified analysis plan

Week	Milestone	Artefact due in supervision
10	Pilot or feasibility work	Pilot findings and amendments
11	Data collection checkpoint	Recruitment and data quality report
12	Analysis progress	First results tables/coding frame
13	Findings and discussion drafting	Draft chapters 4–5
14	Full draft and defence preparation	Complete draft, viva questions

Timeline discipline. A part-time student should assume 12–18 months from registration to defence. Full-time students commonly need 9–12 months. If your plan assumes everything goes right, add 30%.

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3. Choosing a question you can finish

The three-circle test. A viable thesis sits where (1) you are curious, (2) the field needs it, and (3) you have access. If any circle is missing, choose a different question, not a bigger effort.

Good thesis questions in this field (examples).

1. What is the reliability and factor structure of the RDAS in a Nigerian clinical sample?
2. How do fathers describe their experience of being excluded from family therapy?
3. Does a four-session psychoeducation group reduce caregiver burden among families of people with psychosis?
4. What do couples report about the process of rebuilding trust after infidelity?
5. What predicts drop-out after a first family therapy appointment in a community clinic?
6. How do therapists in training describe the influence of their own family of origin on their clinical decisions?
7. Does a culturally adapted version of the speaker–listener technique improve communication in a small clinical series?

Questions to avoid. "The effect of family therapy on family functioning" (too broad); "Why is family therapy better than individual therapy?" (advocacy, not inquiry); anything requiring a sample you cannot access or an instrument you cannot obtain.

> **ARTEFACT**
> A one-page problem statement: what is unknown, why it matters, who it affects, and what you will produce.

---

4. The literature review

Purpose. Not to summarise everything, but to establish (a) what is known, (b) what is contested, (c) what is missing, and (d) why your question follows.

Search discipline.

- At least three databases (PubMed, PsycINFO, Scopus, AJOL, Google Scholar as a supplement).
- Document the strategy: terms, limits, date, number of results, number retained.
- Use a reference manager from day one.
- Read systematic reviews first; they map the field faster than primary papers.

Structure of the chapter. Context → key constructs and definitions → what the evidence shows → what is contested → what is missing (the gap) → your question and aims.

The gap must be specific. "Few studies have examined X in Y population using Z method" — not "more research is needed."

---

5. Design and methods

The rule. The method chapter must be detailed enough that a competent reader could replicate the study, and honest enough that they could see its limits.

Chapter contents.

1. Design and rationale.
2. Setting and context.
3. Participants: inclusion, exclusion, recruitment, consent, sample size justification.
4. Measures: source, psychometrics, scoring, adaptation and translation procedures.
5. Procedure: what happened, in what order, for how long.
6. Analysis: software, models, assumptions, missing data, coding approach, rigour procedures.
7. Ethical considerations: approval reference, consent, confidentiality, risk, data storage.
8. Researcher position and reflexivity.

---

6. Ethics

Required before any data collection. A retrospective application is a serious breach.

Standard components. Purpose and design; participants and recruitment; consent process and documents; risks and benefits; confidentiality and data management; payment; dissemination; and procedures for distress or incidental findings.

Family-specific issues. Consent from guardians plus assent from minors; separate consent for each family member; managing the possibility that one member withdraws; protecting group confidentiality; and a protocol for disclosures of risk during data collection.

ARTEFACT

A complete ethics application with consent forms for an adult and a minor, and a risk protocol for a participant who discloses harm during an interview.

7. Data collection

Pilot first. Five to ten participants will reveal problems that a hundred will not forgive: unclear questions, unworkable recruitment, unusable audio, unrealistic time estimates.

Data quality. Check recordings the same week; transcribe promptly; keep a fieldwork log; record refusals and reasons (they are data).

Recruitment tactics that work in clinical settings. A named contact who can introduce you; a one-page information sheet; flexible times; a clear statement of what participation involves; and a follow-up call at 48 hours. Do not rely on posters.

8. Analysis

Quantitative. Follow the pre-specified plan; document deviations; check assumptions; report descriptives before inference; report effect sizes with confidence intervals; run sensitivity analyses; keep the analysis log current.

Qualitative. Transcribe verbatim; code systematically; keep memos; audit your coding; discuss divergence with a second coder; move from codes to themes with an explicit, documented logic; and present themes with data extracts (anonymised).

Mixed methods. Decide in advance how the strands will be integrated, and produce at least one joint display.

9. Writing the thesis

Standard structure.

Chapter	Content	Typical length
1	Introduction and background	2,000
2	Literature review	5,000
3	Methodology	3,500
4	Findings	4,000
5	Discussion	3,500
6	Conclusion and recommendations	1,000
—	References, appendices	—

Writing rules.

- Write the method first; it is the easiest chapter and it clarifies everything.
- Draft findings as soon as you have data; do not wait for "enough".
- Keep a "parking lot" file for ideas that belong elsewhere.
- Write 500 words a day rather than 5,000 in a panic.
- Read your work aloud; if you run out of breath, the sentence is too long.

Voice. Academic but human. First person is acceptable in reflective and qualitative sections; follow your institution's guidance. Avoid both inflated jargon and chatty informality.

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10. The supervision relationship

What your supervisor owes you. Regular contact; timely feedback; methodological and ethical guidance; honesty about the standard of your work; and reasonable availability.

What you owe your supervisor. Prepared meetings with an agenda and an artefact; drafts submitted on time; decisions recorded; and honesty about difficulties — especially delays.

Managing difficulty. If supervision is not working, raise it early with the programme director. A change of supervisor is legitimate and common; drifting for six months is not.

The meeting agenda template.

1. Progress since last meeting (three bullets).
2. Decisions needed today (the most important item first).
3. Problems I am stuck on.
4. What I will do before the next meeting.
5. Date of next meeting.

---

11. The viva

What examiners test. Whether you understand your own study: why you chose it, what you did, what it means, and what it cannot support.

Preparation.

1. Re-read your thesis and mark every claim you could not defend.
2. Prepare a two-minute summary.
3. Prepare answers to the four standard questions: design, validity, analysis, conclusion.
4. Rehearse with a peer who is instructed to be difficult.
5. Bring a copy of your thesis, your analysis log and a pen.

In the room. Answer the question asked. Say "I don't know" when you don't. Distinguish what the data show from what you hope they show. Correct errors gracefully.

Common reasons for referral or failure. Design cannot answer the question; analysis misapplied; ethics breach; unacknowledged limitations; and evidence of work not being the candidate's own.

12. After the defence

Corrections. Make them promptly, with a tracking table mapping each examiner comment to the change made.

Dissemination. Submit a manuscript to a journal; present at a conference; give a plain-language summary to the participating service. Dissemination is part of the ethical contract with your participants.

Your data. Retain as required by your institution's policy, then destroy securely.

13. Project management

Risk	Early warning	Mitigation
Recruitment failure	Fewer than 50% of target by mid-point	Widen eligibility; add sites; adjust design (with supervisor and ethics approval)
Analysis paralysis	Repeated re-coding without writing	Fixed decision date; write the methods and limitations
Supervisor unavailability	Missed meetings	Written agenda; escalate early
Personal crisis	Missed deadlines	Tell the programme early; request extension with evidence
Scope creep	New questions each week	Freeze the question after Week 4; park ideas

The one-page dashboard. Question, milestones, current position, next action, date, supervisor appointment. Update weekly; it will save your degree.

14. Assessment

Component	Weight
Proposal (approved by panel)	10%
Ethics approval	Pass/fail gate
Thesis	70%
Oral defence	20%

Marking criteria (typical). Originality and significance; command of the literature; appropriateness and rigour of method; quality of analysis; critical discussion; ethical practice; presentation and referencing.

Appendix: thesis checklist

- ■ Question is specific, answerable, and stated in one sentence
- ■ Literature search documented and reproducible
- ■ Gap statement is specific, not "more research is needed"
- ■ Design justified against at least one alternative
- ■ Sample size justification provided
- ■ Ethics reference number included
- ■ Measures described with psychometric properties and any adaptation
- ■ Analysis plan pre-specified; deviations recorded and explained
- ■ Assumptions checked and reported
- ■ Effect sizes with confidence intervals
- ■ Findings presented before interpretation
- ■ Limitations specific and honest
- ■ Implications for practice are actionable
- ■ Referencing consistent and complete
- ■ Plain-language summary prepared for participants

Further reading

- Wisker, G. (2012). The Good Supervisor. Palgrave.
- Booth, W., Colomb, G., & Williams, J. (2016). The Craft of Research (4th ed.). Chicago.
- Swetnam, D., & Swetnam, R. (2009). Writing Your Dissertation. How To Books.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Field, A. (2018). Discovering Statistics Using IBM SPSS Statistics (5th ed.). Sage.

End of Semester Three. Continue to the electives: MFT 722 — Professional Development and Self-Care